Toward Neuroprotective Cooling of the Eye: A Scleral-Contact Thermoelectric Device

BY

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DISSERTATION

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To my parents and brothers, whose love and help were indispensable for this work: thank you for helping me push through.
To my friends, far and close, but particularly to those in Chicago: you were the support I needed, my second family.

Accessibility Statement

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Contribution of Authors

Chapter 1 is an original introduction by the author.

Chapter 2 includes work published as Y. Nakamura, L. Mecacci, and J. R. Hetling, "Therapeutic Levels of Hypothermia Achieved in Isolated Porcine Eyes Using a Scleral Contact Interface," *Annals of Biomedical Engineering*, vol. 54, no. 2, pp. 570–579, 2026 (published online November 2025) [**1**]. Y. Nakamura and the author contributed equally to that article, as stated in its author-contributions declaration; Y. Nakamura worked with the author on the preserved- and fresh-porcine experiments during the author's final year of master's study. J. R. Hetling supervised the work. The article is published open access under a Creative Commons Attribution 4.0 license. The fresh-porcine recordings from that study were reprocessed by the author for Chapter 2 together with the human-donor data.

Chapter 2 also draws on two manuscripts in preparation, on both of which the author is first author: an ex-vivo methods paper with Y. Nakamura, K. Fong, N. Shirdade, and J. R. Hetling, and a human–porcine comparison with J. R. Hetling. K. Fong and N. Shirdade carried out earlier work on the eye-cooling system on which the methods paper builds. As an undergraduate research assistant in 2018–2019, K. Fong designed, machined, and prototyped an earlier ocular cooling device and the instrumentation for measuring its temperature. In 2018, N. Shirdade carried out thermal simulations of the cooling of the porcine eye.

Chapter 3 is the original work of the author.

Chapter 4 is the work of the author. Anesthesia and animal care were provided by the Biologic Resources Laboratory, the histological images by the Robbins and Jacobs laboratory at Rush University Medical Center.

Chapter 5 is an original discussion by the author.

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List of Abbreviations

3D	Three-dimensional
ACC	Animal Care Committee
AI	Artificial intelligence
ARVO	Association for Research in Vision and Ophthalmology
ASA	American Society of Anesthesiologists
ATP	Adenosine triphosphate
AVMA	American Veterinary Medical Association
BRL	Biologic Resources Laboratory
CAD	Computer-aided design
CI	Confidence interval
CLEAr	Contact lens electrode array
CRAO	Central retinal artery occlusion
CV	Coefficient of variation
DAQ	Data acquisition
DC	Direct current
ERG	Electroretinogram
LED	Light-emitting diode
micro-CT	Micro-computed tomography
MOSFET	Metal-oxide-semiconductor field-effect transistor
MRI	Magnetic resonance imaging
NTC	Negative temperature coefficient
ONH	Optic nerve head
OP	Oscillatory potential
PWM	Pulse-width modulation
RM	RetMap electrode (corneal ERG electrode)
RPE	Retinal pigment epithelium
SCEC	Scleral-contact eye cooler
SCR	Scleral contact ring
SD	Standard deviation
TEC	Thermoelectric cooler
TTM	Targeted Temperature Management (trial)
UIC	University of Illinois Chicago
USDA	U.S. Department of Agriculture

Summary

Ischemia of the retina and optic nerve, as in retinal artery occlusion, can cause permanent blindness within hours, and no established treatment protects the tissue. Mild cooling protects neural tissue from ischemic injury in laboratory studies, but whole-body cooling is slow and carries systemic risks, which cooling only the eye would avoid. It is not known how much surface cooling reaches the retina and the optic nerve, or whether it is safe in a living eye.

This work developed and evaluated the scleral-contact eye cooler (SCEC), a thermoelectric device that holds a ring on the anterior sclera at a set temperature, with the overall goal of preparing it for use in patients.

Aim 1 measured the cooling inside isolated human and porcine eyes. Aim 2 redesigned the device to be smaller, more efficient, and safer, and modeled its effect in the perfused human eye. Aim 3 tested, in anesthetized pigs with one eye cooled and the other as control, whether the cooling reaches the retina and is tolerated.

In isolated eyes in a warm bath, the proximal optic nerve, the deepest site measured, cooled by 3.5 to 3.8 °C on average within 20 min. No difference between human and porcine eyes was detected there, and a difference above about 2 °C is unlikely. A reduced-bulk ring of about half the original volume and a ceramic ring of the original shape cooled the optic nerve about as well as the original ring. In the model, body temperature, ring temperature and blood flow set the temperature at the back of the eye far more than the design of the ring did. An integrated control board was built and tested on the bench. Six pigs were each cooled in three sessions, of 30, 60, and 120 min, with the ring at about 9.5 or 4 °C. In the 14 sessions analyzed, from five animals, the b-wave of the electroretinogram of the cooled eye became slower in every session, by 2 to 8 ms at the stronger flash and about 5 ms on average, as expected of a cooler retina, while that of the control eye changed by at most 2.5 ms either way. The slowing largely reversed after the device was switched off and was no longer present at the next session. The intraocular pressure of the cooled eye rose less than that of the control eye in 12 of 14 sessions, redness and staining were worse only occasionally, no adverse event was attributed to the device, and the histology showed no damage in the cooled eye absent from the control eye. One animal died after its third session, most likely of a complication of intubation.

This work shows that a contact device on the sclera cools the back of an isolated eye by a few degrees without surgery and changes living retinal function as cooling would, with no harm found by the measures made. The temperature reached in a perfused human eye, the safety of longer and repeated daily cooling, and protection against ischemia remain to be tested.

CHAPTER 1

Introduction

1.1. Clinical Motivation

Acute retinal ischemia is a cause of sudden and usually permanent monocular vision loss. Its prototype, central retinal artery occlusion (CRAO), is a form of acute ischemic stroke [**2**] with a population incidence of about 2 per 100,000 person-years [**3, 4**]. In Korean nationwide claims data that also count branch and other noncentral occlusions, retinal artery occlusion as a whole is more than three times as common [**5**]. Few affected eyes recover useful vision. Among eyes with permanent non-arteritic CRAO and no cilioretinal sparing that presented at counting fingers or worse, acuity improved at all in only 22% of those seen within a week of onset [**6**], and pooled natural-history data give a 17.7% rate of recovery from 20/200 or worse to 20/100 or better [**7**].

No treatment has proven effective, and the 2026 guideline of the American Heart Association and American Stroke Association for acute ischemic stroke rates the usefulness of intravenous thrombolysis within 4.5 h for disabling non-arteritic CRAO as uncertain [**8**]. In pooled observational data, conservative measures such as ocular massage and anterior-chamber paracentesis were associated with worse recovery than natural history [**7**]. Intra-arterial fibrinolysis matched conservative care at a higher complication rate, and its randomized trial was stopped early [**9**]. The first two randomized trials of intravenous thrombolysis within 4.5 h, THEIA and TenCRAOS, found no significant visual benefit over aspirin [**10, 11**]; one fatal intracranial hemorrhage occurred with tenecteplase [**11**]. A third, placebo-controlled trial, REVISION, was stopped for futility at a prespecified interim analysis after 127 participants had been enrolled, and its results have not been published [**12, 13**]. The TenCRAOS

investigators concluded that effective therapy remains elusive and that alternative approaches warrant investigation [**11**], and the THEIA investigators concluded that the low recovery rates raise questions about how long the retina tolerates ischemia [**10**]. Time is a further barrier. Vision loss is painless, and patients present late. Median symptom-to-door times were about 13 h in a Swiss national stroke registry and a US comprehensive stroke center (788 min and 13.5 h) [**14**, Table 2][**15**, Table 1], and 17 h to first medical contact in an Australian audit [**16**]. In the registry and stroke-center cohorts, only about a quarter of patients arrived within 4–4.5 h.

Retinal and optic-nerve ischemia also complicate non-ocular surgery. Perioperative visual loss follows about 8.6 per 10,000 cardiac operations and 3.1 per 10,000 spinal fusions; after cardiac surgery, most cases are retinal vascular occlusions [**17**]. Retinal artery occlusion alone complicates 7.8 per 10,000 cardiac procedures [**18**], and ischemic optic neuropathy accounts for most cases reported to the American Society of Anesthesiologists (ASA) Postoperative Visual Loss Registry after prone spine surgery [**19, 20**]. Guidance for visual loss after spine surgery is directed at prevention, because no treatment has been shown to work [**20, 21**]. Unlike spontaneous occlusion, the period of surgical risk is known in advance, and a protective intervention could be applied before and during it. External pressure on the globe is itself a recognized cause of perioperative retinal artery occlusion [**19, 21**]. A device that rests on the eye must therefore not raise the intraocular pressure, a safety endpoint of Aim 3 (Section 1.8).

These settings share a need for an intervention, in addition to reperfusion, that extends the retina's tolerance of ischemia. Preclinical evidence motivates localized cooling for that purpose (Section 1.3). Delivering the cooling to the posterior eye through an accessible surface is the engineering problem addressed in this dissertation.

1.2. The Eye

A device that cools the retina from outside the eye works through the layered wall of the globe and against its two circulations. This section describes these structures and the temperature field they sustain.

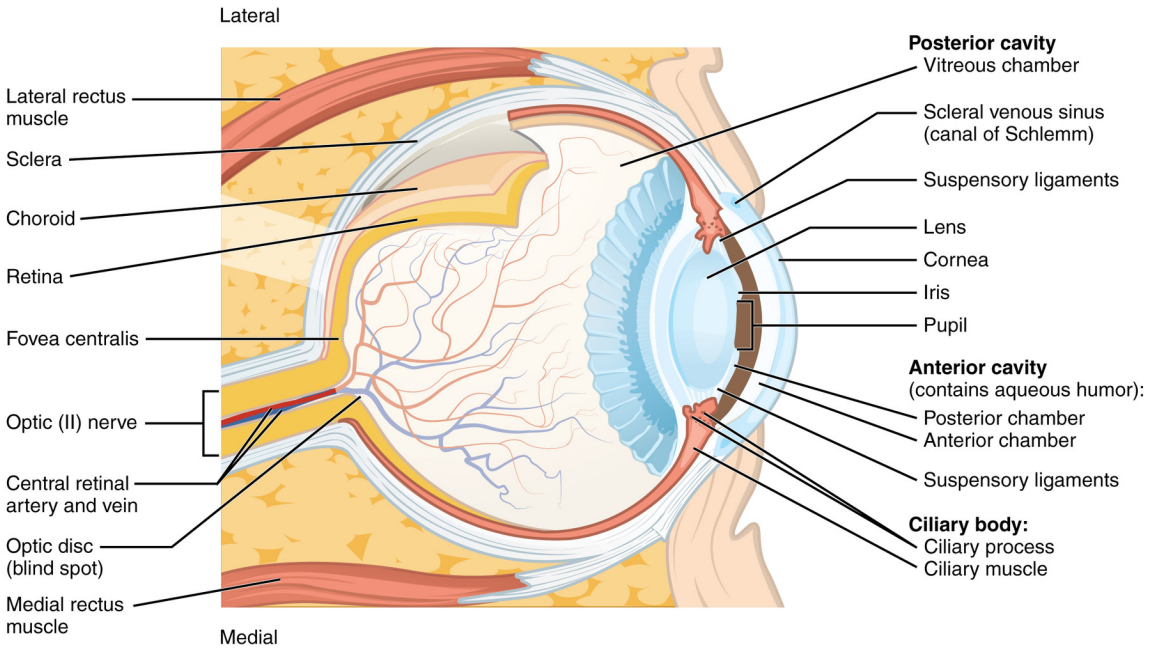


FIGURE 1.1. Cross-sectional anatomy of the human eye: the optical media, the anterior structures, the sclera, choroid, and retina around the vitreous cavity, and the extraocular muscles. From OpenStax College, *Anatomy & Physiology* (Houston: OpenStax, 2013), Fig. 14.13, "Structure of the Eye," via Wikimedia Commons, CC BY 3.0; unaltered.

1.2.1. Gross Anatomy. The human eye is a roughly spherical globe about 24 mm across [**22, 23**], with three concentric coats (Figure 1.1). The outer *fibrous* coat is the transparent cornea, continuous with the opaque sclera. The middle *vascular* coat, or uvea, comprises the iris, ciliary body, and choroid; it carries the greater part of ocular blood flow, although the inner retina is served by a separate retinal circulation (Section 1.2.2). The inner *neural* coat is the retina, the tissue that this work aims to protect. The cooler acts from the outer surface of the fibrous coat, and

the anatomy that matters here is therefore the layered wall between the point of contact and the retina, and the transparent media inside it. The cornea, aqueous humor, lens, and vitreous, a transparent gel filling the vitreous cavity, form the optical path [**24**] and also carry heat between the cooled anterior sclera and the posterior eye.

At the posterior pole the eye wall comprises, from the vitreous outward, the retina, the retinal pigment epithelium (RPE), Bruch's membrane, the choroid, and the sclera [**24**]. Within the retina the photoreceptors lie outermost, apposed to the RPE, and the ganglion-cell axons that form the optic nerve lie toward the vitreous. The retina is among the most metabolically active tissues in the body [**25**], and its outer, photoreceptor layers are avascular, depending on the underlying choroid across Bruch's membrane and the RPE (Figure 1.2).

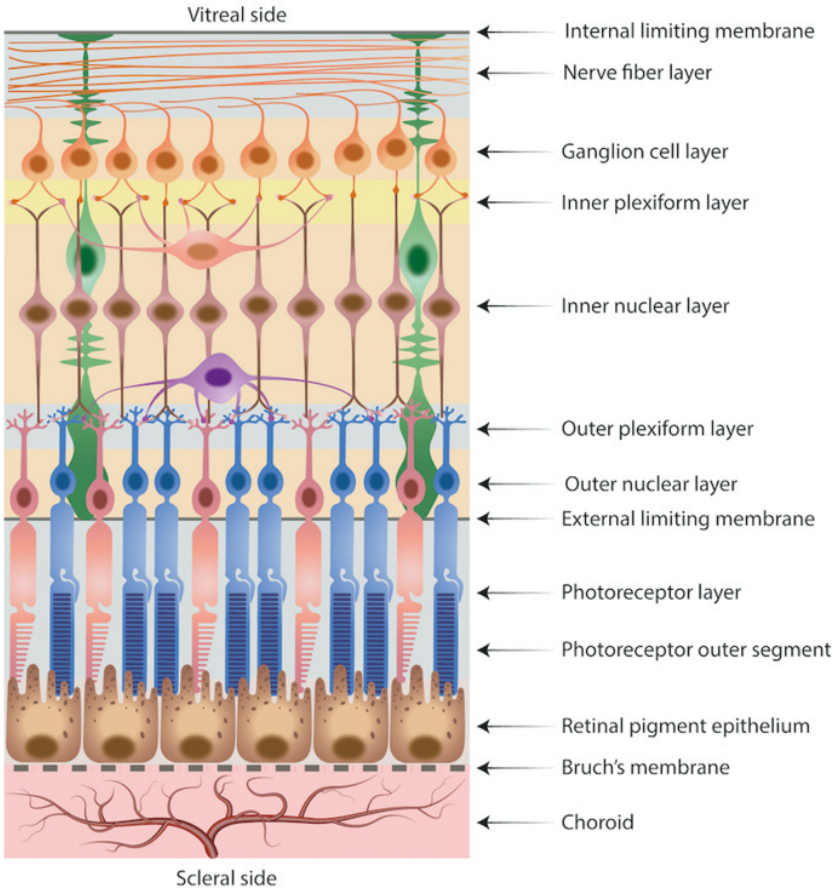


FIGURE 1.2. Retinal layers and their relation to the retinal pigment epithelium, Bruch's membrane, and choroid. Unmodified from Ferrara et al., Fig. 3 [**26**], CC BY 4.0.

The sclera forms five-sixths of the outer coat and is the contact surface of the cooler. Its collagenous stroma is normally avascular and is nourished by the adjacent episcleral and choroidal networks [**27**]. Its thickness is markedly non-uniform: 1.0–1.35 mm at the posterior pole, 0.4–0.6 mm at the equator, about 0.3 mm under the rectus muscles, and roughly 0.8 mm at the limbus [**27**]. Histological measurements give lower values, 0.50 ± 0.11 mm at the limbus (Table 1.3).

1.2.2. Blood Supply. Blood flow matters here as a source of heat. The ocular circulation delivers blood at core temperature, and a cooling device must remove that heat as well as the heat stored in the tissue. The eye's high metabolic demand is met by two largely separate vascular beds, the retinal and the uveal, fed from a common source and drained into a common sink. Arterial inflow arises almost entirely from the ophthalmic artery, usually the first major branch of the internal carotid artery; the anterior ciliary arteries reach the globe through its muscular branches to the rectus muscles. Venous outflow returns through the superior and inferior ophthalmic veins, mainly to the cavernous sinus [**24**].

Figure 1.3 shows the major ocular arterial pathways and venous drainage around the globe, and Figure 1.4 traces the path of the blood from the internal carotid artery to the venous outflow of the orbit.

The retinal circulation is end-arterial. The central retinal artery enters the globe through the optic nerve head (ONH) and supplies the inner retinal layers, which have few collaterals. This helps explain the vulnerability of the retina to arterial occlusion [**30, 31**]; the outer, photoreceptor layers depend on the choroid beneath them. Mean total retinal blood flow in healthy human eyes is roughly 33–80 $\mu\mathrm{L\,min^{-1}}$ by laser Doppler and Doppler optical coherence tomography [**32, 33, 34, 35, 36, 37**]. The uveal circulation, dominated by the choroid, carries the large majority of ocular blood flow. It is supplied by the posterior ciliary arteries, whose short branches also form the circle of Zinn–Haller around the optic nerve, the supply of the ONH (Figure 1.5), and its venous return converges almost entirely into the vortex veins [**24**].

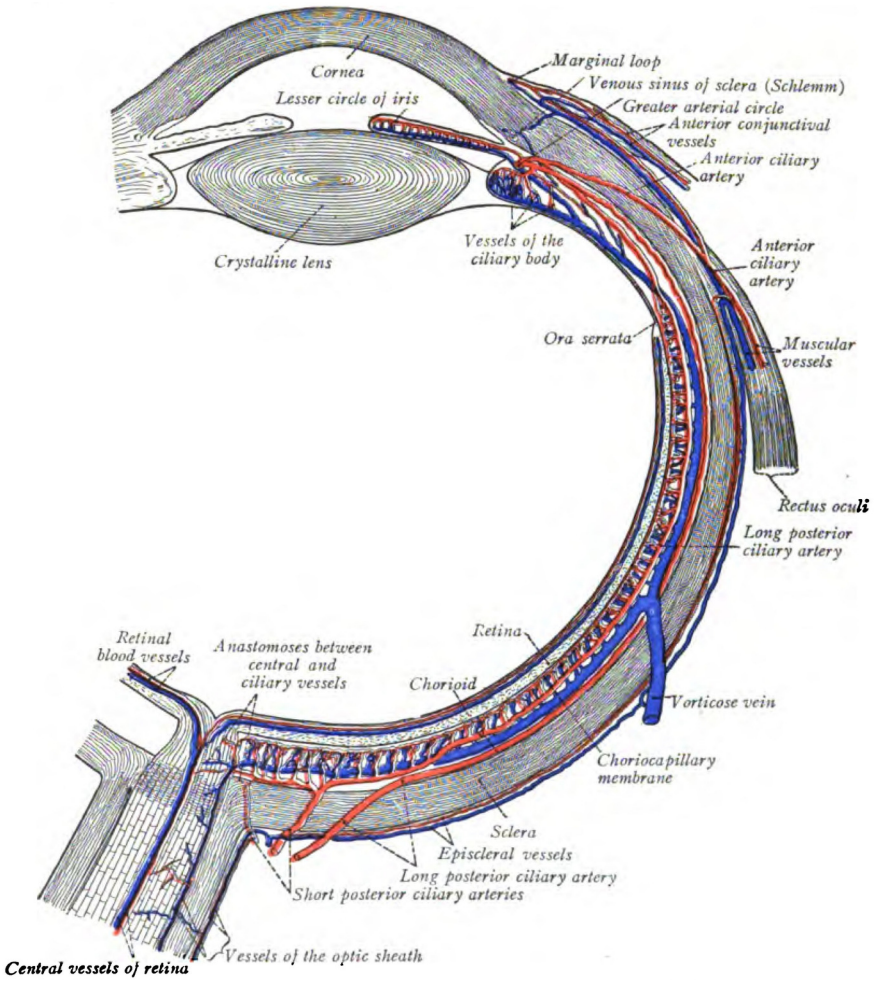


FIGURE 1.3. Blood vessels of the eyeball, diagrammatic section; arteries red, veins blue. From J. Sobotta and J. P. McMurrich, *Atlas and Text-book of Human Anatomy*, vol. 3 (Philadelphia: W. B. Saunders, 1914), Fig. 743, after Th. Leber; artist K. Hajek. Public domain; scan from AnatomyTOOL, with the printed caption cropped and the drawing otherwise unaltered.

Per gram of tissue the choroid has the highest blood flow of any tissue in the body [**28, 38**], roughly four times that of the renal cortex [**39**]. In primates it is about 2000 $\mathrm{mL\,min^{-1}}$ per 100 g, against 60 $\mathrm{mL\,min^{-1}}$ per 100 g for the retina [**24**, pp. 83, 210]. The commonly quoted choroidal share of 85% of ocular blood flow [**39, 40**] rests on monkey measurements in which the choroid received 607 of the 738 $\mathrm{mg\,min^{-1}}$ reaching the retina and uvea, or 82% [**41**]. These values come from microsphere studies in monkeys and cats, and the model of Chapter 3 likewise takes

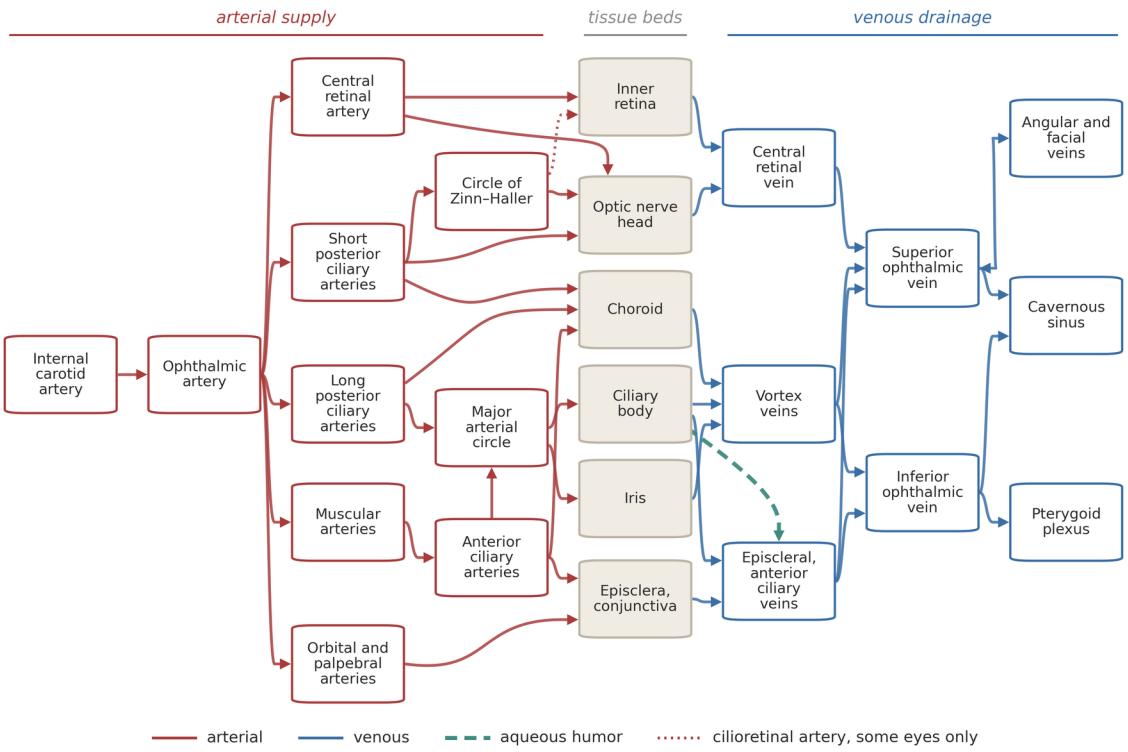


FIGURE 1.4. Flow path of the ocular circulation, from the internal carotid artery to the venous outflow of the orbit; arrows give the direction of flow, and tissue beds are shaded. The two-headed arrow is the connection with the angular and facial veins, in which flow can run either way because the orbital veins have few or no valves [**24, 28**]; the arrow from the central retinal artery to the optic nerve head is its supply to the surface nerve-fiber layer. Not drawn: the central retinal vein may drain directly into the cavernous sinus [**24**], and the prelaminar optic nerve head drains in part to the peripapillary choroidal veins [**29**].

the perfusion of its choroid from microsphere measurements in monkeys [**42**] (Appendix D). In humans, magnetic-resonance perfusion imaging of the posterior pole gives 149 ± 48 mL min^{-1} per 100 mL of tissue for retina and choroid together, in four healthy volunteers [**43**].

The two beds are regulated very differently, which matters for how each clears heat. Retinal blood flow is strongly autoregulated, held nearly constant across changes in perfusion pressure and matched to metabolic demand [**44**], a control also central to ONH perfusion [**29**]. The choroid lacks this classical pressure autoregulation; its flow

follows perfusion pressure and is modulated by autonomic nerves, sympathetic stimulation producing vasoconstriction [**24, 38**]. Some regulation is present: in healthy subjects performing isometric exercise, a 67% rise in mean ocular perfusion pressure raised subfoveal choroidal blood flow by only 12%, and the compensation broke down at larger rises [**45**]. Thermally, choroidal flow is very high and, in animal studies, independent of the metabolism of the outer retina [**38**], and it exchanges heat with the posterior tissues (Section 1.2.3).

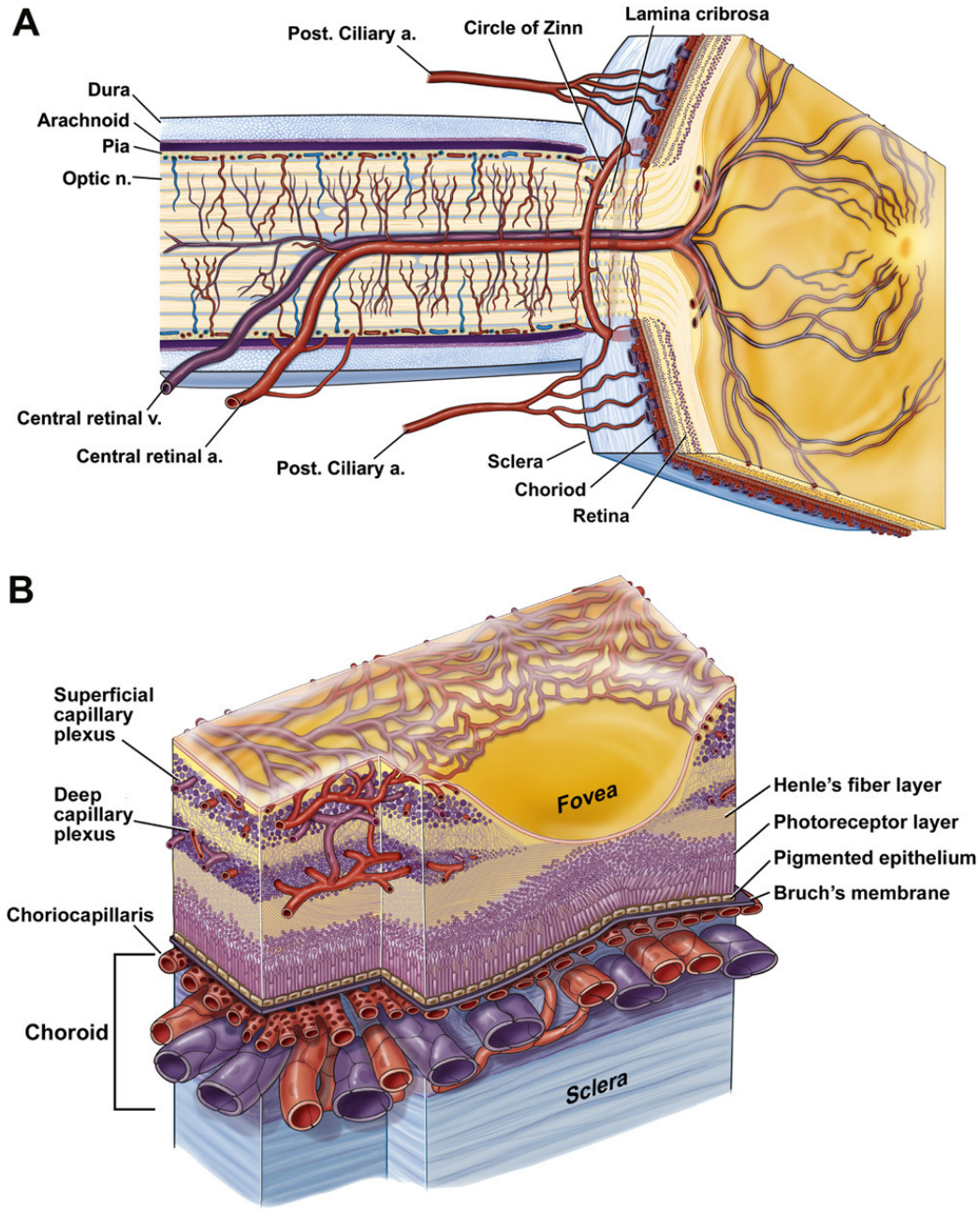


FIGURE 1.5. Retinal and choroidal circulation. (A) Vascular supply at the optic nerve and posterior globe. (B) Retinal capillary plexuses and underlying choroidal vessels. Reprinted from *Progress in Retinal and Eye Research*, vol. 31, J. Kur, E. A. Newman, and T. Chan-Ling, "Cellular and physiological mechanisms underlying blood flow regulation in the retina and choroid in health and disease," pp. 377–406, Fig. 1, Crown Copyright (2012), with permission from Elsevier [**25**]. The drawings, by Dave Schumick, are reprinted from *Encyclopedia of the Eye*, B. Anand-Apte and J. G. Hollyfield, "Developmental anatomy of the retinal and choroidal vasculature," pp. 9–15, Copyright (2010), with permission from Elsevier.

1.2.3. Ocular Thermal Environment. The temperature applied at the front does not set the temperature at the back. Holding a steady temperature requires heat extraction that balances the net heat input. As a result, an anterior contact temperature defines an applied boundary, while the posterior temperature depends on transport through the eye and on physiological heat delivery. Temperatures inside the eye were therefore measured directly, ex vivo, before the device was tested in the living eye.

In patients, temperature rises from the front of the eye to the back (Table 1.1). Shinoda and colleagues measured it at four sites with a needle thermoprobe at the start of vitrectomy, before the infusion was opened, in an operating room set at 25 °C. They attribute the gradient partly to the cooler surroundings and note that their values apply to these conditions only [**46**]. Two later intraoperative series give different absolute values [**47, 48**]. In the series of Ríos-Nequis and colleagues, the vitreous was warmer in front of the optic disc than behind the lens, a rise the authors attribute to the nearby choroidal circulation [**48**].

TABLE 1.1. Intraocular temperatures measured in patients during vitreoretinal surgery, before vitrectomy. The posterior values are vitreous, not retinal, temperatures. —: not measured.

Study	**Anterior chamber**	**Behind the lens**	**Mid-vitreous**	**In front of the optic disc**
Shinoda et al. [**46**]: 30 eyes with intact vitreous	30.0 ± 1.7 °C	32.8 ± 1.2 °C	34.0 ± 0.9 °C	35.0 ± 0.9 °C
Kobylianskyi et al. [**47**]: 23 eyes with proliferative diabetic retinopathy	—	—	35.6 ± 0.7 °C	—
Ríos-Nequis et al. [**48**]: 12 eyes, combined cataract and vitreous surgery[a]	28.7 ± 2.8 °C	29.6 ± 0.7 °C	30.9 ± 0.7 °C	31.9 ± 1.1 °C

[a]Anterior chamber at the start of surgery; vitreous before vitrectomy, after the cataract step.

At the anterior surface, convection, radiation, and tear evaporation remove heat and help explain the cooler cornea and aqueous [**49, 50**]. Their contributions depend on ambient and surface conditions: convection and radiation can supply heat

in sufficiently warm surroundings, while evaporation removes it. Within the globe, heat is conducted through the ocular wall and contents, including the vitreous, and exchanged with the circulation.

The choroid can supply or remove heat according to the blood–tissue temperature difference. In anesthetized monkeys under room illumination, reducing choroidal flow lowered macular retinal–choroidal temperature. Under a focused microscope lamp, tissue temperature exceeded blood temperature and reducing flow raised it further [**39, 40**]. The same circulation that removes absorbed radiant heat can thus oppose externally imposed cooling, and with the high, weakly regulated choroidal flow described in Section 1.2.2 it does so continuously.

Thermal models differ mainly in how they represent heat delivery: a fixed-temperature boundary, a heat-transfer coefficient, a Pennes perfusion term, discrete vessels, or multiphase blood [**51, 52, 53**]. In the model of Geiser and colleagues, choroidal and ciliary perfusion outweighed retinal flow as a heat source, and ambient temperature from −20 to +40 °C changed the predicted retinal temperature by about 1.9 °C, from 35.2 to 37.1 °C [**54**, Table 1]. Flyckt and colleagues found the Pennes perfusion term adequate for first-order predictions and discrete vasculature necessary for finer spatial detail [**52**]. Gongal and colleagues modeled a scleral ring as a 4 °C boundary on the anterior sclera; with Pennes perfusion the warmest point of the ONH settled at 34.0 °C [**55**], whereas a multiphase formulation, in which cooled blood returning from the front of the eye carries the cold backward, predicted 29.6 °C [**53**]. These models and the device were developed in the same research program.

The measurements of this dissertation give temperatures inside the eye during contact cooling, for comparison with the models of contact cooling above. The ex-vivo experiments stand in for the choroidal heat load with a stirred 37 °C canola-oil bath (Chapter 2), and the device was also tested in vivo (Chapter 4).

1.3. Therapeutic Hypothermia and Neuroprotection

Therapeutic hypothermia is the deliberate lowering of tissue temperature to slow metabolism and limit ischemic and hypoxic injury. The clinical evidence concerns mild-to-moderate hypothermia (32–35 °C), which Polderman describes as the first treatment with proven efficacy for post-ischemic neurological injury [**56**], a judgment resting principally on the post-cardiac-arrest literature of that time and not confirmed by later trials (Section 1.3.2).

1.3.1. Mechanisms. Hypothermia acts on many targets at once; in Yenari and Han's review of the experimental literature it affects nearly every investigated cell-death pathway [**57**]. In ischemia, the failure of oxidative metabolism depletes adenosine triphosphate (ATP), disrupts transmembrane ion gradients, and promotes glutamate accumulation and calcium overload, followed on reperfusion by free-radical production and inflammation [**56, 57**]. Cooling lowers metabolism: brain oxygen consumption and glucose metabolism fall by roughly 5% for every degree Celsius, and ATP stores are preserved, which may limit calcium overload and excitotoxic glutamate accumulation [**57**]. Cooling also curbs apoptosis, the post-ischemic inflammatory response, free-radical production, and edema [**56, 57**].

Both reviews caution that lower oxygen and glucose consumption does not by itself explain the protection [**56, 57**]. Metabolic suppression may still matter in the retina, among the most metabolically active tissues in the body (Section 1.2.1). The rate of rewarming also affects the protection conferred by cooling in experimental models, and the systemic hypothermia literature treats controlled rewarming as part of the protocol [**56**].

1.3.2. Evidence in the Central Nervous System. Clinical outcomes depend on the indication and protocol [**57**]. Therapeutic cooling has an established role in neonatal encephalopathy. After cardiac arrest, early trials reported benefits [**58, 59**],

but the Targeted Temperature Management (TTM) and TTM2 trials found no improvement with 33 °C over a controlled 36 °C or over targeted normothermia [**60, 61**]. Current post-arrest guidelines recommend temperature control at a constant target between 32 and 37.5 °C with active prevention of fever, rather than mandatory hypothermia [**62**], and the 2025 European Resuscitation Council and European Society of Intensive Care Medicine guidelines emphasize fever prevention [**63**]. In acute ischemic stroke, the class to which CRAO belongs, the 2026 American Heart Association and American Stroke Association guideline finds no benefit from induced hypothermia in normothermic patients and lists the benefit of selective intra-arterial brain cooling as unknown [**8**].

These systemic trials do not establish the efficacy of localized retinal cooling, but they document a reason to investigate local delivery. Systemic cooling brings coagulopathy and an infection risk that rises with treatment duration [**56**], and in TTM2 more arrhythmia with hemodynamic compromise (24% against 17%) [**61**]. It is also slow: in eleven randomized trials after cardiac arrest the median time to 32–34 °C was 38–360 min [**64**]. During stroke thrombectomy, systemic intravascular cooling below 34 °C was followed by more serious adverse events than standard care, a difference the trial was not powered to test [**65**]. Selective cooling of the brain remains investigational [**62**]. Limiting the cooled region is an engineering strategy to limit this exposure.

1.3.3. Evidence in the Retina. Hypothermia has protected the retina, or reduced its injury-related signaling, in cell-culture, ex-vivo, and animal models at temperatures from about 34 °C down to 30 °C and below, and protection grew as cooling deepened. In isolated rabbit retina after 1 h of simulated ischemia, recovery of light-evoked compound action potentials was 15% at 36 °C, 36% at 33 °C, and 53% at 30 °C [**66**]. Cooling to 30 °C protected porcine retinal explants during 3 h of hydrogen-peroxide exposure [**67**], cooling to 30 and 20 °C protected retinal ganglion cells in hypoxic organ culture [**68**], and cooling to 34 °C reduced vascular endothelial

growth factor secretion from cultured RPE cells by 38% [**69**]. Cooling for 20 min at a retro-ocular temperature of about 32 °C, 24 h before the insult, protected the rat retina from ischemia–reperfusion damage [**70**].

How long the retina survives complete arterial occlusion is uncertain and depends on the model and the perfusion conditions. In Hayreh's experimental model, transient central retinal artery occlusion in elderly atherosclerotic rhesus monkeys produced no detectable damage at 97 min, with progressively more extensive irreversible damage at longer occlusion times up to 240 min [**30**]. Tobalem and colleagues, in a critical review, argue that this figure is an overestimate, attributing it to barbiturate anesthesia, uncontrolled hypothermia, and residual collateral perfusion in the clamp preparation, and propose that infarction of retinal ganglion cells is likely after only 12–15 min of complete occlusion [**31**]. Loss of function and irreversible injury can also diverge. Ex vivo, light responses of ON-bipolar cells recovered after overnight incubation in oxygenated medium at room temperature in human donor retinas collected 50–60 min after circulatory arrest, and in two of five pairs of eyes enucleated 3–4 h post mortem. Mouse retinas recovered fully after 1 h of hypoxia but only in part after 3 h. These preparations lack the vascular and inflammatory components of ischemia in vivo [**71**].

In each of these accounts the window is short relative to the delays with which patients present (Section 1.1) [**14, 15**]. The uncertainty in this window argues for an intervention that can be applied promptly and can extend tolerance (Section 1.8).

In the present work the electroretinogram (ERG) is used to detect temperature-sensitive retinal responses and their recovery after rewarming. Cooling the eye in vivo reduces ERG amplitudes and lengthens their implicit times [**72, 73**]. Temperature-dependent photoresponse features have been used to estimate temperature in isolated mouse retinas [**74**], temperature-dependent photopic ERG kinetics have been shown in isolated porcine retinas and anesthetized pigs [**75**], and the same group has since used focal ERG kinetics to determine retinal temperature during laser heating of the

living pig retina, with a standard error of 0.6 °C for 5-mm spots (1.3 °C for 3.4-mm spots) [**76**]. That calibration applies to focal, light-adapted responses heated above body temperature. For the full-field flash ERG recorded during cooling in this work, ERG changes are therefore indirect evidence of cooling. Tools developed in this laboratory that could show where, and how widely, the retina cools are discussed in Section 5.5.

1.4. Ocular Cooling: Prior Approaches and Their Limitations

Attempts to cool the eye span six decades, from research techniques never intended for clinical use to devices now in early clinical trials (Table 1.2; representative devices in Figure 1.6). Grouped by invasiveness, each falls short of the goal of this dissertation, sustained, controlled hypothermia at the posterior pole.

Invasive routes reached the retina or the optic nerve. Intraocular perfusion cooled the cat retina to 22 °C [**72**]. Anterior-chamber perfusion with a solution held at 11 °C by a feedback-controlled thermoelectric heat pump cooled the rabbit retinal surface by 2–4 °C [**77**]. Cold irrigation during vitrectomy protected the rabbit retina [**78**], and trans-nasal cooling protected the goat optic nerve only with a cold-protectant [**79**]. Cyclocryotherapy shows that cold can cross the sclera, but it destroys tissue by design [**80**]. All of these need intraocular or surgical access.

Research preparations in living animals showed that cold applied at the surface can reach the back of the eye [**70, 72, 73, 81**]. The experiments were short and used exposed eyes or rats. Temperature was measured in few animals or not at all, and none was under feedback control.

Apart from those probe readings [**70, 72**], no non-invasive cooling method has reported a temperature behind the lens. Compresses, a water-cooled contact lens, and handheld cooling-anesthesia applicators act on the surface or the anterior chamber for minutes or seconds [**82, 83, 84, 85, 86**]. The eyelid dressing of Anatychuk and colleagues shows that thermoelectric cooling can be packaged for ocular contact,

with feedback control for up to 48 h, but its posterior effect has only been modeled [**87, 88**]. Devices now in early clinical testing for chronic ocular surface pain show that scleral-contact cooling interfaces are practical [**89, 90, 91, 92**]. They target surface pain through the long ciliary nerves. None of these devices was designed for, or has measured, sustained hypothermia at the retina or the ONH.

TABLE 1.2. Prior approaches to ocular cooling, grouped by invasiveness. Temperatures are of the cooling medium or application site unless a tissue is named. ERG: electroretinogram.

Approach	**Deepest site and temperature reached**	**Why it falls short**
Invasive routes		
Intraocular perfusion, cat [**72**]	Retina, 22 °C; ERG reduced and slowed	Intraocular access; physiology, not rescue
Anterior-chamber perfusion, rabbit [**77**]	Retinal surface 2–4 °C cooler (11 °C, 10 min)	Intraocular access; rewarms within minutes
Cold irrigation, rabbit [**78**]	Retina: ERG and histology better at 8 than at 38 °C	Operating room only (vitrectomy)
Trans-nasal nerve cooling, goat [**79**]	Optic nerve, 4 °C; protected only with a cold-protectant	Surgical access; retina not cooled; vision not restored
Cyclocryotherapy [**80**]	Ciliary body ablated, −80 °C probe, 60 s	Destructive by design
Research preparations in living animals		
Cold-saline cap, cat [**72**]	Retina below 32 °C only with body cooled (probe)	Exposed eye; body and brain cooled too
Scleral cooling coil, rabbit [**73**]	Retina, estimated 15–18 °C (one animal); ERG changed	Eyelids and extraocular tissues removed
Ice pack, rat [**81**]	Inner retina preserved in ischemia	No temperature measured or controlled
Cold gel, rat [**70**]	Behind the globe, about 32 °C (pilot, 4 rats)	Not controlled; rat eye
Non-invasive and clinical devices		
Ice or cold compress [**82**]	Surface: less injection pain	No temperature measured or controlled
Water-cooled contact lens, rabbit [**83**]	Anterior chamber, 10.7–16.1 °C (probe); less corneal laser damage	Anterior only; posterior not tested
Thermoelectric eyelid dressing [**87, 88**]	Retina and vitreous 1–2 °C cooler, modeled only	No measured or clinical data
Cooling-anesthesia applicators [**84, 85, 86**]	Conjunctiva, −15 °C within 10 s	Seconds of cooling; surface anesthesia only
ETX-4143 scleral cooling [**89, 90, 91, 92**]	Conjunctiva over the long ciliary nerves; device about −15 to −5 °C; no temperature reported inside the eye	Targets surface pain, not the retina

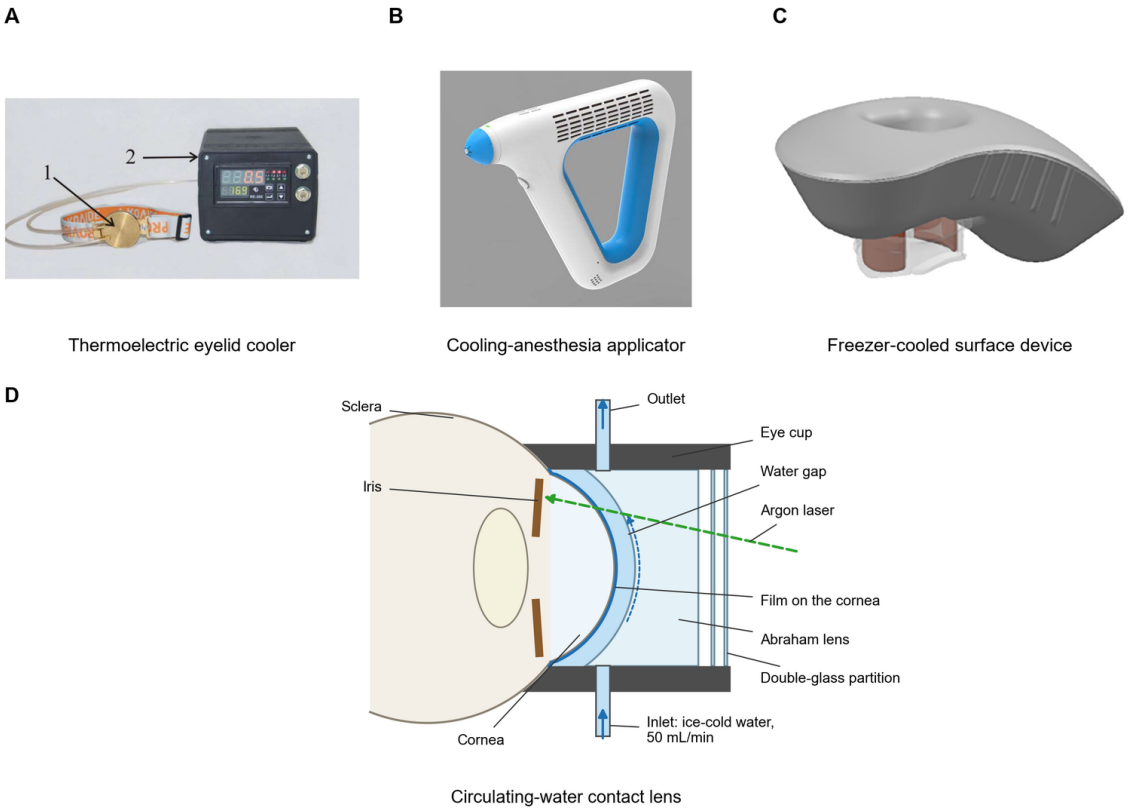


FIGURE 1.6. Representative ocular cooling devices. (A) Thermoelectric eyelid dressing and control unit (1: cooling plate; 2: control and power unit), from Anatychuk et al., Fig. 1 [**87**] (CC BY 4.0). (B) Handheld thermoelectric applicator for injection anesthesia, made by RecensMedical, the sponsor of OcuCool [**86**]; from Chao et al., Fig. 2A, cropped to the device [**85**] (CC BY-NC 3.0). (C) Freezer-cooled ocular-surface device, from Galor et al., Fig. 1 [**90**] (CC BY-NC-ND 4.0); the rabbit-study version used a single conductor. (D) Circulating-water contact lens used during argon laser iridotomy in rabbits: ice-cold water (50 mL min^{-1}) flows between an Abraham lens in an eye cup and a film on the cornea; drawn from the description by Kataoka et al. [**83**].

1.5. Thermoelectric Cooling

The device of this dissertation moves heat by thermoelectric cooling, which relies on the Peltier effect. When current is driven across a junction between dissimilar conductors, heat is absorbed at that junction and released at the other junction of the circuit, and the pair works as a heat pump rather than as a power source [**93**]. A conventional thermoelectric cooler (TEC) connects many n-type and p-type semiconductor legs electrically in series and thermally in parallel between two thin

ceramic plates, which insulate electrically while conducting heat (Figure 1.7) [**94**]. Current pumps heat from one face, the cold side, to the other; reversing the polarity reverses which face is cold, so one module can cool or heat. The module as a whole is not thermodynamically reversible: Joule heating (I^2R) and conduction back along the temperature gradient ($K\,\Delta T$) [**93**] are losses in both modes, where I is the drive current, R the electrical resistance of the module, K its thermal conductance, and ΔT the temperature difference between the hot and cold faces.

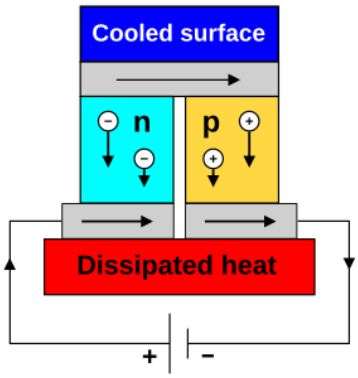


FIGURE 1.7. Operating principle of a thermoelectric couple: an applied current drives heat from the cold side to the hot side through n-type and p-type semiconductor legs. Illustration by Ken Brazier, Wikimedia Commons, CC BY-SA 4.0.

The first consequence for design is that cooling capacity and temperature difference trade off, and a module's headline ratings are therefore not available together. The rated maximum heat-pumping capacity applies at zero temperature difference, and the maximum temperature difference is reached only at zero cooling load [**95**]. Because the Peltier term is proportional to current and Joule heating to its square, the coldest attainable temperature is reached at an optimal current, beyond which more current warms the cold face [**93**]; manufacturers rate this current as the module's maximum current. A module must therefore be selected against the cooling load at the actual temperature difference, hot-side temperature, and electrical operating point, not against no-load ratings; this check is made for the device in Chapter 3 (Section 3.4).

The second consequence is that all the heat delivered to the hot side must be rejected to the ambient. This heat is $Q_h = Q_c + P_{\text{in}}$, where Q_c is the heat pumped from the cold side, P_{in} the electrical power supplied to the module, and Q_h the heat released at the hot side. Hot-side heat rejection is a principal constraint on realizing rated performance [**95, 96**], and the thermal resistance of the whole path, from the tissue through the module to the ambient heat sink, governs how much of a module's capability reaches the point of contact. A perfused eye adds a load that is continuously renewed.

The module is solid-state, without moving parts or a working fluid, and makes no vibration or acoustic noise [**95, 96, 97**], although heat rejection still needs a system around it: the cooler developed here (Section 1.7), like the eyelid device of Section 1.4 [**87**], uses a liquid loop. The annular module adopted here is 26.0 mm in outside diameter with a 14.0 mm central hole and a thickness of 4.7 mm [**95**], which leaves an aperture for the cornea and the optical path. Temperature is set by the magnitude and direction of the drive current and held by closed-loop feedback, and the module is compatible with battery operation, as in portable thermoelectric cooling for personal thermal management [**98**].

1.6. The Porcine Model

Among nonprimate species, the pig is a widely used large-animal model of the human eye, and its use in retinal research continues to grow [**99**]. Its globe approaches human scale, 23.9 ± 0.08 mm in anteroposterior length in enucleated eyes [**100**] against roughly 24 mm in humans, which supports the testing of device-interface tools intended for human use [**99**].

Porcine scleral geometry and water content support its use for interface development, but differences remain; Table 1.3 gives the values with their measurement conditions [**101, 102**]. Two of these differences matter for a scleral-contact device.

Porcine sclera is thinnest only a few millimeters behind the limbus, whereas the human sclera, away from the muscle insertions, is thinnest near the equator. A device 5–6 mm from the limbus in the pig is therefore analogous to one near the equator in a human eye [**101**]. Second, the porcine orbit is small and its extraocular musculature unusually strong, so extraocular devices are difficult to place behind the muscle insertions [**101**], which limits where a ring can sit in this species.

Porcine measurements show a cooler aqueous compartment and warmer posterior vitreous, qualitatively resembling the human gradient [**46, 103**]. Thermal similarity must be assessed experimentally, as in Aim 1.

The retina itself is comparably organized. Porcine retinal circulation is holangiotic, as in humans, but the pig has no central retinal artery, the retina being supplied by several cilioretinal arteries, and its orbital supply arises from the external carotid [**104, 105**]. The retinal layers are distributed as in humans [**100, 106, 107**]. The pig lacks a true fovea and has an elongated visual streak of high cone density (Table 1.3) [**99, 108**]; foveal function is not an endpoint here, since the work targets cooling of the posterior eye, measured at the proximal optic nerve.

The living pig is expected to be a conservative test of posterior cooling, because perfusion is the principal heat load that a cooling device must overcome in vivo (Section 1.2.3) and porcine choroidal flow is high. It exceeds the per-eye totals measured in monkeys by the same microsphere method and, although the methods differ, the per-area values measured in humans by magnetic resonance imaging (MRI) (Table 1.3) [**109, 42, 110**], and human choroidal flow declines further with age [**111**].

Porcine models of ocular hypertension and retinal ischemia are available for future efficacy studies [**113, 114**], but the present work induces neither. The same species serves both phases: eyes from a commercial supplier for ex-vivo thermal characterization, and the live animal for functional and safety assessment with perfusion.

TABLE 1.3. Human and porcine ocular parameters from separate studies. Human scleral thicknesses are, unless another source is given, histological means from non-axially elongated eyes enucleated for melanoma or end-stage glaucoma [**112**]; porcine values are means from formalin-fixed globes grouped by body weight [**101**]. Blood-flow methods are given in each cell.

Parameter	Human	Porcine	Source
Globe axial length	≈24 mm (postmortem anteroposterior mean 24.44 ± 1.03 mm)	23.9 ± 0.08 mm (5 eyes, 6–8 months)	[**22, 23, 100**]
Scleral thickness, limbus	0.50 ± 0.11 mm	0.83, 0.91, 1.12 mm (small, medium, large)	[**101, 112**]
Scleral thickness, equator	0.42 ± 0.15 mm	0.56, 0.73, 0.86 mm (small, medium, large)	[**101, 112**]
Thinnest point away from the muscle insertions, and its location	≈0.4 mm near the equator, ≈13 mm behind the limbus	0.31, 0.35, 0.43 mm at only 5–6 mm behind the limbus	[**101**]
Scleral thickness, under the rectus muscles	≈0.3 mm	Not reported by the cited sources	[**27**]
Scleral thickness, posterior pole	0.94 ± 0.18 mm measured; 1.0–1.35 mm quoted in review	Thickness plateaus from ≈10–12 mm behind the limbus; posterior-pole value not tabulated	[**27, 101, 112**]
Total scleral surface area	≈16–17 cm^2	7.78, 9.66, 11.92 cm^2 (small, medium, large)	[**101**]
Scleral water content (wet mass)	$71.6 \pm 0.63\%$	$69.5 \pm 1.18\%$	[**102**]
Choroidal blood flow	≈1.1 $\mu L\,min^{-1}\,mm^{-2}$ (MRI, retina + choroid, posterior pole); monkey 0.6–1.3 $mL\,min^{-1}$ per eye (microspheres)	3.2–3.7 $mg\,min^{-1}\,mm^{-2}$, ≈1.6–1.7 $g\,min^{-1}$ per eye of blood (microspheres, 6 pigs, 19–27 kg, pentobarbital)	[**109, 42, 110**]
Choroidal response to perfusion pressure	Partial compensation for moderate rises	Falls in proportion to perfusion pressure	[**45, 109**]
Retinal vascular pattern	Holangiotic	Holangiotic; no central retinal artery; orbital supply arises from the external carotid	[**100, 104**]
Fovea	True fovea present	Absent; elongated visual streak (20,000–35,000 cones mm^{-2}, higher local peaks)	[**108**]
Experimentally induced disease models	—	Ocular hypertension (episcleral vein cauterization); retinal ischemia (endovascular occlusion)	[**113, 114**]

1.7. The Scleral-Contact Eye Cooler

The scleral-contact eye cooler (SCEC) applies a controlled thermal boundary at the accessible anterior sclera to remove heat from the globe. It has been developed in the Neural Engineering Vision Laboratory over several student projects. The first prototype (Gen 1) was built by S. J. Dreyer as a portable cooler for traumatic eye injury [**115**]: a machined aluminum ring on the anterior sclera, with three thermoelectric modules on its faces and finned heat sinks cooled by forced air, with the modules driven at a fixed current from a bench supply without closed-loop control. It was tested on thawed porcine eyes with three scleral thermistors in a 37 °C canola-oil bath, chosen because its conductivity is close to that of orbital fat, and the same bath is used in Chapter 2. The posterior sclera cooled by 6.6 °C on average (four of five eyes), some tissue in contact with the ring froze, and the removal of heated air limited how cold the ring could become; liquid cooling was judged more efficient but set aside for a field device. The second prototype (Gen 2), designed and built in 2018–2019 by K. Fong, kept the Gen 1 ring and replaced the air-cooled heat sinks with water cooling. In 2018, N. Shirdade carried out thermal simulations of the cooling of the porcine eye. An ex-vivo methods manuscript in preparation builds on the Gen 2 device and on these simulations (Contribution of Authors).

The third generation (Gen 3), the starting point of this dissertation, was completed by the author during his M.S. study [**1, 116**] (Figure 1.8). A scleral contact ring (SCR), shaped to conform to the anterior sclera, forms the cooled boundary against the eye while leaving a central aperture over the cornea and pupil. The device therefore does not touch the cornea, although the cornea can still cool indirectly, and the visual axis stays unobstructed, which keeps assessments such as the pupillary light reflex accessible. A single annular TEC (Section 1.5) pumps heat from the SCR to a liquid-cooled heat exchanger, or water block, and the coolant carries it to a copper coil in ice water. Control electronics provide power, thermoelectric drive, and thermal sensing. A temperature sensor embedded in the SCR is the feedback

signal by which the ring is held at its set point, 4 °C, the lower limit of hypothermic organ storage (Section 2.2.1); the drive was adjusted at first by the operator and later switched automatically by a comparator. The controlled variable is the ring temperature, and tissue temperatures elsewhere in the globe are responses to the imposed boundary. The tolerance of living ocular tissue to prolonged cooling at this boundary is assessed in Aim 3.

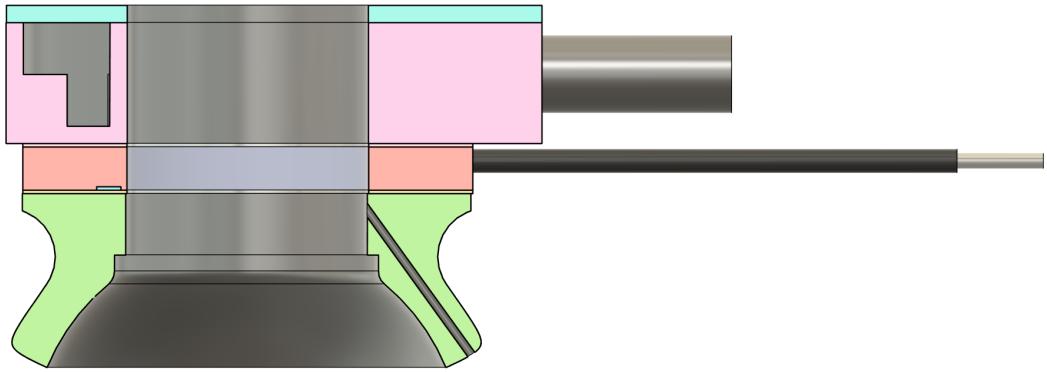


FIGURE 1.8. Cross-sectional computer-aided design (CAD) view of the scleral-contact eye cooler (SCEC): contoured scleral contact ring (green), annular thermoelectric cooler (TEC; orange), and liquid-cooled heat exchanger, or water block (pink and blue). The central aperture gives optical access; the coolant connection and electrical lead extend laterally, and an oblique bore through the ring holds the thermistor.

In the M.S. work, fresh porcine eyes with thermistors were cooled in the oil bath and imaged to document the sensor positions. The next steps set out there (a ceramic ring, automatic control of the TEC, a smaller system, and tests in perfused eyes and human donor eyes) are the subject of this dissertation. Results from the porcine eyes, obtained with Y. Nakamura, were presented at two conferences [**117, 118**] and published online in 2025 [**1**], and the human donor experiments were presented in 2026 [**119**]. Within the same research program, Gongal and colleagues published two finite-element models of the human eye cooled by a scleral ring [**53, 55**] (Section 1.2.3).

Two version names are used for the remainder of the dissertation, because the device evolves across the three aims. *SCEC Gen 3* denotes the ex-vivo prototype

validated in Aim 1 (Chapter 2). *SCEC Gen 4* denotes the enhanced device developed in Aim 2 (Chapter 3), which targets improved efficiency, portability, safety, and patient comfort, including the consolidation of drive and sensing onto an integrated board. The in-vivo study of Aim 3 (Chapter 4) used SCEC Gen 3 fitted with the aluminum reduced-bulk ring of Aim 2, a build that carried over some, but not all, of the Gen 4 enhancements; its configuration is reconciled against the Gen 4 target in Section 5.4, so that "the device" refers to a definite object in every chapter. Heating and analgesic applications are discussed as future work in Section 5.5.

1.8. Knowledge Gap and Specific Aims

Acute retinal ischemia lacks a proven treatment, and most patients present beyond the window of time-limited reperfusion (Section 1.1). Preclinical evidence supports cooling as a means of extending the retina's tolerance of ischemia, but a practical delivery strategy must provide localized, active, and controlled cooling through an accessible interface, sustain it, produce a characterized response at the posterior eye, and be safe in the living eye. To the author's knowledge, no existing approach provides all of these (Table 1.2).

The obstacle is measurement as much as hardware: the temperature of a contact device does not give the temperature at the retina or optic nerve (Section 1.2.3). Before this work, no scleral-contact cooling device had been characterized with spatially verified temperatures throughout the globe, none had been characterized thermally in human eyes, and the porcine eye had not been shown to predict the human thermal response. The scleral-contact devices already applied in patients target surface pain and did not measure the temperatures they produced inside the eye, and in living animals the posterior eye had been cooled from outside only in short research preparations, none under feedback control (Section 1.4). No scleral-contact device had been used for sustained, controlled cooling of the posterior segment of an intact living eye of human size.

The central hypothesis of this dissertation is that a scleral-contact thermoelectric device can produce reproducible posterior ocular cooling under controlled ex-vivo conditions, retain cooling performance through selected design refinements, and produce functional responses consistent with retinal cooling in vivo while maintaining ocular and systemic safety under the tested conditions. It is addressed through three Specific Aims, which assess thermal delivery and preclinical tolerability. Neuroprotective efficacy after ischemia and thermal analgesia motivate the work, but establishing either requires outcome studies with an injury model or a pain measure, which are left to future work (Section 5.5).

Specific Aim 1: Validate SCEC Gen 3 and characterize its thermal performance in ex-vivo eyes. The objective is to characterize, across ex-vivo models using formalin-preserved porcine, fresh porcine, and fresh human donor eyes, the cooling the scleral-contact interface produces at defined measurement sites, and how much it varies between eyes. Temperature is recorded at five ocular sampling locations (anterior, equatorial, and posterior sclera; vitreous; and the proximal optic nerve), whose positions are verified by micro-computed tomography (micro-CT), together with the SCR feedback channel. As a secondary comparison, preserved porcine eyes are compared with fresh porcine eyes to determine whether preserved tissue can serve as a benchtop surrogate for device development.

Two hypotheses are tested. First, SCEC Gen 3 produces a reproducible reduction in temperature at the proximal optic nerve within approximately 15 min (90% of the 20-min excursion), with its size interpreted against a 3 °C benchmark. Second, the cooling excursion at this site differs between fresh human and fresh porcine eyes by an amount that is small against the excursion itself. The absence of a difference cannot be shown directly. This comparison is therefore reported as the difference between the species with its confidence interval, whose limits give the largest difference that the data allow. Cooling is measured as the decrement from each eye's own baseline, because baseline temperature differs between preparations. The 3 °C benchmark is

the smallest cooling step tested in isolated rabbit retina under simulated ischemia, and functional recovery improved at that step [**66**]. It is a benchmark and not an established threshold (Section 2.4). Absolute temperatures are therefore reported alongside the decrement and read against two reference intervals: the 32–35 °C interval of systemic therapeutic hypothermia (Section 1.3) and the roughly 30–34 °C interval of preclinical retinal protection (Section 1.3.3). Performance is characterized by temperature–time profiles, baseline temperature, cooling excursion, time to 90% of the excursion (t_{90}, a measure of kinetics), and variability between eyes within each preparation; the differences between preparations, including the thermal difference between species, are evaluated separately (Chapter 2).

Specific Aim 2: Enhance the device (SCEC Gen 4) for efficiency, portability, safety, and patient comfort. The objective is to develop and evaluate SCEC Gen 4 refinements through computational modeling, experimental assessment of selected scleral contact ring designs, and the design of an integrated printed circuit board for device control. Refinements include reduced-bulk ring geometries with and without an integral corneal lens, alternative ring materials, an insulating outer layer to focus heat extraction, water-block and cooling-circuit miniaturization, and consolidation of control electronics. The TEC was potted in every device used in this work. Reduced form factor and bulk are intended to improve portability, handling, and patient comfort, while material selection and TEC potting address safety considerations at the device–tissue interface and within the thermoelectric assembly.

Computational modeling provides the primary basis for the comparisons, and experiments on selected fabricated rings provide complementary evidence. The hypothesis is that selected geometric and material refinements can reduce device bulk while preserving cooling performance at the proximal optic nerve. Patient comfort itself is not directly measured; improvements relevant to it are assessed through reductions in device dimensions and bulk.

Specific Aim 3: Validation and safety assessment in an in-vivo porcine model. The objective is to evaluate evidence of retinal cooling and assess ocular and systemic safety in six live pigs under a protocol approved by the University of Illinois Chicago (UIC) Animal Care Committee (ACC), the university's institutional animal care and use committee (Section 4.2.1). Each animal undergoes three cooling sessions, lasting 30, 60, and 120 min, followed by a terminal session for final assessments and tissue collection. Testing in the perfused living eye evaluates device performance in the presence of physiological heat delivery, including the choroidal circulation (Section 1.2.3), and allows the systemic response to the device to be assessed.

Changes in the ERG are used as indirect functional evidence of retinal cooling, which reduces ERG amplitudes and lengthens their implicit times [**72, 73, 75, 120**]. The b-wave implicit time and amplitude are used together, a longer implicit time and a smaller amplitude being the expected response. Amplitude is not interpreted alone, because it also varies with electrode contact and position. Changes in the test eye are compared with those in the control eye, which removes systemic and anesthetic effects shared by both eyes. These measurements indicate cooling but do not measure retinal temperature or its spatial extent.

Safety endpoints are retinal functional recovery after rewarming, corneal integrity assessed by fluorescein staining and surface analysis, intraocular pressure measured by tonometry, histology, and systemic stability. ERG responses during cooling and after rewarming are assessed separately to distinguish reversible functional changes from persistent abnormalities within the observation period.

The hypothesis is that device application produces ERG changes consistent with retinal cooling, that the ERG returns to its baseline values after rewarming, and that intraocular pressure, redness, corneal staining, and histology show no increase in the test eye beyond that in the fellow control eye across the tested cooling sessions.

CHAPTER 2

Specific Aim 1: Validate SCEC Gen 3 and Characterize Its Thermal Performance in Ex-Vivo Eyes

Portions of this chapter have been published or presented [**1, 116, 117, 118, 119**]; the Contribution of Authors gives the details.

The fresh-porcine recordings, first reported in the author's M.S. thesis [**116**] and published by Nakamura et al. [**1**], were reprocessed for this chapter together with the human data, through the common quality-control pipeline described in Section 2.2.6. All fresh-porcine values reported here are the reprocessed values. They differ slightly from the published ones, and none of the published conclusions changes. The two sets are compared in Appendix B.

2.1. Background and Rationale

This chapter measures how far cooling applied at the surface reaches into the eye. An ex-vivo preparation permits temperature sensors to be placed at multiple sites throughout the globe, their positions to be verified by imaging, and the resulting temperature fields to be recorded without the variability introduced by blood flow, systemic physiology, anesthesia, and thermoregulation. The absence of perfusion is also the preparation's principal limitation. It was addressed in part by immersing the posterior globe in a continuously mixed 37 °C bath (Section 2.2.2). Like the choroidal circulation, the bath resupplies heat to the globe during cooling. It does so from outside the sclera, however, whereas the choroid delivers heat directly beneath the retina, and the heat the bath delivered was not measured (Section 2.4). The preparation thus establishes the tissue-level response to scleral-contact cooling against a physiological warm boundary.

Three preparations were used because each answers a different question. Formalin-preserved porcine eyes, which can be stored and used without the timing constraints of fresh tissue, were used to develop the platform and to test whether it resolved region-specific cooling reproducibly. Fresh porcine eyes established proof of concept [**1**] and provided the reference against which preserved tissue was assessed as a practical benchtop surrogate. Fresh human donor eyes were required because agreement between the two species cannot be assumed. Porcine and human globes have similar axial lengths, but in separate histological series the porcine sclera is roughly 1.3 to 2.2 times as thick as the human sclera at the limbus and equator, although the porcine sclera is thinnest 5 to 6 mm behind the limbus, close to where the ring sits (Section 1.6, Table 1.3). Because the sclera is the first conductive path between the contact ring and the posterior eye, these differences could alter both the magnitude and the spatial distribution of cooling. The porcine eye can inform device development (Chapter 3) and interpretation of the in-vivo study (Chapter 4) only to the extent that its thermal response matches the human response at the site of interest.

The hypotheses are stated in Section 1.8. The cooling at the proximal optic nerve was judged against a 3 °C benchmark derived from retinal evidence [**66**], and preserved porcine eyes were compared with fresh porcine eyes as a secondary comparison (Section 2.2.6).

2.2. Methods

2.2.1. Device Description: SCEC Gen 3. SCEC Gen 3 (Figure 2.1) consisted of three thermally coupled components: an aluminum scleral contact ring (SCR), an annular thermoelectric cooler (TEC), and a liquid-cooled aluminum heat exchanger (water block). The SCR formed the interface with the anterior sclera and was machined with a curved ocular-contact surface having an intended contact area of approximately 390 mm^2. A 14-mm-diameter central aperture left the cornea exposed

and preserved access to the optical axis. A negative-temperature-coefficient thermistor embedded in the SCR provided continuous measurement of its temperature.

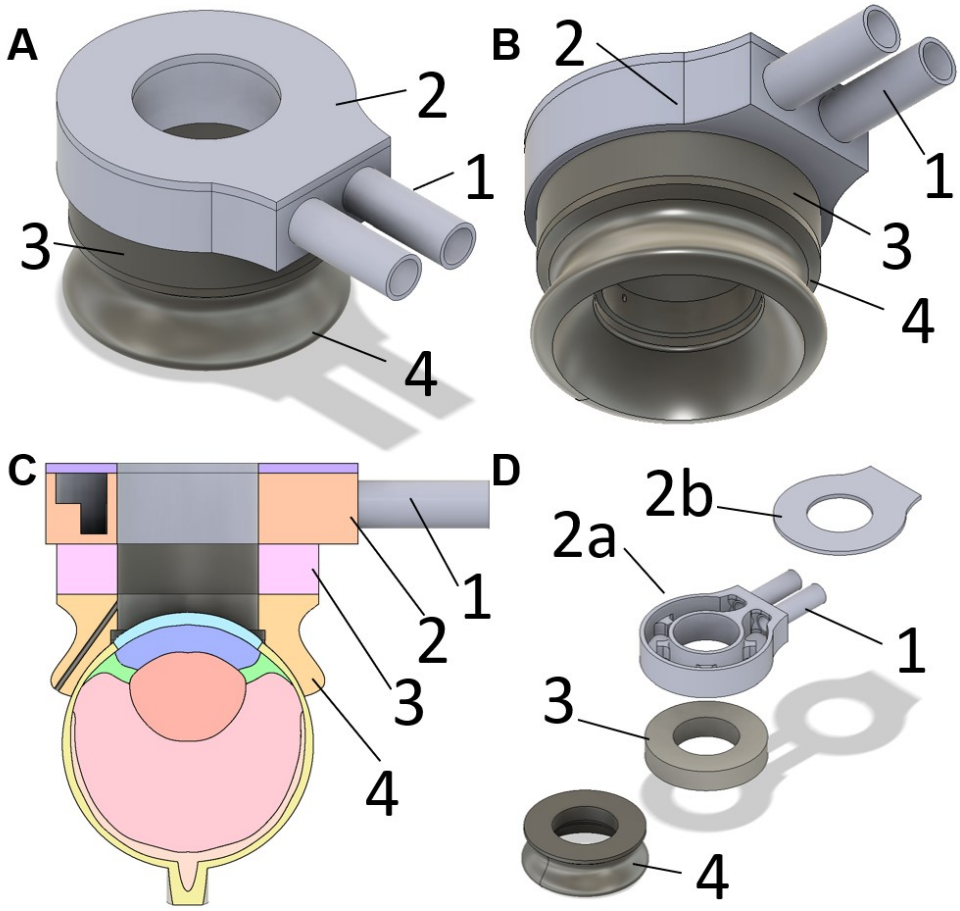


FIGURE 2.1. Components of SCEC Gen 3. (A) Solid model seen from above and (B) from below. (C) Cross-section of the device in position on a schematic porcine eye. (D) Exploded view. 1, coolant inlet and outlet; 2, heat exchanger (2a, body; 2b, lid); 3, thermoelectric cooler (TEC); 4, scleral contact ring (SCR). Panel C is adapted from Nakamura et al., Fig. 2A [**1**], CC BY 4.0.

Heat extracted from the eye entered the aluminum SCR and was pumped across an annular TEC (RH14-14-10-L1-W4.5, Tark Thermal Solutions, Durham, NC, USA). The hot side of the TEC was coupled to the custom-machined aluminum heat exchanger, through which coolant was continuously circulated. The interfaces between the SCR, TEC, and heat exchanger were bonded with thermally conductive epoxy

(OB-101-2, Omega Engineering, USA). The assembled SCEC Gen 3 was approximately 23 mm in height.

The SCR temperature was the controlled variable throughout the experiments, and the cold set point was 4.0 °C in all cooling experiments described in this chapter. The TEC was driven in current-controlled mode at up to 3.9 A, the module's maximum current, about 8 W of electrical power at the supply (3.9 A at 2.1 V). In the porcine experiments, with both preserved and fresh eyes, the operator adjusted the supply to the TEC manually while monitoring the SCR thermistor, holding the SCR at the set point. For the human donor eyes, the platform had been upgraded with automatic feedback control. A comparator with hysteresis (Schmitt trigger) switched the TEC drive in response to the SCR thermistor (Figure A.2 in Appendix A). In the human runs, once at the set point, the ring temperature cycled over a band of about 0.4 °C with the original aluminum ring and with the reduced-bulk ring, and 0.6 °C with the ceramic ring (1st to 99th percentile of the unfiltered SCR channel over the final 4 min). The control method differed between the porcine and human experiments, and device performance was therefore evaluated from the measured SCR temperature together with the simultaneously recorded tissue temperatures.

The set point was chosen at the lower limit of hypothermic organ storage. Organ preservation aims to hold tissue between 4 and 8 °C, because temperatures below about 2 °C increase the risk of cold injury [**121**], and donor corneal endothelium is preserved for up to two weeks at 4 °C in Optisol storage medium [**122**]. Freezing injury requires ice formation, with lethal temperatures from about −2 to −70 °C depending on the tissue [**80, 123**], so a 4 °C boundary cannot produce it. Clinical ocular devices briefly expose the conjunctiva and sclera to much colder surfaces [**86, 90, 91**]. Tissue in contact with the ring remains warmer than the set point: in fresh porcine eyes the anterior sclera equilibrated near 11 °C (Section 2.3.2).

2.2.2. Ex-Vivo Apparatus. The ex-vivo apparatus was built for this work from custom subsystems: a positioning system for the eye and the cooling device, an

intraocular-pressure system, a warm oil bath, and a coolant circuit for the heat exchanger (Figures 2.2 and 2.3). It accepts excised porcine or human eyes instrumented with up to five temperature sensors, and it holds the induced tissue temperatures for as long as cooling is applied.

Eye and device positioning system. An aluminum frame carried two independent vertical translation stages, one for the three-dimensional (3D) printed eye holder, on which the eye rested cornea-up on a perforated support ring, and one for a 3D-printed holder for the SCEC, so that the device could be lowered onto the eye.

Intraocular pressure. Intraocular pressure was maintained near the physiological range using a gravity-fed reservoir of room-temperature phosphate-buffered saline connected to the anterior chamber through tubing, a syringe barrel and a 27-Ga needle inserted through the cornea. In all three cohorts, the reservoir was set about 21 cm above the eye. A column of 20.7 cm of water corresponds to the mean intraocular pressure of normal porcine eyes, 15.2 mmHg [**113**]. In all fresh-eye experiments, fluid movement into the eye was minimal.

Warm oil bath. The posterior globe was partially submerged in a canola-oil bath maintained at approximately 37 °C. The oil was mixed continuously, at first by air bubbled through the oil and later, to reduce oil splatter, by a submersible pump. The bath stood in for the orbital tissues that surround the globe and, as a warm boundary, resupplied heat to the tissue in place of the absent perfusion. The oil level was approximately 4 mm posterior to the limbus, immediately below the SCR.

Coolant circuit. Heat rejected by the TEC was removed through the heat exchanger by coolant that a 12-V pump circulated continuously at approximately 150 $\mathrm{mL\,min^{-1}}$ through a copper coil immersed in a beaker of ice and water. The pump and its setting were the same as in the porcine runs of [**1**]. The coolant was a 50:50 mixture of propylene glycol and distilled water in the porcine experiments and distilled water in the human-donor experiments.

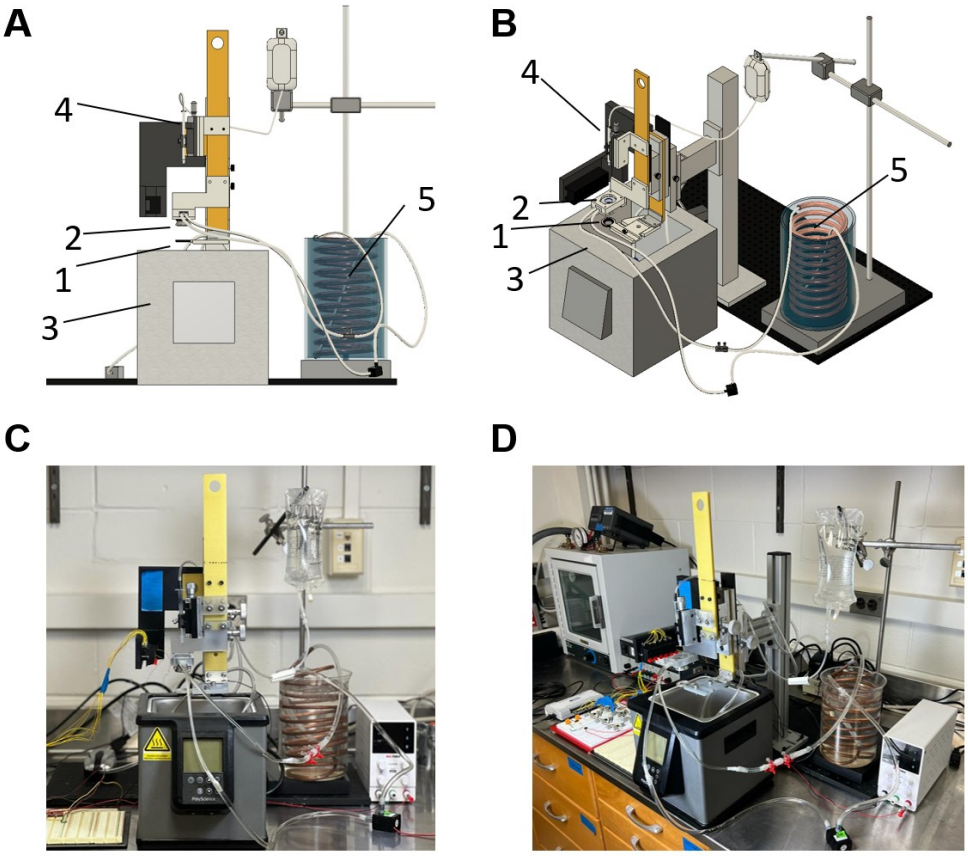


FIGURE 2.2. Ex-vivo apparatus. (A) Frontal and (B) oblique views of the CAD model, and (C, D) the corresponding photographs of the assembled apparatus. 1, eye holder; 2, cooling-device holder; 3, oil bath; 4, intraocular-pressure syringe (fed from the elevated saline reservoir); 5, ice bath.

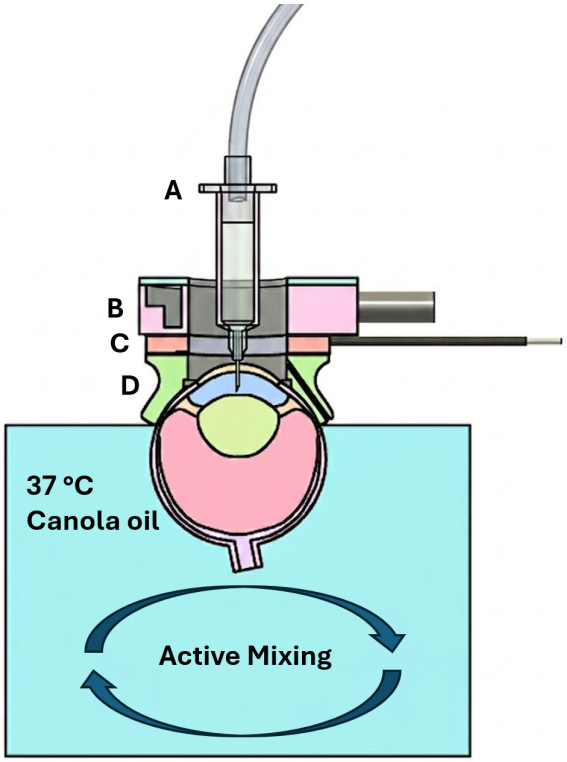


FIGURE 2.3. Schematic cross-section of the ex-vivo cooling setup. The posterior globe is partially submerged in the continuously mixed 37 °C canola-oil bath, and an infusion needle (A) in the anterior chamber connects the eye to the intraocular-pressure reservoir. SCEC Gen 3 rests on the anterior sclera: heat exchanger (B), TEC (C), and SCR (D). Adapted from Nakamura et al., Fig. 2A [**1**], CC BY 4.0.

2.2.3. Ex-Vivo Eye Preparation. Three ex-vivo tissue cohorts were used, in the order in which they were studied: formalin-preserved porcine eyes, fresh porcine eyes, and fresh human donor eyes.

Formalin-preserved porcine eyes (Pig Eyes, Plain Preserved) were obtained from VWR International LLC (Pittsburgh, PA, USA). The eyes are sold as formalin-preserved specimens, without color staining, in a holding fluid that contains less than 0.1% formaldehyde by weight according to the supplier's safety data sheet. The preserved-eye experiments were used to assess the reproducibility and utility of the ex-vivo platform. Five preserved eyes were included in the thermal analysis.

Fresh porcine eyes were obtained through a commercial supplier (Animal Biotech Industries, Doylestown, PA, USA). Globes were harvested with about 13 mm (half an inch) of proximal optic nerve attached and shipped overnight in an insulated box with cold packs; they arrived the day after harvest and were tested on the day of arrival or the day after. Five fresh porcine eyes were included in the thermal analysis.

Fresh human donor eyes were obtained from VisionFirst Indiana Lions Eye Bank (Carmel, IN, USA). Eyes were eligible if they were received within 72 h of enucleation, were grossly intact, and retained at least 10 mm of proximal optic nerve. Eyes with penetrating trauma or gross ocular pathology were excluded. Nine eyes from five donors were included. Donors were male, 43–77 years of age, and the cohort supplied for this study was exclusively White. Enucleation took place 8.5–19.4 h after death (median 12.4 h). The tissue was de-identified and supplied with authorization for research use.

In all three cohorts, excess extraocular tissue, including the rectus muscles and orbital fat, was removed to expose the scleral surface, and each eye was then mounted cornea-up on the eye holder (Section 2.2.2) with a small quantity of cyanoacrylate adhesive.

2.2.4. Temperature Measurement and Spatial Verification. Temperature was measured with negative-temperature-coefficient thermistors (GA10K3MCD1, TE

Connectivity Ltd., China) [**124**]. The sensing tip of each sensor was at most 3.3 mm long and 0.5 mm in diameter, with a nominal response time of 200 ms in liquid. Five sensors were implanted at anatomically defined locations: anterior sclera, equatorial sclera, posterior sclera, vitreous, and proximal optic nerve, adjacent to the optic nerve head. The three scleral thermistors were introduced through small needle incisions and aligned approximately parallel to the scleral surface. The vitreous sensor was advanced toward the center of the globe. The proximal optic nerve sensor was inserted along the axis of the nerve in the porcine eyes, and parallel to the posterior scleral surface, at the junction of the nerve with the globe, in the human eyes (Figure 2.4). Cyanoacrylate adhesive was used to stabilize the leads at the insertion sites.

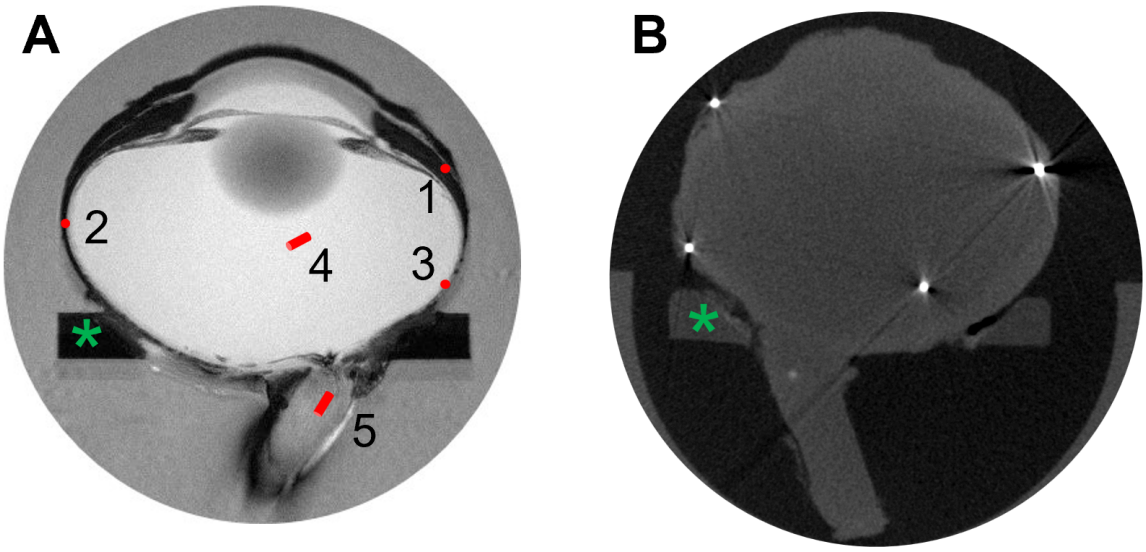


FIGURE 2.4. Target and measured positions of the implanted sensors in porcine eyes [**1**]. Left (A): MRI slice near the midline of the globe, with the target position and orientation of each sensor as a red cylinder: 1, anterior sclera; 2, equatorial sclera; 3, posterior sclera; 4, vitreous; 5, proximal optic nerve. The dark band between the front of the eye and the bath is the displacement of the oil bath relative to the tissue in the image, not anatomy (Section C.4). Right (B): micro-CT slice of an instrumented eye; four of the five sensors are the bright spots (the optic nerve sensor lies just outside the slice), and the green asterisk marks the acrylic eye support. Adapted from Nakamura et al., Fig. 1 [**1**] (scale bars omitted), CC BY 4.0.

A sixth thermistor embedded in the SCR measured the imposed boundary temperature and provided the feedback signal used for device control. Each thermistor formed one arm of a voltage divider whose output was amplified by a non-inverting amplifier, giving approximately −8.0 V at 37 °C and +8.0 V at 4 °C (about 0.48 V °C^{-1}). A Hall-effect current sensor (ACS723 breakout, SparkFun Electronics, Niwot, CO, USA) in series with the TEC supply recorded the drive current. All six temperature channels were acquired simultaneously at 10 Hz using a DI-2108 data acquisition (DAQ) unit (DATAQ Instruments, Akron, OH, USA) and recorded with WinDaq software (Figure A.1 in Appendix A).

Porcine eyes were imaged by 9.4-T MRI before implantation to document gross ocular anatomy, and the implanted thermistors in the porcine and human eyes were localized by micro-CT after the cooling experiment (settings in Section C.4). Reconstructed volumes were imported into 3D Slicer, and sensor coordinates were used to assess spatial consistency and identify sensors that did not sample the intended anatomical compartment.

The calibration procedure evolved during the experimental program. In the porcine experiments, the channels were calibrated before each session in a Fluke 9102S dry-block calibrator [**125**] fitted with a custom insert. With the thermistors held at 20.5 and 37.0 °C, the two potentiometers of each channel's amplifier, which set the intercept and slope, were trimmed until the output read 0.00 V and −8.00 V, respectively. These are the values of the circuit's linear voltage–temperature transfer function at those temperatures, and this single fixed function then converted voltage to temperature for all channels. For the later human-donor experiments, the channels were trimmed in the same way and three calibration plateaus, at 4.0, 20.5, and 37.0 °C, were then recorded in each session. Each plateau was read as the median of its last 3 min before the ramp to the next temperature, the last 10 s excluded; where those 3 min still drifted by more than 5 mV per minute, it was read as the median of the longest stretch whose 10-s medians stayed within ±10 mV. In human

eyes 1 to 4, less than 3 min of the 37.0 °C plateau was recorded, after the block had settled for more than 7 min; there the plateau was read as the median of the whole recorded plateau less its first and last 10 s. The same rule was applied to the in-vivo calibrations (Section 4.2.6). The voltage of each channel was converted to temperature with the least-squares straight line through its three plateaus. The conditioning circuit is linearized. On a bench recording of eleven dry-block temperatures between 2 and 39 °C, a straight line reproduced the block temperature within 0.34 °C.

The expanded uncertainty of an individual absolute temperature measurement was ±1.8 °C ($k = 2$). It is dominated by sensor self-heating, which can only make a reading too low, by at most 1.4 °C, and no correction was applied for it. Because the primary comparative endpoint was a within-run cooling excursion, static common-mode contributions largely canceled. Taking the larger of the human and porcine values, the corresponding differential uncertainty was ±0.40 °C ($k = 2$) for a small excursion, such as that of the proximal optic nerve, and ±1.1 °C for a large one, such as that of the SCR. The uncertainty budget is given in Appendix A.

2.2.5. Experimental Protocol. For each experiment, the instrumented eye was positioned cornea-up in the 37 °C bath and the SCR was lowered onto the anterior sclera using a linear translation stage. Circumferential contact was established by directing light through the central aperture and lowering the device until light was no longer visible between the ring and the scleral surface. The sequence of steps is summarized in Figure 2.5.

The porcine experiments used an active thermal-equilibration step before cooling. The SCEC was operated in reverse polarity to bring the interface toward approximately 34.5 °C, the temperature of the exposed anterior ocular surface, and the preparation was allowed to equilibrate for approximately 30 min. The temperature at each sensor at the first sample of the record defined its initial value.

At time zero, the device was switched to cooling polarity and the SCR set point was changed to 4.0 °C. Temperature at the SCR and all five implanted locations was

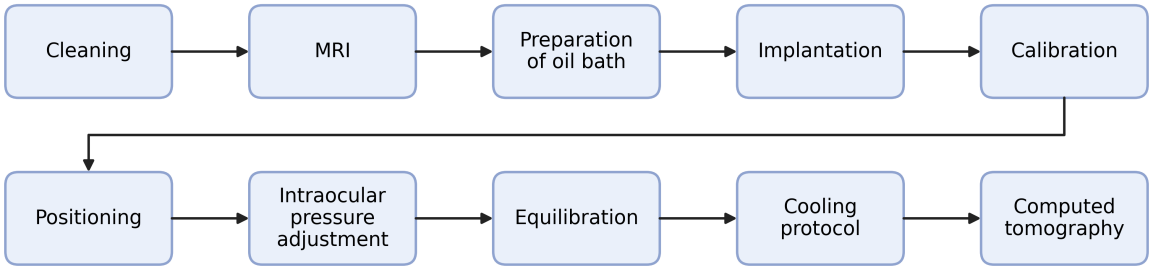


FIGURE 2.5. Experimental protocol for the porcine eyes, from receipt of the eye to post-experiment imaging. Human donor eyes followed the same sequence without MRI and equilibrated passively in the 37 °C bath.

recorded continuously for 20 min. Every trace reached 90% of its excursion before minute 16 (Section 2.2.6).

Human donor eyes were tested using the same 4.0 °C, 20-min cooling protocol, but were allowed to establish their baseline temperatures by passive equilibration in the 37 °C bath. Each human eye was cooled on the same day with three rings (the original ring reported here and the reduced-bulk and ceramic rings compared in Chapter 3) on a single set of implanted sensors and in an order that varied between eyes. Before each run the eye returned passively in the bath to about its resting state. The porcine and human experiments therefore began from different absolute baselines. For this reason, the cross-species comparison was based on the cooling excursion and not on the final absolute temperature.

2.2.6. Data Analysis. All recordings were processed with a single analysis pipeline, implemented as a Python package that runs the complete analysis from the raw files (Appendix C).

Denoising. After conversion to temperature (Section 2.2.4), each trace was median filtered (51 samples, 5.1 s) to suppress the spikes produced by the switching of the TEC relay, and then low-pass filtered with a zero-phase, second-order Butterworth filter (0.01 Hz cutoff). The median filter padded the trace with its edge values, and before the low-pass filter the start of each trace was extended by 300 s of point reflection about its first sample, so that neither step changes the initial temperature.

Quality control. Three objective gates and a final visual review determined which traces entered the analysis. The gates do not use the size of the cooling excursion, whereas the visual review, by its nature, was made with the traces in view. The gates are listed in order of precedence, and a trace failing more than one gate was excluded once, under the first reason in this order. First, sensor placement was verified from the co-registered micro-CT coordinates. Within each cohort, the positions of the four tissue-fixed sensors of every eye (anterior, equatorial, and posterior sclera, and proximal optic nerve) were rigidly aligned, by rotation and translation without scaling, to the mean set of the cohort (generalized Procrustes alignment, Equation C.1 in Section C.2). A tissue-fixed sensor was flagged when its aligned offset from the mean position of its site exceeded one thermistor length, 3.3 mm, the maximum length of the sensing tip given by the manufacturer. A shift of the whole eye in the scanner therefore does not flag a sensor, and the fence is the same for every cohort (Figure 2.6). The vitreous sensor is not fixed to tissue. It therefore did not enter the alignment and had no offset fence. It was flagged when its micro-CT position lay more than 75% of the distance from the center of the segmented globe to its wall in the same direction (Section C.4).

Second, channels whose raw voltage was flat-lined, or lay at the input limit of the acquisition unit ($\pm$10 V) for more than 75% of the record, indicating a disconnected or saturated sensor, were excluded. One further human trace was excluded as a hardware fault although it passed this test. The anterior-sclera sensor of eye 9 was failing. It had reached the input limit in the recording made before the run, and it gave no signal in the two later runs on that eye. Third, a single-exponential decay, used for this screen only, was fitted to each remaining filtered trace, and traces with $R^2 < 0.85$ were excluded as non-monotonic. The fit is made to the low-pass trace, and this gate therefore screens the shape of the cooling curve, not the fluctuation about it, which is reported as the plateau standard deviation (SD). Finally, every trace that passed the three gates was inspected against the other traces of its cohort and site.

Traces showing an obvious transient artifact, a sustained oscillation, or rewarming while the SCR held its set point were excluded. Three traces were removed in this way. Every trace excluded by the hardware gate, the fit gate, or visual review is shown in Figure 2.7. Because the visual review was not blind to the traces, every comparison was repeated with the visually excluded traces retained, as a sensitivity analysis.

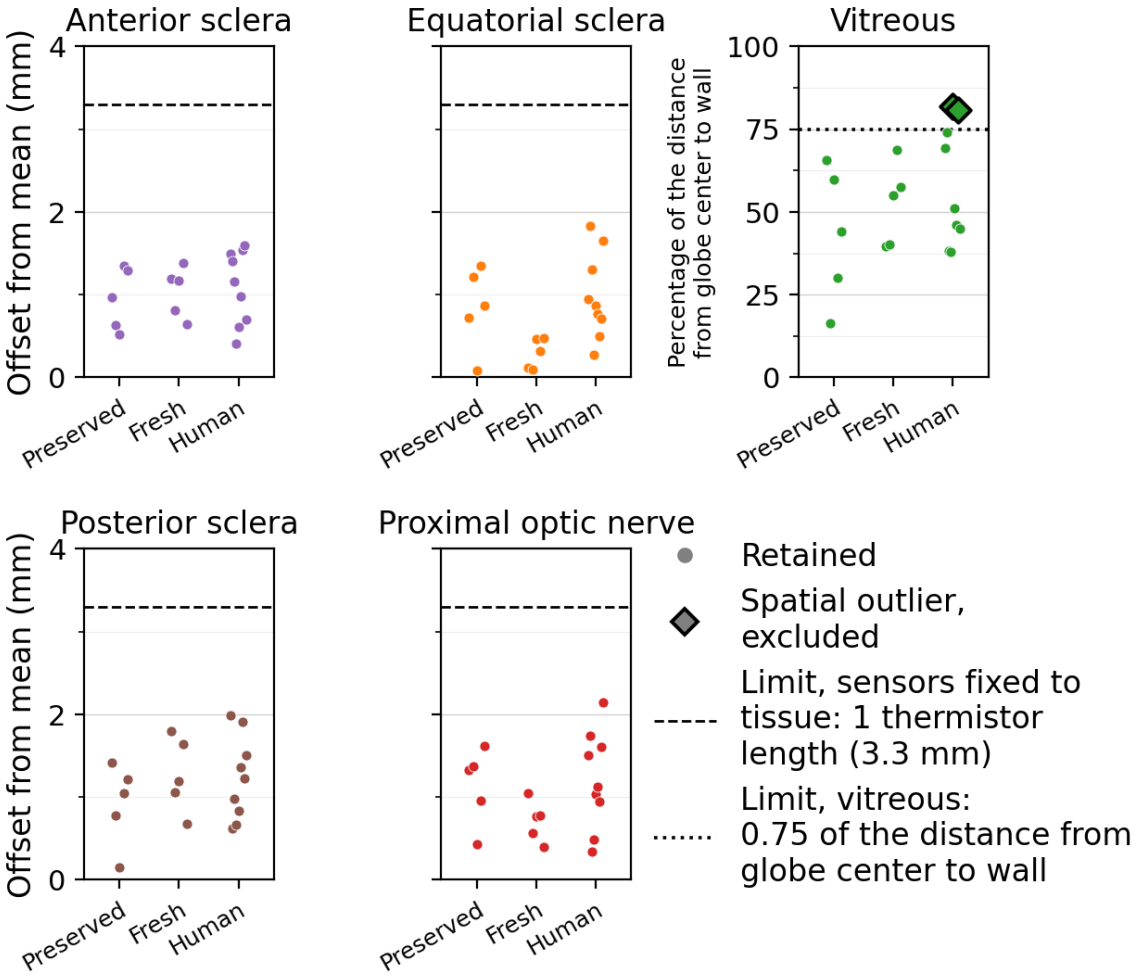


FIGURE 2.6. Spatial screen of sensor placement in preserved porcine, fresh porcine, and human eyes. Scleral and proximal optic nerve panels: offset $r_{e,s}$ of each sensor from the mean position of its site, after the four tissue-fixed sensors of each eye were rigidly aligned to the cohort mean (Equation C.1); dashed lines, the exclusion fence of one thermistor length (3.3 mm). Vitreous panel: position of each vitreous sensor on micro-CT as a percentage of the distance from the center of the globe to its wall in that direction; dotted line, the 75% limit; diamonds, the two excluded sensors (micro-CT sections in Figures C.2 and C.3).

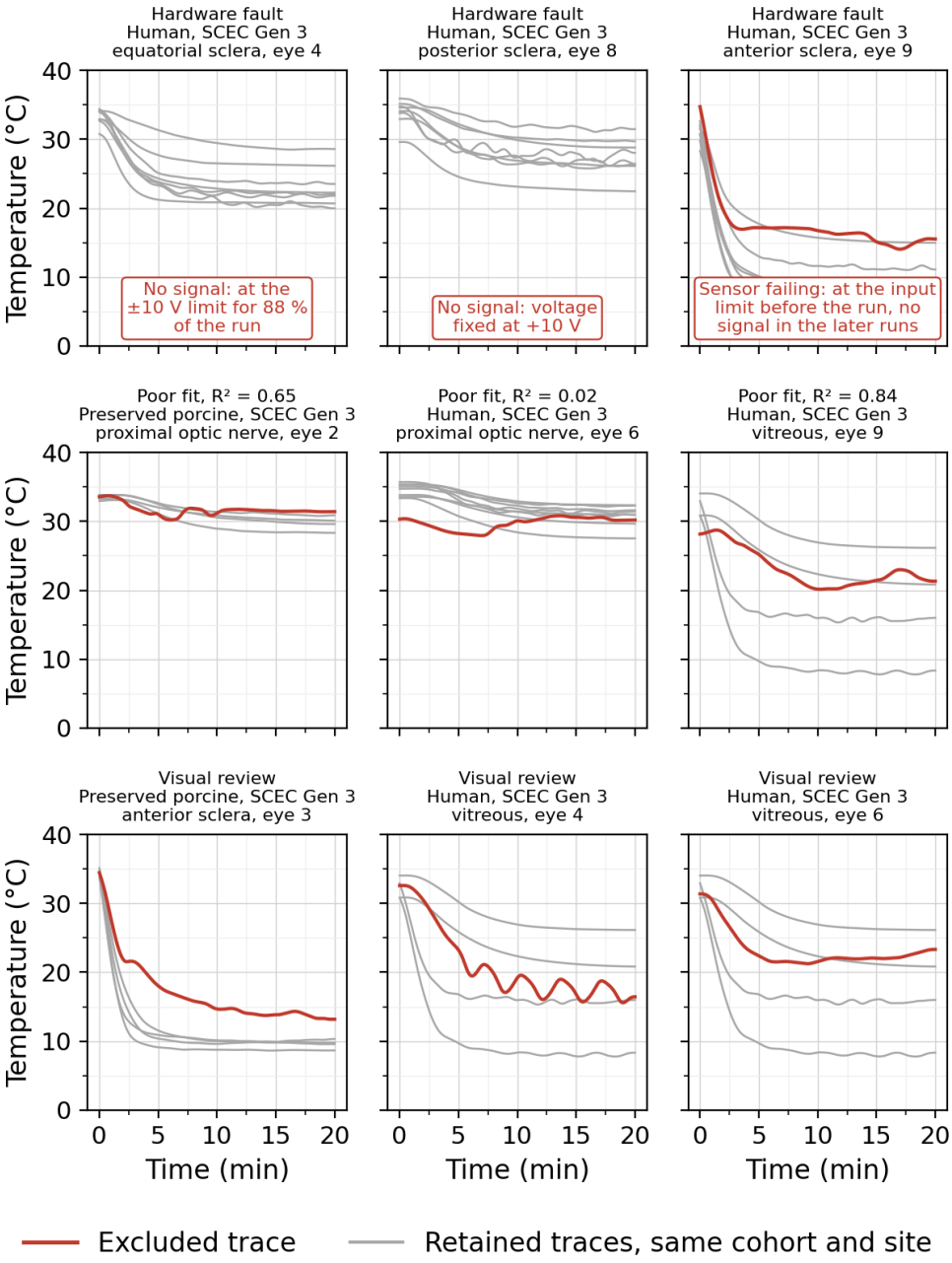


FIGURE 2.7. Traces excluded by the hardware gate, the fit gate, and on visual review (red), against the retained traces of the same cohort and site (gray). A channel with a hardware fault has no temperature signal, so its panel states the fault in the raw voltage; the failing sensor of human eye 9 had a signal in this run and is drawn with its note. R^2 is that of the single-exponential fit. The two sensors excluded by the spatial screen are in Figure 2.6.

Endpoints. For each included trace, the primary endpoint was the cooling excursion,

$$\Delta T = T_{\text{initial}} - T_{\text{final}}, \tag{2.1}$$

where T_{initial} is the temperature at the first sample of the record ($t = 0$), and T_{final} the mean temperature over the last 4 min of the 20-min record (minutes 16–20). In the porcine runs the device had already been running for a few seconds at $t = 0$; in the human runs the drive switched on 1 to 3 s after it. Cooling kinetics were summarized by t_{90}, the first time at which the trace had fallen by 90% of ΔT, and every trace had reached t_{90} before the final window began. The stability of the cooled plateau was summarized by the SD of the median-filtered trace over the same 4-min window. Final temperature was reported descriptively.

Group comparisons. Two comparisons addressed the aim: preserved against fresh porcine eyes, to assess preserved tissue as a benchtop surrogate, and fresh human against fresh porcine eyes, both cooled with SCEC Gen 3, to test porcine–human thermal concordance. Both used the same procedure. Per-site contrasts used two-sided Welch t-tests with Holm correction across the five tissue sites. The SCR, a device endpoint, was analyzed separately and kept outside the Holm family. Effect sizes were reported as small-sample-corrected Hedges' g (the difference over the pooled SD), as a point estimate; the uncertainty of each comparison is given by the confidence interval of the difference in °C. Tests that treat the two eyes of a subject as independent can inflate the type I error rate, whereas a test on the per-subject average holds it at the nominal level [**126**]. The nine human eyes came from five donors, and each test was repeated with fellow eyes averaged to one value per donor, to verify that treating fellow eyes as independent did not drive the result. Each porcine eye was treated as its own donor because the animal of origin was not recorded; the five fresh eyes came from one harvest and may therefore include fellow eyes. Because some sites retained only a few traces, failure to detect a difference was not interpreted as evidence that there is none.

Largest difference compatible with the data. Each comparison is also reported with the 90% confidence interval of the difference in ΔT. A difference beyond the interval limit farther from zero is called unlikely, which corresponds to a one-sided test at the 5% level. That limit is judged against the excursion at the site and the 3 °C benchmark [**66**]. Differences between groups are judged by these intervals, not by the measurement uncertainty of a single excursion (Section 2.2.4). Statistical significance was assessed at $\alpha = 0.05$.

2.3. Results

In all three cohorts, all 17 included proximal optic nerve traces cooled, by between 2.3 and 5.9 °C (Section 2.3.4).

Of the 114 channel traces recorded, 103 passed quality control (Table 2.1). Three cohorts were cooled with SCEC Gen 3 under the original ring: five preserved porcine eyes, five fresh porcine eyes, and nine fresh human eyes. No scleral or proximal optic nerve sensor, in any cohort, lay beyond the spatial fence. The largest offset was 2.1 mm. Two vitreous sensors, both in human eyes, lay more than 75% of the distance from the center of the globe to its wall (82 and 81%) and were excluded by the spatial screen. Three further human channels had a hardware fault, three traces failed the fit gate, and three were removed on visual review. The vitreous, whose sensor is the least mechanically constrained, accounted for five of the eleven exclusions.

Table 2.1. Quality-control outcome by cohort (gates defined in Section 2.2.6); each eye contributed six traces. A trace that failed more than one gate is counted once, under the first reason listed (the reasons are in order of precedence).

Cohort	Traces	Spatial	Hardware	Fit	Visual	Included
Preserved porcine (5 eyes)	30	0	0	1	1	28
Fresh porcine (5 eyes)	30	0	0	0	0	30
Fresh human (9 eyes)	54	2	3	2	2	45
Total	114	2	3	3	3	103

2.3.1. Preserved Porcine Eyes. In the formalin-preserved porcine eyes the cooling excursion fell from the anterior sclera to the proximal optic nerve, which cooled least and slowest (Figure 2.8). All five preserved eyes were analyzed, and 28 of their 30 traces passed quality control. No sensor lay beyond the spatial fence. One proximal optic nerve trace failed the fit gate, and one anterior-sclera trace was removed on visual review. The SCR held the commanded 4 °C set point, with a mean final temperature of 4.1 °C (Table 2.2).

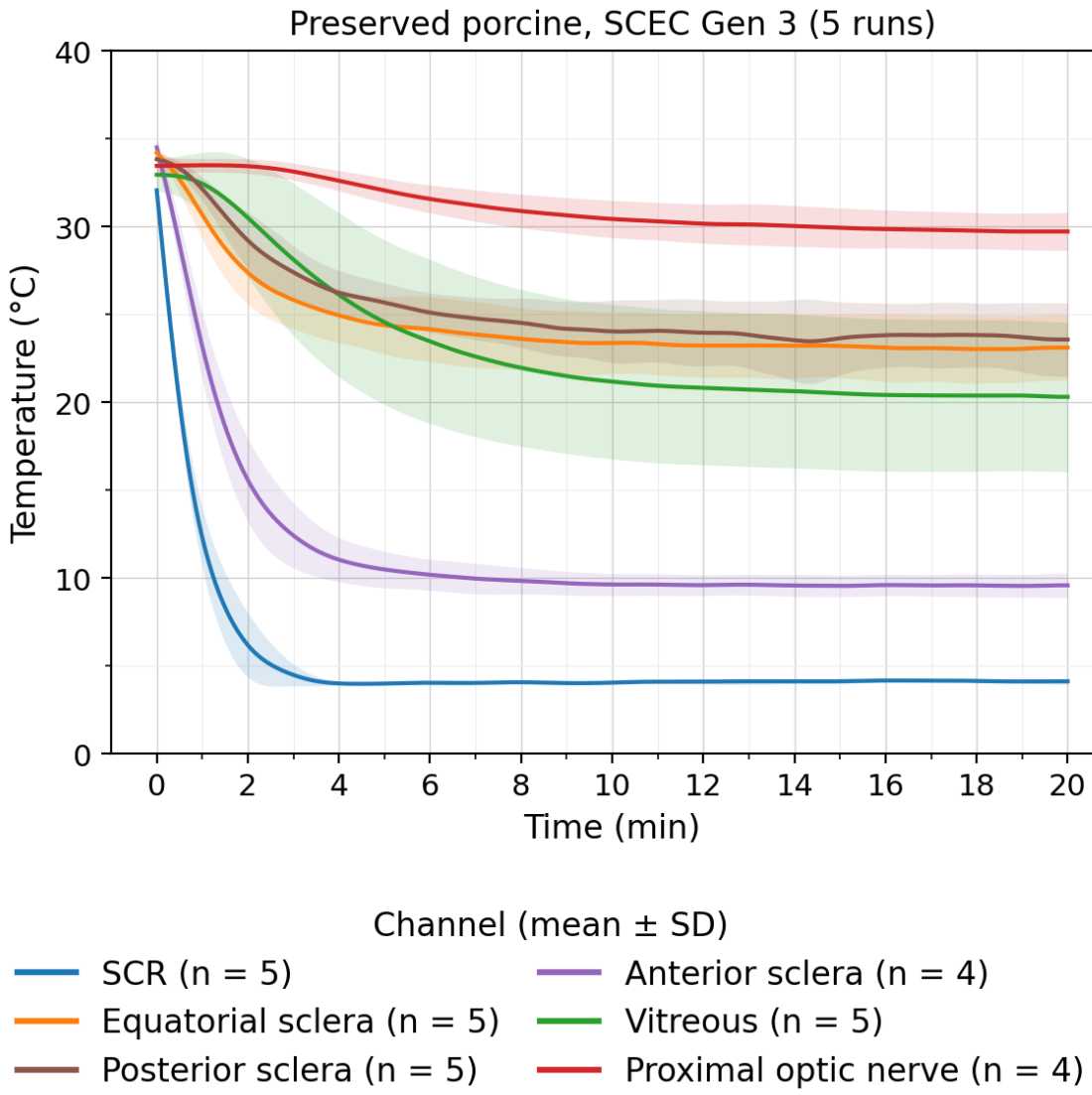


FIGURE 2.8. Temperature at each site during SCEC Gen 3 cooling of five preserved porcine eyes: cohort means and ±1 SD (bands) of the included traces; the legend gives the number of traces per site.

Cooling varied by site (Table 2.2): of the tissue sites, the anterior sclera showed the largest excursion, 24.9 °C, and the proximal optic nerve the smallest and slowest, 3.7 ± 0.8 °C (mean ± SD) with t_{90} of 783 s (13.1 min). Over the final 4 min the deeper sites had approached but not reached a steady state. The largest dispersion between specimens was in the vitreous (final temperature SD 4.3 °C).

TABLE 2.2. Thermal response of preserved ex-vivo porcine eyes during SCEC Gen 3 cooling: mean (SD) over the included traces (n). Initial temperature, final temperature, and ΔT were averaged separately, so ΔT need not equal the difference of the rounded means. Plateau SD is the within-trace SD over the final 4 min.

Location	n	**Initial**	**Final**	ΔT	t_{90}	**Plateau SD**
Units	–	(°C)	(°C)	(°C)	(s)	(°C)
SCR	5	32.1 (0.5)	4.1 (0.1)	27.9 (0.6)	111 (26)	0.06
Anterior sclera	4	34.5 (0.6)	9.6 (0.6)	24.9 (0.9)	188 (46)	0.06
Equatorial sclera	5	34.2 (0.3)	23.1 (1.9)	11.1 (2.0)	324 (123)	0.07
Vitreous	5	33.0 (0.9)	20.4 (4.3)	12.6 (3.4)	515 (72)	0.06
Posterior sclera	5	33.8 (0.4)	23.8 (1.9)	10.1 (1.8)	417 (83)	0.16
Proximal optic nerve	4	33.5 (0.4)	29.8 (1.0)	3.7 (0.8)	783 (95)	0.06

2.3.2. Fresh Porcine Eyes: Proof of Concept. SCEC Gen 3 produced a rapid and spatially ordered decrease in temperature throughout the isolated porcine eye (Figure 2.9). The SCR reached 90% of its excursion in 95 s and held a mean final temperature of 4.1 °C.

TABLE 2.3. Thermal response of fresh ex-vivo porcine eyes during SCEC Gen 3 cooling, as in Table 2.2.

Location	n	**Initial**	**Final**	ΔT	t_{90}	**Plateau SD**
Units	–	(°C)	(°C)	(°C)	(s)	(°C)
SCR	5	32.1 (0.4)	4.1 (0.1)	27.9 (0.4)	95 (12)	0.03
Anterior sclera	5	35.1 (0.5)	10.8 (3.8)	24.3 (3.9)	200 (67)	0.06
Equatorial sclera	5	34.5 (0.2)	25.1 (2.9)	9.4 (3.1)	340 (92)	0.05
Vitreous	5	34.0 (0.3)	22.4 (3.6)	11.6 (3.8)	566 (63)	0.04
Posterior sclera	5	34.3 (0.3)	24.5 (2.1)	9.8 (2.1)	519 (64)	0.08
Proximal optic nerve	5	33.8 (0.3)	30.3 (1.5)	3.5 (1.5)	676 (110)	0.06

Of the tissue sites, the anterior sclera again cooled most, by 24.3 °C, and the proximal optic nerve least, by 3.5 ± 1.5 °C, above the 3 °C benchmark, with t_{90} of

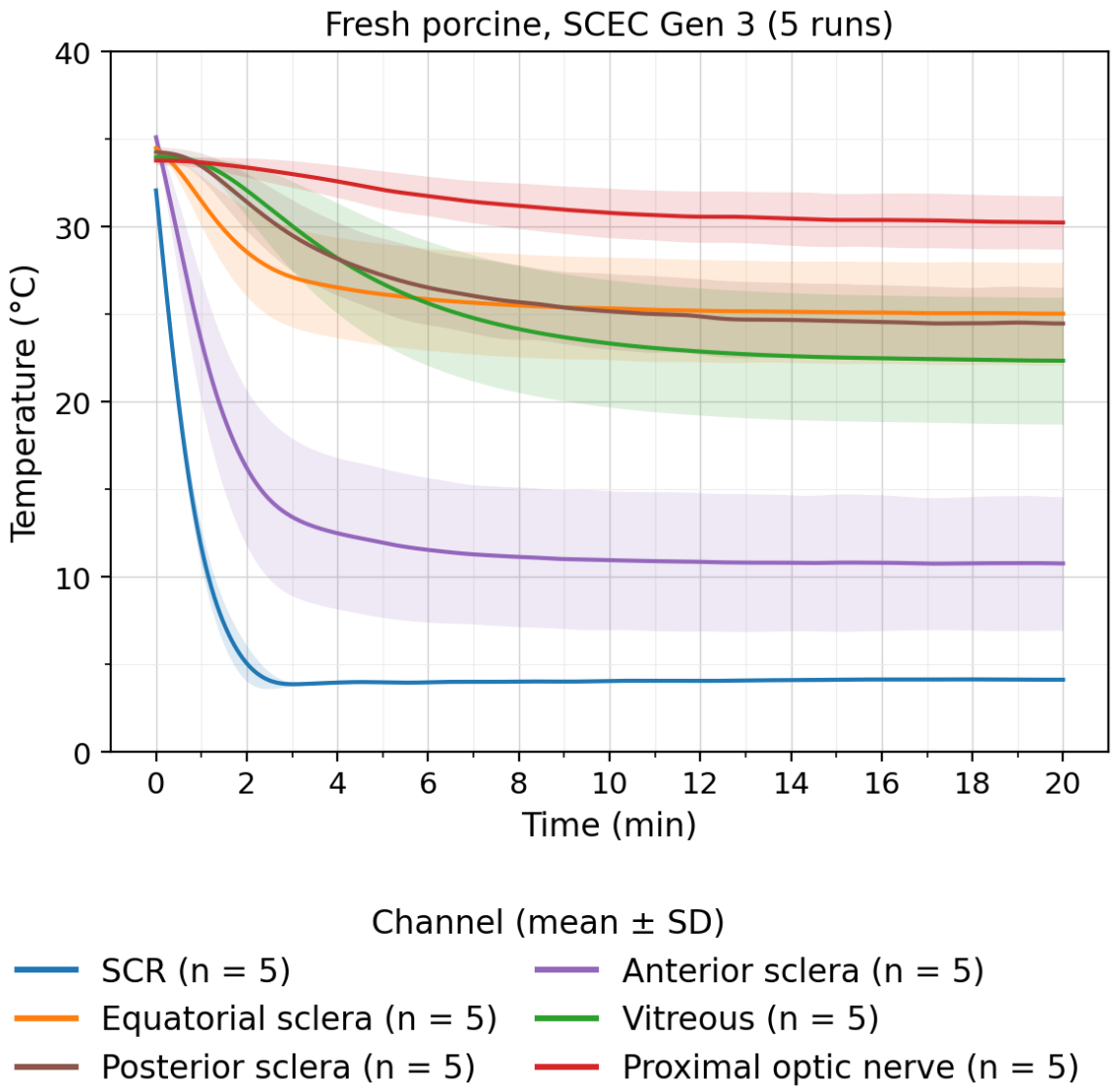


FIGURE 2.9. Temperature at each site during SCEC Gen 3 cooling of five fresh porcine eyes, as in Figure 2.8.

11.3 min (Table 2.3). Its final temperature lay below the 32–35 °C interval of systemic hypothermia [**56**], within which the baseline already fell, and inside the roughly 30–34 °C range of preclinical retinal protection (Section 1.3.3). All 30 traces passed quality control. Relative to its small excursion the proximal optic nerve varied most (coefficient of variation 43%).

2.3.3. Fresh Human Donor Eyes. The same device was then evaluated in fresh human donor eyes (Figure 2.10). Nine eyes from five donors were tested; 45 of 54 traces passed quality control, leaving four to nine traces per site (Table 2.4).

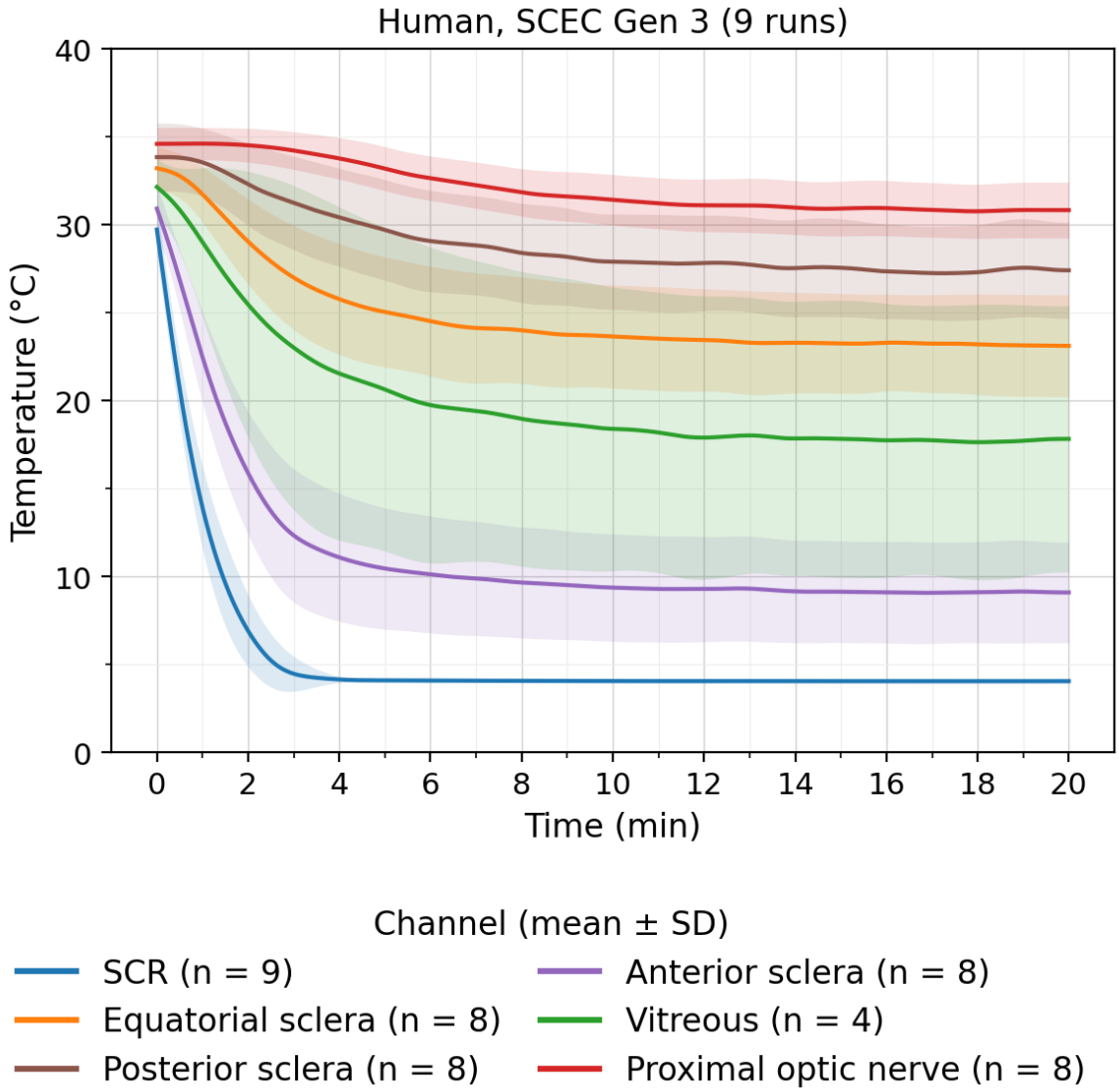


FIGURE 2.10. Temperature at each site during SCEC Gen 3 cooling of nine fresh human donor eyes, as in Figure 2.8.

The spatial pattern seen in porcine eyes was reproduced (Table 2.4). Of the tissue sites, the anterior sclera cooled most, by 21.8 °C, and the proximal optic nerve least, by 3.8 ± 1.1 °C, to 30.8 °C, with t_{90} of 11.1 min. Plateaus were stable (mean within-plateau SD at most 0.24 °C). Human starting temperatures differed from the porcine ones because the cohorts were equilibrated differently (Section 2.2.5). Before

cooling, the human eyes showed a gradient of about 4 °C from the anterior sclera to the proximal optic nerve, similar to that measured in patients [**46**].

TABLE 2.4. Thermal response of fresh human donor eyes during SCEC Gen 3 cooling, as in Table 2.2.

Location	n	**Initial**	**Final**	ΔT	t_{90}	**Plateau SD**
Units	–	(°C)	(°C)	(°C)	(s)	(°C)
SCR	9	29.7 (1.4)	4.1 (0.1)	25.7 (1.4)	124 (27)	0.07
Anterior sclera	8	30.9 (1.4)	9.1 (2.9)	21.8 (2.8)	233 (82)	0.10
Equatorial sclera	8	33.2 (1.2)	23.2 (2.8)	10.0 (2.6)	420 (131)	0.14
Vitreous	4	32.2 (1.6)	17.7 (7.7)	14.4 (6.8)	437 (233)	0.15
Posterior sclera	8	33.9 (1.9)	27.4 (2.7)	6.5 (1.3)	531 (122)	0.24
Proximal optic nerve	8	34.6 (0.9)	30.8 (1.6)	3.8 (1.1)	667 (79)	0.09

2.3.4. Reproducibility of Cooling at the Proximal Optic Nerve. Cooling at the posterior target was present in every eye with a valid proximal optic nerve trace (17 of 19 eyes; the sensor failed quality control in one preserved porcine and one human eye) but varied in magnitude between eyes (Table 2.5). All 17 included proximal optic nerve traces cooled, by between 2.3 and 5.9 °C. All 17 reached 90% of their excursion within 15 min, the slowest at 14.3 min. Eleven of the 17 traces reached the 3 °C benchmark, and a twelfth fell short of it by 0.001 °C. The between-eye coefficient of variation of the proximal optic nerve excursion was 22–43%, against 1–6% at the SCR, whose temperature was controlled. The boundary was thus reproduced closely from eye to eye, and the variation at the posterior target arose within the tissue.

TABLE 2.5. Reproducibility of cooling at the proximal optic nerve across eyes. Counts are of included proximal optic nerve traces. CV is the between-eye coefficient of variation of ΔT; [a]at the proximal optic nerve; [b]one further trace at 2.999 °C.

Cohort	n	**Traces reaching**		ΔT **range**	**CV of** ΔT **(%)**	
	(traces)	$\Delta T \geq 3$ **°C**	$t_{90} \leq 15$ **min**	**(°C)**	**Nerve**[a]	**SCR**
Preserved porcine	4	3	4	2.95–4.56	22	2
Fresh porcine	5	3	5	2.26–5.92	43	1
Fresh human	8	5[b]	8	2.51–5.79	29	6
Total	17	11[b]	17	2.26–5.92		

2.3.5. Fresh Versus Preserved Porcine Eyes. In preserved eyes, cooling closely followed that of fresh eyes (Figure 2.11). The SCR was held at 4.1 °C in both preparations, reaching 90% of its excursion in 111 s in preserved eyes and 95 s in fresh eyes. The spatial pattern of cooling survived chemical fixation. In both preparations the anterior sclera cooled most and fastest, and the proximal optic nerve least and slowest, with t_{90} of 783 s (13.1 min) in preserved and 676 s (11.3 min) in fresh eyes.

No difference in cooling excursion was detected between preserved and fresh tissue at any of the five tissue sites (Table 2.6; all Holm-adjusted $p = 1.00$). The largest nominal difference was at the equatorial sclera, where preserved eyes cooled 1.7 °C more. At the proximal optic nerve the difference was −0.2 °C (fresh minus preserved). The 90% confidence intervals (CI) of the difference at the SCR and the proximal optic nerve (Table 2.6) make a difference larger than 0.6 °C at the SCR, or 1.7 °C at the proximal optic nerve, unlikely. At the remaining sites the intervals were wider and allowed differences of 2.6 to 5.2 °C. Retaining the visually excluded anterior-sclera trace changed the anterior difference from −0.6 to +0.2 °C and left every other value, and every conclusion, unchanged.

TABLE 2.6. Cooling excursion ΔT in fresh and preserved porcine eyes, in °C: mean (SD), with the number of included traces in brackets. Difference, fresh minus preserved, with its 90% confidence interval (Welch), whose limit farther from zero is the largest difference compatible with the data (one-sided, 5% level). p_{Holm} is corrected across the five tissue sites; the SCR is tested separately, and its p (*) is unadjusted. g, Hedges' g (unitless). Differences are computed from unrounded means. Anterior, equatorial, and posterior are the scleral sites.

Location	**Fresh**	**Preserved**	**Diff.**	**90% CI**	p_{Holm}	g
SCR	27.9 (0.4) [5]	27.9 (0.6) [5]	0.0	−0.6, 0.6	0.96*	0.03
Anterior	24.3 (3.9) [5]	24.9 (0.9) [4]	−0.6	−4.4, 3.2	1.00	−0.18
Equatorial	9.4 (3.1) [5]	11.1 (2.0) [5]	−1.7	−4.8, 1.5	1.00	−0.58
Vitreous	11.6 (3.8) [5]	12.6 (3.4) [5]	−1.0	−5.2, 3.3	1.00	−0.25
Posterior	9.8 (2.1) [5]	10.1 (1.8) [5]	−0.3	−2.6, 2.0	1.00	−0.14
Proximal optic nerve	3.5 (1.5) [5]	3.7 (0.8) [4]	−0.2	−1.7, 1.3	1.00	−0.15

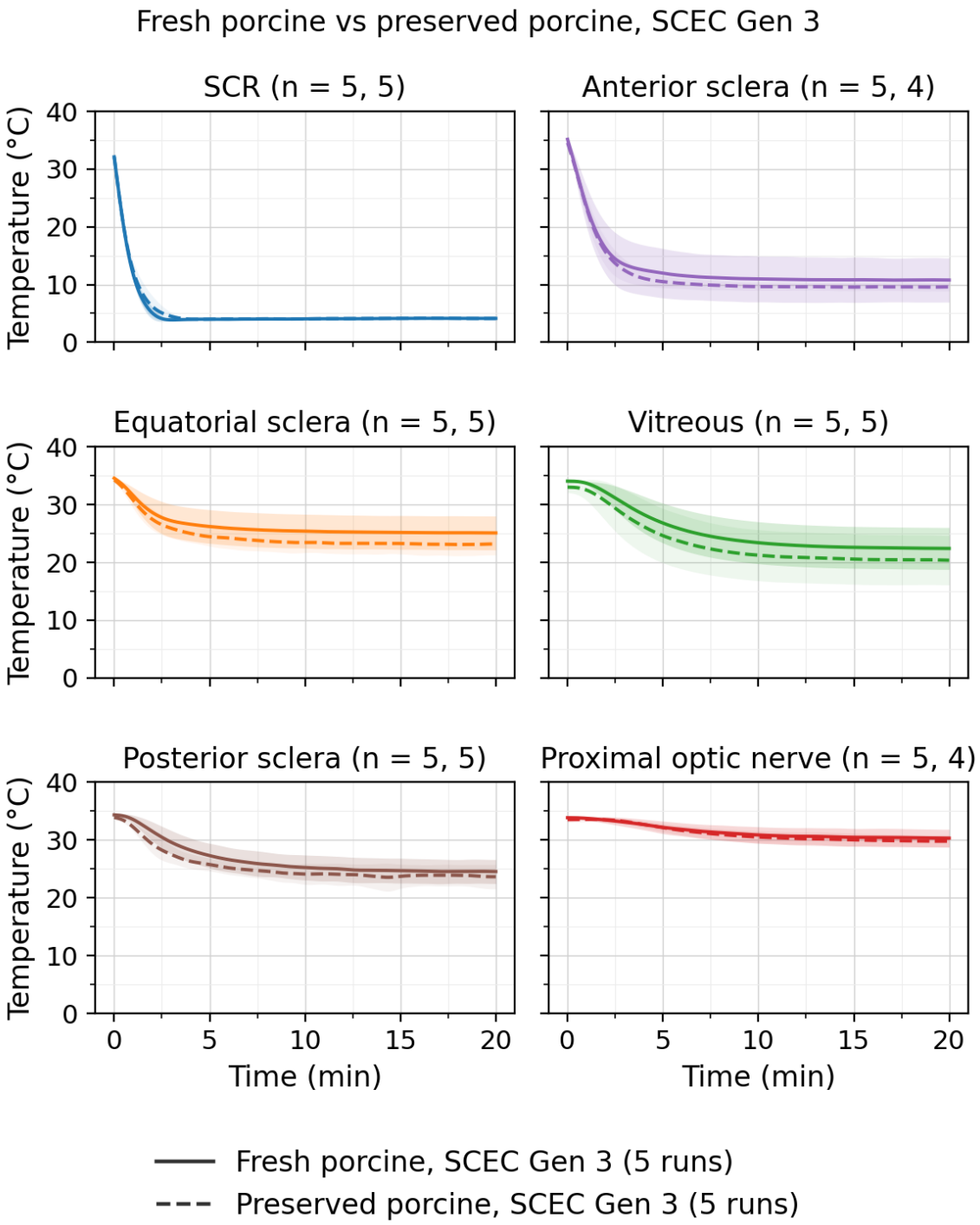


FIGURE 2.11. Temperature at each site during SCEC Gen 3 cooling of fresh (solid) and preserved (dashed) porcine eyes: cohort means and ±1 SD (bands) of the included traces; panel titles give the number of traces (fresh, preserved).

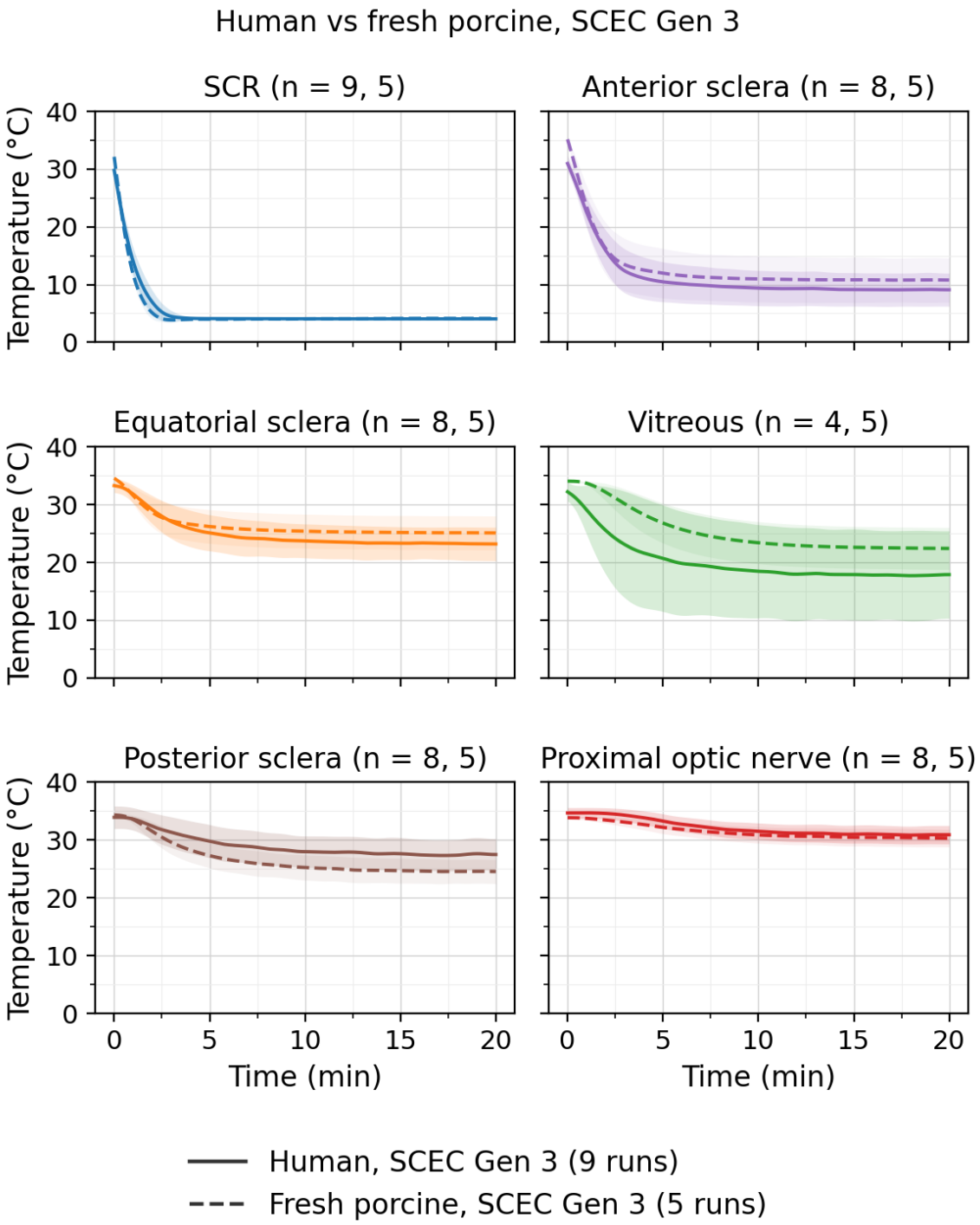


FIGURE 2.12. Temperature at each site during SCEC Gen 3 cooling of fresh human (solid) and fresh porcine (dashed) eyes, as in Figure 2.11; panel titles give the number of traces (human, porcine).

2.3.6. Human–Porcine Thermal Concordance. At the proximal optic nerve, the two species cooled by similar amounts (Figure 2.12, Table 2.7). Both were cooled

under the same ring. As a result, the species effect was not confounded by device geometry. The mean excursion was 3.8 ± 1.1 °C in human eyes and 3.5 ± 1.5 °C in fresh porcine eyes. The human-minus-porcine difference was 0.3 °C (Table 2.7). No difference was detected, and one larger than 1.8 °C lies outside the 90% confidence interval (Section 2.2.6). With fellow human eyes averaged to one value per donor, the difference was much the same, and the result did not depend on treating fellow eyes as independent. The upper limit rests on few porcine eyes: leaving out one porcine eye at a time gave upper limits from 1.74 to as much as 2.29 °C, whereas leaving out one human eye at a time kept it at or below 1.94 °C. Pooling the fresh and preserved porcine traces narrowed the interval to −0.76 to 1.16 °C. In every case the interval included zero.

TABLE 2.7. Cooling excursion ΔT in fresh human and fresh porcine eyes under SCEC Gen 3; difference, human minus porcine; otherwise as in Table 2.6.

Location	**Human**	**Porcine**	**Diff.**	**90% CI**	p_{Holm}	g
SCR	25.7 (1.4) [9]	27.9 (0.4) [5]	−2.3	−3.2, −1.3	0.001*	−1.79
Anterior	21.8 (2.8) [8]	24.3 (3.9) [5]	−2.5	−6.4, 1.4	1.00	−0.71
Equatorial	10.0 (2.6) [8]	9.4 (3.1) [5]	0.6	−2.6, 3.7	1.00	0.19
Vitreous	14.4 (6.8) [4]	11.6 (3.8) [5]	2.8	−5.1, 10.7	1.00	0.47
Posterior	6.5 (1.3) [8]	9.8 (2.1) [5]	−3.3	−5.3, −1.2	0.10	−1.88
Proximal optic nerve	3.8 (1.1) [8]	3.5 (1.5) [5]	0.3	−1.2, 1.8	1.00	0.22

Of the five implanted tissue sites, only at the proximal optic nerve was the species difference bounded below 2 °C. The largest site-specific contrast was at the posterior sclera, where porcine tissue cooled 3.3 °C more than human tissue. Its 90% confidence interval was the only one among the tissue sites to exclude zero (−5.3 to −1.2 °C; $g = -1.88$). The contrast did not remain significant after correction for multiple comparisons (nominal $p = 0.02$, Holm-adjusted $p = 0.10$). The differences at the equatorial sclera and in the vitreous were small against their wide intervals (Table 2.7). With the two visually excluded human vitreous traces retained, the per-site vitreous difference fell from 2.8 to 2.1 °C. No proximal optic nerve trace was excluded visually.

The proximal optic nerve results, including the confidence interval of the species difference, were therefore identical in both analyses. The SCR excursion was 2.3 °C smaller in human eyes ($p = 0.001$). Both cohorts ended at 4.1 °C, and the difference reflects the lower human starting temperature.

2.4. Discussion

Specific Aim 1 established that SCEC Gen 3 delivers controlled trans-scleral cooling to the deep posterior eye. The first hypothesis was supported in occurrence and timing, and in magnitude on average: every included proximal optic nerve trace cooled and all 17 reached 90% of the excursion within 15 min, but only 11 of the 17 reached the 3 °C benchmark, and a twelfth fell short of it by 0.001 °C (Section 2.3.4). The test of the second hypothesis found no detectable difference between the species at that site and bounded any difference at 1.8 °C. Preserved and fresh porcine eyes did not differ detectably at the SCR or the proximal optic nerve.

Magnitude of cooling at the posterior target. The mean proximal optic nerve excursion exceeded the 3 °C benchmark in every cohort. The benchmark is the smallest step tested in isolated retina subjected to simulated ischemia [**66**]. That step ran from 36 to 33 °C, whereas the proximal optic nerve started ex vivo at a lower temperature (Tables 2.3 and 2.4). A 3 °C decrement here is therefore comparable in size to that step but does not reproduce it. The final proximal optic nerve temperatures in fresh porcine and human eyes lie within the roughly 30–34 °C range over which preclinical retinal studies report protection (Section 1.3.3). Both are also close to the 30 °C at which retinal ganglion cells were protected in hypoxic organ culture [**68**]. These absolute values depend on each preparation's starting temperature and carry the absolute measurement uncertainty of Section 2.2.4, most of it a possible underreading from sensor self-heating.

The decrement reached in vivo is expected to be smaller than that reached ex vivo, because perfusion resupplies heat that the bath did not (see *Absence of perfusion*,

below). Delivery at this magnitude for 20 min from a boundary held at 4 °C, well above freezing, is the engineering premise on which the remaining aims depend.

The proximal optic nerve as a conservative index. Most of the retina was likely cooled further than the proximal optic nerve. The proximal optic nerve was chosen as the deep posterior target because it is the site farthest from the ring; its final temperature in fresh porcine and human eyes, 30.3 to 30.8 °C, lies at the lower end of the preclinical retinal range. It does not, however, represent the retina as a whole. The retina lines the globe from just behind the ring to the optic disc, and the cooling imposed at the ring reaches it along a gradient. The retina itself was not instrumented. It lies between the vitreous and the posterior sclera, which in every cohort ended far colder than the proximal optic nerve (Tables 2.2, 2.3, and 2.4), and the peripheral retina lies closer still to the ring. The computational model of Chapter 3 resolves this gradient along the retina.

Preserved tissue as a benchtop surrogate. Preserved and fresh porcine eyes showed no detectable difference at the two sites where the intervals were narrow. The SCR followed the same trajectory in both, and the proximal optic nerve excursions differed by 0.2 °C, with a difference larger than 1.7 °C unlikely. The proximal optic nerve responded more slowly in preserved eyes (Section 2.3.5). At the scleral and vitreous sites, with four or five traces per group, the confidence intervals were too wide to show either a difference or its absence. Preserved eyes are therefore suitable for what the platform is most often asked to do, comparing design variants under identical conditions, where a change in region-specific cooling is the quantity of interest and tissue availability is otherwise limiting. Absolute performance, and any conclusion about sites other than the posterior target, should rest on fresh tissue.

Porcine tissue as a surrogate for the human eye. The cross-species comparison is compatible with using the porcine eye, and a porcine in-vivo model, as a surrogate for the human eye at the posterior target, and only there. At the proximal optic nerve the species differed by 0.3 °C, and by much the same with fellow eyes averaged by

donor. The other tissue sites gave a mixed picture. Porcine eyes cooled 3.3 °C more at the posterior sclera, the largest site-specific contrast, although it did not survive correction for multiple comparisons. At the equatorial sclera and in the vitreous the intervals were too wide to interpret. The posterior-sclera contrast is not explained by the starting temperature, which was nearly the same in the two species at this site, and it is consistent with the anatomy. The porcine sclera is thinnest 5 to 6 mm behind the limbus, close to the ring, whereas the human sclera is thickest at the posterior pole (Table 1.3).

Differences in globe size, scleral thickness, and the depth of each site below the warm bath may contribute to these contrasts. Sensor placement also contributes. The vitreous sensor has no rigid landmark, cannot be seen during insertion, and showed the largest variance in position in both species, and in final temperature in the human eyes.

The state of the vitreous may also differ, as the human vitreous liquefies progressively with age [**127**], and the two cohorts differed in post-mortem age. Vitreous liquefaction was not assessed and the effect of post-mortem age was not tested; both remain possible explanations.

Comparability of the cohorts. The porcine and human experiments differed in three ways that bear on the comparison at the proximal optic nerve, and two of them would both tend to enlarge the human excursion relative to the porcine one, in the direction of the observed difference. First, the control method was manual in the porcine and automatic in the human experiments. The SCR, however, reached the same 4.1 °C plateau in every cohort, and the imposed boundary was therefore comparable. Second, the starting temperature differed. After equilibration with the unpowered ring resting on the eye, the human SCR and anterior sclera began 2.3 and 4.2 °C cooler, respectively, than in the fresh porcine eyes, and the human proximal optic nerve about 0.8 °C warmer. An excursion toward a fixed boundary grows with the distance from that boundary. Third, the proximal optic nerve sensor was oriented

differently, along the nerve in the porcine eyes and parallel to the posterior sclera at the junction of the nerve with the globe in the human eyes. The human sensor thus sampled tissue closer to the cooled globe. The size of these second and third effects could not be estimated from these data.

Absence of perfusion. The ex-vivo measurements likely overestimate the cooling the device produces at depth in a perfused eye. All experiments were performed in non-perfused tissue. The continuously mixed 37 °C bath restored, in part, the warm boundary that surrounds the globe in vivo, but it did not reproduce the choroidal circulation, which delivers heat directly beneath the retina. The perfused model of Chapter 3 cooled the deep sites less than the human eyes did ex vivo (Section 3.3.6). Blood could in principle carry cooling from the ring toward the vortex veins [**1**], but the model indicates that perfusion opposes the cooling at depth. The main indication for the device is ischemia, in which ocular blood flow is by definition reduced. A non-perfused preparation therefore corresponds to part of the clinical scenario. The present measurements are a tissue-level thermal baseline and do not predict in-vivo temperatures.

Measurement limitations. Thermistor implantation was necessarily invasive, and sensor position varied between eyes, most of all in the vitreous. Micro-CT localization and the spatial screen removed sensors that lay far from the intended position of their site but could not remove smaller placement differences. Small air bubbles introduced during implantation could insulate a sensor locally, and variation in the contact pressure of the SCR could change the thermal resistance of the ring–sclera interface. Both would add to the variation between eyes. Sample sizes were modest, particularly for the vitreous after quality-control exclusions, which limited the power of every site-level comparison. The species comparison at the proximal optic nerve is limited in the same way. Its bound of 1.8 °C is about half of the excursion itself, of about 3.5–3.8 °C, and rests on five porcine eyes; with either of two of them left out, the upper limit rose to 2.1 or 2.3 °C. The cohorts also differed in starting temperature

and sensor orientation. The data therefore make a difference of more than about 2 °C between the species at this site unlikely; they do not show that the two cool alike. The human cohort was demographically narrow, five male White donors aged 43–77 years, and cannot show whether the ocular thermal response varies with sex, age, ancestry, or other donor characteristics.

Scope. Aim 1 evaluated thermal delivery only. An ex-vivo preparation cannot show whether the local exposure, about 10 °C at the anterior sclera, is tolerated by living tissue, or whether cooling preserves retinal function during an ischemic insult. The first is assessed in Aim 3 and the second remains future work. The remaining limitations of SCEC Gen 3 (its bulk, heat-rejection hardware, control architecture, and the temperatures it imposes at the tissue interface) motivate Aim 2.

CHAPTER 3

Specific Aim 2: Enhance the Device (SCEC Gen 4) for Efficiency, Portability, Safety, and Patient Comfort

3.1. Background and Rationale

The design choices of SCEC Gen 3 were acceptable for establishing feasibility but limit any use beyond it. Specific Aim 1 showed that SCEC Gen 3 can lower the temperature of the deep posterior eye by holding a ring on the anterior sclera at 4 °C (Chapter 2). Its use outside a laboratory remained to be shown. The ring was machined from bulk aluminum, 2336 mm^3 of metal resting on the conjunctiva and under the eyelids, and it placed an electrical conductor in direct contact with the eye. Heat was carried away by a heat exchanger of similar volume and an ice-bath circuit, and the controller was assembled from separate boards.

The bulk of the ring also bears on where its cooling goes. A ring that fills the fornices contacts the eyelids over a larger area than the sclera. In the computational model of Gen 3 developed in this chapter, about 66% of the heat extracted over 35 min of modeled cooling came from the eyelids, about 2% from the air, and only 33% from the eye. Heat drawn from the eyelids does not cool the target tissue, yet the module, the heat-rejection hardware, and the power supply are all sized by the total heat they must move.

Efficiency, the first of the four enhancement targets, is the share of the extracted heat drawn from the eye. Portability is the size of the ring, the heat-rejection hardware, and the control electronics. Safety covers the electrical isolation of the tissue from the powered module, surface temperatures below 43 °C (the limit for applied parts that a clinical device will have to meet [**128**]), and the behavior of the controller

when a sensor or the module fails. Comfort is judged only by the dimensions and volume of the parts that contact the eye and eyelids.

No change may cost the cooling that Aim 1 established. Tissue is limited, each eye carries a single set of implanted sensors, and a new ring takes weeks to fabricate. For these reasons the refinements were explored first in a computational model, which holds every other condition fixed and evaluates each variant over a range of operating conditions. The designs that the model favored and that could be built were then tested in human donor eyes on the ex-vivo platform of Aim 1, paired within the same eyes. The control electronics were consolidated into a single board and verified on the bench.

The hypothesis is stated in Section 1.8. Whether cooling is preserved is judged at the proximal optic nerve, the one site at which the hypothesis is tested, from the difference from Gen 3 and the largest difference that its confidence interval allows, set against the excursion itself. Changes in efficiency and in the other targets are reported as measured differences from Gen 3.

3.2. Methods

3.2.1. Design Requirements and Refinements. Each refinement addresses one or more of the four targets and is judged against the Gen 3 device of Chapter 2. Table 3.1 records how far each was taken and where its evidence is reported.

Ring material. Aluminum conducts heat well but is an electrical conductor in contact with the eye. Candidate replacements were screened for thermal conductivity, electrical insulation, optical transparency, hardness, and use in medical devices, and two were carried forward. Aluminum oxide is a ceramic that combines moderate thermal conductivity with electrical insulation and can be made translucent or transparent, depending on how it is processed [**129, 130**]. Fused silica glass is transparent and insulating but conducts heat poorly. Fused silica was also retained because of its manufacturing route: parts of fused silica glass can be molded or printed from a silica

TABLE 3.1. Refinements of SCEC Gen 4, the target of each, and how far each was taken; modeled refinements were evaluated only with the computational model of Section 3.2.2.

Refinement	**Target**	**Status**	**Evidence**
Ceramic and glass rings	Safety	Ceramic ring built and tested; glass modeled	Model (three materials); paired tests in human donor eyes (aluminum against ceramic)
Reduced-bulk ring (v2)	Portability, comfort	Built and tested	Model; paired tests in human donor eyes
Integral corneal lens (v3)	Efficiency, safety, comfort	Modeled	Model
Insulating outer layer	Efficiency	Modeled	Model (0.5 and 1.0 mm silicone)
Smaller heat exchanger	Portability	Modeled	Water-block model (three designs)
Integrated control board	Portability, safety	Built and bench-tested	Bench verification
Potting of the TEC	Safety	Built	Silicone injected with a curved-tip syringe to fill the annular module between its two ceramic plates and waterproof it, in every device used in this work
Coolant circuit	Portability	Built	Replacement pump and push-to-connect tubing fittings
Higher post density and printing on the ring	Efficiency, portability	Concept only	None; future work
Contact-pressure sensor, ERG electrode, injection port	Safety	Concept only	None; future work
Patient comfort	Comfort	Not measured; implied	Dimensions and volume only

composite and then sintered [**131, 132**], a process now offered commercially (Glassomer GmbH, Freiburg, Germany), which would combine transparency with ease of manufacture.

Ring geometry. The Gen 3 ring (v1) is a bulk aluminum part that conforms to the anterior sclera and leaves the cornea exposed, with a volume of 2336 mm^3 and a scleral contact area of approximately 390 mm^2. The reduced-bulk ring (v2) keeps a similar contact area, 393 mm^2, with a thinner profile that nearly halves the volume, to 1191 mm^3. The third design (v3) adds an integral lens of 105 mm^3 over the cornea to the v2 profile (1296 mm^3, 45% less than v1). The lens extends the contact area to 619 mm^2, protects the cornea, and is intended to limit corneal drying.

Outer layer, heat rejection, and control. A silicone layer, 0.5 or 1.0 mm thick, on the outer walls of the ring was evaluated as a way to direct the extracted heat toward the eye and away from the eyelids. Three heat exchangers of decreasing volume were evaluated (Section 3.2.2.3), cooled with distilled water (Appendix E). The controller must hold the ring at its set point, nominally 4 °C, and stop the module when a sensor or the module fails. A clinical device will also have to keep every patient-contacting surface, in practice the hot side of the module and the heat exchanger, below 43 °C. Here that limit was used only in the heat-exchanger model, as the safety limit for the hot side.

3.2.2. Computational Model. Two finite-element models were first built and solved in COMSOL Multiphysics 6.4 (COMSOL AB, Stockholm, Sweden): one of the eye, orbit, and eyelids with the ring in place, to compare ring materials, geometries, and the insulating layer, and one of the heat exchanger on the hot side of the module. A Python program then replaced COMSOL, and the results of this chapter are those of the Python runs. The analysis is described in Section C.6.

3.2.2.1. *From COMSOL to Python.* Both COMSOL models were exploratory, built to compare design variants under identical conditions rather than to predict the temperatures of a particular eye. The eye model coupled conduction to flow in

the humors and the air in front of the eye, represented the module as a layer on the ring, and began cooling with the ring held at ambient temperature. Its results were computed on the finest of three meshes per design, about one million elements (Table 3.2 and Figure D.3; settings in Section C.6).

Errors in how the boundary conditions were applied were then found, among them the starting state of the ring and the emissivity of the ceramic and glass rings. By then the COMSOL license had ended, so the models could not be corrected in COMSOL. A finite-element program was therefore written in Python with Claude Code (Appendix K; folder `code/ch3-heat-replica` of the code repository, Section C.7), reading the meshes, materials, and boundary conditions from the COMSOL model files. In the eye model it solves heat transfer only, with the humors and the air as stagnant conductors; in the heat exchangers it also solves the coolant flow. It was validated against the stored COMSOL solutions, with the same setup (Section 3.3.1).

With the errors fixed, the eye now starts at rest, with the device in place and switched off, so that the anterior sclera begins near 36 °C instead of 27 °C. The air domain and the module layer are removed, each ring takes the emissivity of its material, and the tissues take the property values of Appendix D. The heat is computed from the solved temperature field, across each surface of the ring and from the sources of the model. The same 27 operating points, three materials, two insulating layers on each ring, and rewarming cases were solved in this set-up. Runs beyond the COMSOL cases are called extension runs. For the heat exchanger, the Python runs give each design the same heat.

3.2.2.2. *Scleral Contact Ring and Eye.*

Geometry. The model represents a human eye in the orbit, with the ring on the anterior sclera and the eyelids closed over its outer surface (Figure 3.1). The globe is divided into cornea, tear film, aqueous humor, lens, ciliary body, vitreous humor with Cloquet's canal, retina, choroid, and sclera, and is surrounded by orbital fat, the extraocular muscles, and the eyelids. The eye has the dimensions of an adult

TABLE 3.2. Meshes on which the results were computed: the finest mesh of the eye model for each ring, without the air and the module layer, and the intermediate mesh of each heat exchanger. Element quality is one minus the equiangular skewness of the dihedral angles of each tetrahedron (1 for a regular tetrahedron, 0 for a degenerate one).

	Eye model			Heat exchanger		
Statistic	**v1**	**v2**	**v3**	**v1**	**v2**	**v3**
Mesh vertices	165 472	163 912	171 043	35 977	54 051	35 449
Tetrahedra	946 374	940 447	979 821	207 341	309 912	192 068
Boundary triangles	123 762	121 606	130 148	20 643	34 437	27 349
Minimum element quality	0.100	0.030	0.008	0.066	0.159	0.030
Average element quality	0.669	0.669	0.666	0.675	0.669	0.673
Mesh volume (cm^3)	19.88	19.53	19.59	4.52	2.65	0.94

human globe (Appendix D), and the orbit is a cylinder of radius 17.5 mm. The three rings were placed on the same eye. The model is symmetric about the vertical plane through the optical axis and only half was solved; heat flows are reported for the whole device.

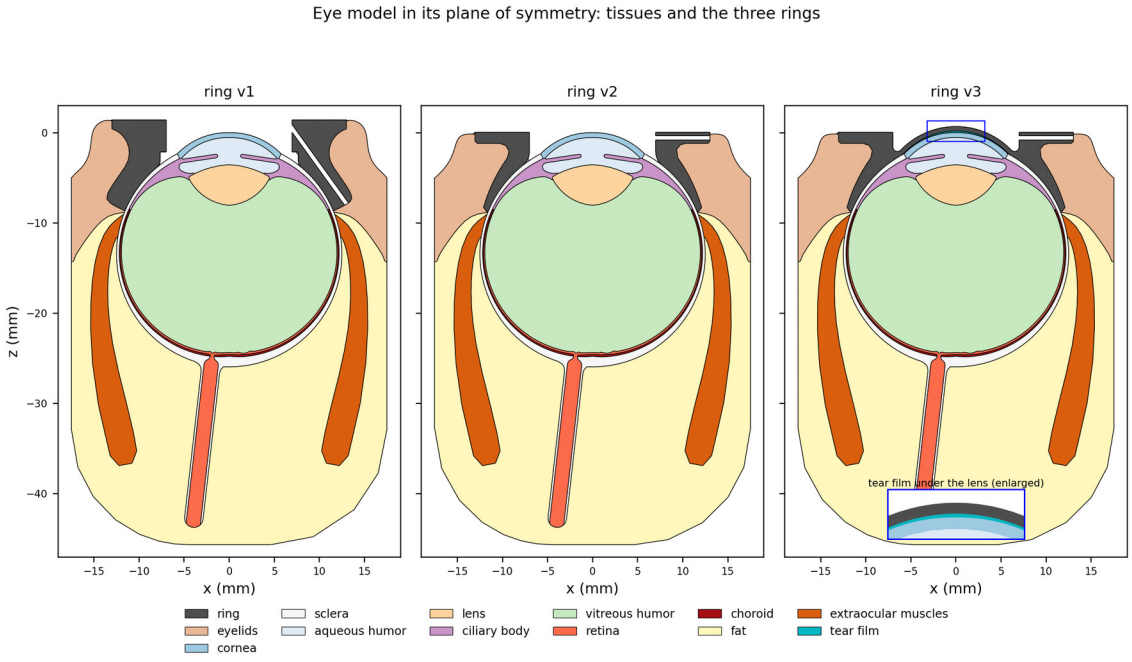


FIGURE 3.1. The eye model with the three scleral contact rings: v1, the original ring of SCEC Gen 3; v2, the reduced-bulk ring; v3, the reduced-bulk ring with an integral corneal lens. The plane shown is the plane of symmetry, the cut of the half model that was solved; colors mark the domains, and the inset enlarges the tear film beneath the lens of v3.

Heat transfer and boundaries. Heat conduction was solved in the tissues and the ring, with the humors at rest. In the perfused tissues (choroid, retina, ciliary body, extraocular muscles, eyelids, and orbital fat), the heat equation carries the Pennes terms for perfusion and metabolic heat [**133**],

$$\rho c_p \frac{\partial T}{\partial t} = \nabla \cdot (k \nabla T) + \omega \, \rho_b c_b \, (T_b - T) + Q_{\mathrm{met}}, \tag{3.1}$$

where T is the temperature of the tissue at time t, ρ, c_p, and k are the density, specific heat, and thermal conductivity of the tissue, ω its perfusion rate, ρ_b and c_b those of blood, T_b the body temperature, and Q_{met} the metabolic heat (Table 3.3; sources in Appendix D). The choroid, with the highest perfusion, is the dominant perfused heat source opposing cooling. Its perfusion rate is taken from microsphere measurements in monkey eyes [**42**].

The outer shell of the orbit is held at T_b. The exposed surfaces lose heat to the room, at the ambient temperature T_0, by convection with a heat-transfer coefficient of 10 $\mathrm{W\,m^{-2}\,K^{-1}}$ and by radiation, and the cornea also by tear evaporation. In v3 the lens covers the ocular surface, so neither tear evaporation nor exposure applies to the eye (Figure D.1). In the starting state, the eye at rest with the device switched off, the face of the ring that meets the module is insulated; it is then brought from its starting temperature to the set point T_c along a smoothed step lasting 90 s. The transient was computed for 35 min. Further details are given in Appendix D.

Conditions. At the baseline condition (ambient 25 °C, body 37 °C, set point 4 °C) the ring material was varied among aluminum, aluminum oxide, and fused silica, and the three geometries were compared directly. Each geometry was also solved for every combination of ambient temperature (10, 25, and 35 °C), body temperature (35, 37, and 39 °C), and set point (4, 8, and 12 °C), 27 conditions per ring, to test whether the comparison depends on the operating point. A silicone layer of 0.5 or 1.0 mm was added to the outer walls, and rewarming after switch-off was simulated for a subset of conditions.

TABLE 3.3. Thermal properties, perfusion rates, and metabolic heat of the domains of the eye model. The properties of the tear film and the ring (and the silicone layer) depend on temperature and are given at 20 °C (aluminum oxide, for example, 37.0 at 4 °C and 32.1 $\mathrm{W\,m^{-1}\,K^{-1}}$ at 37 °C).

Domain *Symbol (units)*	**Conductivity** k ($\mathrm{W\,m^{-1}\,K^{-1}}$)	**Density** ρ ($\mathrm{kg\,m^{-3}}$)	**Specific heat** c_p ($\mathrm{J\,kg^{-1}\,K^{-1}}$)	**Perfusion** ω ($\mathrm{s^{-1}}$)	**Metabolic heat** Q_{met} ($\mathrm{W\,m^{-3}}$)
Cornea	0.56	1070	3707	–	–
Aqueous humor	0.593	1005	4159	–	–
Lens	0.43	1126	3255	–	–
Vitreous humor	0.59	1010	4134	–	–
Ciliary body	0.49	1088	3556	0.0064	685
Retina	0.55	1014	3958	0.0039	3443
Choroid	0.51	1074	3682	0.0927	624
Sclera	0.545	1102	3431	–	–
Extraocular muscles	0.49	1090	3421	0.0045	686
Eyelids (skin)	0.34	1070	3660	0.0011	644
Orbital fat	0.2245	920	2500	0.0004	199
Tear film	0.60	998	4180	–	–
Ring, aluminum	236	2700	903	–	–
Ring, alumina	34.3	3990	764	–	–
Ring, fused silica	1.37	2220	705	–	–
Silicone layer	0.26	1280	1781	–	–

3.2.2.3. *Heat Exchanger.* Each heat exchanger is an aluminum annulus on the hot side of the module, with fins on alternating walls that force the coolant along a zigzag path (Figure 3.2 and Table 3.4). The v3 block has no lid of its own and is closed by the hot-side ceramic plate of the module, which the coolant touches directly. Each was modeled whole. The heat from the module entered as a uniform flux over its footprint, an annulus of inner radius 7 mm and outer radius 13 mm, the same in the three designs, and each received the same two heat loads: 10 W, the operating load, and 15.5 W, a conservative upper value. In v3 most of that annulus is the coolant itself, because the model has no plate closing the block. The heat left with the coolant; the outer walls and the module plane outside the footprint lost a small amount of heat to the ambient air.

The water flow and the temperature were solved with the Python program for every combination of flow rate (0.10, 0.25, and 0.50 $\mathrm{L\,min^{-1}}$) and inlet temperature (4, 10, and 20 °C), and the temperature was advanced in time with the flow where

the flow was unsteady. The surfaces on which these conditions act are shown in Figure D.2.

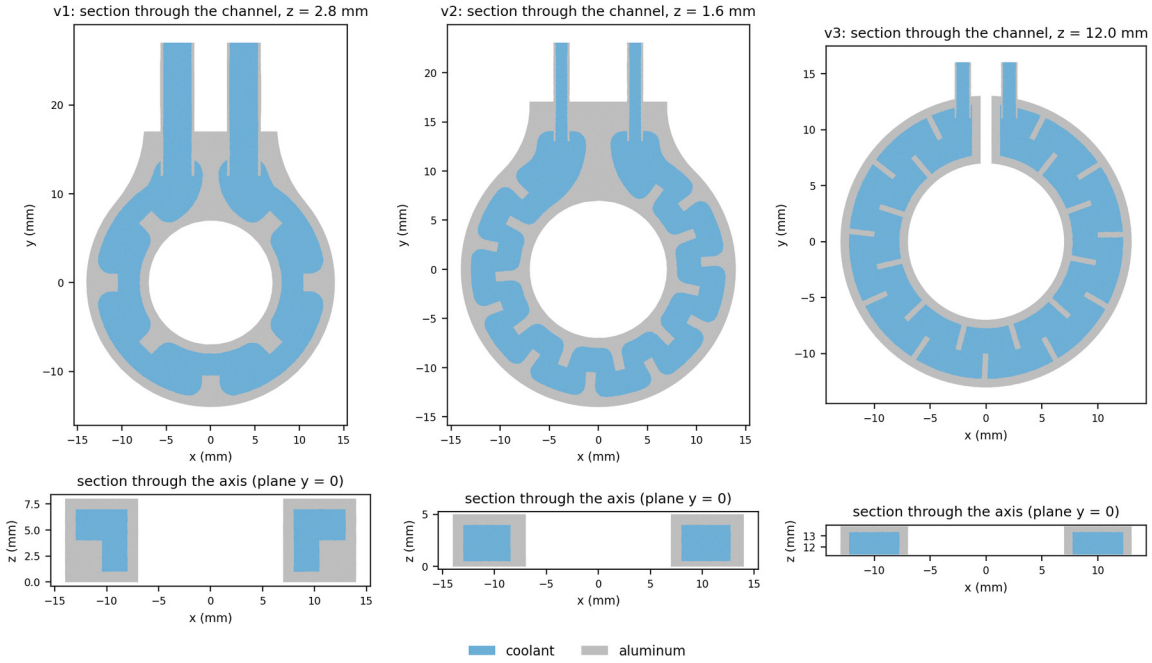


FIGURE 3.2. The three heat-exchanger designs as modeled: v1, that of SCEC Gen 3, with seven fins filling half the channel height; v2, a lower channel and 18 thinner fins; v3, the lowest channel and 21 fins, closed by the hot-side ceramic plate of the module instead of a lid. Coolant (blue) and aluminum (gray) in a horizontal section at 30% of the channel height, where the fins of v1 are (top), and in the vertical section through the axis (bottom). Dimensions in Table 3.4.

TABLE 3.4. Geometry of the three heat-exchanger designs. Volumes measured on the CAD models.

Feature	**v1 (Gen 3)**	**v2**	**v3**
Volume (mm^3)	2337	1619	435
Block, inner/outer radius (mm)	7.0/14.0	7.0/14.0	7.0/13.0
Channel height (mm)	6.0	3.5	2.0
Fins: number, thickness (mm)	7, 2.0	18, 1.0	21, 0.5
Fin height	half channel	full channel	full channel
Inlet and outlet diameter (mm)	4.00	2.25	1.41
Lid	separate	separate	module plate

3.2.2.4. *Outcome Measures.* Temperatures were read at the sites measured in Chapter 2 (Figure 3.3): the anterior, equatorial, and posterior sclera, the vitreous,

and the proximal optic nerve, at its junction with the globe. A distal optic nerve probe, 1 cm behind the globe, recorded the effect of cooling behind the eye, and domain probes recorded the average, minimum, and maximum temperature of the retina and the whole eye. The steady temperature at each site is a weighted mean of the body and ring temperatures. The principal outcome was thus the cooling effectiveness

$$\varepsilon = \frac{T_b - T_{\text{site}}}{T_b - T_{\text{ring}}} \times 100\%, \tag{3.2}$$

where T_b is the body temperature, T_{site} the steady temperature at the site, and T_{ring} the final temperature of the ring. It is 0% for a site left at body temperature and 100% for a site cooled to the temperature of the ring. It does not depend on the operating point: a linear fit of the steady temperatures over the 27 operating points of each ring reproduced every site to within 0.003 °C, and less than 1% of a change in ambient temperature reached any of the measurement sites (1.3 to 1.4% on average over the whole eye with v1 and v2, under 0.1% with v3). It also gives directly the ring temperature needed to reach a target. Kinetics were summarized by t_{90}, rewarming by the time each site stayed below 35 °C after switch-off, and efficiency by the share of the heat drawn from the eye rather than the eyelids and air. For the heat exchanger, the outcomes were the rise of the average temperature of the heated face above the coolant inlet temperature and the peak temperature of that face against the 43 °C limit.

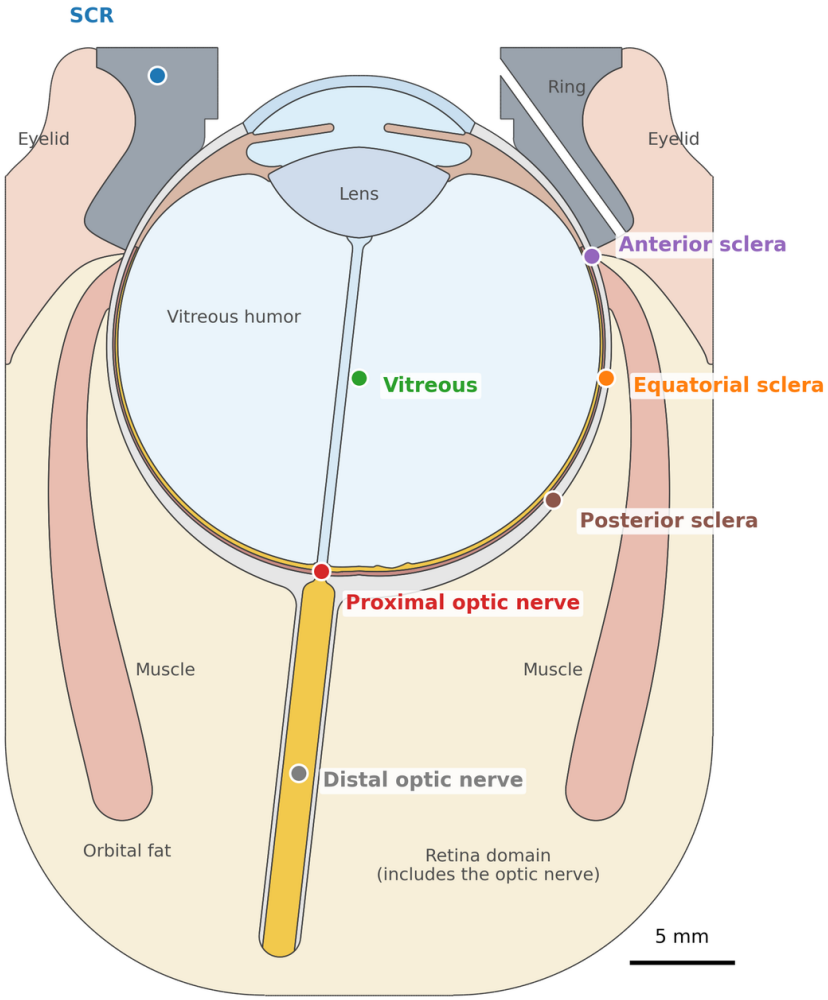


FIGURE 3.3. Probe locations on the section of the model in its plane of symmetry (v1 ring): the anterior, equatorial, and posterior sclera, the vitreous, the proximal optic nerve, the distal optic nerve (1 cm behind the globe), and the SCR. The SCR probe, on the rim of the ring 10 mm from the axis and out of this plane, is drawn turned into it. Yellow, the retina domain, including the optic nerve, over which the average retinal temperature is taken.

3.2.3. Experimental Ring Comparisons. A ceramic ring of the original geometry and the reduced-bulk ring were each compared with the original aluminum ring in human donor eyes, to test whether an electrically insulating material, or halving the volume, costs cooling.

Rings. The ceramic ring reproduced the original geometry in alumina, fired from the same design file without further machining (Ceramco, Inc., Center Conway, NH,

USA). The reduced-bulk ring (v2) was machined from aluminum with the contact surface of the original ring and a thinner profile, which reduced the height of the assembled device from 23 to 20 mm. The three rings are shown in Figure 3.4; each was assembled into a complete device built as SCEC Gen 3.

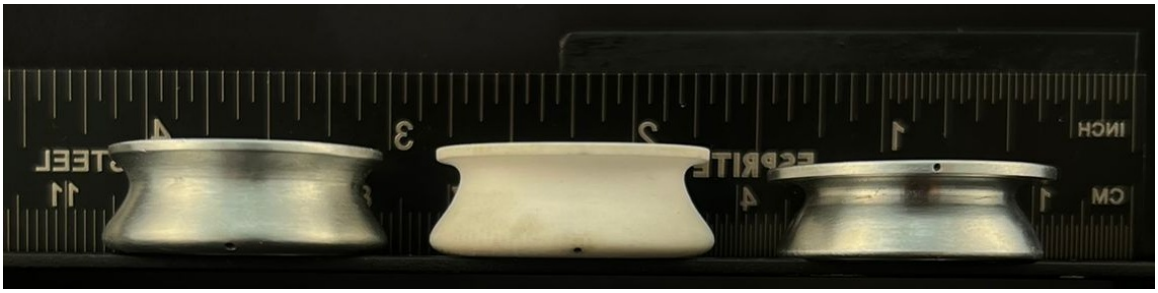


FIGURE 3.4. The three scleral contact rings tested in human donor eyes, side view on a ruler (inches above, centimeters below): (left) the original aluminum ring of SCEC Gen 3 (v1), (center) the alumina ring of the original geometry, and (right) the reduced-bulk aluminum ring (v2). The photograph is mirrored, so the ruler reads in reverse.

Protocol. The nine human donor eyes of Chapter 2 were each cooled with all three rings on the same day, with the same implanted sensors, under the protocol of Section 2.2.5, re-equilibrating in the bath between rings, in an order that varied between eyes. The comparisons are therefore paired within each eye, and differences in sensor placement, donor, and anatomy cancel.

Analysis. The recordings were processed with the pipeline and endpoints of Section 2.2.6. A sensor excluded by the spatial screen was excluded under all three rings, and one further trace, the vitreous sensor of eye 4 under the reduced-bulk ring, was removed on visual review for the same oscillation it showed under the original ring. In all, 23 of the 162 traces recorded with the three rings were excluded (11 for a hardware fault, 3 for a poor fit, 6 by the spatial screen, and 3 on visual review). This left 45, 47, and 47 traces for the original, ceramic, and reduced-bulk rings. One of the excluded traces is the proximal optic nerve of eye 6 under the original ring, and the comparisons with that ring at the proximal optic nerve rest on eight eyes. Each ring was compared with the original by the procedure of the species comparison in

Chapter 2. Paired t tests were run at each site with Holm correction across the five tissue sites. Each difference in °C is reported with its 90% confidence interval, whose limit farther from zero is the largest difference that the data allow (Section 2.2.6). The hypothesis is tested at the proximal optic nerve.

Comparison with the model. As a supplementary step, the measurements were set beside the model through the same quantity for both, the fraction of each site's own starting difference from the ring that the device removed,

$$\varepsilon_{\Delta} = \frac{T_{\text{initial}} - T_{\text{final}}}{T_{\text{initial}} - T_{\text{ring}}} \times 100\%, \tag{3.3}$$

where T_{initial} and T_{final} are the temperatures of the site when the ring was switched on and at the end of cooling, and T_{ring} the final temperature of the ring. Model and measurements were also compared through t_{90}. Both start from an eye at rest with the unpowered ring in place: in the model the eye has settled with the device off, and ex vivo the ring equilibrated with the eye in the 37 °C bath. The model is read after 35 min and the experiments over minutes 16 to 20. Each ring was matched to its model design (original ring to v1, reduced-bulk ring to v2, ceramic ring to the alumina v1 run), at a body temperature of 37 °C and a set point of 4 °C. Because the model includes perfusion and the eyelids and the experiments do not, the comparison is not a validation of the model. It was read for the order of the sites, by Spearman's rank correlation (not tested, with five sites), and for the direction of the differences, by the ratio of measured to modeled effectiveness at each site.

3.2.4. Integrated Control Board. The separate boards of the Gen 3 controller, a thermistor conditioning circuit, a comparator that switched the module through a relay, and a laboratory supply (Section 2.2.1), were consolidated for SCEC Gen 4 onto a single printed circuit board designed for this work (Figure 3.5). Its firmware is described, and its schematics and drawings are given, in Appendix F.

The microcontroller runs a digital proportional–integral–derivative loop on the ring thermistor at 5 Hz and sets the drive of a thermoelectric-module controller.

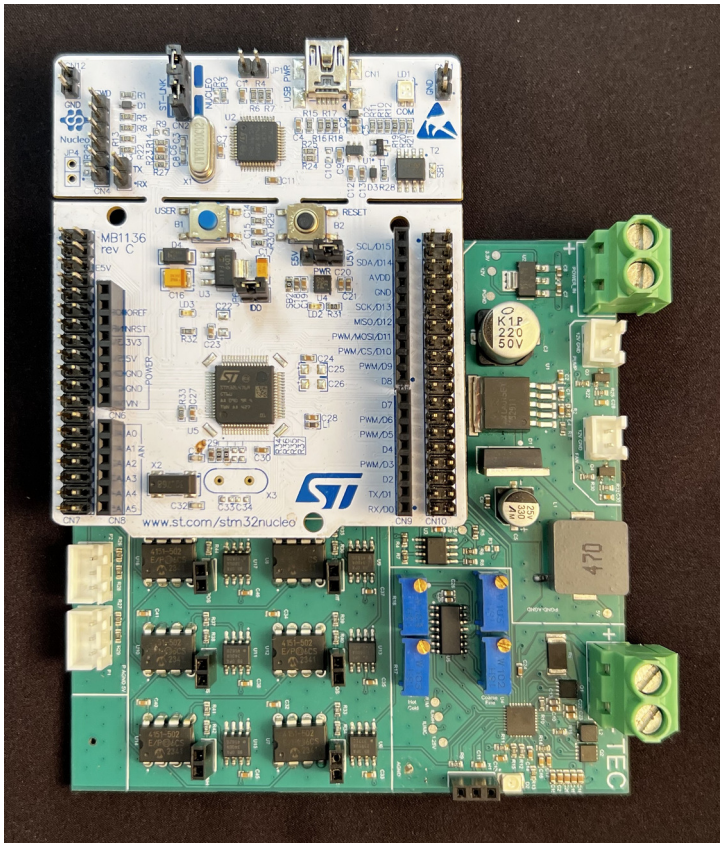


FIGURE 3.5. The integrated control board (revision 1.0): the microcontroller board on its headers (upper left), the six thermistor channels (lower left), and the power and drive stage (right), with the screw terminals for the 12 V supply (upper right) and the thermoelectric module (lower right) and the pump and fan connectors between them.

Hard limits on the module current and voltage are set by trimmers, independently of the firmware, and the module is off by default. A safety routine running every 50 ms stops the module and latches the fault if the ring thermistor reads open or out of range, the 2.5 V reference leaves its band, or the module current is abnormal. A hardware watchdog resets the microcontroller if its program stops (Section F.1).

Bench verification. The board was verified on the bench in stages, from unpowered checks and operation into dummy loads to closed-loop regulation of a ring on a preserved porcine eye in the bath, with a pull-down to 4 °C and hold, 18 min in all (Section F.2). Three safety cutouts were triggered on purpose, by disconnecting the ring thermistor and the module and by stalling the program. The current

monitor was compared with a multimeter in series with a 0.24 Ω resistor in place of the module. Regulation was summarized by the time to reach the set point and the largest deviation while held, the current monitor by its largest deviation from the multimeter.

3.3. Results

3.3.1. Validation of the Python Program. The checks below show that the program solves the COMSOL models correctly. The Python program agreed with the solutions stored by COMSOL within 0.06 °C at every probe and stored time of the 104 cooling cases, and within 0.008 °C at the end of cooling; the nine material cases agreed within 0.031 °C. In the 22 rewarming cases it agreed within 0.054 °C at the end, and within 0.075 °C at every tissue probe during the run with steps of 0.5 to 3 s in the first minute after the release (0.32 °C with the standard steps); the largest difference, 0.11 °C, was at the v3 ring 0.3 min after the release, with the ring at 4 °C and the body at 39 °C. The corrected runs use the same solver and the same meshes, without the air and the module layer.

The eye model converged at the deep sites, except at the posterior and equatorial sclera with v3, where the change grew from the coarsest to the finest mesh: from the intermediate to the finest mesh the final temperature changed by at most 0.04 °C at the proximal and distal optic nerve and the posterior sclera and 0.11 °C at the equatorial sclera, the vitreous, and the average retina. Differences between designs smaller than this are not interpreted. The anterior sclera, next to the ring, did not converge, and refinement around the ring put it up to about 1.3 °C higher. Temperatures there are therefore lower bounds (Section D.6).

In the COMSOL heat-exchanger model the heat carried away by the coolant exceeded the heat supplied by 0.9 to 1.2% on average over the 36 operating points of each design and differed from it by at most 5% at any one point, most when the room was much warmer or colder than the coolant, and the Python runs closed within 0.2%.

The COMSOL solution changed by 0.03 °C or less between the two finest meshes, but the Python program agreed with it only within 0.6 °C at the module face and 0.04 °C at the outlet. The temperature of the module face is thus known to about 1 °C, and the values on these meshes are low (Section D.6). The COMSOL sweep gave v3 about a fifth less heat than v1 and v2. The results below therefore come from the Python runs with the same heat on the three designs, on the intermediate mesh. The flow was steady only in heat exchanger v1, at 0.10 $\mathrm{L\,min^{-1}}$ with every inlet and at 0.25 $\mathrm{L\,min^{-1}}$ with the 4 °C inlet, and elsewhere the pressure drop oscillated with an amplitude (half its detrended peak-to-peak range) of 0.6 to 49% of its mean, above 46% in 21 of the 23 unsteady cases.

3.3.2. Ring Material. In the model, an aluminum-oxide ring cooled the eye almost as well as an aluminum ring of the same geometry, and a fused-silica ring did not (Figure 3.6A). After 35 min at the baseline condition, the aluminum-oxide ring kept 89 to 95% of the cooling effectiveness of aluminum at every site and with every geometry. With the original geometry it kept 94% at the proximal optic nerve, and the final temperatures were 0.2 °C higher there and 2.2 °C higher at the anterior sclera, the site next to the ring. Fused silica kept only 29 to 46% of the effectiveness of aluminum. With the original geometry and fused silica, the proximal optic nerve ended at 35.7 °C against 33.8 °C with aluminum, and the anterior sclera at 28.4 °C against 9.3 °C. The ring itself, held at the same cold face, ended at 7.7 °C instead of 4.1 °C. The glass conducted too little heat to cool the eye in a useful way, and on this result fused silica was dropped without a ring being built.

In the human donor eyes, the ceramic ring and the aluminum ring of the original geometry cooled the proximal optic nerve by similar amounts, 4.0 and 3.8 °C on average in the eight eyes with both readings. The paired difference, aluminum minus ceramic, was −0.20 °C (90% confidence interval −0.82 to +0.43 °C, eight eyes), and no difference was detected. A difference larger than 1.0 °C is unlikely with the eyes of each donor averaged, and one larger than 0.8 °C with each eye counted separately

(Figure 3.7A; Section 3.3.3). The same held for the ring itself (+0.16 °C, −0.41 to +0.73), which ended at 4.1 °C with either material. The ceramic ring was slower, however, with a t_{90} of 174 s against 124 s on average. At the other four tissue sites the ceramic ring cooled more on average, by 0.9 to 1.7 °C (Figure 3.7A). No difference was significant after Holm correction ($p \geq 0.38$). With intervals that allow differences of 2.4 to 4.3 °C, these sites say little either way; only at the posterior sclera did the interval exclude zero (−2.40 to −0.11 °C).

The model and the experiment agree at the proximal optic nerve, where neither finds a difference between the two materials larger than 1 °C. The model predicts 0.23 °C more cooling with aluminum there, inside the experimental confidence interval. At the other tissue sites they differ in direction. The model predicts a small advantage for aluminum, at most 2.3 °C, whereas the experiment found a trend, not significant, toward more cooling with ceramic, and the model's difference lies outside the experimental interval at the anterior and posterior sclera. An electrically insulating alumina ring thus preserved cooling at the proximal optic nerve, and the glass alternative was excluded by the model alone.

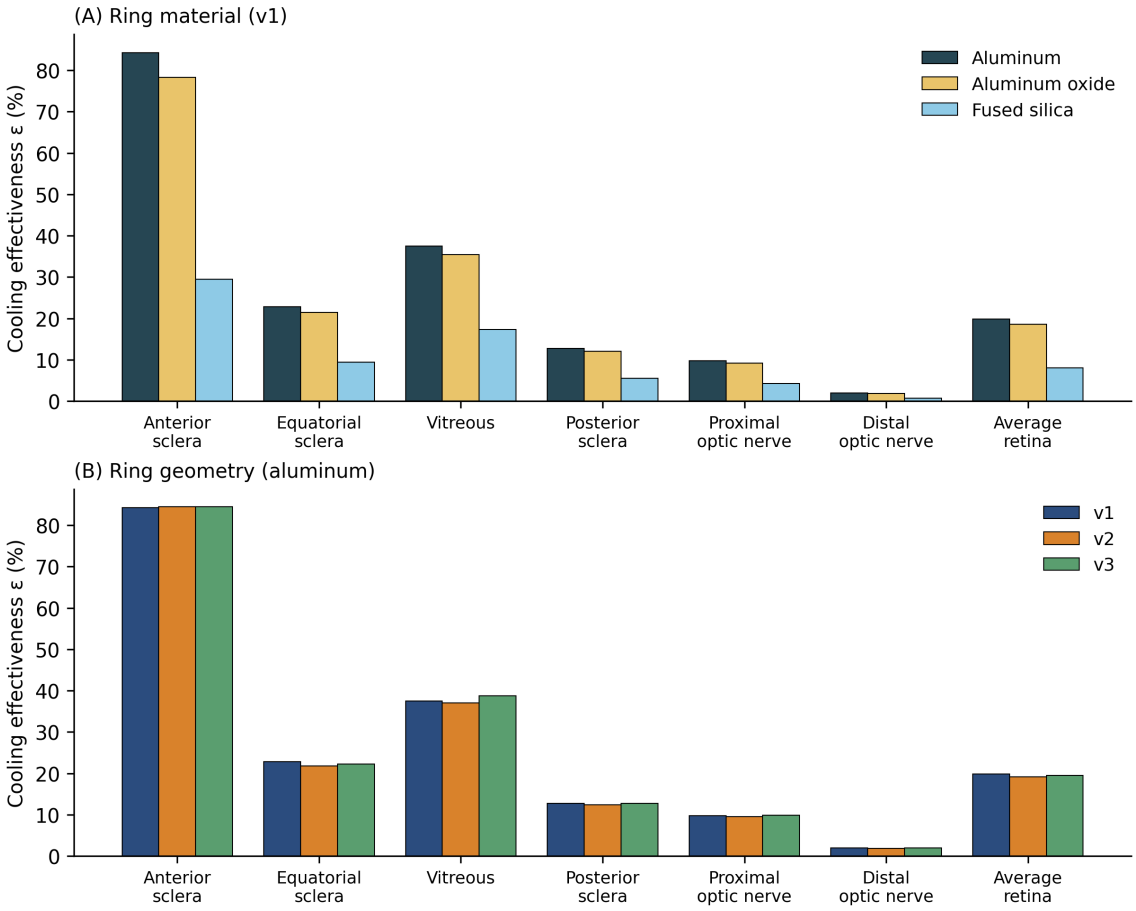


FIGURE 3.6. Cooling effectiveness ε at each site in the model after 35 min of cooling at the baseline condition: (A) v1 rings of aluminum, aluminum oxide, and fused silica; (B) aluminum rings of the three geometries.

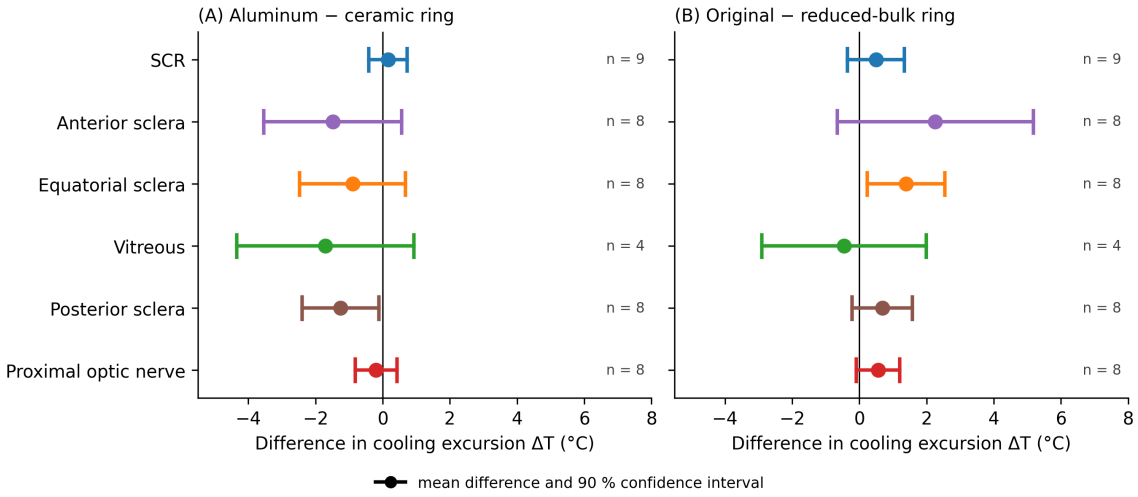


FIGURE 3.7. Paired differences in cooling excursion at each site in human donor eyes between the original ring and (A) the ceramic ring, (B) the reduced-bulk ring: mean with its 90% confidence interval, whose limit farther from zero is the largest difference compatible with the data (one-sided, 5% level). n, eyes with a valid pair.

3.3.3. Ring Geometry. In the model, the three geometries cooled the eye almost identically (Figure 3.8). The cooling effectiveness of the three rings differed by at most 2 percentage points at any site, and by 3 for the average over the whole eye. At the proximal optic nerve it was 9.8, 9.6, and 9.9% for v1, v2, and v3 (Figure 3.6B).

Across all 27 operating points the cooling excursion with each ring was a nearly fixed fraction of that with the original ring at every site (with the reduced-bulk ring, 97.8 to 99.5% of it at the proximal optic nerve). Thus the comparison does not depend on the body, ambient, or set-point temperature. At the baseline condition the cooling excursion with the reduced-bulk ring was slightly smaller than with the original: by 0.05 °C at the proximal optic nerve, 0.09 °C at the posterior sclera, 0.07 °C at the vitreous, and 0.3 °C at the equatorial sclera. Except at the vitreous, these differences exceed the change of the results between the two finest meshes (Table D.2), but they are small, 0.6 to 3.9% of the excursion of each of these sites (at the proximal optic nerve, 1.7% of an excursion of 3.1 °C). Adding the corneal lens (v3) restored the original ring's final temperatures at the proximal optic nerve and posterior sclera, to within 0.04 °C. Its excursion at the vitreous was larger than with either open ring, by 0.7 °C relative to v1 and 0.8 °C relative to v2, the largest difference between any two designs. The t_{90} was the same for all three rings to within 1 min at every site (Section 3.3.5).

The reduced bulk did change how much heat the ring drew (Figure 3.9). Over 35 min of cooling the reduced-bulk ring drew 14% less heat than the original, and almost all of the saving came from the eyelids. The heat drawn from the eye was nearly unchanged, and its share, the measure of efficiency, rose from 33% to 37%. The ring with the corneal lens (v3) drew about as little heat as the reduced-bulk ring, and 38% of it from the eye, because the lens adds contact with the cornea. Followed back to its sources, the 7.7 kJ that left through the cooling face of the original ring came 48% from the wall of the orbit, 33% from the blood, 14% from heat stored in the tissues, and 6% from the room.

In the human donor eyes, the reduced-bulk ring cooled the proximal optic nerve by 3.2 °C on average against 3.8 °C for the original ring in the eight eyes with both readings. The paired difference, original minus reduced-bulk, was +0.56 °C (90% confidence interval −0.09 to +1.20 °C, eight eyes). The reduced-bulk ring thus cooled the proximal optic nerve 0.6 °C less on average, not significantly, and by no more than 1.2 °C (Figure 3.7B). At the posterior sclera the difference was +0.68 °C (−0.21 to +1.58). At the other three sites the intervals were wider, 2.3 to 5.8 °C across (Figure 3.7B), and at the equatorial sclera the interval excluded zero (+0.23 to +2.54 °C). No difference was significant after Holm correction ($p \geq 0.29$). The ring itself held its set point equally well with either geometry, ending at 4.0 to 4.1 °C, and the reduced-bulk ring had the shorter t_{90}, 82 s against 124 s on average. The comparisons were repeated with the eyes of each donor averaged (three to five donors; five at the proximal optic nerve) and with the vitreous traces removed on visual review put back. Neither repeat changed a conclusion.

Model and experiment agree at the proximal optic nerve, the site at which the hypothesis is tested. The model predicts that the original ring cools it 0.05 °C more than the reduced-bulk ring, inside the experimental interval, and in both the reduced-bulk ring cools it less, by under 1.2 °C. Both also find the original ring cooling the posterior and equatorial sclera slightly more, by 0.7 and 1.4 °C in the experiment and by 0.09 and 0.3 °C in the model. Compared, as for the materials, on the cooling excursion, the model's differences lie inside the experimental confidence intervals at all five sites. At the vitreous and the anterior sclera the two disagree in sign. Halving the volume of the ring kept the cooling at the proximal optic nerve, the site at which the hypothesis of this aim is tested, at a cost of about 0.6 °C, and of no more than 1.2 °C, of the 3.8 °C excursion.

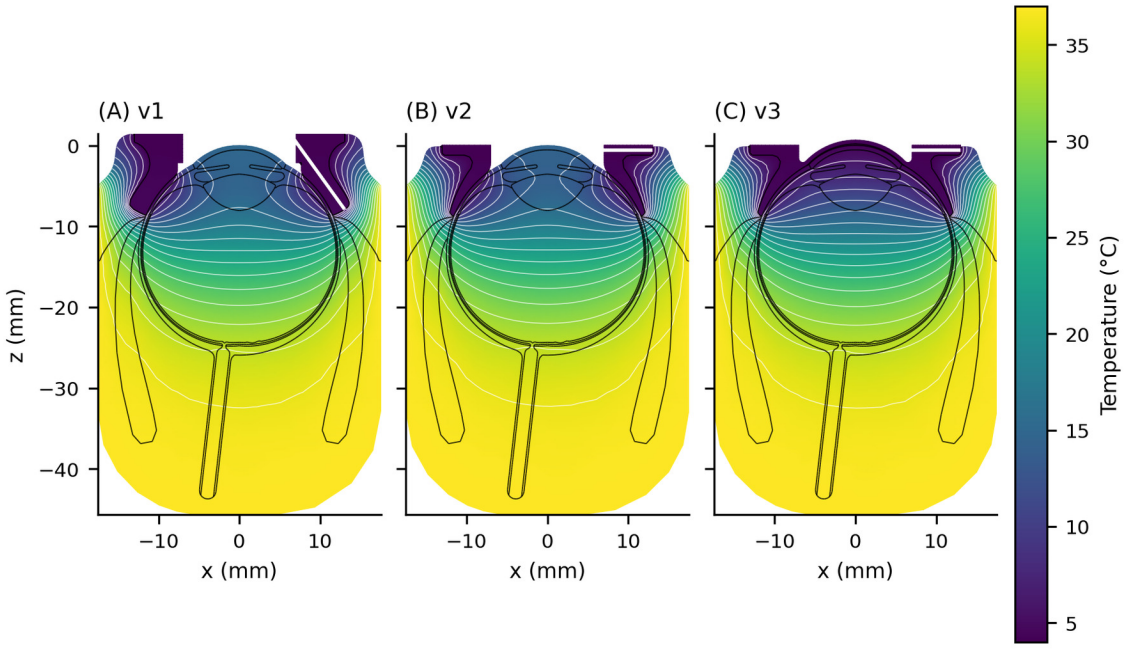


FIGURE 3.8. Temperature in the plane of symmetry after 35 min of cooling with (A) v1, (B) v2, and (C) v3, at the baseline condition, on a common color scale. White lines are isotherms every 2 °C; black lines outline the ring and the tissues.

3.3.4. Insulating Outer Layer. The insulating layer did not improve cooling. It was evaluated only in the model. With a layer of 0.5 or 1.0 mm on the outer walls of any of the three rings, the sites ended within −0.05 to +0.33 °C of their temperatures with the same ring without a layer after 35 min at the baseline condition. The change was largest at the anterior sclera, 0.3 °C with the 1.0-mm layer on each ring, within the mesh uncertainty at that site (up to about 1.3 °C, Section D.6) and so not resolved by the model. It was smallest at the deep sites: at most 0.03 °C at the proximal optic nerve, comparable to the change between the two finest meshes there, and at most 0.01 °C at the distal optic nerve. The cooling effectiveness fell by no more than 1 percentage point at any site.

The layer did change where the ring drew its heat (Figure 3.9). For the original ring the total heat drawn over 35 min fell by 5 and 7% with layers of 0.5 and 1.0 mm and that drawn from the eyelids by 8 and 11%, so the share drawn from the eye rose from 33% to 35 and 36%. The other rings behaved alike.

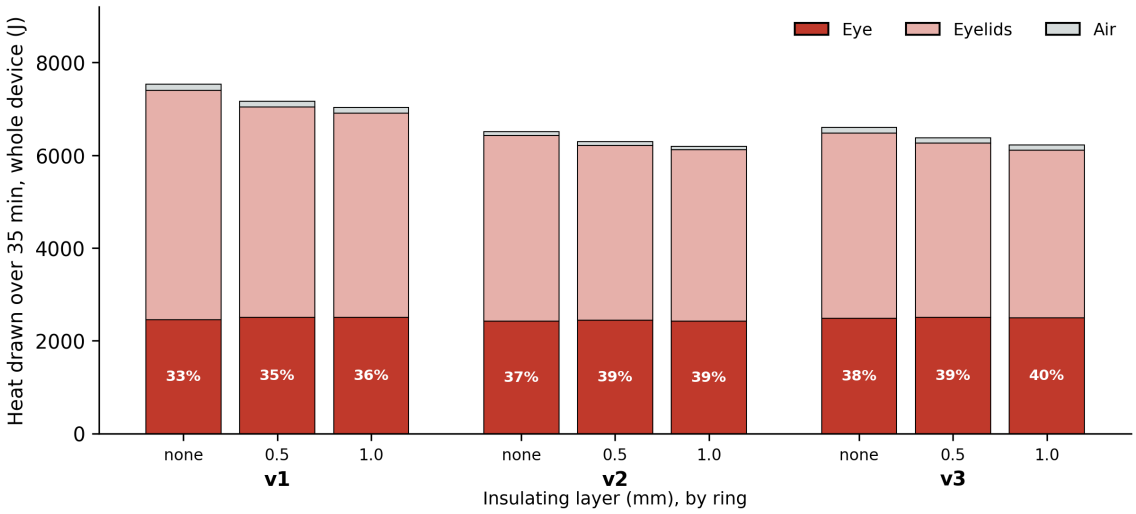


FIGURE 3.9. Heat drawn by the whole device over 35 min from the eye, the eyelids, and the air, for each ring without and with a 0.5 or 1.0 mm insulating layer; the share from the eye is printed in the eye part of each bar. Heat reaching the lens of v3 through the tear film counts with the eye, and heat through a layer with the eyelids.

3.3.5. Cooling Targets and Kinetics. The steady temperature at each site depends linearly on the body and ring temperatures. Each site is thus a weighted mean of the body and ring temperatures, $T_{\text{site}} = (1-\varepsilon)\,T_b + \varepsilon\,T_{\text{ring}}$, with ε as a fraction and the symbols as in Equation 3.2, which gives the ring set point that brings a site to a target (Figure 3.10). The retina itself is cooled along a gradient, from its anterior edges next to the ring to the posterior pole (Figure 3.12). At a body temperature of 37 °C, the proximal optic nerve reaches 35 °C with the ring at 16.6 °C or colder, and with the ring at the 4 °C floor of the device it settles at 33.8 °C. Bringing it to 32 °C would need a ring at −14.1 °C, which the device cannot provide. The shallower sites need less: the average retina reaches 32 °C with the ring at 11.7 °C (Figure 3.10). The distal optic nerve, 1 cm behind the globe, stays within 1 °C of body temperature at every operating point and cannot be brought to either target.

The body temperature shifts every requirement: at 35 °C the proximal optic nerve reaches 32 °C with the ring at 4.3 °C, while at 39 °C it needs −1.9 °C to reach even 35 °C. The three ring geometries require ring temperatures within about 2.1 °C of

one another (2.9 °C at a body temperature of 39 °C) for every site other than the distal optic nerve and for both targets.

Cooling was fastest next to the ring and slowest at depth (Figure 3.11). Over the operating points, the median time to 90% of the final cooling rose with depth, from the anterior sclera to the distal optic nerve (Table 3.5). These times varied by at most about 0.5 min with the operating point and the ring geometry.

After the device was switched off, the eye rewarmed over minutes (Figure 3.11). At a body temperature of 37 °C, the proximal optic nerve and the posterior sclera rose above 35 °C first and the vitreous and the anterior sclera last, the time at each site depending on the set point and the geometry (Table 3.5). At 39 °C the proximal optic nerve never fell below 35 °C.

TABLE 3.5. Times in the model: median time to 90% of the final cooling over the operating points; and, at a body temperature of 37 °C over the set points and ring geometries, the time from switch-on until each site fell below 35 °C and the time it stayed below 35 °C after switch-off. –, the distal optic nerve did not fall below 35 °C.

Site	**Time to 90% (min)**	**Time to below 35 °C after switch-on (min)**	**Time below 35 °C after switch-off (min)**
Anterior sclera	1.6	0.3	13.5 to 15.9
Equatorial sclera	8.8	2.0 to 2.5	7.9 to 9.8
Vitreous	10.4	2.5 to 2.8	13.1 to 15.1
Average retina	10.0	1.6 to 2.4	7.4 to 9.5
Posterior sclera	12.4	5.5 to 7.2	6.7 to 9.0
Proximal optic nerve	14.3	8.4 to 12.1	5.8 to 8.6
Distal optic nerve	20.0	–	–

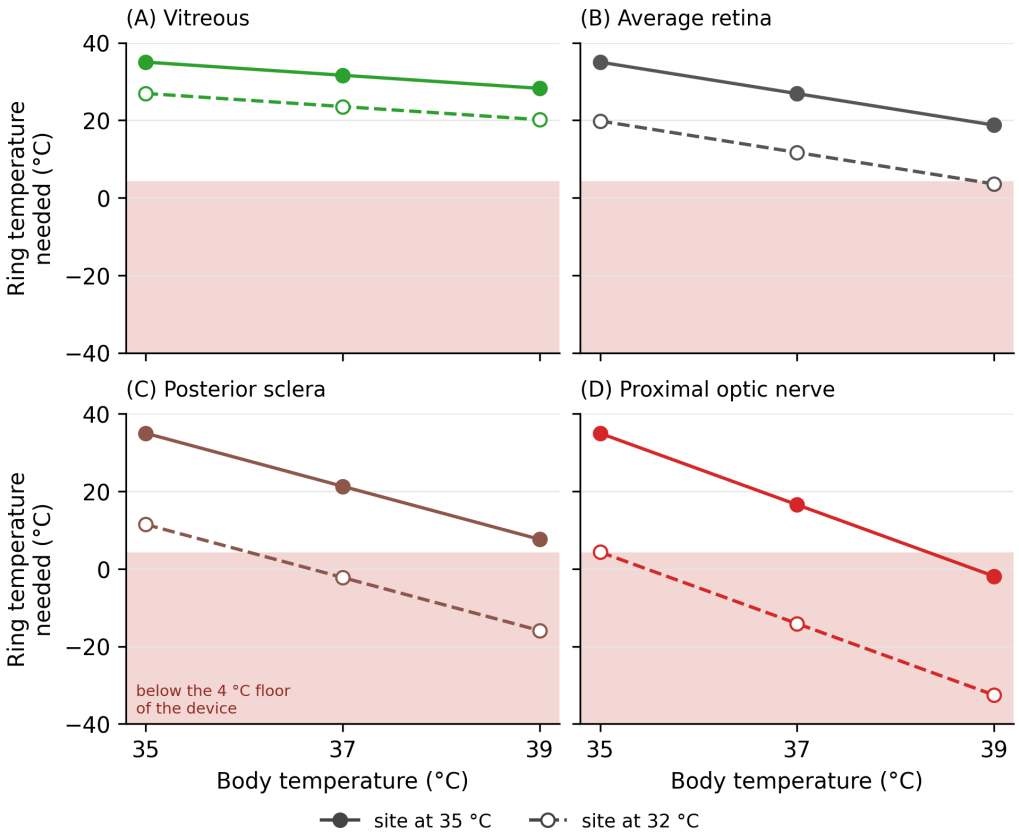


FIGURE 3.10. Ring temperature needed, from the effectiveness of v1, to bring (A) the vitreous, (B) the average retina, (C) the posterior sclera, and (D) the proximal optic nerve to 35 °C (solid, filled markers) and 32 °C (dashed, open markers) at steady state, against body temperature. Shaded, below the 4 °C floor of the device.

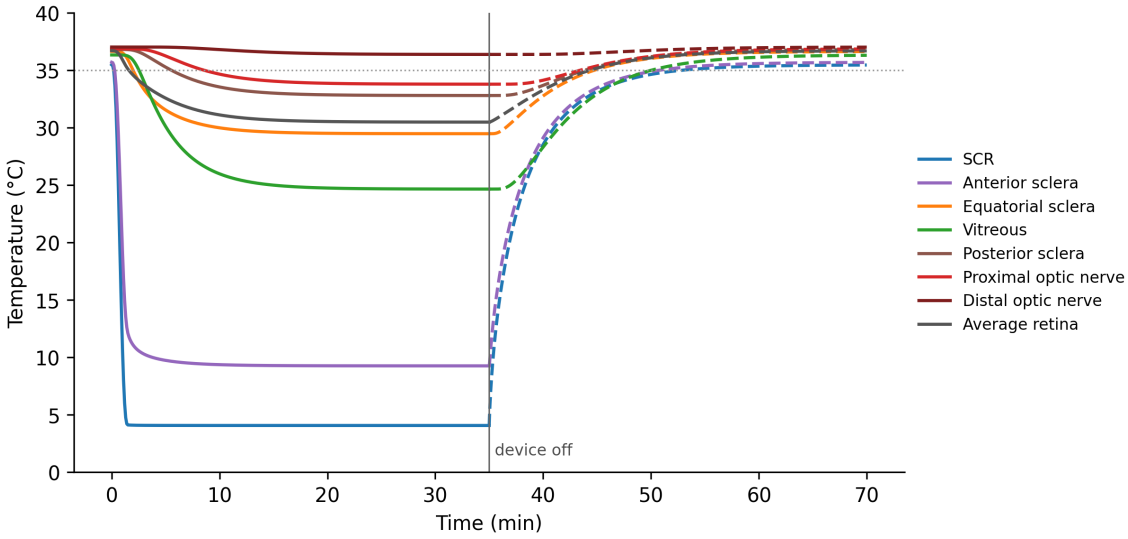


FIGURE 3.11. Temperature at each site of the model during 35 min of cooling with the original ring (v1, solid) and after switch-off (dashed), at the baseline condition (body 37 °C, ambient 25 °C, set point 4 °C); dotted line, 35 °C.

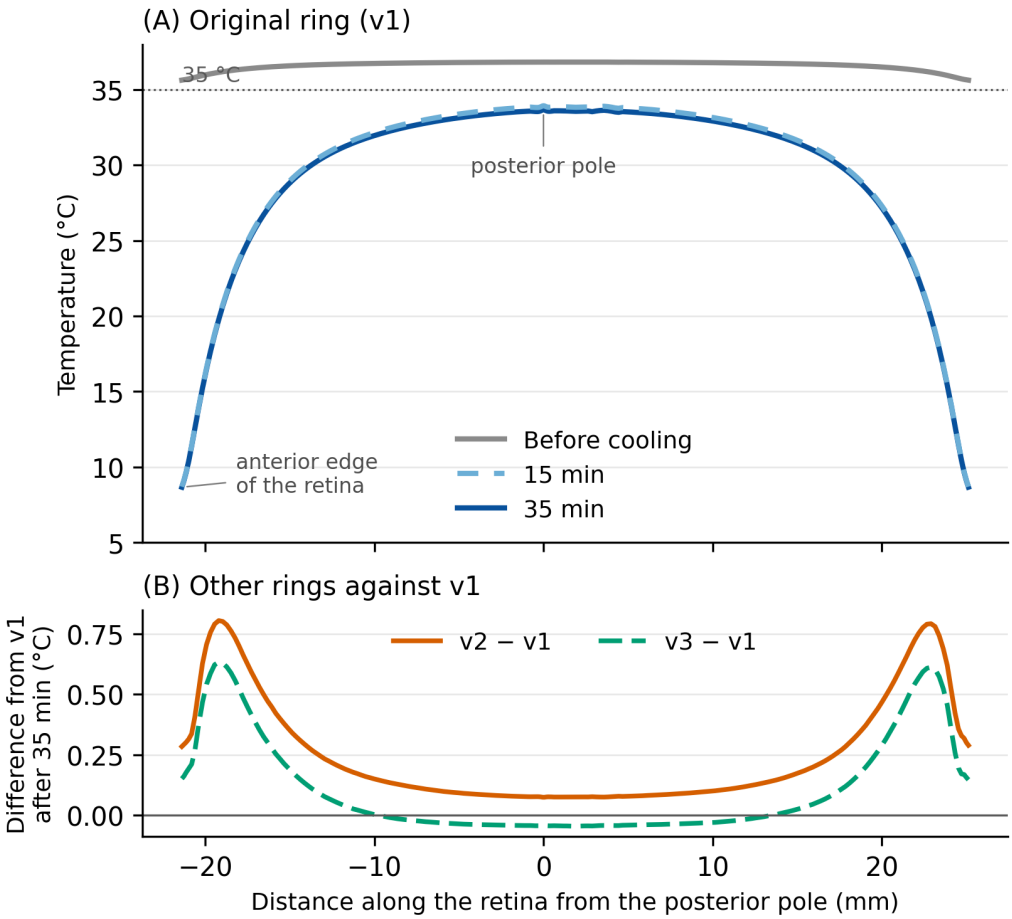


FIGURE 3.12. Temperature along the whole arc of the retina in the plane of symmetry of the model, from one anterior edge through the posterior pole to the other, at the baseline condition. (A) Original ring (v1), before cooling and after 15 and 35 min (these two nearly coincide); dotted line, 35 °C. (B) Difference of v2 and v3 from v1 after 35 min.

3.3.6. Model Versus Ex-Vivo Measurements. With each of the three rings, the model and the donor eyes ranked the five sites in the same order (Spearman $\rho = 1$): both cooled the anterior sclera the most and the proximal optic nerve the least. The amounts were closest at the anterior sclera, next to the ring, where the measured ε_Δ was 0.87 to 1.12 times the modeled value (Figure 3.13). At the vitreous with every ring, and at the proximal optic nerve with the original and reduced-bulk aluminum rings, the modeled value also lay inside the wide 95% confidence interval of the measurements. At the deep sites the donor eyes cooled more than the model, 1.1 to 2.2 times as much, with the smallest gap at the proximal optic nerve (1.1 to 1.4 times). These factors are approximate, because the sensors at depth did not sit exactly at the points of the model. The sensors named posterior sclera lay at 66% of the axial length from the anterior pole on average, and the posterior point of the model at 81%.

Most of this difference is likely to come from the difference between the two conditions: the model includes perfusion and the eyelids, and the donor eyes do not. No model of the bath set-up is reported, because in a bath the heated walls supply heat to the eye, so such a model overstates the warming. The donor eyes also cooled faster at depth, reaching 90% of their cooling in 11.0 to 11.4 min at the proximal optic nerve against 14.3 to 14.6 min in the model, 8.8 to 9.1 against 12.4 to 12.7 min at the posterior sclera, and 6.9 to 7.6 against 10.4 to 10.8 min at the vitreous, and more slowly at the anterior sclera, 3.6 to 3.9 against 1.5 to 2.2 min.

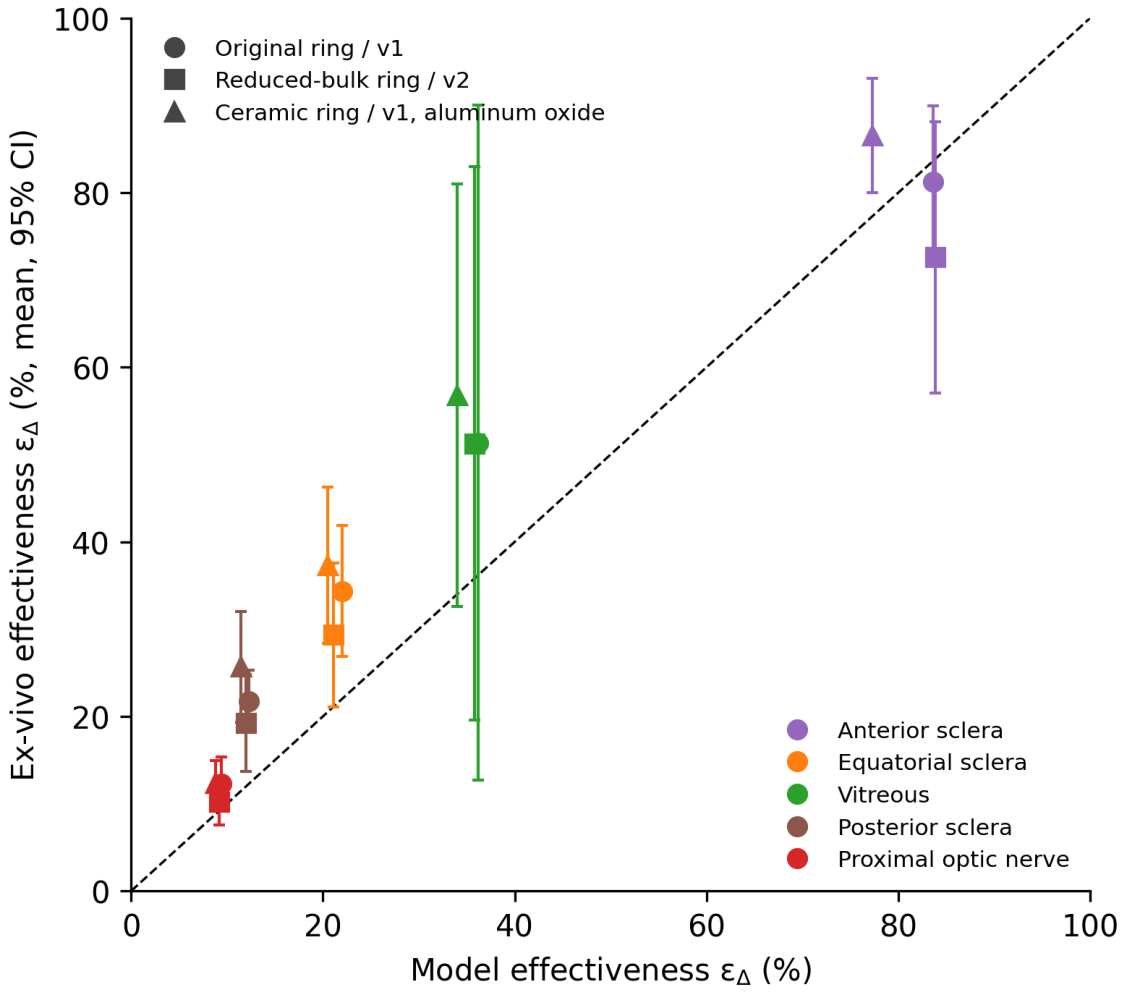


FIGURE 3.13. Cooling effectiveness ε_Δ (Equation 3.3) computed in each human donor eye (mean over the eyes, with its 95% confidence interval from the t distribution with $n-1$ degrees of freedom) against the model, for each site and ring; dashed, the line of identity. Points above it cooled more ex vivo than in the model. n, 4 or 5 eyes at the vitreous and 8 or 9 elsewhere. The model values are lower than ε in Figure 3.6 because ε_Δ is referred to the starting temperature of each site, not to body temperature (proximal optic nerve, v1: 9 against 10%).

3.3.7. Heat Exchanger and Heat Rejection. With the flow fixed, the temperature problem is linear, so each solved run gives the temperature of the heated face for any heat load and ambient temperature,

$$T_f - T_{\text{in}} = R\,P + c\,(T_a - T_{\text{in}}), \tag{3.4}$$

where T_f is the average temperature of the heated face in °C, T_{in} the coolant inlet temperature in °C, R the rise per watt in °C W^{-1}, P the heat in W, c the dimensionless coupling to the air, and T_a the ambient temperature in °C. Checked against direct calculations, this relation held within 0.013 °C. With R and c of the nine runs of each design, the two heat loads (10 and 15.5 W) and two ambient temperatures (10 and 35 °C) give 36 operating points per design.

With the same heat, the smaller heat exchangers kept the heated face closer to the coolant than the original did at the same flow, on average over that face (Section D.5). Over the 36 operating points the heated face stayed 0.7 to 3.9 °C above the coolant inlet in v1, 0.4 to 2.3 °C in v2, and 0.3 to 1.9 °C in v3. At the same flow and heat, v2 rose about 60% as much as v1 and v3 about 45%. These are lower bounds, because the meshes do not resolve the wall layers (Section D.5).

In all three designs, the temperature of the heated face was set almost entirely by the temperature of the coolant. The inlet temperature explained 97.6 to 99.3% of the variance of the heated-face temperature, and the flow rate less than 2%. The 43 °C limit applies to the hottest point and was therefore checked against the peak temperature of the heated face, computed on the time-averaged flow field. At the lowest flow, the warmest inlet, and the higher heat load the peak was 24.5 °C in v1, 23.7 °C in v2, and 28.4 °C in v3, at least 14.6 °C below the limit (Figure 3.14). At 15.5 W the peak of v3 reached 10 °C above the inlet at 0.10 L min^{-1} and 1.9 °C at 0.50 L min^{-1}. This peak is probably too high, because in the model the heat enters the coolant directly, with no ceramic plate to spread it. The pressure drop rose steeply as the channels narrowed (Table 3.6). The smaller channels of v2 and

v3 need about 40 and 200 times the pressure of the original, so at the same pumping power the order reverses: with water at 4 °C and 10 mW the rise was 0.067, 0.105, and 0.143 °C per watt in v1, v2, and v3. The 12 V diaphragm pump of the coolant loop (CrocSee CRS-7A016) is listed at 1.3 L min^{-1} and a head of at least 5 m, about 49 kPa. It covers v1 at every flow in Table 3.6. The 42 kPa of v2 at 0.50 L min^{-1} is close to that head, and v3 at 0.25 L min^{-1} and above exceeds it, before the tubing is counted. These blocks would therefore run at lower flow on this pump or need a stronger one.

TABLE 3.6. Pressure drop of the three heat exchangers at each coolant flow rate, computed with the Python program from the solved flow, and hydraulic pumping power at the highest flow rate (pressure drop times volume flow rate; pump efficiency not included).

Quantity, flow rate	**v1 (Gen 3)**	**v2**	**v3**
Pressure drop (kPa), 0.10 L min^{-1}	0.05	1.9	9
Pressure drop (kPa), 0.25 L min^{-1}	0.25	11	52
Pressure drop (kPa), 0.50 L min^{-1}	0.9	42	206
Pumping power (W), 0.50 L min^{-1}	0.007	0.35	1.7

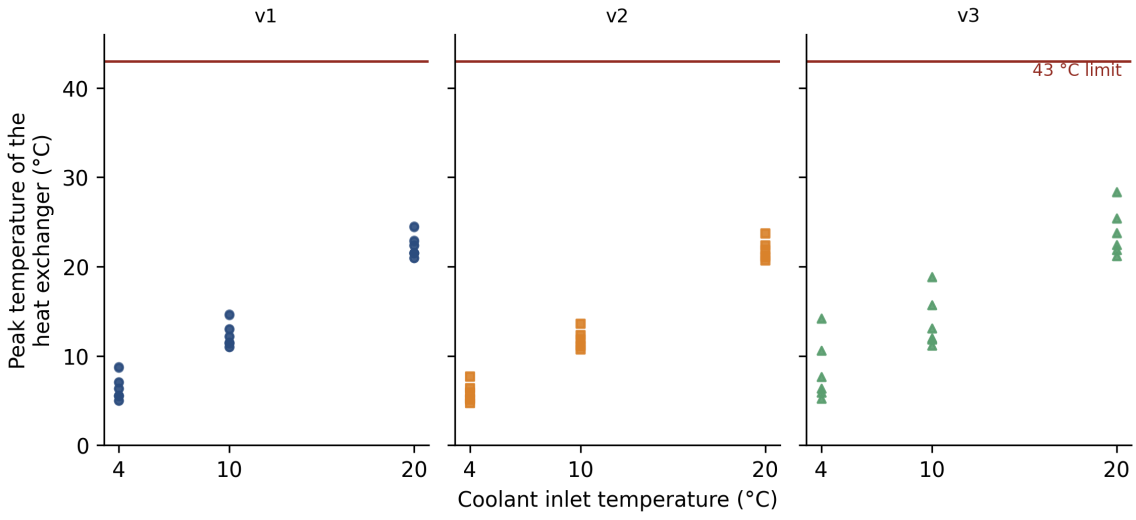


FIGURE 3.14. Peak temperature of each heat exchanger, the hottest point of the aluminum and the heated face, at its 36 operating points, with 10 and 15.5 W on every design, against the coolant inlet temperature; line, the 43 °C limit. The hottest point always lay on the heated face.

3.3.8. Integrated Control Board. The planned bench stages were completed, except that the reference did not stay in regulation under drive (below). The hard limits clamped the module at 3.93 A in cooling, 0.94 A in heating, and ±1.69 V. In closed loop the ring reached a cooling set point in about 30 s and held it within ±0.3 °C for 8 min. With the device on a preserved porcine eye in the bath, as in the ex-vivo experiments, the ring was pulled down from 31 to 4 °C in about 5 min and undershot to 2.0 °C. It then held 4.0 °C (3.8 to 4.3 °C) for the last 7 min of a run of 18.4 min (Figure 3.15A). The current monitor agreed with a multimeter in series within ±0.03 A from 0.48 to 3.93 A in cooling (Figure 3.15B). The safety cutouts that were tested stopped the module when triggered on purpose (Figure 3.15C).

The other functions of the board worked: the flow, pressure, and ambient sensors read, and the board switched the pump and the fan (Section F.2).

One fault remains to be fixed. While the module is driven, the 2.5 V reference of the board at times loses regulation, at the enable step and also during steady drive. The hardware current limits are set from that reference and cannot be relied on while it is disturbed. The module current is then limited only in duration, by the firmware cutouts, which stopped the module in every logged fault but one. At the start of the run in Figure 3.15A the reference dipped and the current reversed for a few seconds without a cutout, and the run continued. The cause was not found, and the board was therefore not used in the in-vivo study (Section 4.2.4).

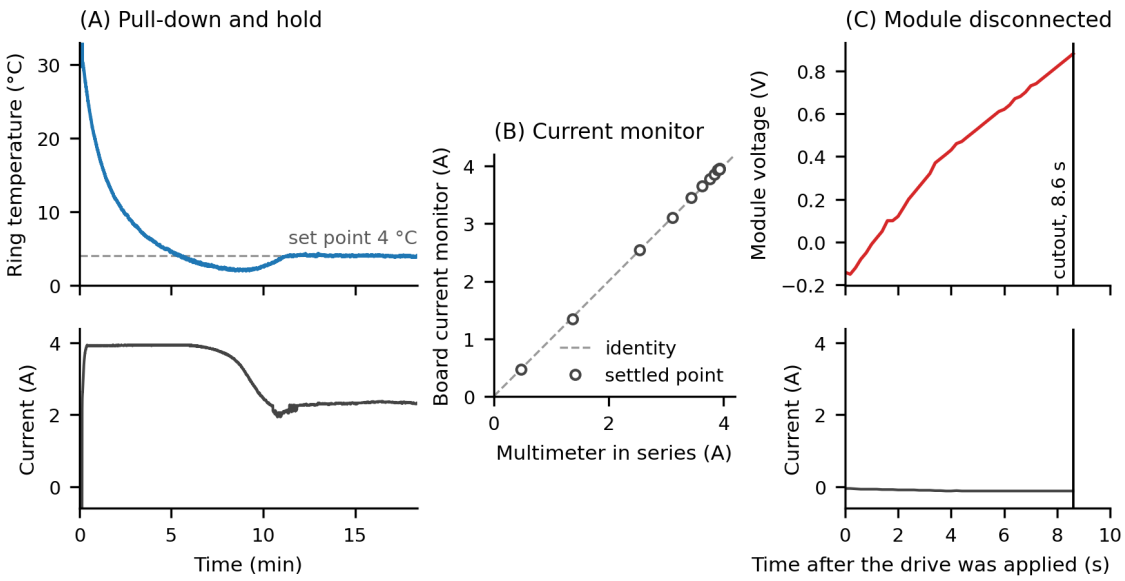


FIGURE 3.15. Bench performance of the control board. (A) Closed-loop pull-down of the ring from 31 °C to the 4 °C set point (dashed) and hold, with the module current, over 18.4 min; the current axis is clipped at −0.6 A, and in the first 9 s the current reversed to −1.5 A. (B) Current reported by the board against a multimeter in series, at 13 settled drive levels in cooling into a resistor; dashed line, identity. (C) Open-circuit cutout with the module disconnected on purpose: the voltage applied by the control loop rose while almost no current flowed, and the cutout stopped the drive 8.6 s after it was applied.

3.4. Discussion

Both refinements that were built mostly kept the cooling at the proximal optic nerve, the site of the hypothesis: the alumina ring within 1.0 °C of aluminum in paired human donor eyes, and the reduced-bulk ring about 0.6 °C less on average, by no more than 1.2 °C (0.05 °C in the model). The smallest heat exchanger was 81% smaller than that of Gen 3, and the controller was consolidated onto one board. None of them cooled the proximal optic nerve detectably more than Gen 3 did.

In the model, alumina kept 89 to 95% of the effectiveness of aluminum, whereas fused silica conducted too little heat to cool the eye as well as aluminum and was excluded without being built. At the other tissue sites the donor eyes showed a trend, not significant after correction, opposite to the model.

That no refinement cooled the proximal optic nerve detectably more than Gen 3 is consistent with the model. At every site the three ring geometries differed in cooling effectiveness ε by no more than 2 percentage points (9.6 to 9.9% at the proximal optic nerve), and the time to 90% of cooling did not change with the geometry. The temperature reached at depth is set mainly by the body temperature. At the proximal optic nerve a change of 1 °C in body temperature moves the final temperature by 0.90 °C and a change of 1 °C in ring temperature by 0.10 °C. At steady state in the model, with its nominal perfusion, the posterior eye sits between a cold source on the anterior sclera and a warm, perfused choroid and orbit, and its temperature is a fixed weighting of the two over the operating points tested, with an effectiveness of about 10% at the proximal optic nerve. The geometry of the ring therefore has a small effect on the cooling at depth, and deeper cooling would have to come from a colder ring or a different route to the posterior eye.

The insulating layer shows the difference between efficiency and effectiveness: on the original ring it raised the share of heat drawn from the eye from 33 to 36% without improving the temperatures reached. It is worth having for the module and the heat exchanger, which carry less load, and possibly for the patient, since the eyelids are cooled less.

Around the optic nerve head the retina is cooled no more than the proximal optic nerve, and the distal optic nerve, 1 cm behind the globe, is barely cooled at all. At a body temperature of 39 °C the proximal optic nerve would not reach 35 °C (35.6 °C with the ring at 4 °C), although the drop below body temperature, 3.4 °C, is no smaller than at 37 °C. The average retina reaches 32 °C with the ring at 11.7 °C, inside the 30 to 34 °C range of preclinical retinal protection, but the average is carried by the periphery. With the ring at 4 °C and the body at 37 °C the retina ranges from about 9 °C at its anterior edge to 33.7 °C at the posterior pole (Figure 3.12).

In the model of a perfused eye, at its nominal perfusion rates (Table 3.3) and a body temperature of 37 °C, a ring at the 4 °C floor of the device brings the proximal

optic nerve to 33.8 °C, inside the 32 to 35 °C range of systemic therapeutic hypothermia, but 32 °C there would need a ring well below freezing. This value depends more on blood flow than on anything else in the model. In extension runs of the Python program under the same conditions (ring v1 at 4 °C, body at 37 °C, 35 min of cooling), with every perfusion rate halved the proximal optic nerve reached 32.1 °C, and with every rate doubled 35.2 °C. The Pennes term assumes that blood leaves the tissue at tissue temperature [**133**, p. 116]. A lumped model of retinal heat balance that uses nearly the same choroidal perfusion rate (0.0963 s^{-1}) assumes instead an equilibration factor of 0.5 between blood and choroid, which halves the heat exchanged, as in the run with every perfusion rate halved [**134**]. The wall of the orbit, held at body temperature in the model, is the second influence: it keeps supplying heat to the orbital tissues, which can then cool only as far as their perfusion and metabolic heat allow. With the wall held there, the statement that 32 °C cannot be reached at the proximal optic nerve holds at the nominal perfusion rates and at half of them (32.1 °C).

An earlier Pennes model of a scleral ring from this research program reached a similar value at the optic nerve head, a maximum of 34.0 °C against 33.7 to 33.8 °C here, with different boundary conditions and perfusion rates [**55**]. Its multiphase successor, in which cooled blood carries the cooling posteriorly, predicts deeper cooling, 29.6 °C [**53**].

Before cooling the model is warmer than the eyes of patients measured at the start of surgery [**46**], 34.4 against 30.0 °C in the anterior chamber and 36.8 against 35.0 °C in front of the optic disc. The cooling it predicts for a given target temperature could therefore be conservative.

Model and donor eyes rank the sites identically and agree next to the ring, so the model reproduces the path of heat from the anterior sclera into the globe. At depth the donor eyes cooled more, probably because of the difference between the

two conditions (Section 3.3.6). The cooling measured ex vivo at depth is therefore likely to overstate that of a perfused eye, by a factor that remains to be measured.

Heat rejection did not constrain the design. On average the hot side ran within a few degrees of the coolant in every design, well under the 43 °C limit, which was examined in the model only because the hot side is not instrumented. With the coolant touching the module directly, the smallest heat exchanger kept the hot side closer to the coolant than the largest at the same heat and flow. As a result, most of the volume of the Gen 3 heat exchanger could be removed without thermal penalty. The price is pressure: the narrower channels of v2 and v3 need about 40 and 200 times the pressure of v1 (Table 3.6), and at the same pumping power v1 had the smallest rise. At 0.10 $\mathrm{L\,min^{-1}}$, v3 needs about 9 kPa, within the reach of a small pump.

The module itself has less margin than the heat exchanger. After 35 min the ring of the model drew 3.42 W through the cold face of the module, held at 4 °C (ring v1; 2.93 and 2.95 W for rings v2 and v3). In the heat-exchanger model the hot face of the module ran 0.3 to 20 °C above the cold face, from the coldest to the warmest coolant. At its maximum current of 3.9 A the module pumps about 3.5, 3.0, and 2.5 W with 0, 10, and 20 °C across it, read from the manufacturer's curves [**95**]. The margin is therefore small or negative: at full current the module about matches the load of the original ring with coolant at 4 °C (margin +0.07 to −0.09 W), and falls short by up to 0.4 W with coolant at 10 °C and 0.9 W at 20 °C; rings v2 and v3 fall short by up to 0.3 W at 20 °C. A module of the same series with shorter elements, RH14-14-06-L1-W4.5, is listed at 5.4 W with no temperature difference and is 3.8 mm thick against 4.7 mm [**135, 95**]. Scaled in the same way it would cover the load of every ring at all three coolant temperatures, but it needs 6.0 A, above the 3.9 A to which the present board is limited.

The integrated control board met its requirements on the bench, with one exception that prevented its use: the reference on which the hardware current limits

and the temperature measurements depend can lose regulation under drive. The firmware stopped the module in every logged fault but one, at the start of the run of Figure 3.15A, when the current reversed for a few seconds without a cutout. A safety argument cannot rest on the firmware alone, and the next revision will set the limits from an independent reference.

The model was not validated against a perfused eye, and its tissue properties and perfusion rates come from the literature (Appendix D). Perfusion was varied only in extension runs. The points sampled in the model are not in the same places as the implanted sensors at depth, and the model and the measurements are therefore compared only approximately there. They agree most closely at the anterior sclera. The lens of v3, the insulating layer, and the smaller heat exchangers were evaluated only in the model, and the ceramic ring only in the original geometry. The ex-vivo comparisons rest on nine eyes from five donors, with four to eight valid pairs per site. One ring of each kind was tested, so a difference between rings cannot be separated from the individual ring and its seating on the eye. The integrated control board was tested on the bench only. Patient comfort remains to be measured. The refinements carried into the in-vivo study were those that had been built and verified: the reduced-bulk aluminum ring on the Gen 3 heat exchanger, regulated by the Gen 3 controller (Section 4.2.4).

CHAPTER 4

Specific Aim 3: Validation and Safety Assessment in an In-Vivo Porcine Model

4.1. Background and Rationale

Neither the isolated eyes nor the computational model of Chapters 2 and 3 can answer the two questions on which any clinical use depends: whether cooling of this kind is tolerated by living ocular tissue, and whether it reaches the retina against an intact circulation. The isolated eye has no perfusion, and the bath that stood in for it resupplied less heat than a perfused eye receives. The ex-vivo excursions are likely an upper estimate of the cooling reached at depth (Section 2.4). Only in vivo is the choroidal heat load present (Section 1.2.3), and only in vivo can retinal function be measured while the eye is cooled (Section 1.3.3).

The tolerance of living ocular tissue to non-freezing cooling lasting tens of minutes to hours has not, to the author's knowledge, been established. The storage evidence that motivated the 4 °C set point (Section 2.2.1) comes from metabolically arrested, non-perfused tissue held in preservation solution. Non-freezing cold between 0 and 15 °C lasting several hours or days can injure perfused tissue, in four stages with vascular and sensory signs [**136**]. Cooling also has reversible effects specific to the eye: donor corneas swell when stored at 4 °C and thin again over the first hours of rewarming, as the endothelial pump resumes [**137**]. Pressure applied to the globe, as by a contact device, can also raise intraocular pressure and can itself occlude the retinal circulation [**6, 21**] (Section 1.1). These effects define the safety endpoints of this aim, listed in Section 1.8.

The objective of this aim was to evaluate, in a live porcine model, whether the device produces changes in the electroretinogram (ERG) consistent with retinal cooling and whether repeated cooling sessions of increasing duration are tolerated by the ocular surface, the globe, and the animal. The hypothesis is stated in Section 1.8.

4.2. Methods

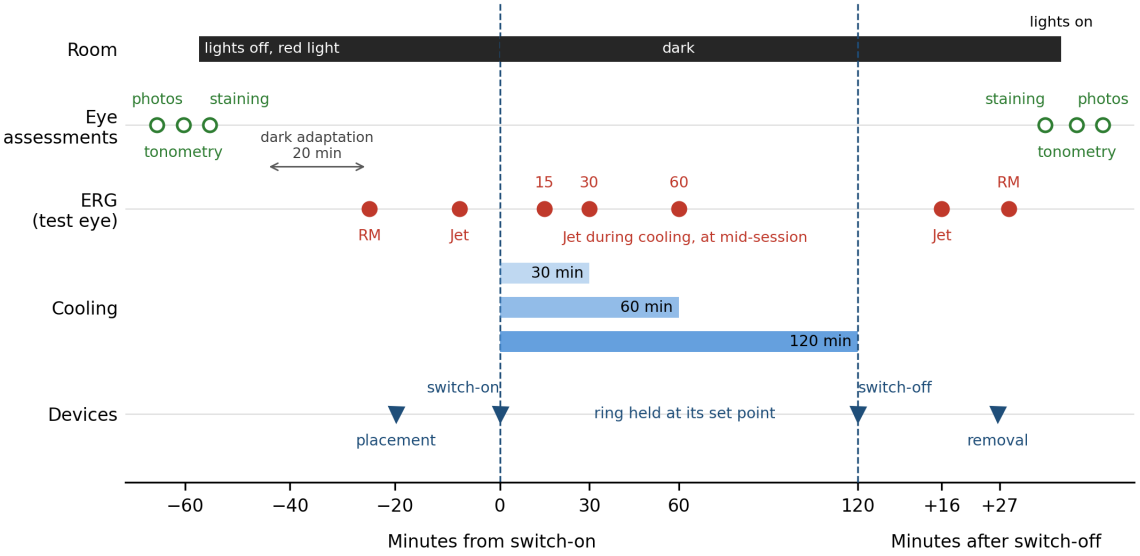


FIGURE 4.1. Sequence of a cooling session: baseline assessments, dark adaptation and baseline ERG, placement of both devices, switch-on, cooling with ERG recordings, switch-off, removal, and post-cooling assessments. Device and ERG times are typical of the fifteen sessions of animals 2 to 6; in animals 1 and 2 the number and times of the recordings during cooling varied. Black bar, room lights off and red light, from the corneal staining before cooling to that after.

4.2.1. Study Design and Approval. All procedures were approved by the University of Illinois Chicago Animal Care Committee under protocol `IPROTO202600000055` (approved April 30, 2026), with amendments `IAMEND202600000339`, `IAMEND202600000409`, `IAMEND202600000587`, and `IAMEND202600000642` (Appendix G). Each animal was scheduled for three cooling sessions of increasing length, 30, 60, and 120 min, and a terminal session, with daily observation on the intervening days. Figure 4.1 shows the sequence of a cooling session. The sessions were planned for days 1, 4, 7, and 10; the days performed and

the weight of each animal are given in Table 4.1. In the terminal session, except in animal 1, both devices were placed and a Jet ERG recorded, without cooling, before euthanasia, enucleation, and histology. Animal 3 died after its third cooling session and had no terminal session (Section 4.3.1). Baseline assessments were repeated before every cooling session, so that each session was referenced to the state of the eye on that day. Each session was performed under general anesthesia. In every session the right eye was the test eye, fitted with the active device, and the left eye was the control eye, fitted with an identical device that was not activated. Each animal thus served as its own control for the mechanical effects of the device, for anesthesia, and for systemic changes shared by both eyes.

FIGURE 4.2. Animals 3 and 4 in a pen of the Biologic Resources Laboratory the day after arrival, with wood-shaving bedding and toys for enrichment. During the study each animal had its own pen.

4.2.2. Animals. Six domestic pigs (*Sus scrofa*; farm strain, not genetically modified), the number the protocol provides for, were obtained from an approved vendor (Oakhill Genetics) and housed individually in the Biologic Resources Laboratory (BRL) of the University of Illinois Chicago (Figure 4.2). Five completed the study

and one died during it (Section 4.3.1); their weights are given in Table 4.1. They were less than a year old. The species was chosen for the similarity of its eye to the human eye in size, structure, and retinal vascular pattern (Section 1.6). The study was classified in U.S. Department of Agriculture (USDA) pain category D.

The number of animals was set for a paired comparison. Assuming a 30% change in the primary ERG measure, a standard deviation of 20%, a two-sided α of 0.05, and 80% power, the normal approximation gave 3.5, rounded to four animals, and two further animals were approved to allow for attrition, including equipment failure. The quantity of interest is the change of each eye from its own recording before cooling: a change in the test eye and none in the control eye over the same interval. The power obtained with the measured variability is given in the discussion (Section 4.4).

TABLE 4.1. Sex of each animal, day of each session (session 1 is day 1; session 4 is the terminal session), and weight (kg) from the BRL record of the session; ∼, written as approximate on the record. Animal 3 died after session 3.

Animal	Sex	Days	Session 1	Session 2	Session 3	Session 4
1	Male	1, 4, 8, 11	26.3	26.2	28.0	28.6
2	Male	1, 4, 8, 12	32.5	32.5	33.8	36.7
3	Male	1, 4, 8, –	33.4	∼33	34.8	–
4	Male	1, 4, 8, 11	32.8	∼33	35.1	36.0
5	Male	1, 5, 9, 15	28.9	30	32.8	35.2
6	Male	1, 7, 10, 15	30.4	32.7	∼33	34.1

4.2.3. Anesthesia and Physiological Monitoring. Anesthesia was induced and maintained by BRL staff under the approved protocol; the drugs and doses of each session are given in Appendix H. Each pig was sedated, intubated, ventilated, and maintained on isoflurane (1–2%).

From animal 2 onward, after the protocol had been amended, rocuronium was given intravenously once adequate anesthetic depth had been verified, to abolish eye movement; no neuromuscular block was used in animal 1. Animal 2 moved once, during session 2, and the active device came briefly out of the eye and was reinserted at once.

Flunixin was given for analgesia. Propofol, hydromorphone, and an intravenous infusion of lidocaine and ketamine were given at the discretion of the veterinarian or veterinary technician. The infusion was added after animal 2 moved, as an adjunct to isoflurane.

Electrocardiogram, pulse oximetry, capnography, indirect blood pressure, and rectal temperature (core temperature below) were monitored continuously and recorded every 10 to 20 min. Anesthetic depth was assessed from jaw tone and the palpebral reflex together with heart rate, respiratory rate, and blood pressure. Room temperature and relative humidity were logged at the start and end of each session. Forced warm air was used when needed and was withdrawn when core temperature rose.

If physiological parameters left the safety range during cooling, the protocol suspended cooling for 15–20 min. A session was ended if core temperature fell below 35 °C and did not stabilize with warming, if oxygen saturation remained below 90%, or if heart rate fell below the limits set by the veterinary staff. Heart rate was watched at device placement and at the start of cooling, because of the oculocardiac reflex.

4.2.4. Device Configuration. The device used in vivo was SCEC Gen 3 fitted with the aluminum reduced-bulk ring (the geometry modeled as v2 in Chapter 3), mounted on the Gen 3 heat exchanger. In the donor eyes this ring gave the proximal optic nerve 0.6 °C less cooling than the original ring on average, a difference unlikely to exceed 1.2 °C. The Gen 3 controller was set to hold the ring at a nominal 4 °C (Section 2.2.1), with the interruptions and the calibration error of Sections 4.3.2 and 4.2.6. In the first two animals the switching element was a relay, as in Aim 1. After the relay had failed in session 2 of animal 1 and in session 3 of animal 2, it was replaced, from session 1 of animal 3 onward, by a trigger switch module with two metal-oxide-semiconductor field-effect transistors (MOSFETs; ANMBEST, rated by the seller at 5–36 V direct current (DC) and 15 A). The module switched reliably and silently. A pump supplied the heat exchanger with distilled water cooled in an

ice bath (Appendix E). The control eye was fitted with an identical, inactive replica of the device, so that the two eyes received the same mechanical contact.

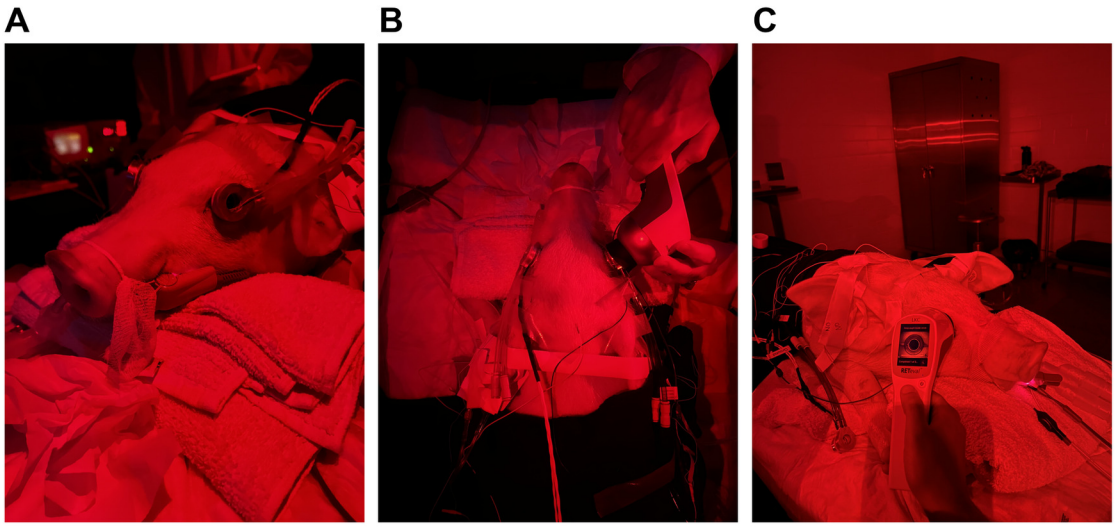


FIGURE 4.3. The cooling session in the animal laboratory, under red light: (A) the device on the eye of an anesthetized pig and (B) an ERG recorded with the Jet electrode through its central opening (animal 1, session 1); (C) the screen of the ERG recorder during a recording (animal 2, session 3).

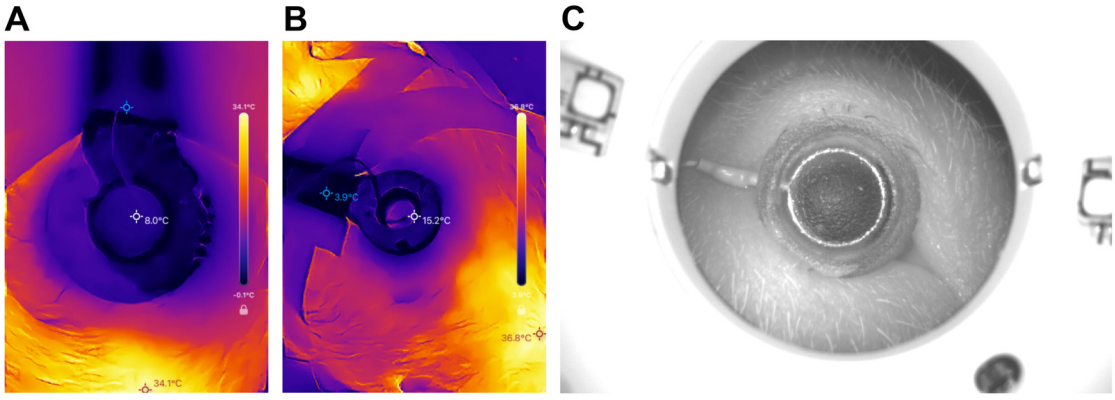


FIGURE 4.4. The device on the eye in thermal images ((A) animal 6, session 1; (B) animal 4, session 3) and (C) the RetMap (RM) corneal electrode on the eye seen by the pupil camera of the ERG recorder (animal 5, session 1). The thermal images are from a phone camera without emissivity correction; the reflective metal parts do not read true temperature, so the images show only the pattern of cooling.

4.2.5. Device Application. From animal 2 the room lights were turned off for corneal staining. The room was kept dark (Figure 4.3C), with every light source covered by a red filter screen, from the start of dark adaptation until after the last corneal staining of the post-cooling assessments. After the baseline assessments (Section 4.2.8), an ophthalmic lubricant ointment (Puralube Vet Ointment, white petrolatum and mineral oil, Dechra) was applied to the eye-contacting side of the ring of each device. The ring was placed on the eye by sliding its skirt under the eyelids with the help of tissue tweezers. The active device was placed on the right eye first and the control device on the left eye 2 to 6 min later, and both were secured with tape (Figure 4.4). The coolant circuit, temperature sensor, and power of the active device were connected while the coolant pump and the thermoelectric drive were off; both were started together at switch-on. A corneal ERG electrode (Jet) was placed through the central opening of each device onto the cornea.

The ring of the active device read about 16 to 21 °C before it was placed. Both devices stayed on until the ring had rewarmed to between 20 and 35 °C and were then removed; the post-cooling assessments followed.

4.2.6. Temperature Monitoring. The temperature of the ring of the active device was measured by the thermistor embedded in it, through the ring feedback channel and data-acquisition unit of Aim 1 (Section 2.2.4; DI-2108 unit, WinDaq software), at 10 Hz. The drive current was recorded on a second channel.

Before each session the temperature channel was calibrated in the dry-block calibrator at 37.0, 28.7, 20.5, 12.3, and 4.0 °C. This channel is not the linearized channel of Aim 1. Its readings were converted to temperature with a model of the measuring circuit fitted to the plateaus of that session (Section A.1). A fit to the three original plateaus reproduced the other two within 0.06 to 0.23 °C, and within 0.03 °C with the corrected calibration described below; this is taken as the accuracy of the ring temperature.

Ring temperatures of the sessions before the well thermistor was held against the aluminum are reported with 5.5 °C added, and the set point that read 4 °C in those sessions was about 9.5 °C. Up to and including session 1 of animal 5 the well thermistor used for this calibration did not touch the aluminum of the well and could not shed its self-heating as the ring thermistor does in the ring. From the next session, session 1 of animal 6 on the following day, a custom piece held it in contact with the aluminum; this arrangement, the corrected calibration, is taken as correct. Before session 2 of animal 6 the channel was calibrated both ways on the same morning: against the corrected calibration, the earlier arrangement read the ring 5.5 to 5.6 °C low from 4 to 25 °C and 5.2 °C low at 35 °C. Animal 5 shows the same shift between its first two sessions, 5.3 to 6.0 °C before placement and on the eye after switch-on.

No sensors were implanted in vivo. Core temperature was recorded rectally by the anesthesia team.

4.2.7. Assessment of Retinal Function. Full-field ERGs were recorded under dark adaptation. The corneas were anesthetized with proparacaine hydrochloride (0.5%) and the pupils dilated with phenylephrine hydrochloride (2.5%) and tropicamide (1%), one to two drops each, given in that order, and the eyes were dark adapted in the darkened room (Section 4.2.5). Dark adaptation was planned to last 15 min in animal 1 and 20 min from animal 2 onward, and the first recording was made at its end. In the 120-min sessions the drops were applied again during the session.

An ophthalmic lubricant gel (Celluvisc, carboxymethylcellulose sodium 1%, Allergan) was applied to the cornea as the coupling medium of the electrode. Sterile subcutaneous needle electrodes served as the ground, on top of the neck, and as the references, one for each eye between the eye and the ear, and stayed in place for the whole session. The active electrode was an RM (RetMap) corneal electrode before the device was placed and after it was removed, and a Jet electrode, through the central opening of the device, with the device in place. Responses were recorded with

a handheld full-field ERG system with an integrated Ganzfeld stimulator (RETeval, LKC Technologies, Gaithersburg, MD, USA), using its scotopic protocol for rabbit, mouse, and pig, with white flashes of three strengths: 0.06, 8, and 25 cd·s/m^2, presented at 0.5, 0.1, and 0.05 Hz. The two eyes were recorded in turn, the test eye first. The response to the weakest flash was too small and variable to be measured reliably, and the results are those at 8 and 25 cd·s/m^2.

In animals 1 and 2 the number and the times of the recordings differed between sessions. In three of these sessions (session 2 of animal 1, sessions 1 and 2 of animal 2) the Jet recording after switch-off was made within 2 min of switch-off, with the ring still below 13.5 °C. In these sessions it records the end of cooling rather than recovery. Session 3 of animal 1 had no Jet recording before cooling or after switch-off. From animal 3 the times were fixed: one recording at about the middle of cooling, and the Jet recording 14 to 25 min after switch-off (median 18 min), with the ring partly rewarmed. The last recording of each session was made with the RM electrode after the devices had been removed. The ring temperature at each recording was read from the cooling record (mean of the 2 min around the recording) in all sessions except session 3 of animal 3, whose cooling record is missing (Section 4.3.2).

The analysis used the raw record of each flash step, as set out in Section C.3: a 0.3 Hz high-pass filter removed drift [**138**], each sweep was referred to the 20 ms before its flash, and sweeps with artifacts were rejected before averaging. For the a-wave and b-wave the average was low-pass filtered at 75 Hz, so that an oscillatory potential could not displace the b-wave peak, a departure from the standard. At 8 and 25 cd·s/m^2 the oscillatory potentials (OPs) were band-pass filtered from 75 to 300 Hz, as the standard sets out, and measured as the sum of the amplitudes of their positive peaks, OP1 to OP4, or OP1 to OP3 where only three were present (79 of the 388 traces of animals 1 to 6). The a-wave amplitude was measured from the pre-stimulus baseline and the b-wave amplitude from the a-wave trough.

Dark adaptation in animals 1 and 2 lasted 14 to 23 min, and in animals 3 to 6 20 to 23 min, against the standard's minimum of 20 min.

Recordings were grouped by electrode and phase (RM before placement; Jet before switch-on; Jet during cooling; Jet after switch-off; RM after removal).

4.2.8. Ocular Safety Assessments. Both eyes were assessed at the start of each session, before the devices were placed, and again after they were removed. At the start, the corneal staining came after the photographs and tonometry and before the drops and the ERG; it preceded the ERG by 24 to 37 min in animals 1 to 6 (in animal 1 the order differed; Section 4.2.9). From animal 2, after the session the corneal staining was done with the lights off, and tonometry and the photographs followed with the lights on.

Ocular irritation. The ocular surface of each eye was photographed, and bulbar and limbal redness were graded from the photographs in whole steps on the 0-to-4 scales of the Johnson & Johnson Clinical Assessment Guide named in the protocol [**139**], which are adapted from the Efron grading scales [**140**]. Tarsal redness, also listed in the protocol, was not graded.

Intraocular pressure. A handheld rebound tonometer (TonoLab, Icare Finland Oy, Vantaa, Finland) was held perpendicular to the central cornea, with the lids retracted without pressure on the globe, and one to three readings per eye were averaged. The instrument is calibrated for the rodent eye and not validated for the porcine eye, and its readings might carry a systematic offset. They are used only to compare the two eyes of an animal and the same eye before and after a session. At the start of the session the pressure was measured before any topical anesthetic had been applied.

Corneal staining. The eye was flushed with sterile saline and stained with fluorescein, from a wetted sterile strip touched to the conjunctiva away from the central cornea. The excess was flushed away and the surface was photographed in the dark through a cobalt-blue filter. The staining was graded from 0 to 4 for each eye by the author on one photograph per eye and time point, chosen as for redness.

Observation. Between sessions each animal was observed daily for ocular swelling, squinting, and corneal ulceration, and was weighed, scored for body condition, and checked for food and water intake.

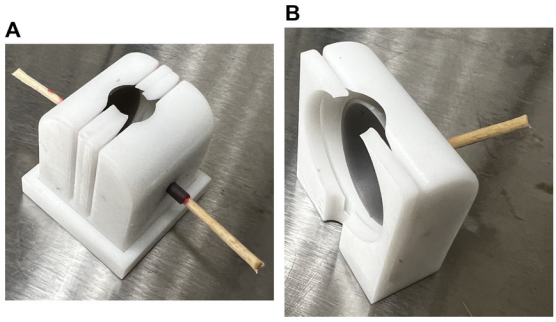


FIGURE 4.5. The 3D-printed guide used to trim the fixed globes for histology, front (A) and side (B). The globe sits in the spherical cavity, centered by a small shell pushed in with the two sticks; the two parallel slots, about 5 mm apart, guide the blade.

Histology. In the terminal session, after the assessments, each animal except animal 3, whose eyes were enucleated after its death (Section 4.3.6), was euthanized under anesthesia with an intravenous overdose of pentobarbital sodium (Fatal-Plus, 390 mg/mL; 10 mL), a method acceptable for swine under the American Veterinary Medical Association (AVMA) Guidelines for the Euthanasia of Animals [**141**]. Death was confirmed by the absence of respiration, cardiac activity, corneal reflex, and muscle tone. Both eyes were enucleated immediately and fixed, in animal 1 by immersion in formalin and in animal 2 by immersion in formalin with an injection of formalin into the globe. From animal 3 onward modified Davidson's fixative was used, injected into the globe and by immersion, to limit retinal detachment. After fixation each globe was trimmed in a 3D-printed guide designed for this study (Figure 4.5), leaving a central slab containing the optic nerve head and the cooled region. The eyes were sectioned from the cornea to the optic nerve and processed by the UIC histology core for hematoxylin and eosin staining of the cornea, sclera, conjunctiva, and peripheral and posterior retina. The stained sections of all six animals were imaged together with a digital microscope (VHX-6000, Keyence, Osaka, Japan) at the Robbins and

Jacobs Family Biocompatibility and Implant Pathology Laboratory (Rush University Medical Center, Chicago, IL, USA), and were read by the author.

4.2.9. Data Analysis.

Animals and groups. Animal 1 is exploratory, as the procedure was still being set up (no neuromuscular block, other premedication, assessments reordered, staining not yet in the dark); it is left out of the ERG, intraocular pressure, and ocular surface results and of every test, and appears, marked as exploratory, only in the device operation, ring temperature, systemic course, and histology. The tests were made over animals 2 to 6, the primary animals, and all results are those of animals 2 to 6 unless stated otherwise. Session 3 of animal 3, after which the animal died (Section 4.3.6), is left out of the ERG, intraocular pressure, and ocular surface results: its cooling was not recorded, and after switch-off the ERG of both eyes changed together, pointing to a systemic cause. During cooling its test eye changed and its control eye did not, and including it would not change the result for the b-wave implicit time. This left 14 cooling sessions for analysis. The results are given over both ring temperatures, about 9.5 °C (animals 2 to 4, session 1 of animal 5) and about 4 °C (animal 5 from session 2, animal 6); the two are given separately only where that changes a result.

ERG measures. Cooling prolongs the implicit times and reduces the amplitudes of the ERG [**72, 73, 75**]. By slowing the response it should also widen the b-wave, and more so if the retina cools unevenly, which spreads the timing of its parts. The width was therefore measured on the filtered trace: the time above 80% of the b-wave amplitude, from the a-wave trough, up to the first trough after the peak, so that a later wave is not counted. The primary measures were the b-wave implicit time and amplitude and the summed oscillatory potentials, each at 8 and 25 cd·s/m^2. The width of the b-wave was a secondary measure.

Evidence of retinal cooling. Each eye was compared with itself, from its Jet recording before cooling to that during cooling, with the device installed: the test eye shows

the effect of cooling, the control eye whether the same change occurred through anesthesia, time, or mechanical contact. The two changes were also compared with each other, which is the more sensitive test. The b-wave amplitudes of the two eyes of normal subjects differ by a median of 8 to 11% [**142**], in minipigs no ERG measure differed significantly between right and left eyes [**143**], and anesthesia and core temperature act on both alike. In normal subjects, by contrast, one eye recorded on different days must fall in amplitude by 17 to 29%, or rise by 20 to 42%, before the change is significant [**144**]. A change in amplitude alone was not taken as evidence of cooling, because amplitude also depends on electrode contact and position. A session was counted as showing cooling when the b-wave of the test eye had both a longer implicit time and a smaller amplitude than before cooling at both primary flash strengths. As an exploratory addition, the same session-by-session judgment was made on the oscillatory potentials, from a later OP2 peak and a smaller summed amplitude. Each change is given in the unit of the measure; the fall of the summed oscillatory potentials is also given as a percentage of the eye's own earlier value.

Safety. The first safety criterion was the return of the test eye's ERG to its pre-cooling values after rewarming, judged on the Jet recordings before cooling and after switch-off and confirmed on the RM recordings before installation and after removal of the devices, which span the session. A measure was restored when the test eye's change from its own baseline lay within twice the standard deviation of the control eye's change over the same interval, pooled over the primary animals and sessions, for each electrode, flash, and measure. A Jet recording after switch-off counted as a recovery recording only at least 10 min after switch-off; these were made at 11 to 25 min, with the ring 73 to 97% rewarmed. The two made within 2 min (sessions 1 and 2 of animal 2), ring below 13.5 °C, record the end of cooling; they were left out of the comparison and its spread, and those sessions were judged on the RM pair alone. The change from the recording during cooling to the recovery recording shows how far the ERG came back as the ring rewarmed. A change persisting between sessions was

sought by comparing the Jet recordings at the start of sessions 2 to 4 with session 1, confirmed with the RM recordings, together with the core temperature, which alters the ERG [**120**]. Within a session the control eye shows the effect of core temperature. When the test eye had not returned, the control eye was checked. If both eyes had changed in the same direction by more than the control eyes usually changed, the change was attributed to a systemic cause. If the test eye had come part of the way back from its change during cooling while the control eye had not changed, the pattern was read as a recovery still under way when recorded.

The second criterion was no harmful increment: intraocular pressure, redness, and corneal staining should rise in the test eye no more than in the control eye, with no increase persisting to the next session, and the histology of the test eye should show no damage absent from the control eye. Safety was not inferred from the absence of a significant difference, which is expected with five animals even when a real effect exists.

Statistical considerations. Each session length was analyzed separately. The change of each eye from before to during cooling, and the test eye's change against the control eye's, were tested with paired t tests over the animals. For each measure and flash, the p value of the test-against-control comparison was Holm-corrected across the three session lengths. Between sessions, the same comparison was made for each later session against session 1, Holm-corrected across sessions 2 to 4. Effect sizes are Hedges' g of the paired test-minus-control differences. For restoration, intraocular pressure, and the comparison between sessions, the 90% confidence interval of the mean test-minus-control difference gives, by its limit farther from zero, a difference larger than which is unlikely (one-sided 95%). The redness and staining grades are given as the median and range over the animals for each eye and time point, with the number of sessions in which the grade of the test eye changed by at least one step more toward harm than that of the control eye. The session lengths

were always given in the order 30, 60, 120 min in the same animals, and a trend across sessions is confounded with repeated anesthesia and cumulative cooling.

4.3. Results

4.3.1. Animals and Sessions. Animals 1, 2, 4, 5, and 6 completed all four sessions. Animal 3 died in recovery from its third cooling session (Section 4.3.6) and had no terminal session. Animal 1, exploratory, appears only in the device operation, ring temperature, systemic course, and histology (Section 4.2.9). All 18 cooling sessions were given for their planned length.

4.3.2. Device Operation and Ring Temperature. The ring temperature of the 17 sessions with a record, corrected for the calibration error (Section 4.2.6), is shown in Figure 4.6. The cooling of session 3 of animal 3 was not recorded: the recording, started at 09:22, stopped at 10:32, before switch-on, after the recording computer had gone into standby. The session log gives switch-on at 10:42 and switch-off at 12:42.

In all the recorded sessions, the ring, placed on the eye, read 27.5 to 32.3 °C before switch-on (27.6 to 31.0 °C in animals 2 to 6). In the sessions at about 9.5 °C it came within 0.5 °C of its level 0.8 to 2.9 min after switch-on, and the controller began to switch the drive 1.0 to 3.1 min after switch-on. The ring was then held at a mean of 9.1 to 10.0 °C, with a standard deviation of the 1-s means within a session of at most 0.14 °C (0.29 °C in session 2 of animal 1). With the corrected calibration it took 3.9 to 10.3 min to reach its level, 4.0 to 4.1 °C. In the first two (session 1 of animal 6, session 2 of animal 5) the drive stayed on nearly all the time, the device at its limit near a true 4 °C.

With the relay (animals 1 and 2) the drive was interrupted in two of the six sessions. The ring never fell below 3.8 °C, above the 2 °C below which the risk of cold injury to stored organs rises [**121**] (Section 2.2.1). With the MOSFET module (animals 3 to 6, 11 sessions with a record) the drive ran uninterrupted. In its sessions

at about 9.5 °C the ring stayed between 8.9 and 10.2 °C from 2 min after the controller began to switch until 1 min before switch-off.

Each of animals 2 to 6 had the device switched on for 210 to 211 min over its three cooling sessions.

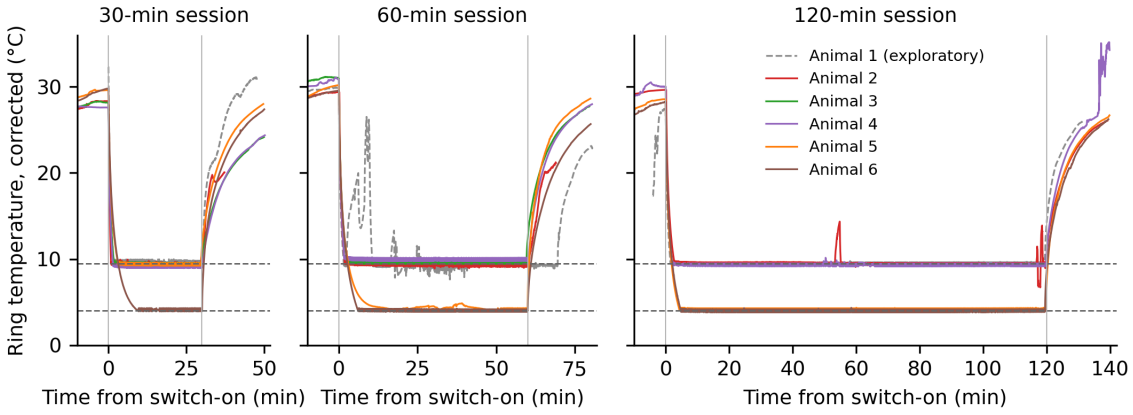


FIGURE 4.6. Temperature of the ring of the active device in each cooling session against time from switch-on, from placement or 10 min before switch-on, whichever is later, to removal or 20 min after switch-off, whichever is earlier; 1-s means of the 10-Hz record, corrected for the calibration error. Dashed lines, the set point in the two calibrations, 9.5 and 4 °C; vertical lines, planned switch-on and switch-off. Animal 1, exploratory, dashed gray line. Session 3 of animal 3 has no record of the cooling. The rise in the trace of animal 4 before removal in the 120-min session coincides with the post-cooling ERG, recorded with the device still on the eye.

4.3.3. Electroretinogram. The analysis used 78 recordings of both eyes at the two stronger flashes (312 traces; mean waveforms in Figure 4.7). The trace checks of Section C.3 flagged 16 of the 156 traces at 25 cd·s/m^2 and 15 at 8 cd·s/m^2, 12 and 15 of them for residual drift; the checks leave the values unchanged.

Cooling. Of the 15 cooling sessions of animals 2 to 6, 14 were analyzed. Session 3 of animal 3, after which the animal died (Section 4.3.6), was left out: in it the ERG of both eyes moved beyond the usual range of the control eyes in the same direction, with both electrodes, pointing to a systemic cause rather than the cooling. Of the 14, nine were with the ring at about 9.5 °C (four, three, and two after 30, 60, and

120 min) and five at about 4 °C (one, two, and two). The changes are shown in Figure 4.8 and Tables 4.3 and 4.4.

The b-wave of the test eye slowed in all 14 sessions, by 2.0 to 8.0 ms at 25 cd·s/m^2 and 1.5 to 8.5 ms at 8 cd·s/m^2, while the control eye changed by at most 2.5 ms either way. The mean slowing of the test eye was 5.1 to 5.8 ms at each session length and flash, and the mean change of the control eye −0.5 to 0.8 ms (Tables 4.3 and 4.4). Against the control eye the slowing was significant after Holm correction at every session length and at both flashes, with large effects.

The b-wave amplitude of the test eye fell in 10 of the 14 sessions at each flash, by a mean of 67 μV (18%) at 25 cd·s/m^2 and 56 μV (14%) at 8 cd·s/m^2. Against the control eye no session length was significant after Holm correction (Tables 4.3 and 4.4). The amplitude of the control eye fell in about half the sessions and rose in the rest, by up to about 130 μV either way, and its mean change was near zero (+12 μV at 25 cd·s/m^2, −2 μV at 8 cd·s/m^2).

The b-wave of the test eye widened in all 14 sessions at each flash, and that of the control eye in 13. On average the test eye widened by 4.1 ms against 1.6 ms in the control eye at 25 cd·s/m^2, and by 3.3 against 1.8 ms at 8 cd·s/m^2.

The oscillatory potentials of the test eye were slower in 13 of the 14 sessions at 25 cd·s/m^2 and 12 at 8 cd·s/m^2, the OP2 peak later by a mean of 1.5 and 1.1 ms, and those of the control eye in 1 and 3, with a mean change of the OP2 peak of −0.5 ms at each flash. Their summed amplitude fell in the test eye in 11 of the 14 sessions at 25 cd·s/m^2 and 13 at 8 cd·s/m^2, and in the control eye in 5 and 6, no more often than it rose. Against the control eye no session length was significant after Holm correction at either flash (Tables 4.3 and 4.4). At 25 cd·s/m^2 the fall in the test eye grew with the session length: 19, 49, and 55% after 30, 60, and 120 min; at 8 cd·s/m^2 it was 39% over the three session lengths.

With implicit time and amplitude taken together (Section 4.2.9), a slower and smaller b-wave was found at both flashes in the test eye in 10 of the 14 sessions and

in the control eye in 3, and slower and smaller oscillatory potentials, the exploratory judgment, in the test eye in 9 and in the control eye in none.

Restoration. Of the 14 sessions, 12 had a recovery recording with the Jet electrode, made 11 to 25 min after switch-off with the ring 73 to 97% rewarmed. In the other two, sessions 1 and 2 of animal 2, the Jet recording after switch-off was made within 2 min, too early to count as a recovery recording, and those two sessions were judged on the RM recordings only. The b-wave implicit time of the test eye was restored in 11 of the 12 at 25 cd·s/m^2 (limit ±3.7 ms) and in 8 of the 12 at 8 cd·s/m^2 (limit ±2.8 ms). In the others it was still 3.0 to 4.5 ms slower than before cooling: session 1 of animal 3 at both flashes, and session 3 of animal 2, session 2 of animal 3, and session 1 of animal 4 at 8 cd·s/m^2. In each of these the test eye had come part of the way back, by 25 to 46% of its slowing during cooling, while the control eye changed by at most 2 ms: the pattern of a recovery not yet complete in the 11 to 25 min between switch-off and the recording, rather than of a lasting change.

The amplitude was restored in all 12 at each flash, the width in 11 at 25 cd·s/m^2 (not session 2 of animal 6) and all 12 at 8 cd·s/m^2, and the summed oscillatory potentials in 11 at 25 cd·s/m^2 (not session 2 of animal 3) and 10 at 8 cd·s/m^2 (not session 3 of animal 2 or session 2 of animal 3). In these three the oscillatory potentials of the test eye were still as low as during cooling, but those of the control eye had also fallen after switch-off, by 107 to 132 μV, so the pattern does not separate an incomplete recovery of the test eye from a change in both eyes. With the RM electrode, over the whole session, the implicit time was restored in all 14 sessions at 25 cd·s/m^2 and 13 at 8 cd·s/m^2, the amplitude in 13 at each flash, the width in all 14 at each flash, and the oscillatory potentials in all 14.

By the 90% confidence intervals, a lasting lengthening of the implicit time by more than 3.6, 3.2, and 5.8 ms after 30, 60, and 120 min at 25 cd·s/m^2, and 3.4, 2.4, and 4.8 ms at 8 cd·s/m^2, is unlikely with the Jet pair. A fall of the amplitude by more than 92 and 231 μV after 60 and 120 min at 25 cd·s/m^2, and 41 and 197 μV

at 8 cd·s/m^2, is also unlikely. After 30 min the amplitude of the test eye came back higher than that of the control eye (mean +89 μV, 90% confidence interval +34 to +144, at 25 cd·s/m^2; +122 μV, +36 to +208, at 8 cd·s/m^2). With the RM pair, as confirmation, after 60 min the amplitude of the test eye was lower than that of the control eye (mean −122 μV, 90% confidence interval −173 to −70), although each session but one lay within the restoration limit.

In two of the measures that were not restored, both eyes changed together, pointing to a systemic cause: with the RM electrode, the amplitude at both flashes in session 2 of animal 3.

Between sessions. The Jet recording at the start of each later session was compared with that of session 1 (Table 4.2). There was no significant difference between the test and the control eye in the b-wave implicit time, amplitude, or width, or in the summed oscillatory potentials, at either flash. The mean change of the width from session 1, test minus control, lay between −2.2 and +1.8 ms at every session. By the 90% confidence intervals, a lasting slowing of the test eye by more than 1.3 ms at the start of session 2 is unlikely at 25 cd·s/m^2, and a slowing of any size at session 3, and at sessions 2 and 3 at 8 cd·s/m^2, where the upper limits were below zero. A lasting fall of the amplitude is bounded in the same way by the lower limits in Table 4.2. The terminal session rests on four animals, and its intervals are wider. The RM recordings before installation showed implicit-time changes of the same size, so no slowing of the test eye persisted to the next session; their summed oscillatory potentials were larger in the test eye than in the control eye at the terminal session (+184 and +144 μV at 25 and 8 cd·s/m^2, p_{Holm} 0.055 and 0.018). At the Jet recordings the core temperature was within 1.3 °C of that in session 1 (1.8 °C at the RM recordings).

TABLE 4.2. Change of the Jet ERG at the start of each later session from that of session 1, test minus control eye, mean (90% confidence interval) over animals 2 to 6; p_{Holm}, the smallest over the three sessions of the paired t test. Flash in cd·s/m^2; session 3 of animal 3 is left out; n, animals.

Measure	**Flash**	**Session 2** ($n = 5$)	**Session 3** ($n = 4$)	**Terminal** ($n = 4$)	p_{Holm}
Implicit time (ms)	25	−1.1 (−3.5, +1.3)	−1.8 (−3.0, −0.5)	+0.4 (−1.4, +2.2)	0.13
	8	−1.4 (−2.5, −0.3)	−1.2 (−2.0, −0.5)	+0.1 (−0.6, +0.9)	0.09
Amplitude (μV)	25	−8 (−74, +58)	−32 (−113, +49)	−108 (−267, +51)	0.63
	8	+14 (−65, +94)	−17 (−122, +89)	−55 (−158, +48)	0.90
Summed OPs (μV)	25	+69 (−37, +176)	+54 (+12, +97)	−64 (−184, +56)	0.17
	8	+92 (−2, +187)	+82 (+7, +158)	−17 (−164, +129)	0.25
Width at 80% (ms)	25	−2.2 (−4.5, +0.2)	−0.8 (−3.0, +1.5)	+1.1 (−2.0, +4.2)	0.36
	8	+0.0 (−1.5, +1.5)	+0.1 (−1.6, +1.7)	+1.8 (−1.6, +5.2)	0.89

TABLE 4.3. The b-wave and the summed oscillatory potentials at 25 cd·s/m^2 (the width is a secondary measure): mean change of each eye from the Jet recording before cooling to that during cooling, over both ring temperatures (five, five, and four sessions at 30, 60, and 120 min), with the 95% confidence interval of the test-minus-control difference in parentheses. p_{Holm}, paired t test of the change of the test eye against that of the control eye, corrected across the three session lengths within each measure and flash. g, Hedges' g of the test-minus-control differences, a point estimate (four or five sessions). Session 3 of animal 3 is left out.

Measure	**Min**	**Test eye**	**Control eye**	**Test against control**	p_{Holm}	g
Implicit time (ms)	30	+5.8	+0.8	+5.0 (+1.7, +8.3)	0.027	+1.53
	60	+5.5	+0.2	+5.3 (+1.8, +8.8)	0.027	+1.52
	120	+5.2	−0.5	+5.8 (+2.7, +8.8)	0.027	+2.21
Amplitude (μV)	30	−56	−3	−52 (−100, −4)	0.116	−1.08
	60	−101	+46	−147 (−326, +32)	0.170	−0.82
	120	−41	−13	−28 (−248, +192)	0.714	−0.15
Oscillatory potentials (μV)	30	−40	+46	−86 (−185, +13)	0.137	−0.86
	60	−132	+5	−137 (−258, −17)	0.103	−1.13
	120	−129	+36	−164 (−352, +23)	0.137	−1.01
Width at 80% (ms)	30	+4.8	+1.7	+3.1 (−2.0, +8.3)	0.337	+0.60
	60	+4.5	+1.4	+3.1 (−0.2, +6.4)	0.177	+0.94
	120	+2.6	+1.5	+1.1 (−3.0, +5.2)	0.454	+0.31

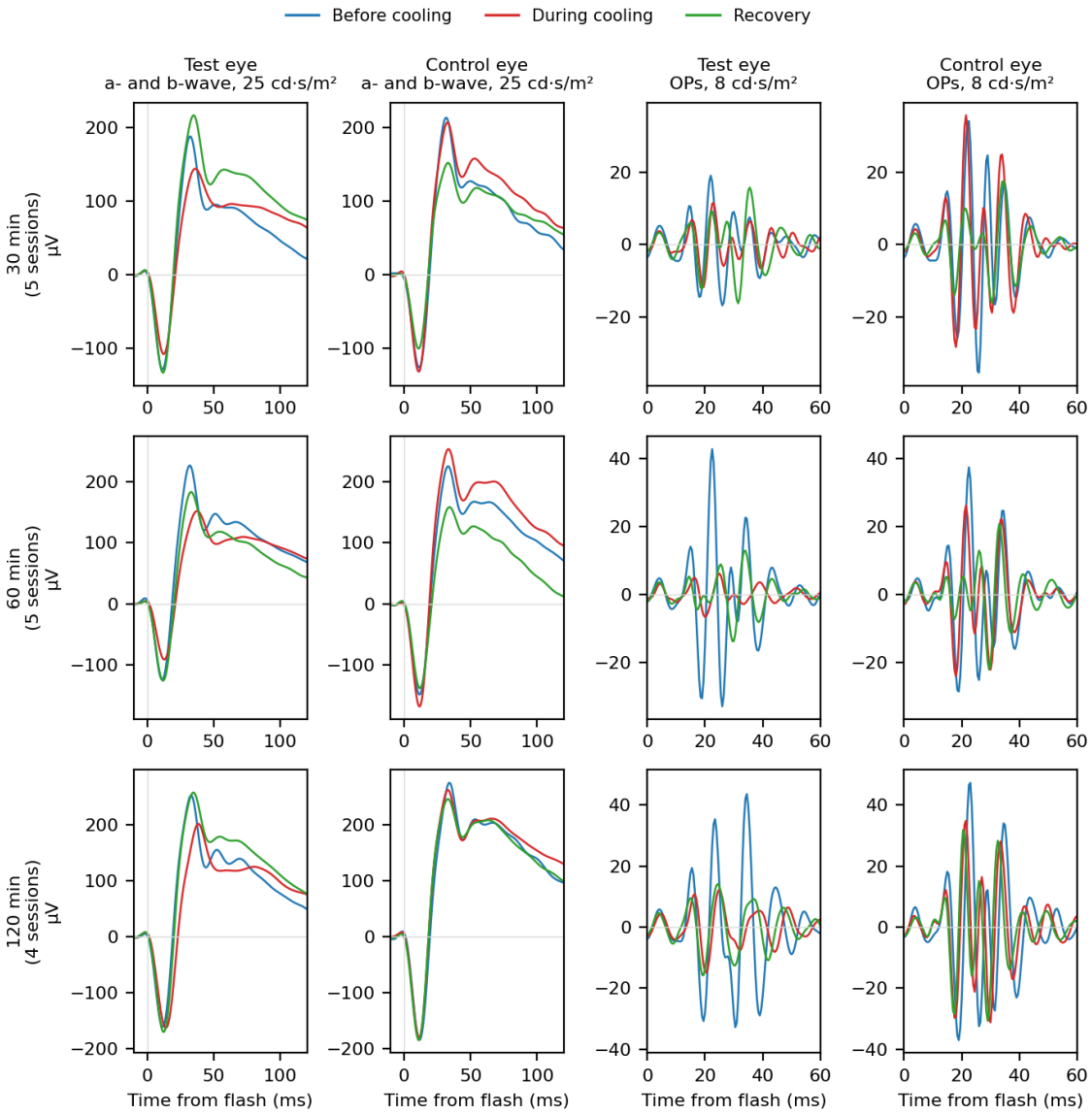


FIGURE 4.7. Mean electroretinograms of animals 2 to 6 in the 30-, 60-, and 120-min sessions (rows; five, five, and four sessions, both ring temperatures), Jet electrode: before cooling (blue), during cooling (red), and at the recovery recording after switch-off (green); the recovery mean includes only recordings made at least 10 min after switch-off, so it leaves out animal 2 at 30 and 60 min, recorded 0 and 2 min after switch-off (four sessions in each row). Session 3 of animal 3 is left out. First two columns, a- and b-wave of the test and control eyes at 25 cd·s/m^2; last two, oscillatory potentials at 8 cd·s/m^2 (75–300 Hz). Averaging recordings whose peaks fall at different times lowers and broadens the mean peaks, the oscillatory potentials most.

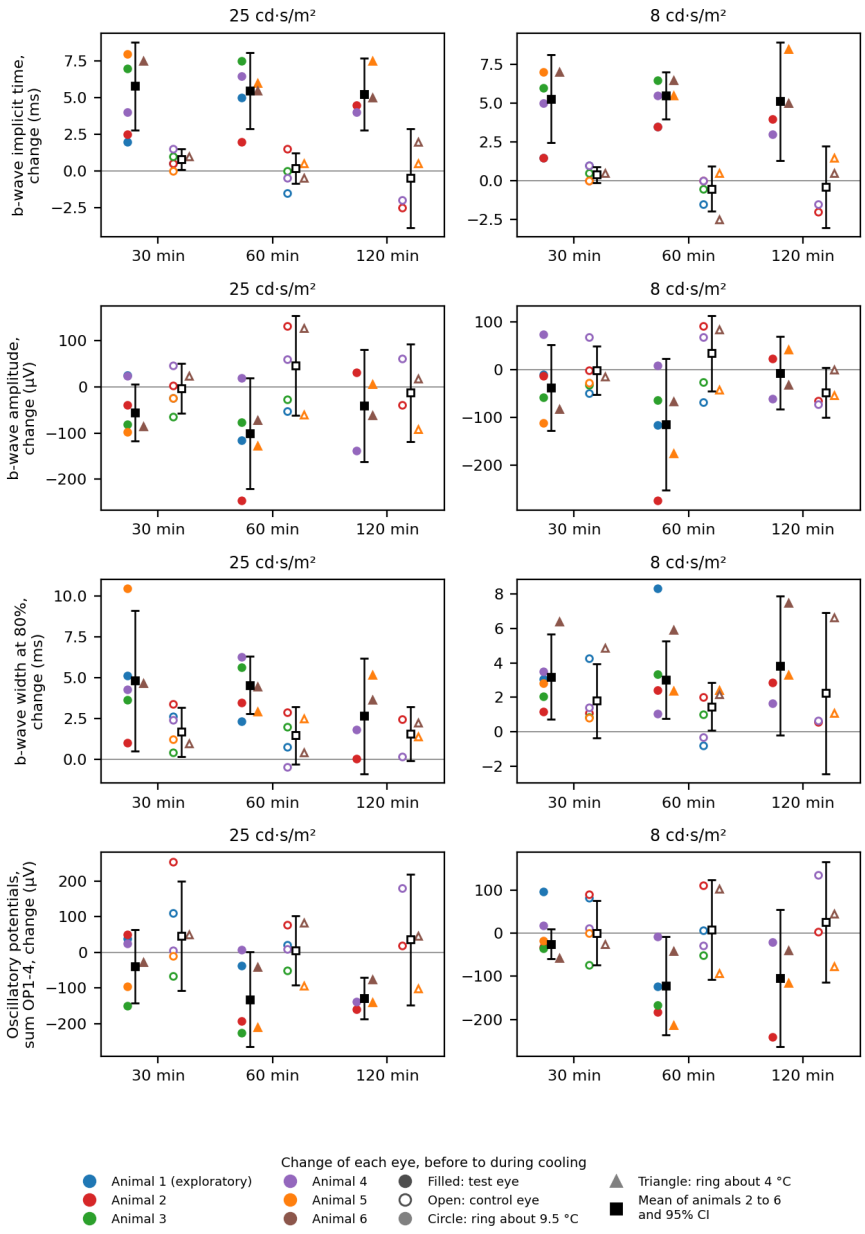


FIGURE 4.8. Change of each eye from its recording before cooling to that during cooling (Jet electrode), test (filled) and control (open), for each animal, by session length: b-wave implicit time, amplitude, and width at 80% of the amplitude, and summed amplitude of oscillatory potentials 1 to 4 (bottom), at 25 and 8 cd·s/m^2. Circles, ring at about 9.5 °C; triangles, ring at about 4 °C; black squares, mean of the sessions of animals 2 to 6 with its 95% confidence interval (animal 1, exploratory, is drawn but not in the mean). Session 3 of animal 3 is left out.

TABLE 4.4. The same measures at 8 cd·s/m^2, as in Table 4.3.

Measure	Min	Test eye	Control eye	Test against control	p_{Holm}	g
Implicit time (ms)	30	+5.3	+0.4	+4.9 (+2.1, +7.7)	0.009	+1.77
	60	+5.5	−0.5	+6.0 (+3.4, +8.6)	0.009	+2.29
	120	+5.1	−0.4	+5.5 (+3.6, +7.4)	0.009	+3.27
Amplitude (μV)	30	−38	−1	−37 (−84, +11)	0.195	−0.77
	60	−115	+35	−150 (−311, +12)	0.186	−0.92
	120	−7	−48	+41 (−57, +140)	0.275	+0.48
Oscillatory potentials (μV)	30	−25	+1	−26 (−100, +49)	0.394	−0.34
	60	−122	+8	−130 (−268, +9)	0.179	−0.93
	120	−104	+26	−131 (−272, +10)	0.179	−1.07
Width at 80% (ms)	30	+3.2	+1.8	+1.4 (+0.4, +2.4)	0.057	+1.36
	60	+3.0	+1.5	+1.6 (−0.3, +3.5)	0.083	+0.82
	120	+3.8	+2.2	+1.6 (+0.4, +2.8)	0.057	+1.50

4.3.4. Intraocular Pressure. From before placement of the devices to after their removal (Figure 4.9), the reading of the control eye rose in all 14 sessions, by a mean of 12.4 mmHg and up to 33.7 mmHg. That of the test eye rose in 7 and fell in 7, with a mean rise of 3.7 mmHg. Measured from where each eye started, the test eye therefore ended lower than the control eye in 12 of the 14 sessions, by 6.7 to 22.3 mmHg; only in sessions 1 and 2 of animal 2 did it end higher, by 18.0 and 7.5 mmHg. Averaged over the five animals, the test eye rose 8.8 mmHg less than the control eye (95% confidence interval from 18.8 mmHg less to 1.3 mmHg more). By session length (Table 4.5) the unadjusted 95% confidence interval excluded zero only after 120 min (p_{Holm} 0.075). By the upper limits of the 90% confidence intervals, a rise of the test eye exceeding that of the control eye by more than 7.9 and 0.5 mmHg after 30 and 60 min is unlikely. At the start of the later sessions the reading was within 9 mmHg of that of session 1 in the test eye and within 15 mmHg in the control eye (Figure 4.10).

TABLE 4.5. Change in the intraocular pressure reading from before placement of the devices to after their removal, as in Table 4.3 (relative values: the tonometer is calibrated for the rodent eye).

Measure	**Min**	**Test eye**	**Control eye**	**Test against control**	p_{Holm}	g
Intraocular pressure (mmHg)	30	+10.2	+14.5	−4.3 (−20.2, +11.6)	0.491	−0.27
	60	+1.1	+10.2	−9.1 (−21.6, +3.4)	0.228	−0.72
	120	−1.3	+12.3	−13.6 (−23.9, −3.3)	0.075	−1.52

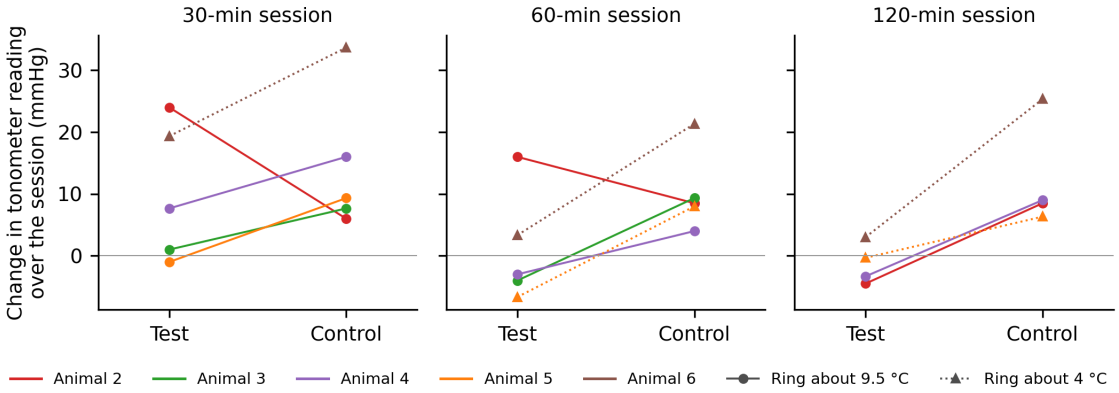


FIGURE 4.9. Change in the tonometer reading of the test and control eyes over each cooling session, from before placement of the devices to after their removal, for animals 2 to 6: ring at about 9.5 °C, solid line; about 4 °C, dotted line. Session 3 of animal 3 is left out.

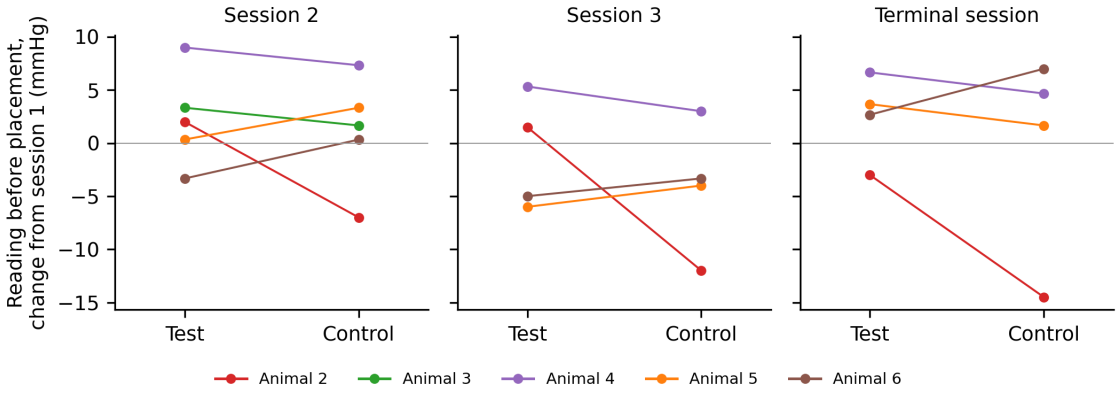


FIGURE 4.10. Tonometer reading of the test and control eyes before placement of the devices at each later session minus that before session 1, for animals 2 to 6. Session 3 of animal 3 is left out.

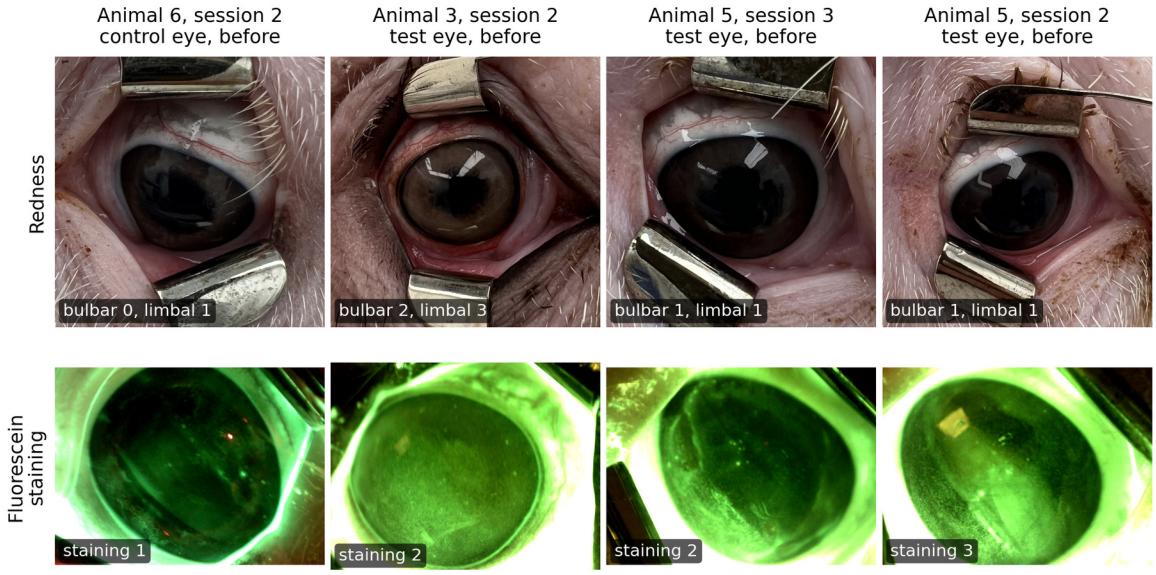


FIGURE 4.11. Photographs of eyes before four cooling sessions, chosen to show staining of grades 1 to 3: conjunctival redness (top) and fluorescein staining (bottom), with the recorded grades (0 to 4; bulbar and limbal redness, corneal staining). Control eye of animal 6 before session 2, test eye of animal 3 before session 2, test eye of animal 5 before session 3, and test eye of animal 5 before session 2.

4.3.5. Ocular Surface. Table 4.6 gives the grades at each session, and Figure 4.11 shows the eyes before four sessions, with corneal staining of grades 1 to 3. Before the first session, before either eye had carried a device, in the ten eyes bulbar redness was grade 0 to 2, limbal redness grade 0 or 1, and corneal staining grade 1 to 3, grade 3 in three eyes, all three later cooled (all selected photographs in Appendix I). Over the 14 sessions the grade of the test eye changed by at least one step more toward harm than that of the control eye in 2 sessions for bulbar redness, 3 for limbal redness, and 1 for staining. In most of these the grade of the test eye did not rise: the control eye improved and the test eye improved less or not at all. The redness of the test eye rose only in session 1 of animal 4, limbal, from 0 to 1. Its staining rose in five sessions, in four of them with the control eye rising as much or more. In the fifth, session 3 of animal 2, it rose from 2 to 4 against 3 to 1 in the control eye, and was still grade 4 in the terminal session, against grade 2 in the control eye.

TABLE 4.6. Grades of the ocular surface of the test and control eyes before and after each cooling session and at the terminal session, median (range) over animals 2 to 6 (0-to-4 scales). Session 3 of animal 3 is left out; n, animals.

			Bulbar redness		**Limbal redness**		**Corneal staining**	
Session		n	Test	Control	Test	Control	Test	Control
1 (30 min)	Before	5	1 (1–2)	2 (0–2)	1 (0–1)	1 (0–1)	3 (1–3)	1 (1–2)
	After	5	1 (0–1)	1 (0–2)	1 (0–1)	1 (0–1)	3 (1–3)	3 (1–3)
2 (60 min)	Before	5	1 (1–2)	1 (0–2)	1 (0–3)	1 (0–3)	2 (1–3)	2 (1–2)
	After	5	1 (0–1)	1 (0–2)	1 (0–1)	0 (0–2)	2 (1–3)	3 (2–3)
3 (120 min)	Before	4	1 (0–1)	1 (0–1)	0.5 (0–1)	0.5 (0–1)	2 (2–3)	2.5 (1–3)
	After	4	0 (0–1)	0.5 (0–1)	0 (0–1)	0 (0–1)	2.5 (1–4)	2 (1–2)
Terminal		4	1 (1–2)	1 (1–2)	1 (0–2)	1.5 (0–2)	2.5 (1–4)	1 (1–2)

4.3.6. Systemic Course and Adverse Events. No session was paused or ended for a physiological reason (Figure 4.12). Oxygen saturation stayed at or above 93%, heart rate between 74 and 153 beats per minute, end-tidal carbon dioxide between 35 and 53 mmHg, and core temperature at or above 35.1 °C, its lowest in session 2 of animal 5.

On the day after session 1 of animal 2 the left eyelid, on the control side, was moderately swollen, closing the eye, and the animal scratched at it; the right eyelid was only very mildly swollen, and the right eye appeared normal. The next day both eyes were equally open and the mild inflammation was resolving. After session 2 animal 3 had expiratory wheezing and tracheal irritation, with blood on the endotracheal tube, which may have been too large; it was treated with meloxicam (0.1 mg/kg by mouth) and was much improved the next day. After session 3 it arrested during recovery and was euthanized by the BRL staff with the agent used in the terminal sessions (pentobarbital sodium, Fatal-Plus, 10 mL intravenously). Necropsy found laryngeal necrosis, which the BRL veterinarian and veterinary pathologist attributed most likely to a traumatic intubation in session 2, and cardiac hypertrophy, judged incidental and not the cause of death; neither was attributed to the device (Figure H.1, Appendix H). Both eyes were collected after death.

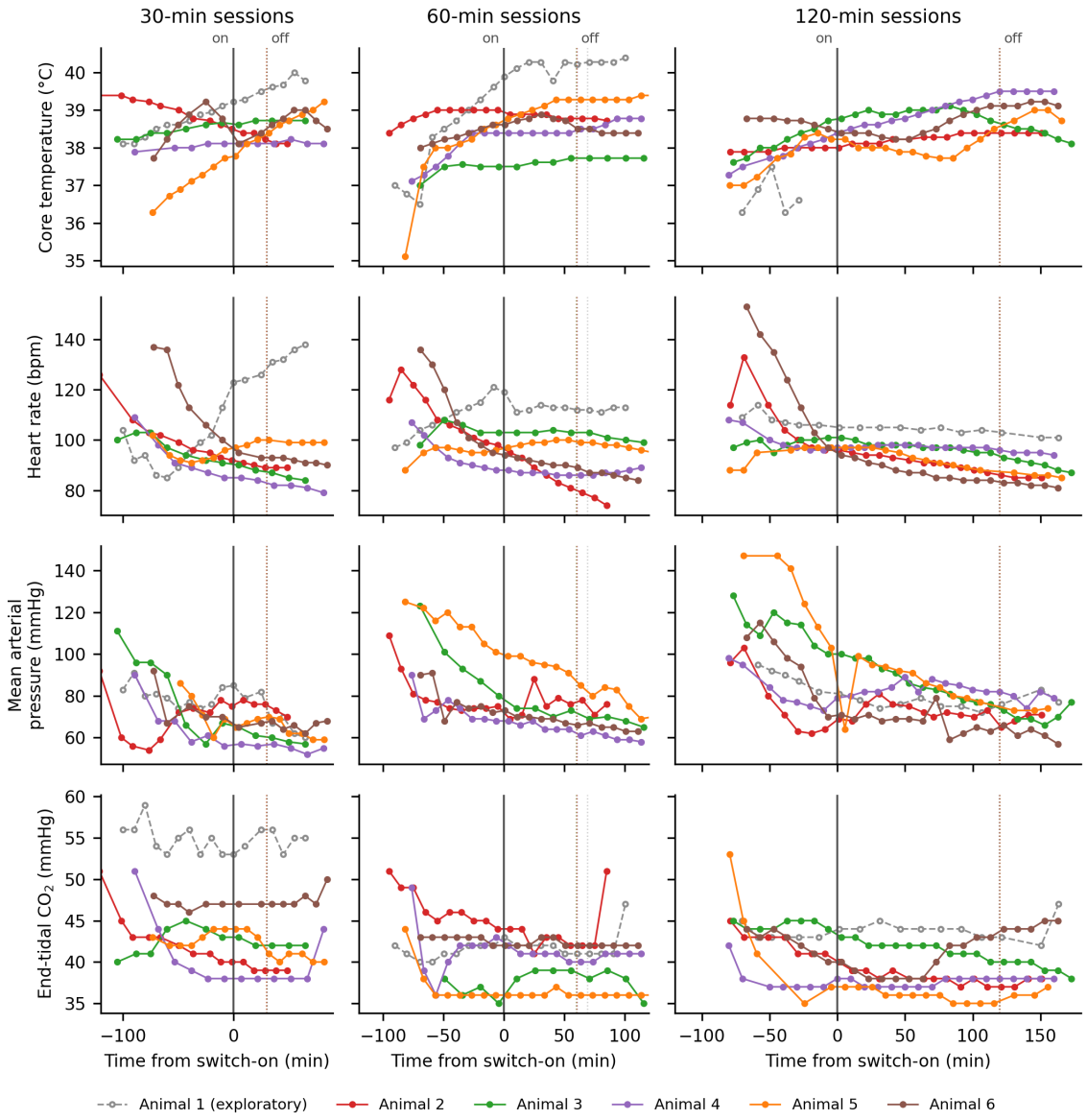


FIGURE 4.12. Core temperature, heart rate, mean arterial pressure, and end-tidal carbon dioxide during each cooling session against time from switch-on, from the anesthesia record (a reading about every 10 min). Solid vertical line, switch-on; dotted lines, switch-off of each session. Animal 1, exploratory, dashed gray line with open circles; its mean arterial pressure was not recorded in session 2, nor its core temperature after switch-on in session 3. Session 3 of animal 3, left out of the ocular measures, is shown.

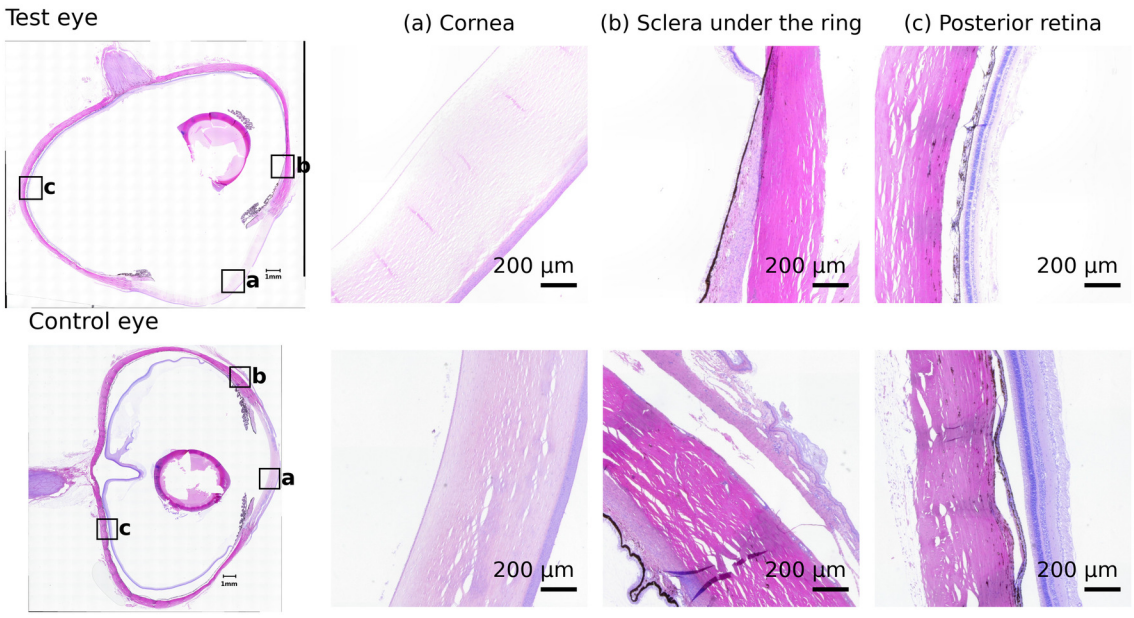


FIGURE 4.13. Hematoxylin and eosin sections of the test (top) and control (bottom) eyes of animal 4, slide 5: the whole section and, enlarged, regions chosen on it: (a) the cornea, (b) the sclera behind the limbus, under the ring, and (c) the posterior retina.

4.3.7. Histology. Both eyes of each animal were enucleated and fixed, those of animal 3 after its death, with a different interval from death to fixation. The sections of the six animals were read (Figure 4.13, animal 4; one slide of each animal in Appendix I). Apart from a processing artifact in animal 3 (below), in no animal did the test eye show a change absent from the control eye. Neither eye of any animal showed inflammation, necrosis, or thrombosis, and the conjunctiva, limbus, and sclera under the ring looked the same in both eyes. The control eyes of animals 3 and 4 had a focal loss of the corneal epithelium at the same place in each section, with no inflammation or healing, and so recent, from handling, tonometry, or the electrode near the end of the session. It was not in the test eye. In the test eye of animal 3 the corneal epithelium was lifted off Bowman's layer as an intact sheet in all three sections, with no damage to the cells beneath, most likely in processing. A first set of scans of animal 1, fixed in formalin, showed the retina detached in both eyes, which led to the change of fixative from animal 3 onward; its retina was not graded. In both

eyes of every animal the retina was detached, its inner layers vacuolated, and the lens cracked, all processing artifacts.

4.4. Discussion

Only the Gen 3 device, with the reduced-bulk aluminum ring, was used on an animal (Section 4.2.4); the rest of the Gen 4 design remains to be tested in vivo (Section 5.4). The relay of the Gen 3 controller was the weak element, and the MOSFET module that replaced it ran without an interruption (Section 4.3.2). Apart from the relay interruptions, the device held the ring steady at its operating temperature for up to 2 h against the heat of the living eye and lids.

Three practical problems affected the earlier sessions. The device did not always fit under the eyelid until the lid was drawn over it with tweezers. The ring thermistor sometimes came out of the device, giving poor readings or none, until its insertion point was fixed with silicone. The channel was first calibrated with a second thermistor that sat in the well without touching its walls. As a result, the ring read about 5.5 °C low until a custom piece put the thermistor in contact with the aluminum (Section 4.2.6). Corrected for this error, most of the exposure was at about 9.5 °C, above the 4 °C intended.

During cooling the b-wave of the cooled eye became slower and wider in every session, at both primary flashes, while the implicit time of the control eye changed by at most 2.5 ms either way (Section 4.3.3, Table 4.3). A longer implicit time is the response expected of a cooler retina [**72, 73, 75, 120**], and the control eye, which carried an identical ring through the same anesthesia, core temperature, and time, shows that the change did not arise from these. The b-wave of the control eye also widened, by less, and part of the widening is shared; the larger widening of the cooled eye fits a slower and possibly uneven response of its retina. The slowing diminished as the ring rewarmed after switch-off, which also points to temperature rather than to a lasting effect of the contact. The five sessions at about 4 °C slowed within the

range of the others at 25 cd·s/m^2 and up to 1.5 ms beyond it at 8 cd·s/m^2, and are too few to show a relation to the ring temperature. Although ischemia caused by the cold, or a difference in the pressure of the two rings, cannot be fully excluded, the slowing of the test eye alone, with a ring on both eyes, points to cooling of the retina.

The implicit time, with the control eye that changed by at most 2.5 ms, not the amplitude, is the better evidence that the slowing of the test eye arises from its temperature. The slowing, about 5 ms, is several times the variability of b-wave timing between sessions on this device, under 1 ms in healthy eyes under light adaptation [**145**]. The fall of about 14 to 18% in the b-wave amplitude of the test eye, by contrast, is within the normal difference between the two eyes of a subject, whose 95th percentile is 27 to 36% [**142**]. The amplitude also varied widely between recordings, and of the six confidence intervals of the fall of the summed oscillatory potentials only one, after 60 min at 25 cd·s/m^2, excluded zero, and none was significant after Holm correction (Tables 4.3 and 4.4). Anesthesia, too, lowers the amplitude more than it delays the b-wave: in dogs under isoflurane the b-wave amplitude was 54% lower than in the same dogs awake, whereas its implicit time did not differ significantly [**146**]. In anesthetized pigs the amplitude of the multifocal ERG fell over the 150 min after induction [**147**], and a decline of the amplitude in either eye over a long session is therefore expected.

The hypothesis that the ERG returns to its baseline after rewarming was tested only in part. The recovery recording was made before the ring had rewarmed fully (Section 4.2.9), and whether the difference between the eyes closes completely was not measured. At 8 cd·s/m^2, where the limit is narrower, the b-wave implicit time of the test eye returned to within the limit in only 8 of the 12 recovery recordings, and in the other four it was still 3.0 to 4.0 ms slower, consistent with a retina not yet back to its temperature. At 25 cd·s/m^2 it returned to within the limit in 11 of the 12.

Whether the cooling left a lasting change in the retina is answered better by the start of the next session than by the recovery recording. At the start of each later

session, compared with session 1, before either eye had been cooled, no difference between the test and the control eye was significant after Holm correction in the b-wave implicit time, the b-wave amplitude, or the summed oscillatory potentials, at either flash (Section 4.3.3, smallest p_{Holm} 0.09). Where the means moved, they moved against a lasting effect of the cold: relative to the control eye the b-wave of the test eye was 1.1 to 1.8 ms faster than in session 1 at sessions 2 and 3, not slower, and its summed oscillatory potentials were larger by 54 to 92 μV. In the terminal session the b-wave timing was near zero, +0.4 and +0.1 ms, but relative to the control eye the b-wave amplitude of the test eye was lower than in session 1 by 108 and 55 μV, continuing a decline over sessions 2 and 3, and the summed oscillatory potentials by 64 and 17 μV, at 25 and 8 cd·s/m^2 (Table 4.2); these negative values come mainly from rises in the control eyes of animals 4 to 6.

The slowing still present in four recovery recordings was not seen at the start of the next session (for session 2 of animal 3, whose next session is left out of the analysis, the start of that session gives +1.0 ms at 8 cd·s/m^2, test eye against control eye relative to session 1). The amplitude, with intervals that include zero, can neither show nor exclude a cumulative effect. The b-wave timing, by contrast, showed no cumulative effect over the sessions of an animal. By the 90% confidence intervals a lasting slowing of more than 1.3 ms is unlikely at the start of sessions 2 and 3 (Table 4.2); the terminal session, on four animals, bounds it less tightly at 25 cd·s/m^2 (2.2 ms).

The safety endpoints showed no harm attributable to the cooling within the measurements made. The reading of the control eye, which carried an identical ring that was not cooled, rose in all 14 sessions analyzed, through the anesthesia [**148, 149**], the fluids, or the device, whereas that of the cooled eye rose less, or fell, in 12 of the 14 (Section 4.3.4). The cooling may have reduced or reversed that rise, although the difference was not significant after Holm correction. Intraocular pressure under anesthesia moves by several mmHg with induction, intubation, and the agents given, and in anesthetized rats it falls with body temperature when no heat support is given

[**150**]. The readings before and after a session were taken at different stages of anesthesia, but both eyes shared the anesthetic and the body temperature, and these effects cancel in the difference between the eyes.

That local cooling lowers the pressure is established in humans; its mechanism remains to be established. A stream of cold air on one eye lowered the pressure from 14.1 to 12.6 mmHg, with no change in aqueous flow or outflow facility, and the fall was attributed to the episcleral venous pressure, which fell too [**151**]. In a randomized trial, a cold mask worn for 10 min lowered the pressure by 2.0 mmHg and a heated mask raised it [**152**]; the authors attributed the fall to less aqueous production, through vasoconstriction and slower metabolism of the ciliary body, but did not measure it. A coil on the rabbit sclera left the pressure unchanged [**73**]. No study has tested whether cooling offsets a rise produced by a device on the eye, and neither the episcleral venous pressure nor the aqueous flow was measured here. The episcleral veins offer one route, which remains a hypothesis: the rise under a scleral lens is attributed to their compression [**153**], the ring rests on the same region of the sclera, and cold lowers their pressure [**151**]. Cooling also thickens the cornea, which alters applanation readings [**154**] and may alter a rebound reading. Corneal thickness was not measured, and the present readings cannot separate these effects.

Redness and staining were worse in the cooled eye in only a few sessions, at each of the three session lengths (Section 4.3.5). Bulbar redness fell over a session in both eyes, and in all but two of the 14 sessions that of the cooled eye changed no more toward harm than that of the control eye; limbal redness showed no pattern. Although these counts are too few, and the grades too coarse, to show it, a lower bulbar redness in the cooled eye is plausible. Cold is known to reduce conjunctival redness: in a masked trial a cold compress applied for 5 min reduced bulbar hyperemia faster than no treatment [**155**], and cold air on the eye lowered the pressure of the episcleral veins, which drain the same vascular bed [**151**].

Corneal staining rose over a session in the control eye at least as often as in the cooled eye. Most of it therefore arises from what the two eyes shared: the ring, the corneal ERG electrode, the topical anesthetic, tonometry, and hours of exposure under anesthesia. Corneal ERG electrodes alone cause transient epithelial changes, seen with fluorescein in 62% of normal subjects and associated with the topical anesthetic [**156**]. As both eyes carried all of these, the data cannot say which caused the staining, only that the cooling added nothing that could be measured. The staining grades are weak evidence in either direction: three of the ten eyes were graded 3 before any device had touched them, and pooled dye cannot be told from staining (below). Of the five rises in the cooled eye, only one had returned to its earlier grade by the next session; the highest, grade 4 persisting to the terminal session in the cooled eye of animal 2, had no counterpart in its histology, in which the corneas of the two eyes looked alike. Neither redness nor staining points to harm from the cooling.

The one death in the study followed a complication of intubation, and the eyelid swelling of animal 2 was on the control side.

The retina was detached and vacuolated in both eyes of every animal by processing, so the sections can show only gross damage to it. The only losses of corneal epithelium were in control eyes, so they came from what the two eyes shared, not from the cold. The histology showed no change from the cooling: the conjunctiva, limbus, and sclera under the cooled ring looked like those under the ring that was not cooled (Section 4.3.7).

The quality of the data improved from animal to animal as the procedures became routine with experience. From animal 2 rocuronium was given to abolish eye movement and the corneal staining was done in the dark. From animal 3 the MOSFET module switched the drive without an interruption, the times of the ERG recordings were fixed, and dark adaptation lasted 20 to 23 min in every session. From session 1 of animal 6 the channel was calibrated with the well thermistor against the aluminum, so that no correction was needed. The improvement was not uniform: the cooling

record of session 3 of animal 3 was lost, and the share of ERG traces flagged by the trace checks did not fall after animal 2 (Section 4.3.3).

The in-vivo study is based on six animals, and the ERG after 120 min on four (animals 2, 4, 5, and 6: animal 1 is exploratory and session 3 of animal 3 is left out). The main results pool sessions at two ring temperatures, about 9.5 and 4 °C, and describe cooling over that range rather than at one set point. With the measured variability and five animals the paired comparison of the test eye with its own baseline has a power above 99% for the implicit time, about 73% for a 30% change in amplitude, and 77% for the width; the comparison of the test eye with the control eye has 97%, 43%, and 59%. The fixed order of the three session lengths confounds any trend across session lengths with repeated anesthesia and cumulative cooling. The test eye was always the right eye, fitted and recorded first, and the control eye was dark-adapted slightly longer, in every recording including that before cooling. In the porcine ERG the b-wave implicit times at strong flashes do not differ significantly between 15 and 25 min of dark adaptation [**157**].

The tonometer gives relative values only and was not used with the devices on the eyes, although a device resting on the sclera can raise the pressure while it is in place [**153, 158**]. Redness and staining were graded by the author, who knew which eye had been cooled, on one photograph per eye and time point that the author also chose. The grades are whole steps of a 0-to-4 scale, and as a result most sessions tie and one step is the smallest change that can be seen. Dye pooled on the surface cannot be told from staining on some photographs. The histology was read by the author, who knew which eye had been cooled, and not by a pathologist; it covers sections of a central slab of each globe, from the cornea to the optic nerve head, and there was no slit-lamp, fundus, or corneal endothelial examination. With the RM electrode the restoration limits were wider (implicit time ±11.6 and ±6.7 ms, amplitude ±285 and ±287 μV at 25 and 8 cd·s/m^2) than the cooling effect, so the RM recordings confirm the Jet result only coarsely. Two b-wave latencies of animal 6 were read by hand

at the local peak. The core temperature of the anesthetized animals should be read against the normal porcine range of 38.5 to 39.5 °C. No temperature was measured inside the eye, and the relation between b-wave timing and retinal temperature has not been established in this preparation. The size of the delay therefore cannot be converted into a retinal temperature.

CHAPTER 5

General Discussion and Conclusions

5.1. Synthesis Across Aims

The three aims approach one question from three sides: how much of the cooling applied to the anterior sclera reaches the back of the eye. Aim 1 measured it directly, in eyes without a circulation. Aim 2 compared ring designs in the same preparation and used a computational model to estimate what a circulation does to it. Aim 3 looked in the living eye, where no temperature could be measured, for its functional trace, a slowing of the electroretinogram (ERG).

In the isolated eye a ring held at 4 °C produced a steep gradient from front to back (Figure 5.1A). The anterior sclera cooled the most and the proximal optic nerve the least. At the proximal optic nerve no difference between the species was detected, and one larger than 1.8 °C is unlikely (Table 2.7). Human and porcine eyes cooled there by 3.5 to 3.8 °C.

The refinements of Aim 2 left this deep cooling largely unchanged (Chapter 3). In the same donor eyes, the reduced-bulk ring, with about half the volume, and the ceramic ring both gave the proximal optic nerve about the same cooling as the original ring. The reduced-bulk ring cooled it slightly less, by less than the spread between eyes.

The isolated eye has no blood flow, and the choroid of the living eye carries heat to the tissue the device is trying to cool. The model of the perfused eye therefore gives less cooling than the bath experiments at the sites away from the ring (Figure 5.1B). This gap is not a measurement of the effect of perfusion: the model has not been compared with a temperature measured in a perfused eye, and the two conditions differ in more than perfusion: the model also includes the eyelids, and the donor eyes

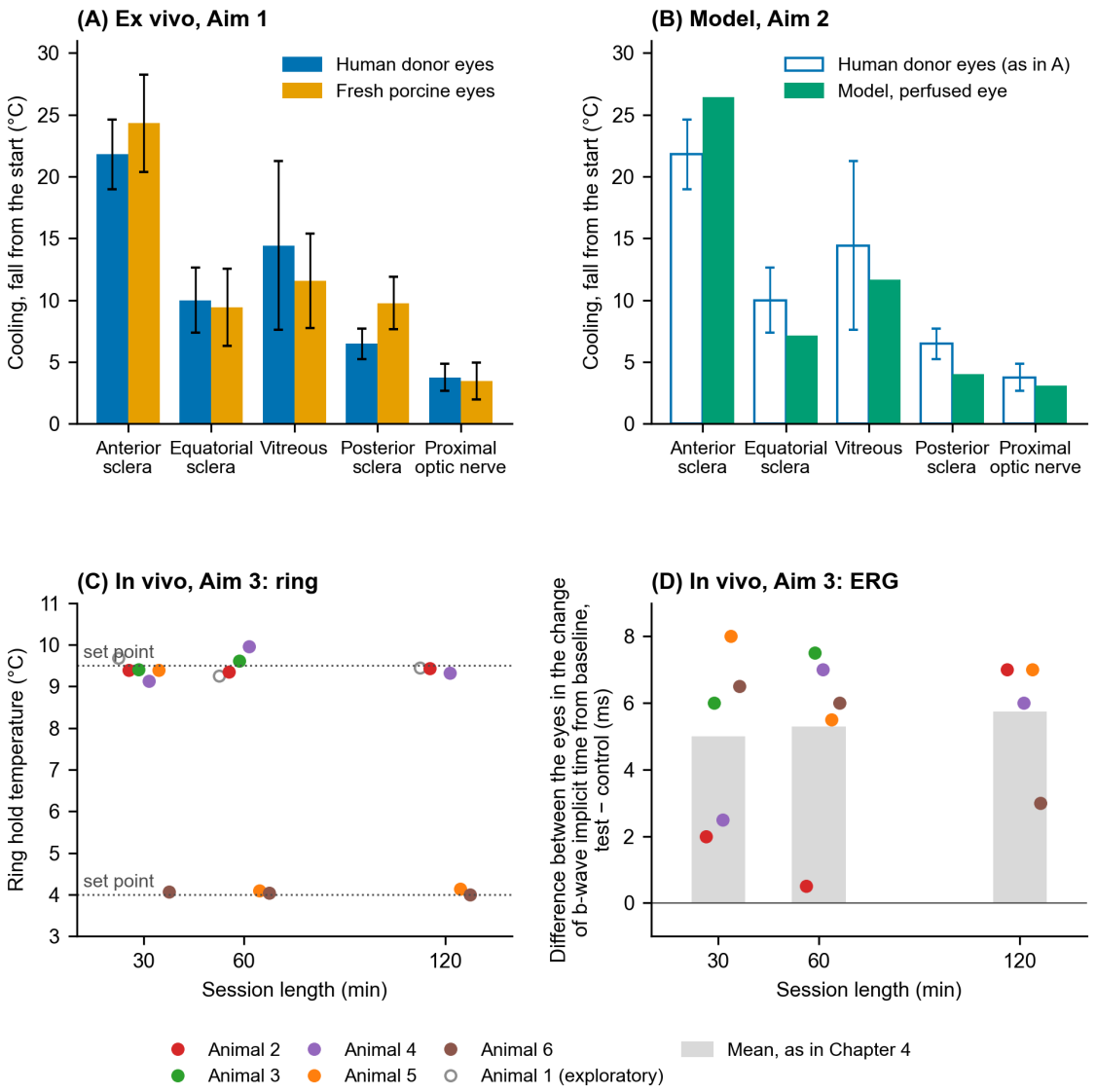


FIGURE 5.1. Cooling at the sites shared by the three aims. (A) Fall in temperature from the start of cooling over the 20-min record in human donor and fresh porcine eyes with the original ring (mean and SD; Tables 2.3 and 2.4). (B) The same fall in the human donor eyes (open bars, as in A) and in the model of the perfused eye in the orbit (Section 3.3.6); the perfused model is read after 35 min, the measurements over minutes 16 to 20 (Section 3.2.3). (C) Mean ring temperature held in each in-vivo session, corrected, by session length. (D) Change in b-wave implicit time from the Jet recording before cooling to that during cooling (from animal 3 at about the middle of cooling), cooled eye minus control eye, by session length, at 25 cd·s/m^2; bars, the means over both ring temperatures (Section 4.3.3). Session 3 of animal 3 is left out; animal 1, exploratory, open gray circles (panel C only).

were held in a heated bath (Section 3.3.6). Across the model runs, body and ring temperature set the temperature at the back of the eye far more than the design of the ring did.

In vivo, the cooling applied at the anterior sclera reached the retina closely enough to change its function, but the ERG does not show by how much the retina cooled. With no temperature measured in vivo, Chapter 4 addresses the question only through the ERG, whose timing slows as the retina cools [**72, 73, 75**]. The ring was held close to its set point for up to 2 h. During cooling the b-wave of the cooled eye slowed from its own baseline in all 14 sessions analyzed, by 2.0 to 8.0 ms, whereas that of the control eye changed by at most 2.5 ms either way (Figure 4.8; Section 4.3.3). Figure 5.1D shows the slowing relative to the control eye, and Figure 5.1C the ring temperature.

The isolated eyes fix the size and the pattern of the cooling that the ring produces when no circulation opposes it. They also show that at the proximal optic nerve it differs by less than about 2 °C between human and porcine eyes and is much the same with the reduced-bulk and ceramic rings. The model places that cooling against a circulation and identifies body temperature and ring temperature as what sets the temperature reached at depth. The in-vivo study shows that the reduced-bulk ring, held near 9.5 °C in most sessions and near 4 °C in five, on a perfused eye, changes retinal function in the direction expected of cooling. The sessions lasted up to 2 h, and the eye and the animal tolerated them as far as was measured.

5.2. Comparison with Prior Work

The approaches reviewed in Section 1.4 fall into two groups. The first cools the inside of the eye or the optic nerve directly, but each method requires surgical access [**72, 78, 79**]. The second acts at the surface without surgery [**87, 84, 86**], and for none of these devices has a temperature at the back of the eye been reported. Research preparations in living animals had shown that cold applied outside the eye

lowers posterior temperatures [**70, 72, 73**], and of the methods that reached the retina the one with feedback control delivered its fluid into the anterior chamber [**77**].

The SCEC belongs to the second group by its access and aims at the target of the first. It is placed without surgery, it holds the sclera at a set temperature for tens of minutes to hours, and its effect has been measured at located sites inside the eye, up to the optic nerve. The fresh-porcine part of this work, published by Nakamura et al. [**1**], established that effect in porcine eyes. This dissertation extends it to human donor eyes and to two further ring designs, and applies the device to the living animal. To the author's knowledge it is the first scleral-contact cooler whose temperature has been measured in human donor eyes at sites verified by micro-CT. The SCEC also differs from the other contact coolers that come closest. The ETX-4143 scleral devices cool for 4 to 10 min from a reservoir frozen before use and are aimed at ocular surface pain [**89, 90**]. Kataoka's circulating-water contact lens protected the cornea and was not tested at the back of the eye [**83**]. Lachapelle's coil cooled the retina but had to be glued to the sclera with the eyelids removed [**73**].

The SCEC trades depth of cooling for the absence of surgery. Direct methods bring cold fluid to the retina or the nerve itself, whereas a ring on the anterior sclera cooled the proximal optic nerve of an isolated eye by a few degrees. In a Pennes model a circulation reduces this further, although at a body temperature of 37 °C the model still brings the proximal optic nerve to 33.8 °C, inside the range of systemic therapeutic hypothermia (Section 3.4); a multiphase blood model instead predicts deeper cooling [**53**].

5.3. Clinical Translation

The two clinical settings that motivate the device (Section 1.1) ask different things of it: speed of application in retinal artery occlusion, and tolerance of long exposures in perioperative visual loss. In retinal artery occlusion the injury has already begun when the patient is seen, usually hours after onset [**2, 14, 15, 16**], and cooling could

only follow the insult. In perioperative visual loss the period of risk is known in advance, and cooling could begin before it and continue through it. Supine cardiac surgery is the natural setting for the device, because direct pressure on the eye is to be avoided in prone surgery [**21**]. Most ischemic optic neuropathy after spine surgery is posterior [**19**], whereas the model places the reach of the device short of the nerve 1 cm behind the globe, at the retina and the optic nerve head (Section 3.4).

The effect is best described as a temperature reduction of a few degrees at the back of the eye, not as the attainment of a target temperature. What the device delivers has to be set against what the laboratory evidence suggests is needed. The measured effect at the proximal optic nerve of the isolated eye was a fall of 3 to 4 °C. Protection of the retina has been reported for temperature reductions of this size (Section 1.3), including the 3 °C step used here as a benchmark [**66**]. Whether a perfused human eye cools by as much is not known. The model suggests somewhat less. With fever the reduction is about the same, but the temperature reached is higher, because it follows body temperature.

The in-vivo safety evidence covers three cooling sessions of 30, 60, and 120 min with the ring near 9.5 °C, or near 4 °C in five sessions of animals 5 and 6, given over about ten days to an anesthetized pig with an uninjured eye. Within that envelope, in the animals studied, the cooled eye showed a smaller rise in the tonometer reading than the control eye in 12 of the 14 sessions once the device was removed. Redness and corneal staining were rated worse in the cooled eye in only a few sessions, without a relation to session length. No adverse event was attributed to the device, and the histology showed no injury to the cornea or sclera of the cooled eye that was absent from the control eye. The b-wave timing was restored after switch-off in 11 of the 12 recovery recordings at the stronger flash and 8 at the weaker (Section 4.4).

5.4. Limitations

Each chapter gives the limitations of its own work (Sections 2.4, 3.4, and 4.4).

No temperature was measured in a perfused eye. The bath is not a model of perfusion, the computational model has not been validated against such a measurement and depends on perfusion rates taken from the literature, and the ERG is indirect evidence of retinal cooling. Its relation to retinal temperature has not been established in the pig under these conditions, and the delay of the b-wave cannot yet be set against the cooling that the model predicts. The control eye carried the same ring, which argues against pressure on the globe as the cause, but a difference in pressure between the two rings, or ischemia caused by the cold, can also slow the ERG. The histology was read unmasked and could show only gross injury to the retina, which processing had detached in both eyes.

The groups were small: nine human eyes from five donors, five fresh and five preserved porcine eyes, and six animals, the first of them exploratory. Conclusions from the eyes rest on excursions, which are better determined than absolute temperatures (Appendix A).

SCEC Gen 4 was not assembled as a whole. The in-vivo device was Gen 3 with the reduced-bulk ring (Section 4.2.4). The ceramic ring was tested ex vivo and in the model. The insulating layer, the integral corneal lens of ring v3, and the smaller heat exchangers were evaluated only in the model. The integrated board was tested only on the bench, where the fault of its reference voltage remains open.

Several steps separate the present device from clinical use. The exposures tested in vivo, up to 2 h, are shorter than the operations in which perioperative visual loss occurs (a mean anesthetic duration of 9.8 h in the 83 registry cases of ischemic optic neuropathy after spine surgery [**19**]), and the tolerance of the anterior sclera, cornea, and lens to longer contact at low temperature is not established. The intraocular pressure with the device on the eye has not been measured. No limit for the cold side has been based on tissue tolerance: the board stops below 0 °C only to prevent freezing, and the controller used in vivo has no cold-side limit at all. The limit of 43 °C used for the warm surfaces is taken from the standard for medical electrical equipment

[**128**] and was examined in the model only. Biocompatibility of the materials in contact with the eye, sterility, electrical safety, and the software of the controller remain to be addressed.

5.5. Future Work

The first need, and the link still missing between the three aims, is a direct measurement of temperature at the back of a perfused eye during cooling, for example with a single fine probe in a terminal experiment. It would test the model, fix the factor by which a circulation reduces the deep cooling, and turn the ERG from the only evidence of retinal cooling into a measure that can be calibrated against temperature.

The model should also be run over the plausible range of choroidal blood flow and then compared with that measurement. The control board needs a revision in which the reference voltage cannot disturb the current limits, a sensor on the warm side, a cold-side limit based on tissue tolerance, and a test of each safety cutout on the hardware. The thermoelectric module can also be developed further, with a higher density of thermoelectric posts and a module printed directly on the ring. The in-vivo evidence is confined to the envelope tested, and longer exposures, repeated daily use, awake patients, and eyes that are already ischemic remain to be studied. The question that motivates the device, whether this degree of cooling protects the retina or the optic nerve, requires a study in an injury model, with cooling begun at a clinically realistic time relative to the insult.

The computational work would be extended in two steps. A complete model of the setup would couple the eye and orbit to the ring, the thermoelectric module with its Peltier, Joule, and conduction terms, the heat exchanger, and the coolant loop under the control law. It would thus predict tissue temperatures from the drive current instead of from an imposed ring temperature. An optimization on that model would then replace the comparison of a few chosen designs. It would range over the geometry and material of the ring, the module, and the heat exchanger, constrained

by the 43 °C limit for warm surfaces, the power of a portable supply, and the size of the device. Both should be run with more than one representation of perfusion, because Pennes and multiphase models predict opposite effects of the circulation on posterior cooling [**53**].

Several additions to the device itself were conceived in this work and remain to be built or evaluated (Table 3.1). A contact-pressure sensor in the ring would report the force applied to the eye, which bears on the intraocular pressure during treatment. Under scleral lenses the rise in pressure is attributed to compression of the episcleral veins, whose pressure is about 7 to 11 mmHg, and a lens exerts its full compression only after more than 4 h of wear [**153**]. The intraocular pressure would also be measured during application and over exposures of several hours, as it has been during scleral-lens wear [**158**]. The present ring leaves no access for a rebound tonometer. An electrode in the ring would allow the ERG to be recorded through the device, without a separate corneal electrode. Other tools developed in the author's laboratory could turn the ERG into a spatial measure of cooling. The contact lens electrode array (CLEAr) records the ERG at many sites on the cornea at once, and the differences between sites reflect where on the retina the response arises [**159, 160, 161**]. The hemispherical stimulus domes of the laboratory stimulate selected regions of the retina. PeriStim presents patterns to the mid- and far-peripheral retina [**162, 163**], and its light-emitting diode (LED) successor, HoneyDome, delivers full-field and partial-field flashes [**164, 165, 166**]. Used together during cooling, they could show where the retina cools and how far the cooling extends.

Thermistors built into both ceramic plates of the thermoelectric module would measure its cold and its warm side directly, which the 43 °C limit for the warm surfaces and a future cold-side limit based on tissue tolerance both require. The silicone potting between the plates of the module, present in every device used here, remains to be evaluated on its own for electrical isolation and for protection against moisture. A ring that detaches from the module could be replaced or sterilized between patients,

and an injection port through the ring would give access to the eye with the device in place.

The reverse polarity used for experimental equilibration could also give the device a therapeutic heating mode, which would need its own validation of temperature control, safety, and efficacy. Moderate heating of the eyelids is used in the treatment of dry eye from meibomian gland dysfunction [**167**], a use to which a heating mode could be suited.

Thermal analgesia is another possible application. Whether the SCEC relieves pain would have to be tested with a pain endpoint and an appropriate exposure protocol. Cooling affects corneal sensory-nerve activity [**168**].

Part of this work has been published, and the rest is to be submitted as a series of papers. The fresh-porcine experiments of Aim 1 were published with Y. Nakamura [**1**]. Two manuscripts on the ex-vivo work are in preparation: a methods paper on the ex-vivo system, and the comparison of human donor and porcine eyes. The computational model and the ring designs of Aim 2 will be the subject of a computational manuscript. The in-vivo safety and ERG study of Aim 3 will be submitted for publication, and is also planned for presentation at the Association for Research in Vision and Ophthalmology (ARVO) Annual Meeting.

5.6. Conclusions

A thermoelectric ring held at 4 °C on the anterior sclera cooled the proximal optic nerve of isolated human and porcine eyes by a few degrees within the record. The first hypothesis of Aim 1 held for the occurrence and timing of this cooling, and for its size on average: the mean excursion exceeded the 3 °C benchmark in every cohort, although only 11 of the 17 traces reached it. Every included trace at that site cooled and approached its final temperature within 15 min. No difference was detected at that site between the species, or between preserved and fresh porcine eyes. The hypothesis of Aim 2 was supported at the proximal optic nerve, where

the reduced-bulk ring and the ceramic ring each gave about as much cooling as the original ring. A model of the perfused eye indicates that blood flow reduces the deep cooling, by a factor that remains to be measured, and that body temperature, more than the design of the ring, sets the temperature reached.

In anesthetized pigs the device, fitted with the reduced-bulk ring, held the ring for up to 2 h at about 9.5 °C, above the 4 °C intended, through a calibration error, and at about 4 °C in five sessions of animals 5 and 6 (Section 4.3.2). The b-wave of the cooled eye slowed and widened from its own baseline while that of the control eye did not slow, a change consistent with retinal cooling, and its summed oscillatory potentials fell. The first part of the hypothesis of Aim 3 was supported, as a longer implicit time and a smaller b-wave amplitude were found together at both primary flash strengths in 10 of the 14 sessions (3 in the control eye), and slower and smaller oscillatory potentials, judged in the same exploratory way, in 9, against none in the control eye. After switch-off the b-wave timing was restored, by the criterion of Section 4.2.9, in 11 of the 12 recovery recordings at 25 cd·s/m^2 and in 8 of the 12 at 8 cd·s/m^2, and the summed oscillatory potentials in 11 and 10 of the 12. These recordings were made before rewarming was complete. The second part was therefore tested only in part, but the return of the timing toward baseline in most recovery recordings points to recovery. At the terminal session the difference in b-wave timing between the eyes did not differ from that of session 1. In the animals studied the intraocular pressure after removal of the device, the ocular surface, and the systemic course showed no harm attributable to the device, and the histology of the cooled eye showed no damage absent from the control eye.

The contributions of this dissertation are the following.

(1) The temperature field produced by a scleral-contact cooler was measured in human donor eyes, at sites inside the eye located by micro-CT.

(2) Human donor and fresh porcine eyes were compared with the same ring. In both the anterior sclera cooled most and the proximal optic nerve least, and at the proximal optic nerve the two species differed by less than about 2 °C.

(3) A computational model showed that the reduced-bulk ring, of about half the volume, and the ceramic (alumina) ring cool almost as well as the original aluminum one, that body and ring temperature matter more at depth than the design of the ring, and that blood flow reduces the deep cooling.

(4) An integrated control board for the device was designed and tested on the bench.

(5) The device was applied to living eyes for the first time, in anesthetized pigs, where it held the ring at about 9.5 or 4 °C, slowed the ERG of the cooled eye, and caused no harm attributable to it in the measures made.

Before clinical use, the temperature at the back of a perfused eye must be measured, longer exposures must be shown to be tolerated, and protection of the retina or the optic nerve must be tested in a model of ischemic injury.

Appendices

APPENDIX A

Temperature Measurement Uncertainty

This appendix gives the budget behind Section 2.2.4: an expanded uncertainty ($k = 2$) of ±1.8 °C for a single reading and, for a within-trace excursion ΔT, ±0.40 °C at the proximal optic nerve and ±1.1 °C at the ring, the larger of the human and porcine values. It follows the Guide to the Expression of Uncertainty in Measurement [**169**]: standard uncertainties u_i are combined in quadrature, and Type B contributions bounded by a half-width a are rectangular ($u_i = a/\sqrt{3}$) unless stated otherwise.

A.1. Measurement Chain

Each thermistor (negative temperature coefficient, NTC; $R_{25} = 10$ kΩ, $\beta = 3976$ K) sat in a linearized divider and was read through an amplifier by a 16-bit data-acquisition system with a 20 V input range (slope 2.06 °C V^{-1} between the 37.0 and 20.5 °C plateaus). Human-donor channels were converted with a per-channel least-squares line through plateaus at 4.0, 20.5, and 37.0 °C in a dry-block calibrator, and porcine channels with a fixed linear transfer function trimmed at 20.5 and 37.0 °C (Section 2.2.4). The human lines miss their own plateaus by up to 0.57 °C, more than the 0.34 °C measured on the porcine chain in 2022, so the linearity terms of the human eyes come from them. The tables below are those of the human eyes. The porcine budget, computed in the same way, gives ±0.33 °C on an excursion at the proximal optic nerve, ±1.1 °C at the ring, and ±1.8 °C on a single reading; this dissertation uses the larger value of each, ±0.40, ±1.1, and ±1.8 °C, for both species.

The ring channel of the in-vivo study (Section 4.2.6) is not linearized: a straight line misses its plateaus by about 1 °C. Its readings were converted with a model of

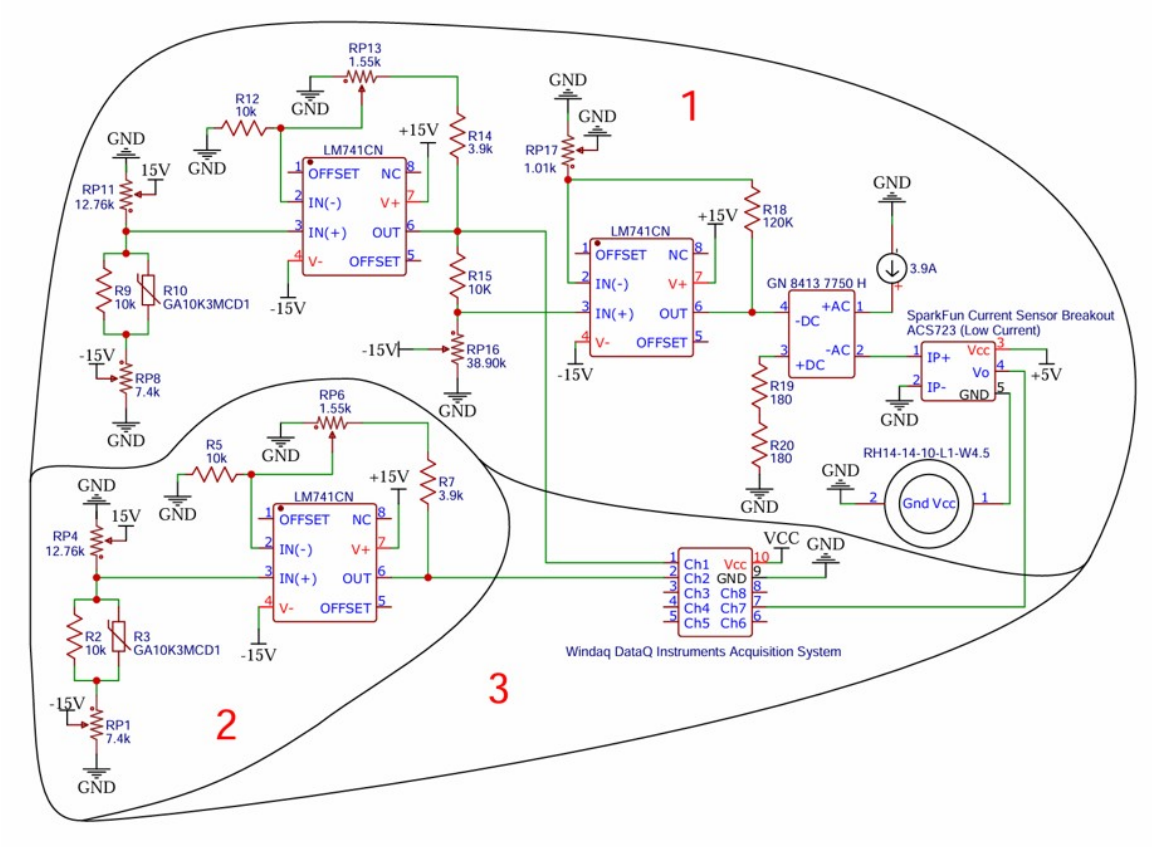


FIGURE A.1. Signal-conditioning and acquisition circuit: area 1, the channel of the SCR feedback thermistor with the automated supply circuit and the current-sensing board; area 2, one of the five identical channels of the implanted sensors; area 3, the data-acquisition unit.

the circuit,

$$V = \frac{aR + b}{R + R_{\mathrm{d}}}, \tag{A.1}$$

fitted to the plateaus recorded in each session, where V is the output voltage of the channel, R the resistance of the thermistor, a (V) and b (V Ω) constants set by the gain and offset of the amplifier, and R_{d} (Ω) the effective resistance of the divider in which the thermistor sits. The resistance follows the Steinhart–Hart law of the thermistor type (10K3A),

$$\frac{1}{T} = A + B \ln R + C(\ln R)^3, \tag{A.2}$$

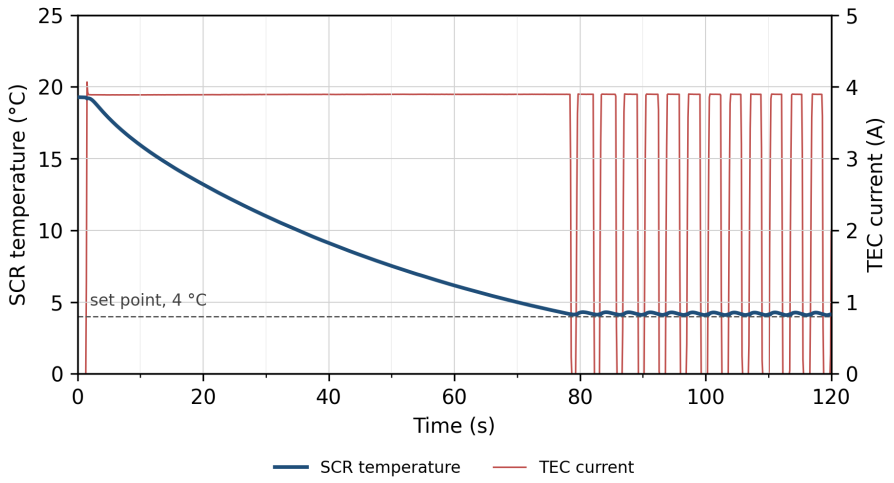


FIGURE A.2. Automatic feedback control of the SCR: the TEC drive current (right axis) is held at its maximum until the SCR temperature from the feedback thermistor (left axis) reaches the 4 °C set point, then the comparator switches the drive on and off to hold it. Bench recording on a preserved porcine eye (2023), not an analyzed run.

where T is the absolute temperature and $A = 1.129241 \times 10^{-3}$, $B = 2.341077 \times 10^{-4}$, and $C = 8.775468 \times 10^{-8}$ are the manufacturer's coefficients for R in ohms. Only a, b, and R_{d} were fitted; with five plateaus the residuals were at most 0.16 °C.

A.2. Absolute Uncertainty

Table A.1 lists the contributions to the uncertainty of a single reading. One term, sensor self-heating, accounts for 80% of the variance; without it the expanded uncertainty would be ±0.81 °C.

TABLE A.1. Uncertainty budget for a single absolute temperature reading. u_i is the standard uncertainty; the share is u_i^2/u_c^2.

Source	Basis	u_i (°C)	Share
Self-heating	one-sided bound 1.43 °C, not corrected (see below)	0.826	80%
Linear conversion	largest miss of a human calibration line at its own plateaus, 0.57 °C	0.329	13%
Calibrator accuracy	dry-block specification ±0.25 °C [**125**]	0.144	2%
Calibrator block uniformity	between wells of the aluminum insert, ±0.2 °C (the manufacturer's figure for the 9100S; none is given for the 9102S with a custom insert)	0.115	2%
Sensor interchangeability	thermistor tolerance ±0.2 °C [**124**]	0.115	2%
Dynamic lag	response time in liquids 0.2 s [**124**] at the fastest cooling rate measured, 0.54 °C s^{-1}	0.062	<1%
Drift after calibration	0.05 V recorded after calibration	0.059	<1%
Calibrator stability	±0.05 °C [**125**]	0.029	<1%
Reading noise (Type A)	standard deviation on the settled calibration plateaus	0.021	<1%
Lead resistance	9.4 Ω in series with the sensor, computed from the length and gauge of the leads and cable	0.022	<1%
DAQ accuracy	±5 mV	0.006	<1%
Combined standard uncertainty u_c		0.920	
Expanded uncertainty U ($k = 2$)		1.84	

Self-heating. The current through a thermistor warms it above the tissue it measures. Raising the excitation to the operating supply raised the reading by several degrees in the calibrator (Table A.1). The calibration was made at the operating

supply, and heat leaves the sensor more readily in tissue and oil than in the well, so a reading can only be too low. This underreading was bounded from the study's own data, since at steady state no submerged tissue can be warmer than the oil: in the eleven runs with a recorded oil temperature (36.9 to 37.5 °C) the oil exceeded the starting reading by at least 1.3 °C at the proximal optic nerve and 1.2 °C at the posterior sclera. Under the earlier calibration the proximal optic nerve could read at most 1.2 °C low, 21% of its rise in the dry block (5.66 °C); the same fraction of the largest dry-block rise, that of the vitreous sensor (6.74 °C), gives 1.43 °C, and this bound was used for every channel. No correction was applied, so the error is taken as rectangular between zero and the bound $a = 1.43$ °C, and its root-mean-square about zero, $u = a/\sqrt{3}$, enters the budget. The reported absolute temperatures can therefore be up to about 1.4 °C lower than the true tissue temperature, and not higher. The underreading largely cancels in ΔT; only its change with temperature remains (Table A.2).

A.3. Differential Uncertainty

A cooling excursion is the difference between two readings of one sensor, so the calibrator offset, sensor interchangeability, lead resistance, and most of the self-heating cancel. Four terms grow with the excursion: the slope of the calibration line, the curvature that a straight line ignores, the change of self-heating between the two temperatures, and the β tolerance of the implanted bead. Table A.2 gives the budget for a small excursion, the proximal optic nerve (34.6 to 30.8 °C), and a large one, the ring (29.7 to 4.1 °C).

The differential value at the proximal optic nerve is the uncertainty of a single excursion at the primary site; differences between groups are judged by their confidence intervals (Section 2.2.6).

TABLE A.2. Uncertainty budget for a within-trace cooling excursion ΔT: standard uncertainty u_i, in °C, for an excursion at the proximal optic nerve and at the ring.

Source	**Basis**	**Proximal optic nerve**	**Ring**
Bath stability	±0.2 °C	0.115	0.115
Drift during the run	as Table A.1	0.059	0.059
Linear conversion, on a difference	human calibration lines, 0.24 and 0.54 °C	0.139	0.311
Reading scatter	standard deviation of single samples about the filtered trace over the final window, median over the included traces	0.054	0.054
Slope of the calibration line	calibrator accuracy and uniformity at two set points, over the excursion	0.030	0.203
Change of self-heating with temperature	bound of 1.43 °C scaled by the change in dissipated power (source resistance 10.8 kΩ, the median estimated from 36 calibration channel-sessions)	0.026	0.140
Thermistor β tolerance	±0.5% [**124**]; the implanted beads are not the calibration set	0.012	0.070
Combined standard uncertainty u_c		0.202	0.427
Expanded uncertainty U ($k = 2$)		0.40	0.85

APPENDIX B

Reconciliation of Published and Reprocessed Fresh-Porcine Values

The fresh-porcine recordings of Nakamura et al. [**1**] were reprocessed for Chapter 2 with the pipeline of Section 2.2.6, which retained all 30 channel traces, so its gates change no value. The initial temperatures agree to 0.1 °C at every site, and the differences come from two definitions. The published final temperature is that of the averaged curve at 1200 s and the reprocessed one the mean over minutes 16–20 of each trace; this change of window raises the final temperature by 0.01 to 0.07 °C, and the rest of the −0.1 °C difference in ΔT comes from the reprocessed averaged curve ending up to 0.1 °C warmer at 1200 s than the published one, with the same initial temperatures. The published t_{90} is read from the averaged curve and the reprocessed one is the mean over the traces, so the largest difference, −27 s at the proximal optic nerve, is mostly one of definition. Tables B.1 and B.2 compare the two sets; no difference in ΔT or final temperature exceeds 0.1 °C at the published precision (0.13 °C against the two-decimal values of [**116**]), and none changes a conclusion of the published study.

TABLE B.1. Fresh-porcine cooling excursion ΔT, in °C, as published [**1**] (the difference of the published cohort-mean initial and final temperatures) and as reprocessed for Chapter 2; difference, reprocessed minus published. A conclusion is unchanged (last column) when the difference is below the differential uncertainty at the proximal optic nerve (±0.40 °C, Appendix A), as every difference is.

Location	**Published**	**Reprocessed**	**Difference**	**Unchanged?**
Anterior sclera	24.4	24.3	−0.1	Yes
Equatorial sclera	9.5	9.4	−0.1	Yes
Vitreous	11.7	11.6	−0.1	Yes
Posterior sclera	9.9	9.8	−0.1	Yes
Proximal optic nerve	3.6	3.5	−0.1	Yes

TABLE B.2. Fresh-porcine final temperature and time to 90% of the cooling excursion (t_{90}), as published [**1**] and as reprocessed for Chapter 2.

	Final temperature (°C)		t_{90} **(s)**	
Location	**Published**	**Reprocessed**	**Published**	**Reprocessed**
Anterior sclera	10.7	10.8	194	200
Equatorial sclera	25.0	25.1	338	340
Vitreous	22.3	22.4	569	566
Posterior sclera	24.4	24.5	532	519
Proximal optic nerve	30.2	30.3	703	676

APPENDIX C

Analysis Code

This appendix documents the code used to analyze the experiments of Chapters 2 and 4 and the computational model of Chapter 3. The ex-vivo and model analyses are each a Python package that runs from the raw data to every table and figure with one command, holds its parameters in one configuration file, and records the software versions and file checksums with its results.

C.1. Ex-Vivo Analysis Pipeline

The ex-vivo package reproduces the study's analysis notebook (version 4.28) exactly when the settings added after that version are switched off (Figure C.1).

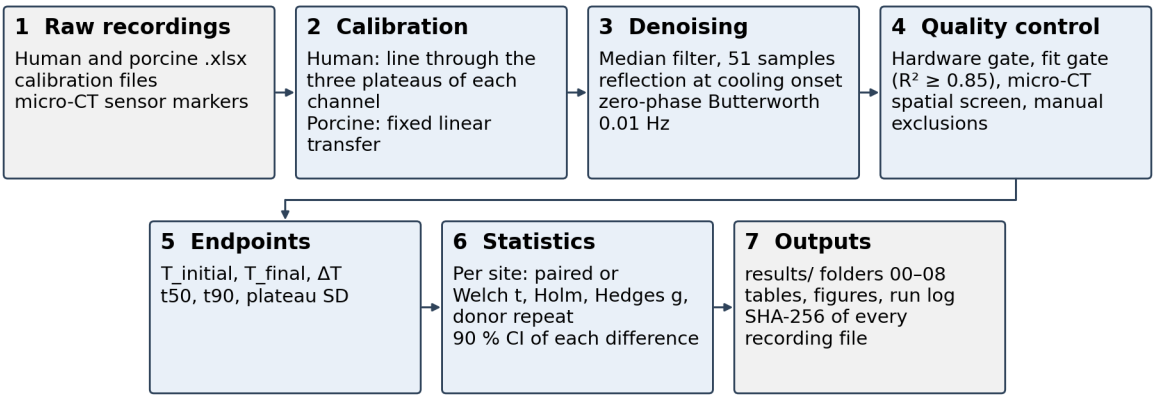


FIGURE C.1. Stages of the ex-vivo analysis, from the raw recordings to the tables and figures of Chapter 2.

C.2. Analysis Parameters

Table C.1 lists the parameters of the method (Section 2.2.6).

Table C.1: Parameters of the ex-vivo analysis.

Stage	**Parameter and value**
Acquisition	sampling interval 0.1 s
Calibration	human: per-channel least-squares line through the plateaus at 4.0, 20.5, and 37.0 °C, each plateau located by k-means clustering of the voltages (three clusters, seed 42) and read as the median of its last 3 min before the next ramp (last 10 s excluded), or of its longest stretch of 10-s medians within ±0.01 V where those 3 min drifted by more than 5 mV per minute, or of the whole recorded plateau, first and last 10 s excluded, where less than 3 min 10 s was recorded; porcine: $T = -2.0656\,V + 20.439$ °C for all channels, with V the recorded channel voltage in volts and T in °C; readings outside 0–43 °C set to missing
Denoising	median filter 51 samples (5.1 s), edge-value padding; point-reflected extension of 300 s before the first sample; second-order Butterworth low-pass, 0.01 Hz, applied forward and backward (zero phase)
Channel corrections	human eye 4: equatorial and anterior channels exchanged (the anterior sensor was recorded on the equatorial channel); eye 6: the calibration lines of the anterior and posterior channels exchanged, each column converted with the other channel's calibration
Hardware gate	at least 100 samples; flat line if the raw-voltage SD is below 10^{-4} V; stuck if more than 90% of samples sit at the median value; railed if more than 75% of samples lie beyond ±9.9 V; and by hand, the failing anterior-sclera sensor of human eye 9 (original ring)
Fit gate	single-exponential fit $T = A\,e^{-kt} + C$ (bounds $0 \le A \le 50$, $0 \le k \le 1$ s^{-1}, $0 \le C \le 40$ °C), $R^2 \ge 0.85$, at least 20 samples
Spatial screen	sensors identified by their marker labels; the four sensors fixed to tissue (anterior, equatorial, and posterior sclera, proximal optic nerve) of each eye rigidly aligned (rotation and translation) to the cohort mean; limit on the offset from the mean position of one thermistor length (3.3 mm); vitreous sensor excluded if more than 75% of the distance from the center of the globe to its wall on micro-CT; a spatial breach takes precedence over an earlier reason
Manual exclusions	four traces: three in the original-ring data of Chapter 2 (Section 2.2.6) and one, the vitreous sensor of human eye 4 under the reduced-bulk ring, in the ring comparisons of Chapter 3
Endpoints	final temperature and plateau SD over minutes 16–20 (240 s); t_{50}, t_{90} as the first times the trace has fallen by 50% and 90% of ΔT

Table C.1: Parameters of the ex-vivo analysis (continued).

Stage	Parameter and value
Statistics	paired t test when the two groups share at least two eyes, otherwise Welch's t test; Hedges' g; $\alpha = 0.05$; Holm correction over the five tissue sites; 90% confidence interval of each difference (its limit farther from zero is the largest difference the data allow); species comparison repeated on donor means

In the spatial screen, the offset of sensor s in eye e from the mean set of the cohort was

$$r_{e,s} = \left\| \mathbf{R}_e \, \mathbf{x}_{e,s} + \mathbf{t}_e - \bar{\mathbf{x}}_s \right\| , \tag{C.1}$$

where $\mathbf{x}_{e,s}$ is the micro-CT position of sensor s in eye e, $\mathbf{R}_e$ and $\mathbf{t}_e$ the rotation and translation that best map the sensor set of eye e onto the mean set in the least-squares sense, and $\bar{\mathbf{x}}_s$ the mean aligned position of sensor s over the eyes of the cohort. The alignment removes the position and orientation of each eye in the scanner.

C.3. ERG Processing

The width of the b-wave and the number of its peaks (Section 4.2.9) are computed by `erg_cohort.py` from the same filtered trace as the picks. The analysis used the raw record of each flash step, sampled every 0.512 ms, not the instrument's reported waveform. Drift was removed with a 0.3 Hz high-pass filter (second-order Butterworth, forward and backward) [**138**]. Sweeps from 20 ms before to 200 ms after each flash were referred to their 20-ms pre-flash mean and interpolated onto a 0.5-ms grid. A sweep was rejected when the noise in the 1 s before its flash exceeded three times the median over the recording's sweeps, or its b-wave amplitude differed from the median of those sweeps by more than half.

The accepted sweeps were averaged and low-pass filtered at 75 Hz (fourth-order Butterworth, forward and backward). The a-wave was the minimum 2 to 26 ms after the flash, measured from the pre-flash baseline, and the b-wave the maximum 8 to 50 ms after the flash and after that trough, measured from the a-wave trough [**138**].

For the oscillatory potentials, the accepted sweeps of each flash step, without its first flash, were averaged without the 75-Hz low-pass filter and then band-pass filtered from 75 to 300 Hz (eighth-order Butterworth from a fourth-order prototype, forward and backward) [**138**]. The oscillatory potentials were the positive peaks on the rising limb of the b-wave, from the a-wave trough (not earlier than 10 ms after the flash) to 5 ms after the b-wave peak, each measured from the trough before it. The measure is the sum of the first four positive peaks, or of three where only three were present, with the root-mean-square over the same window alongside, at 8 and 25 cd·s/m^2.

Replacing the high-pass filter with a straight line fitted to the 1 s before each flash and subtracted changed the b-wave amplitude by a median of 1 μV and the implicit time by a median of 0 ms (at most 77 μV and 1.5 ms, apart from the two implicit times corrected by hand) over the 194 traces at each of the two stronger flashes. The values reported by the instrument were not used. A trace was flagged when the residual drift of its recording before the flash exceeded three times the median of all recordings (42 of the 388 traces of animals 1 to 6 at the two stronger flashes), when a pick fell on the edge of its search window (9), when the mean level between 150 and 200 ms after the flash was beyond $\pm$150 μV, a sign of drift or a slow potential (5), or when the noise before the flash had a standard deviation above 15 μV (2) or its mean was more than 10 μV from zero (none). Flagged traces were checked against the waveform; none of the checks changes a value. Two implicit times were corrected by hand and are kept in a separate file with the detector's values, and the phase of each recording was assigned by hand from the session log.

C.4. Globe Outline and Position of the Vitreous Sensor

For micro-CT after the experiment the thermistor wires were cut; for both MRI (porcine eyes, before implantation) and micro-CT the eye on its holder was sealed in a container (Figure C.4), filled with oil for most porcine eyes and for one scan of human eye 2, and with air otherwise. MRI at 9.4 T (Agilent, Santa Clara, CA, USA) used

a multislice gradient-echo sequence with $0.078 \times 0.078 \times 0.8$ mm voxels (repetition time 550 ms, echo time 7.5 ms, flip angle 20°, 36 contiguous slices, 16 averages) and a 72-mm volume coil; the oil is displaced in these images by about 1.1 mm along the readout direction relative to the water-bearing tissue, which is imaged in place. Micro-CT (North Star Imaging, Rogers, MN, USA) used 70 kV and 1200 μA (60 kV and 550 μA for human eyes 5 to 9), reconstructed at isotropic voxels of 32 to 79 μm.

The outline of each globe on micro-CT was found automatically: a smooth closed surface fitted to the tissue edges along rays from the center of the globe in the fresh eyes, and the tissue mask after a 3 mm morphological opening in the folded preserved porcine eyes. Every outline was inspected on fifteen sections and moved by hand onto the wall where it departed from it (18 of the 19 eyes), after which it followed the drawn lines to a median distance of 0.2 mm or less. The center of the globe is the centroid of the automatic outline; the axis is the normal of the holder ring for human eyes 3 to 9, the line from the center through the lens for human eyes 1 and 2 and the fresh and three preserved porcine eyes, and the line from the optic-nerve sensor through the center for preserved eyes 4 and 5. In human eye 2 the direction of the nerve stump, which disagreed with the lens, was rejected because it placed the optic-nerve sensor 92° from the anterior pole. In preserved eyes 1 to 3 the lens axis places the optic-nerve sensor 88 to 113° from the anterior pole, and no result uses these axes. The code is archived with the ex-vivo data (`CT Segmentation/code`) and in `code/ch2-ct-segmentation` of the repository; its results enter the package as `config/vitreous_position_ct.csv`.

The position of the vitreous sensor was the ratio r/R, where r is the distance from the center of the outline to the sensor and R the distance from the center to the outline in the same direction (0 at the center of the globe, 1 at its wall). A vitreous sensor was excluded when r/R exceeded 0.75 (Figures C.2 and C.3), as in human eyes 5 and 8 (0.82 and 0.81).

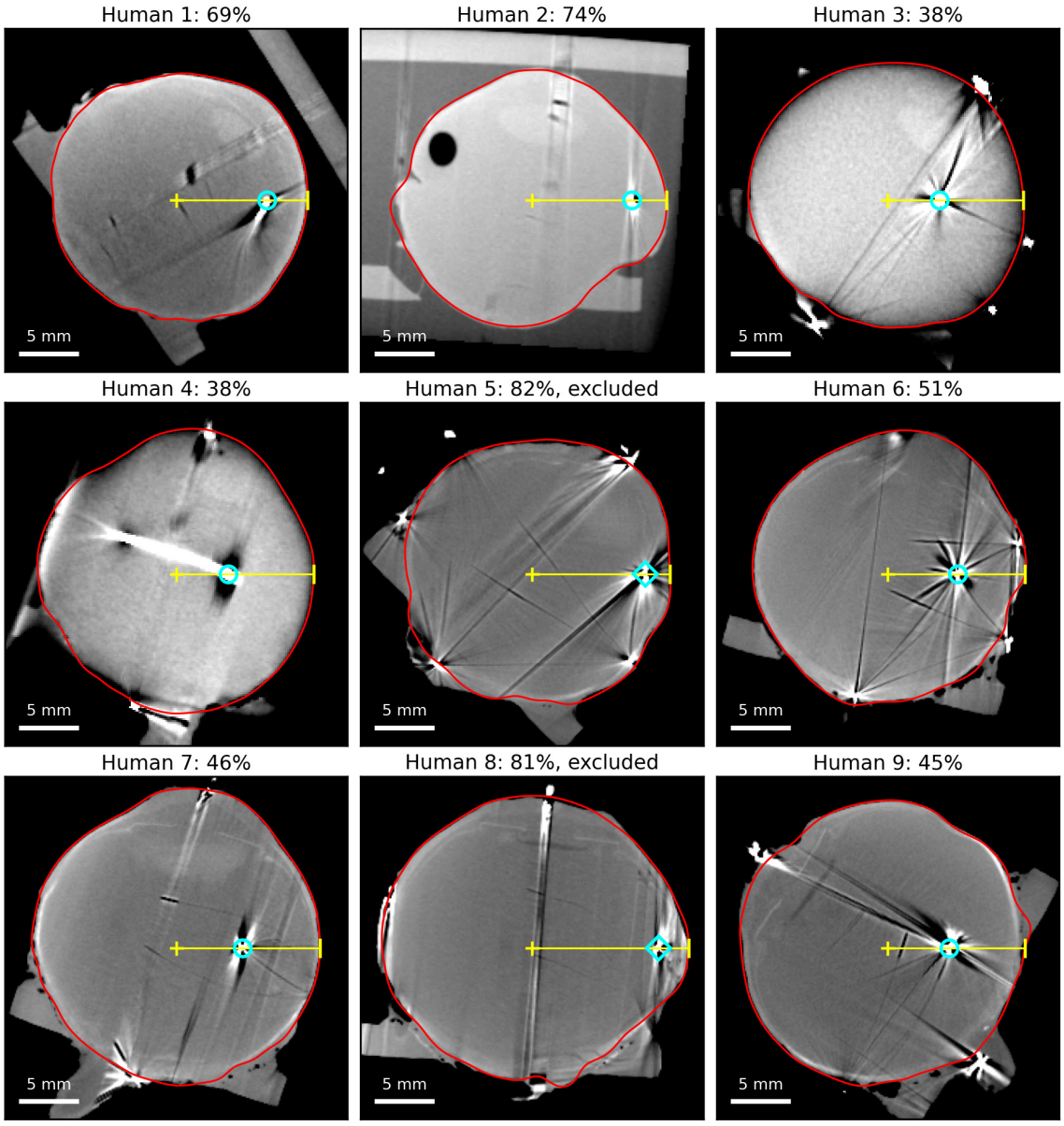


FIGURE C.2. Position of the vitreous sensor in the nine human eyes: micro-CT section through the center of the globe outline (cross) and the sensor (cyan marker), with the outline in red and the line from the center to the wall in yellow. The number is the ratio r/R of Section C.4, as a percentage; a diamond marks a sensor beyond the limit of 75%, which was excluded.

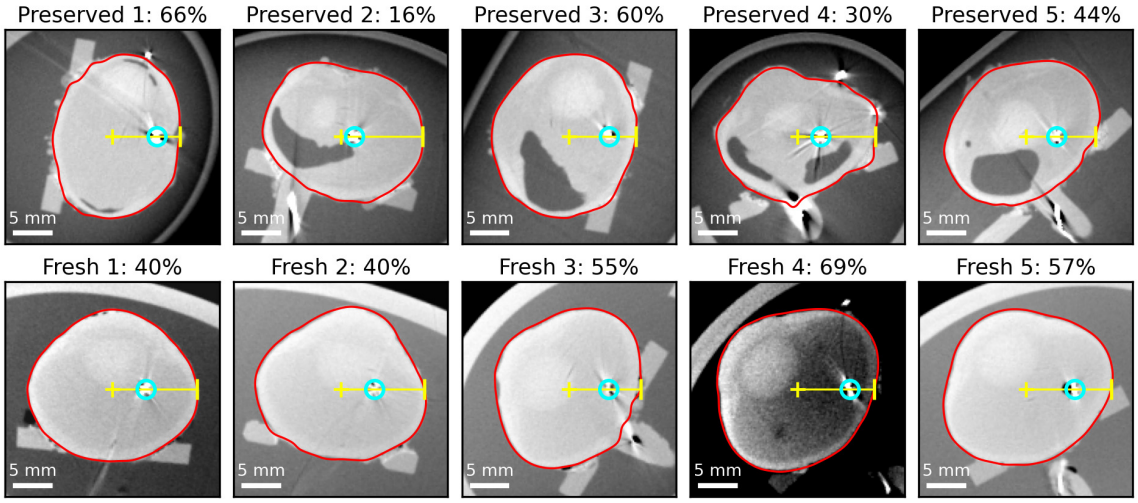


FIGURE C.3. Position of the vitreous sensor in the five preserved (top) and five fresh (bottom) porcine eyes, drawn as in Figure C.2. No porcine vitreous sensor lay beyond the limit of 75%.

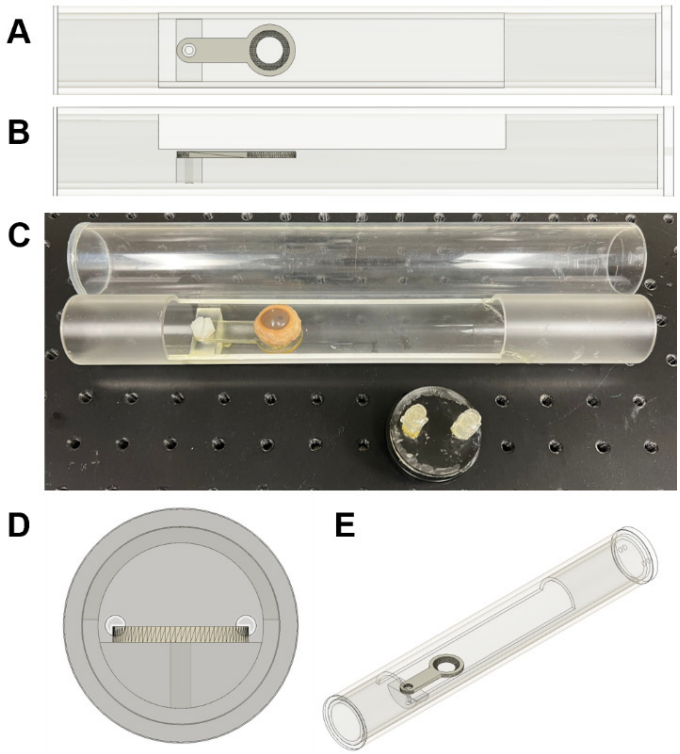


FIGURE C.4. Sealed container, filled with oil or air, used for MRI and micro-CT. Top: top and side views of the CAD model. Middle: the container with an eye on its holder, and the cap with the two valves used to evacuate air. Bottom: front view of the cap and panoramic view of the container (E).

C.5. Reproducing the Analysis

The package was run with Python 3.13, NumPy 2.2.6, pandas 2.3.2, SciPy 1.16.1, statsmodels 0.14.6, scikit-learn 1.9.0, Matplotlib 3.10.6, and openpyxl 3.1.5, pinned in its requirements file because later releases no longer reproduce the stored references exactly. The analysis is run from `code/ch2-exvivo-analysis` with

```
pip install -r requirements.txt
python run_analysis.py --data <recordings folder> --out results
python tests/test_reproduce.py <recordings folder>
```

`tests/test_default.py` is a regression test of the current settings. Both tests pass on the version used in this dissertation (Section C.7).

C.6. Computational Model Analysis

The COMSOL exports, which the Python program of Chapter 3 was validated against, were analyzed with a second package of the same design (Figure C.5), which reads the ex-vivo results from `code/ch2-exvivo-analysis/results` and so is run after the ex-vivo package. Eleven corrections, listed in the package README, are applied by default, among them the doubling of the heat exported for the face of the module and the division of the heat among eye, eyelids, and air, which is read from tables recomputed from the boundary fluxes stored in the COMSOL files (`code/ch3-heat-replica`).

The figures of Chapter 3 are drawn by the scripts in `code/figures`, each listed with its inputs and their checksums in the manifest of its folder. The eye-model temperatures and heat values come from the corrected runs, the validation from the replication runs, and the heat-exchanger results from the sweep of `code/ch3-heat-replica`. The effectiveness, the ring temperatures needed for the targets, and the cooling and rewarming times are computed from those runs by `code/figures/aim2_python_corrected.py`; the package computes the same

quantities from the COMSOL exports, and these are stored with its results but not reported.

Mesh sensitivity. In COMSOL, temperature, velocity, and pressure were interpolated with linear elements, the stationary initial state was solved to a relative tolerance of 10^{-3}, and the transient was integrated with a variable-step backward differentiation formula of order 1 to 2 at a relative tolerance of 0.005. The eye model was solved for each ring on three meshes, from COMSOL's *Coarser*, *Coarse*, and *Normal* presets with local refinement of the retina, choroid, and tear film (about 14 000, 35 000, and 164 000 to 171 000 vertices without the air and the module layer, as solved; Table 3.2), and the heat exchangers on three meshes in the same way.

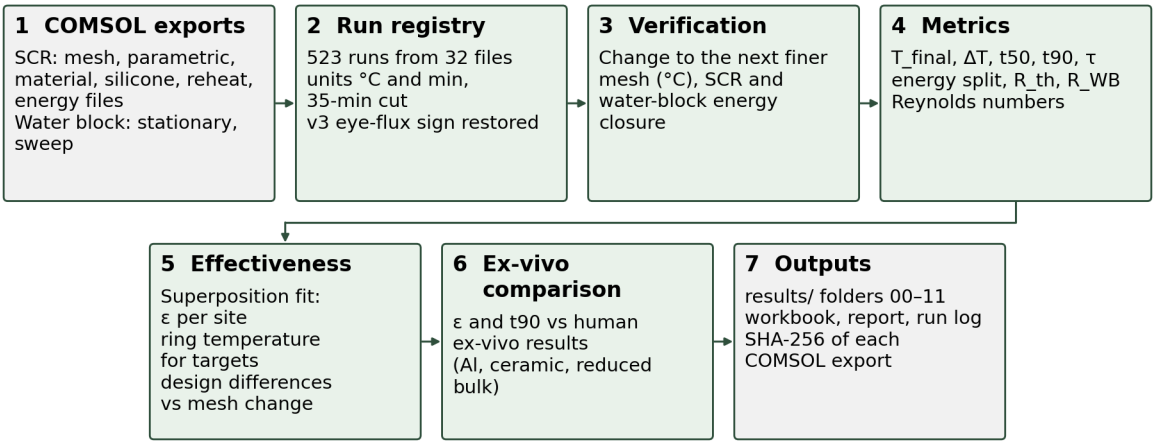


FIGURE C.5. Stages of the computational model analysis of the COMSOL exports. The eye-model results of Chapter 3 come from the Python runs; this package supplies the ex-vivo side of the model comparison.

Python finite-element program. The program reads the meshes, the material definitions, and the boundary conditions from the COMSOL model files and solves the heat equation with linear tetrahedral elements and a second-order backward differentiation formula, with fixed steps of 6 s for the first 5 min, 15 s to 15 min, and 30 s afterwards, the first step by backward Euler. Within each step, radiation was solved by Newton iteration until the largest change of temperature was below 10^{-5} °C (10^{-8} °C for the stationary starting state), and the temperature-dependent properties were taken at the temperature extrapolated from the two previous steps; the linear

systems were solved with PARDISO, and those of the coolant flow were checked to a relative residual of 10^{-9} and solved again with SuperLU when PARDISO did not reach it. After switch-off the face of the ring that met the module was left insulated, and the steps were 0.5 s to 6 s, 1 s to 18 s, and 3 s to 1 min, then as during cooling; with one 6 s step across the switch-off the ring was up to 0.33 °C off COMSOL. In the heat exchangers the coolant flow was solved as laminar incompressible flow (Navier–Stokes, quadratic velocity and linear pressure elements), with no-slip walls, a uniform inlet velocity giving the set flow rate, zero pressure at the outlet, the viscosity of water at the inlet temperature, and no buoyancy. Where the flow did not settle it was advanced in time, and the temperature was advanced with it and averaged over three transits of the coolant. The corrected and extension cases follow Section 3.2.2.1. The flow solver and the reading of the model files are verified only by the agreement with COMSOL.

C.7. Code Availability

The code is kept in a public repository rather than reproduced here: `https://github.com/GigiosGros/eyecooler-thesis`. Folder `code/ch2-exvivo-analysis` holds the ex-vivo analysis of Chapter 2, in the version used here, `code/ch2-ct-segmentation` the segmentation of the micro-CT and MRI scans, `code/ch3-model-analysis` the analysis of the COMSOL exports and `code/ch3-heat-replica` the finite-element program of Chapter 3, and `code/ch4-live-pigs` the processing of the in-vivo recordings (temperature conversion, ERG splitting and extraction, the summaries of intraocular pressure, ocular surface grades, cooling and systemic parameters, and the cohort statistics of Chapter 4). The scripts that draw the figures are in `code/figures`, the figures in `figures`, and this dissertation in `thesis`. The raw recordings, the COMSOL model files, and the inputs and the list of cases of the finite-element program are available from the author on request; the probe temperatures and heat flows of the corrected

runs, from which the numbers of Chapter 3 are computed, are available from the author on request.

APPENDIX D

Tissue Properties, Boundary Conditions, and Meshes of the Computational Model

The eye measures 24.8 mm from the corneal apex to the choroid at the posterior pole and 25.0 mm across the outer surface of the sclera; the cornea is 11.5 mm across and 0.55 mm thick at its center, the anterior chamber 3.0 mm deep, the lens 9.0 mm across and 4.5 mm thick, and the sclera 0.42 mm thick on average.

Published thermal properties of the eye differ widely and most come from animals [**51**], so each property of the model (Table 3.3) was estimated from the composition of the tissue, averaged over published values, or taken from the IT'IS database [**170**]; the tear film and the ring materials came from the COMSOL library. Regional variation was not represented. The mesh and boundary figures, like the schematics of Appendix F, are at full resolution in the electronic copy.

D.1. Estimates from Tissue Composition

Most ocular tissues were treated as water containing a dispersed solid phase, with the conductivity given by the Hashin–Shtrikman upper bound, with water as the continuous phase [**171**],

$$k = k_w + \frac{1-\phi_w}{\dfrac{1}{k_s - k_w} + \dfrac{\phi_w}{3k_w}}, \tag{D.1}$$

where k is the thermal conductivity of the tissue, ϕ_w its water fraction, $k_w = 0.6\ \mathrm{W\,m^{-1}\,K^{-1}}$ the conductivity of water, and k_s that of the solids: 0.41 (cornea), 0.33 (vitreous), 0.23 (ciliary body), 0.20 (retina), 0.22 (choroid), and $0.43\ \mathrm{W\,m^{-1}\,K^{-1}}$ (sclera). The specific heat c_p and the density ρ followed from the same fractions, $c_p = [\phi_w + 0.4\,(1-\phi_w)] \times 4184\ \mathrm{J\,kg^{-1}\,K^{-1}}$ and $\rho = [\phi_w + (1-\phi_w)\,\rho_s] \times 1000\ \mathrm{kg\,m^{-3}}$,

where 4184 $\mathrm{J\,kg^{-1}\,K^{-1}}$ and 1000 $\mathrm{kg\,m^{-3}}$ are the specific heat and the density of water, 0.4 the specific heat of the solids relative to water, and ρ_s the density of the solids, 1.37, 1.48, 1.35, 1.16, 1.37, and 1.34 $\mathrm{g\,cm^{-3}}$ in the same order (1.34 for the lens).

Checked against published compositions [**172, 27, 173, 174, 175, 176, 177, 178, 179, 180, 170**] and component properties [**181, 182, 183, 184**], the solid-phase conductivities computed from these compositions reproduce the conductivities used within 0.01 $\mathrm{W\,m^{-1}\,K^{-1}}$, except for the cornea and the sclera (0.51 and 0.47 against 0.56 and 0.545 used, which lie closer to the measured means [**170**]).

D.2. Perfusion and Metabolic Heat

The perfusion rate of each tissue followed from its measured blood flow, $\omega = Q_{\mathrm{blood}}/(60\,V)$, where ω is the perfusion rate in $\mathrm{s^{-1}}$, Q_{blood} the blood flow in $\mu\mathrm{L\,min^{-1}}$, V the volume of the domain in $\mathrm{mm^3}$, and 60 converts minutes to seconds. The choroid took the mean total choroidal flow per eye measured with microspheres in four eyes of three monkeys, 917.5 $\mu\mathrm{L\,min^{-1}}$ [**42**], over 165 $\mathrm{mm^3}$ (0.0927 $\mathrm{s^{-1}}$; indirect estimates for the human eye are of the same order [**185, 43, 110**]); the retina the mean of six laser Doppler studies in humans (33, 80, 38.1, 73, 64.9 and 38.1 $\mu\mathrm{L\,min^{-1}}$), 54.5 $\mu\mathrm{L\,min^{-1}}$ [**32, 33, 34, 36, 35, 186**], over 236 $\mathrm{mm^3}$ (0.0039 $\mathrm{s^{-1}}$); and the ciliary body 133 $\mu\mathrm{L\,min^{-1}}$ [**187**] over 348.25 $\mathrm{mm^3}$. The extraocular muscles, eyelids, and orbital fat used published flows per unit mass [**188, 189**] and the density of the tissue; for the eyelids no measured value was found and the rate is an assumption.

The metabolic heat was the specific metabolic rate of each tissue [**190**] as a volumetric source, $Q_{\mathrm{met}} = \dot{m}\,\rho \times 4184/86\,400$, where Q_{met} is the metabolic heat in $\mathrm{W\,m^{-3}}$, $\dot{m}$ the metabolic rate in $\mathrm{kcal\,kg^{-1}\,day^{-1}}$, ρ the density in $\mathrm{kg\,m^{-3}}$, 4184 the number of joules in a kilocalorie, and 86 400 the number of seconds in a day; for the retina it was derived from the oxygen consumption [**191**], 3443 $\mathrm{W\,m^{-3}}$. The blood density and specific heat, $\rho_b = 1068\ \mathrm{kg\,m^{-3}}$ and $c_b = 3682\ \mathrm{J\,kg^{-1}\,K^{-1}}$, were composition estimates close to the IT'IS values [**170**].

D.3. Values Used

Table D.1 lists the values used, their basis, and the published values compared. The rings of aluminum, aluminum oxide (alpha, polycrystalline), and fused silica had emissivities of 0.05, 0.95, and 0.90 (the COMSOL models used 0.05 for every ring).

TABLE D.1. Thermal properties of the eye model, their basis, and the published values they were compared with (k in $\mathrm{W\,m^{-1}\,K^{-1}}$, ρ in $\mathrm{kg\,m^{-3}}$, c_p in $\mathrm{J\,kg^{-1}\,K^{-1}}$, ω in $\mathrm{s^{-1}}$).

Domain	**Used (k; ρ; c_p; ω)**	**Basis**	**Published values compared**
Cornea	0.56; 1070; 3707; –	k set near the measured mean (the composition gives 0.51); ρ, c_p composition estimate (81% water [**170**])	k 0.35–0.58 [**192, 51, 170, 193**]
Aqueous humor	0.593; 1005; 4159; –	k the mean of four published values [**192, 194, 195, 196**]; ρ from [**195**]; c_p composition estimate	k 0.58 [**170**]
Lens	0.43; 1126; 3255; –	k from its water content [**196**]; ρ, c_p composition estimate (63% water, the nuclear value [**176**])	k 0.40–0.43 [**197, 170**]
Vitreous humor	0.59; 1010; 4134; –	Composition estimate (98% water); viscosity 0.7 Pa s, within the 0.2–1 Pa s range of the complex viscosity of the liquid phase [**177**]	k 0.58–0.60 [**51, 197, 192, 170**]
Ciliary body	0.49; 1088; 3556; 0.0064	Composition estimate; perfusion from ciliary blood flow [**187**]	k 0.49–0.50 [**170**]
Retina	0.55; 1014; 3958; 0.0039	Composition estimate (91% water [**178**]); perfusion from retinal blood flow [**32, 33, 34, 36, 35, 186**]	k 0.55–0.57 [**198, 170**]
Choroid	0.51; 1074; 3682; 0.0927	Composition estimate (80% water; the IT'IS database, which gives the choroid the properties of blood, lists 80.4% [**170**]); perfusion from choroidal blood flow [**42**]	k 0.52–0.53 [**198, 170**]
Sclera	0.545; 1102; 3431; –	k set near the measured mean (the composition gives 0.47); ρ, c_p composition estimate (70% water [**172**])	k 0.58–1.0 [**199, 198, 192, 170**]; the value of 1.0 traces to a single measurement and exceeds the conductivity of water
Extraocular muscles	0.49; 1090; 3421; 0.0045	IT'IS database (muscle) [**170**]; perfusion from extraocular muscle blood flow in primates [**188**]	k 0.57 [**52**]
Eyelids	0.34; 1070; 3660; 0.0011	Eyelid dermis [**200**]; perfusion from skin blood flow	k 0.37 (skin) [**170**]
Orbital fat	0.2245; 920; 2500; 0.0004	k the mean of published values; ρ, c_p [**198**]; perfusion from adipose blood flow [**189**]	k 0.20–0.25 (subcutaneous) [**195, 198**]; IT'IS 0.21 [**170**]
Tear film	water	COMSOL library (water); evaporation 12 $\mathrm{W\,m^{-2}}$	12.7–20.5 $\mathrm{g\,m^{-2}\,h^{-1}}$ [**201, 202**]

D.4. Fluid Domains and Boundary Conditions

This section describes the COMSOL models, which the replication followed; the corrected runs differ as stated in Section 3.2.2.1, and their boundary conditions are drawn in Figure D.1.

The aqueous humor, the vitreous humor, and the air were laminar, non-isothermal fluids under gravity (the viscosity of the vitreous is in Table D.1). Aqueous humor entered and left at 1.23×10^{-6} $\mathrm{m\,s^{-1}}$, and the intraocular pressure was 2000 Pa (15 mmHg). The optic nerve had an outer sheath with the properties of the sclera and a core with those of the retina. The outer shell of the orbit was held at body temperature and that of the air domain at ambient temperature; where the two meet on the eyelid, COMSOL and the Python program impose their mean on 227 nodes. The cornea (v1, v2) lost heat by tear evaporation, and surfaces exposed to air radiated to the surroundings with emissivities of 0.98 for tissue and 0.05 for every ring. The ring was in perfect contact with the eye and the eyelids; a 5-μm film in the Python program raised the anterior sclera by at most 0.14 °C.

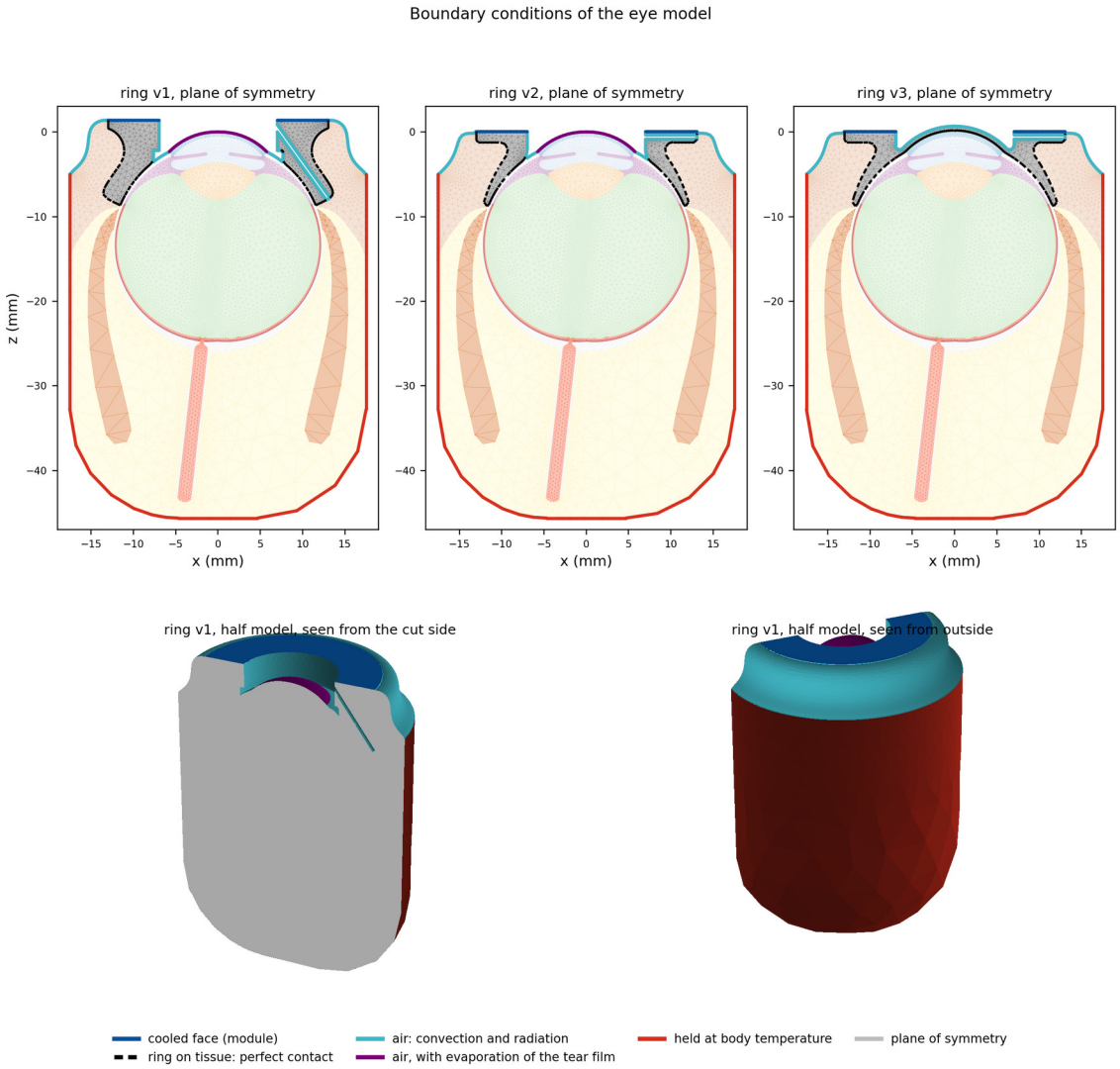


FIGURE D.1. Boundary conditions of the eye model in the corrected runs, in the plane of symmetry for the three rings and in three dimensions for v1. The face of the ring that meets the module is cooled; exposed surfaces lose heat to the room by convection and radiation, and the cornea also by evaporation of the tear film (covered by the lens in v3); the outer boundary of the orbit is held at body temperature; the ring is in perfect thermal contact with the tissue.

D.5. Boundary Conditions and Meshes of the Heat Exchanger

Figure D.2 shows the boundary conditions of each heat exchanger (Section 3.2.2.3).

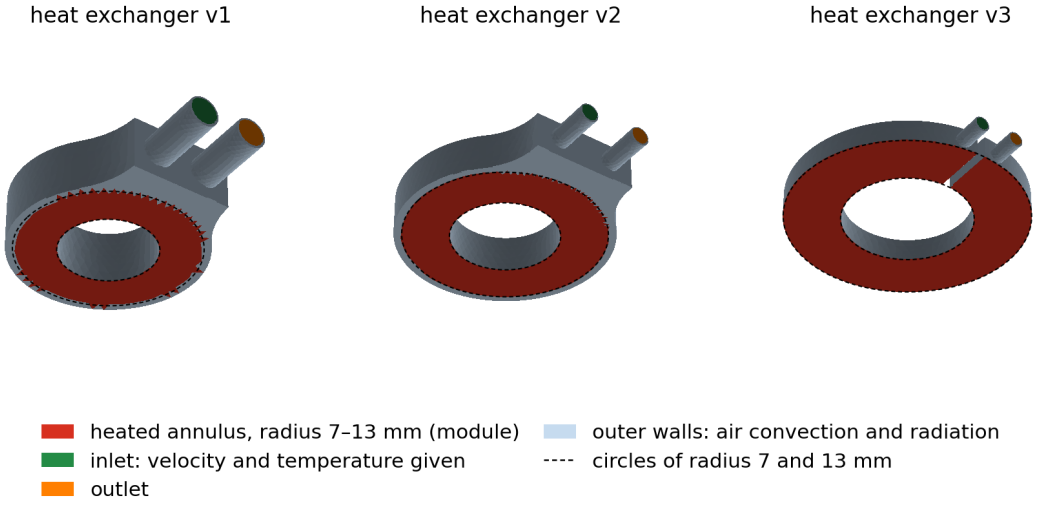


FIGURE D.2. Boundary conditions of the heat exchangers, seen from the module side: the module footprint, the annulus of radius 7 to 13 mm on which the heat enters (red; dashed circles); the inlet, with the coolant velocity and temperature given (green); the outlet (orange); and the outer walls, which exchange heat with the ambient air by natural convection and radiation (gray-blue).

The heated face rose above the coolant by 0.24, 0.12, and 0.07 °C per watt in v1 at 0.10, 0.25, and 0.50 L min^{-1}, by 0.15, 0.07, and 0.04 in v2, and by 0.12, 0.05, and 0.03 in v3 (means over the three inlet temperatures, intermediate mesh). In the converged cases these values changed by up to 12% from one mesh to the next, and the order of the designs was the same on every mesh. They are lower bounds: on meshes with resolved wall layers the rise of v1 converges to about 0.32 and 0.18 °C per watt at 0.10 and 0.25 L min^{-1}, and the values here lie 24 to 35% below that limit.

The Reynolds number at the narrowest section of the channels was 170 to 1240 in v1, 360 to 2690 in v2, and 510 to 3800 in v3: laminar in v1 and laminar to transitional in v2 and v3.

D.6. Meshes and Mesh Sensitivity

Figure D.3 shows the meshes of the eye model.

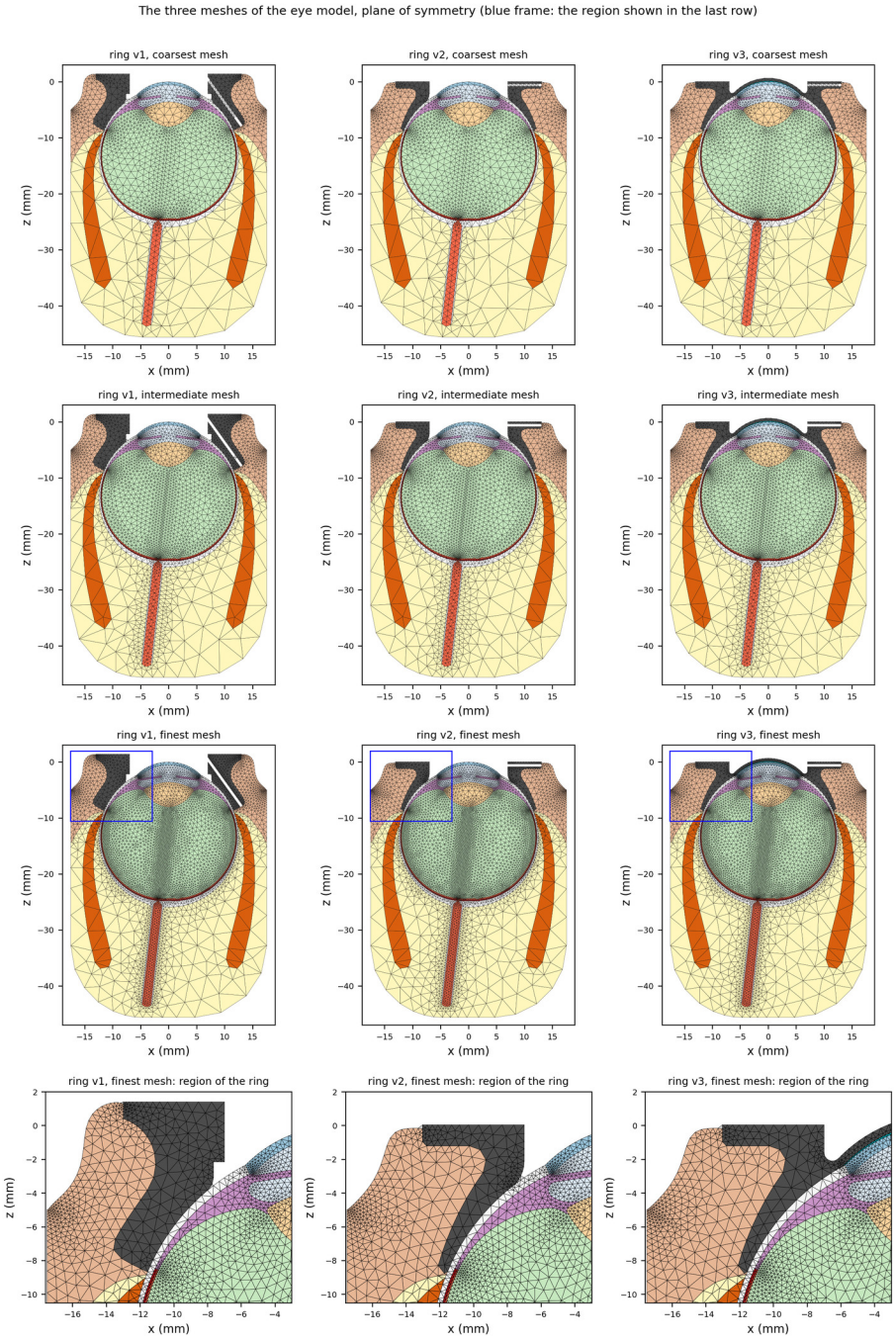


FIGURE D.3. The three COMSOL meshes of the eye model for each ring, in the plane of symmetry, without the air and the module layer as in the corrected runs, from the coarsest (top row) to the finest (third row), on which the results were computed. Bottom row, the region of the ring on the finest mesh enlarged (blue frame in the row above).

In COMSOL the energy balance of the ring closed to within 0.2% after the first 5 min in eight of the nine ring and layer configurations; in the ninth, v2 with the 1.0 mm layer, whose model file stores no mesh, the exported faces account for 84% of the heat. From the intermediate to the finest mesh the final temperature rose by 0.01 to 0.04 °C at the optic nerve and the posterior sclera, 0.04 to 0.08 °C at the equatorial sclera and the vitreous, and 0.07 to 0.11 °C in the average over the retina (Table D.2); with v3 the change at the posterior and equatorial sclera grew from the first step to the second, so these sites had not converged; at the anterior sclera the two steps had opposite signs, so no trend can be read there.

This change (bars in Figure D.4) is the threshold below which differences between designs are not interpreted, not the error of the finest mesh, which reads low (proximal optic nerve, vitreous, anterior sclera) by 0.02, 0.06 to 0.08, and 0.36 to 0.62 °C against the same mesh with every element divided into eight, and by 0.03 to 0.04, 0.10 to 0.14, and 0.75 to 1.28 °C against the extrapolated value; anterior-sclera temperatures are therefore lower bounds. Table D.2 and Figure D.4 come from runs of the Python program on these meshes in the corrected set-up, without the air and the module layer (`mesh_steps.csv` in `figures/aim2/sources/data/python/corrected`).

TABLE D.2. Change of the final temperature at each site of the eye model from one mesh to the next finer, in °C, at the baseline condition, for each ring: (a) coarsest to intermediate and (b) intermediate to finest (about 14 000, 35 000, and 164 000 to 171 000 vertices, without the air and the module layer).

Site	**v1 (a)**	**v1 (b)**	**v2 (a)**	**v2 (b)**	**v3 (a)**	**v3 (b)**
Anterior sclera	−0.21	+0.20	−0.10	+0.29	−0.12	+0.37
Equatorial sclera	+0.07	+0.04	+0.12	+0.07	+0.06	+0.08
Posterior sclera	+0.02	+0.01	+0.04	+0.03	+0.01	+0.04
Vitreous	+0.09	+0.04	+0.12	+0.08	+0.10	+0.08
Proximal optic nerve	+0.03	+0.02	+0.05	+0.03	+0.03	+0.03
Distal optic nerve	+0.02	+0.01	+0.02	+0.02	+0.02	+0.01
Average retina	+0.16	+0.07	+0.20	+0.11	+0.18	+0.11
SCR	0.00	0.00	0.00	0.00	0.00	0.00

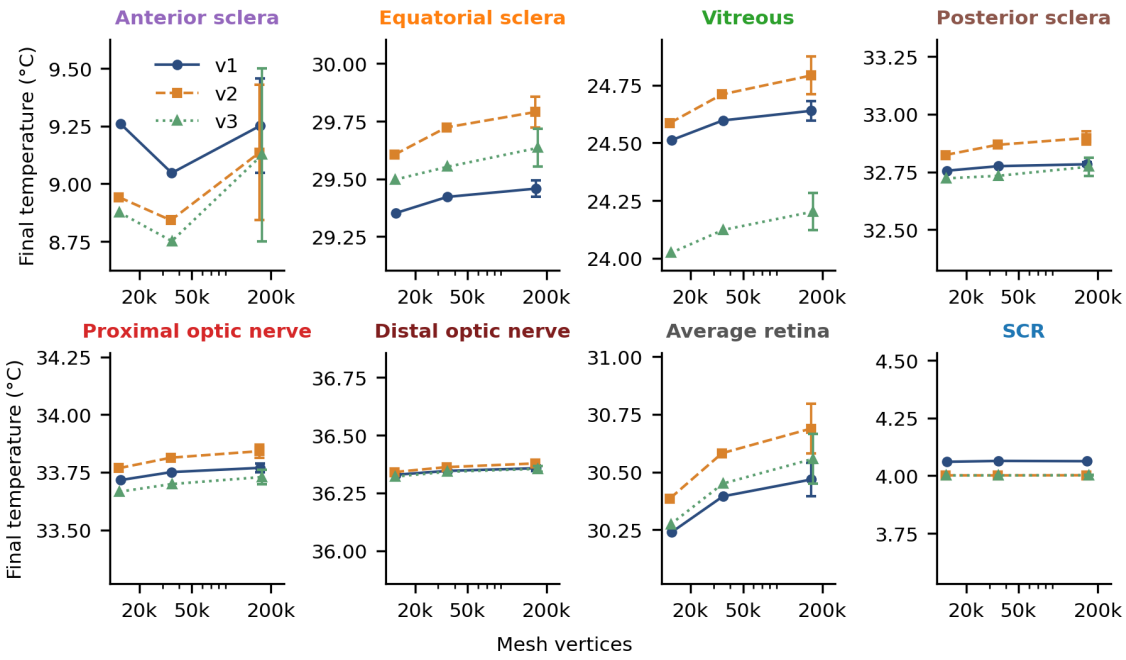


FIGURE D.4. Final temperature at each site of the eye model on the three meshes (vertex counts), for each ring. Each panel spans 1 °C around its own values, so changes can be compared between sites. The bar on the finest mesh spans ± the change from the intermediate to the finest mesh (Table D.2), a step between meshes, not the error of the finest mesh (see text).

From the intermediate to the finest mesh the laminar COMSOL solution of the heat exchanger changed by 0.03 °C at the module face and by 0.02 to 0.03 °C in the average temperature of the blocks. The Python program on the same meshes agreed within 0.04 °C at the outlet but only within 0.6 °C at the module face, whose temperature is therefore known to about 1 °C.

APPENDIX E

Selection of the Water-Block Coolant

The heat exchanger of SCEC Gen 3 was run with a 50:50 mixture of propylene glycol and distilled water in the porcine experiments of Chapter 2, and with distilled water alone in the human-donor experiments, in every in-vivo session, and in the water-block model of Chapter 3.

Propylene glycol lowers the freezing point, which was not needed here, since the ice bath could not cool the loop below 0 °C. Every thermal property that governs the loop is less favorable with it. Relative to water, pure propylene glycol has about one third of the thermal conductivity, 60% of the specific heat, and about 55 times the viscosity at 25 °C [**170, 203**], and water–glycol mixtures lie between the two.

The Lynch laboratory at the University of Illinois Chicago allowed the heat flow of each fluid to be measured by differential scanning calorimetry (Figure E.1). Samples of 11.6 to 16.5 mg in aluminum pans were heated at 10 °C min^{-1} from 1 to 40 °C. Per gram, the 50:50 mixture absorbed 0.79 and propylene glycol 0.57 times as much heat as distilled water at 10 °C, and 0.74 and 0.48 times at 25 °C. The heat flow of water rose 2.6-fold from 5 to 35 °C, whereas its specific heat is nearly constant, so the signal is not a measurement of specific heat and only the order of the three fluids is used: water absorbed the most heat at every temperature. Distilled water was therefore used from the human-donor experiments onward.

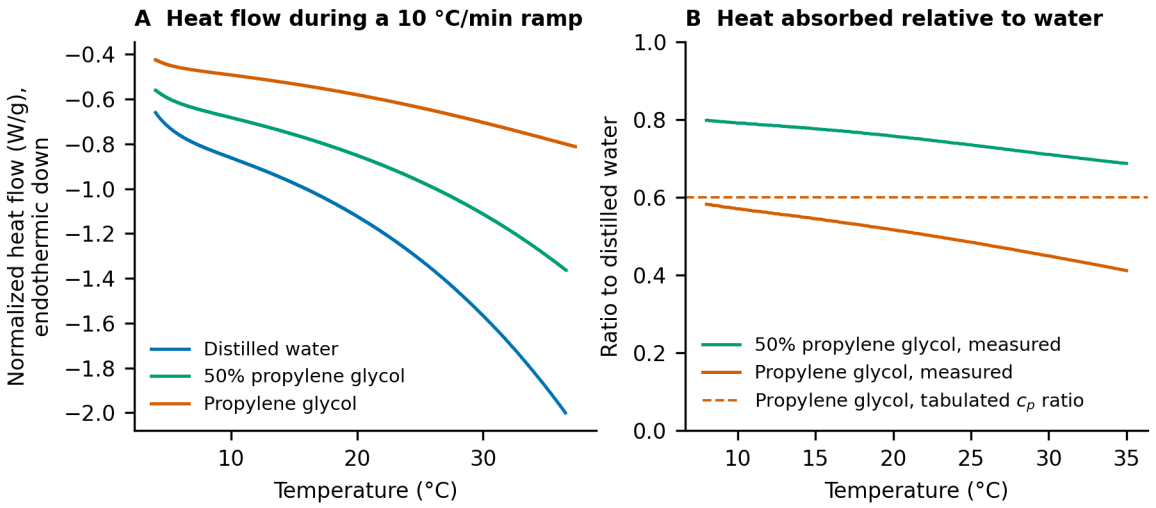


FIGURE E.1. Differential scanning calorimetry of the candidate coolants. (A) Normalized heat flow. (B) Heat flow relative to distilled water (solid) and tabulated ratio for propylene glycol (dashed) [**170, 203**]. Only the comparison between samples is interpreted.

APPENDIX F

The Integrated Control Board

F.1. Hardware and Firmware

An STM32L476RG microcontroller [**204**] sets the drive of an ADN8831 thermoelectric-module controller (Analog Devices) [**205**], which powers the module through an H-bridge of four MOSFETs and senses its current across a 10 mΩ resistor. Trimmers set hard limits on the module current and voltage, independently of the firmware, and the module is off by default. Six thermistor channels read the ring and five ocular sites from a buffered 2.5 V reference; only the ring channel is used for control. The board also measures the module current and voltage, reads two coolant-flow sensors (SLF3S-4000B, Sensirion) [**206**] and an ambient temperature and humidity sensor (AHT20, Aosong) [**207**], has inputs for two coolant-pressure transducers, and drives the pump and a fan (Figure F.1). As designed, the ring signal fed an analog loop in the module controller. On the board as used that connection was cut, and the control loop runs in the microcontroller, which sets the drive through its digital-to-analog converter.

The ring is regulated by a digital proportional–integral–derivative controller at 5 Hz. A safety routine running every 50 ms stops the module and latches the fault when the ring thermistor reads open or outside 0–45 °C, when the 2.5 V reference leaves its band, when the module current exceeds 5 A for 500 ms or, in closed loop, is absent, or when the link to the operator falls silent for more than 2 s over the binary protocol. A hardware watchdog resets the microcontroller if its program stops. Faults on the five measurement channels are reported without stopping cooling. After a fault the pump and fan run at full speed for 30 s (3 s in console runs) to clear the stored

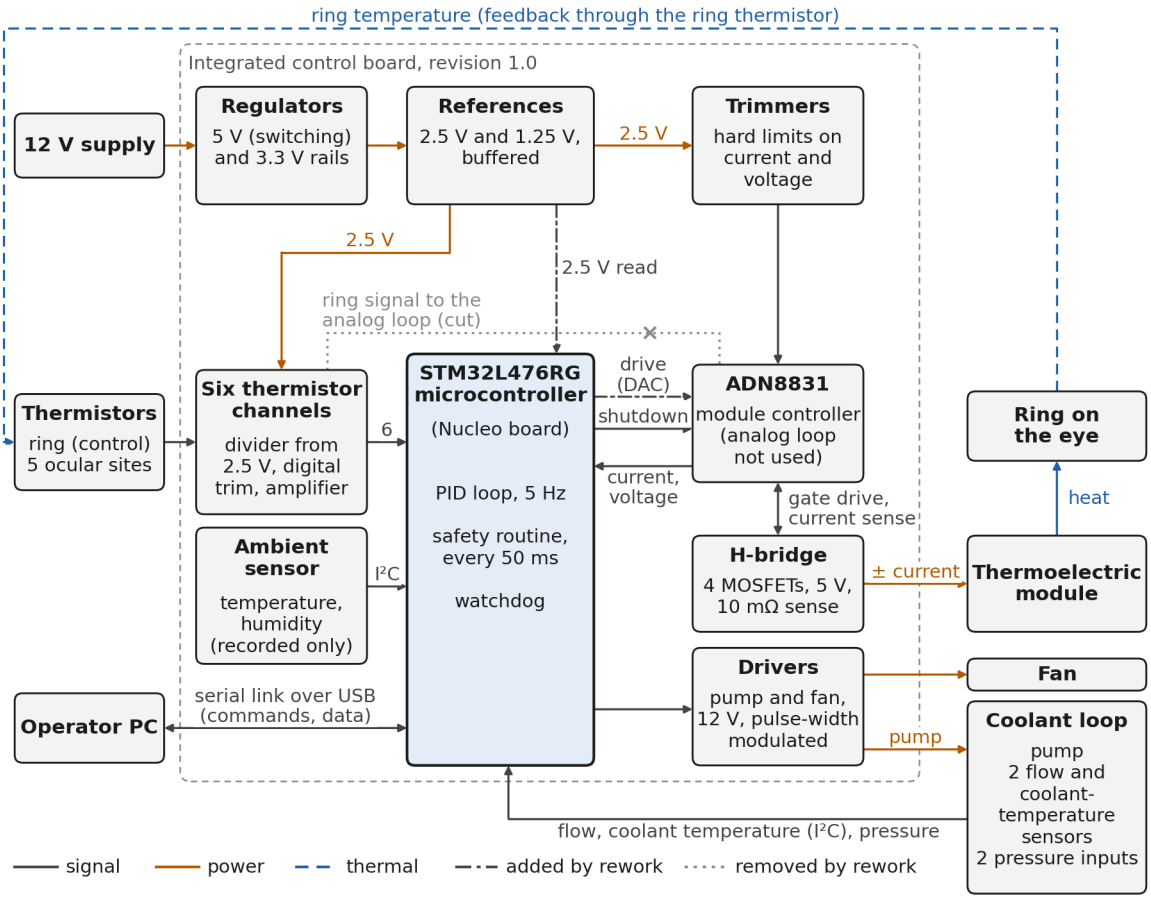


FIGURE F.1. Block diagram of the integrated control board (revision 1.0) as used: solid gray lines, signals; orange, power; blue, the thermal path (dashed where it closes the loop through the ring thermistor); dash-dot, connections added by rework (the drive set directly by the microcontroller and the reading of the 2.5 V reference); dotted, the connection removed by rework (the ring signal to the analog loop of the module controller).

heat. The bench logs of Section F.2 were recorded with firmware versions 0.9.47 to 0.9.59. The firmware source is available from the author on request.

The module voltage is five times the departure of the drive signal from a 1.25 V center, cooling below it and heating above, with one leg of the H-bridge linear and the other switched at 500 kHz. The board runs from an external 12 V supply, through a buck converter to 5 V and a linear regulator to 3.3 V. Each thermistor forms a divider with a digital potentiometer, followed by a buffer amplifier, and is converted to temperature with a Steinhart–Hart model.

F.2. Bench Verification

Before it drove a ring, the board was verified in stages: the bridge transistors, power-up, the reference, operation into dummy loads to confirm the hard limits, the thermal response, the polarity of the drive, and closed-loop regulation. The last stage was a soak at 4 °C with the device on a preserved porcine eye in the bath, which ended with the ring thermistor disconnected. In a later session the module was disconnected and the program stalled (the latter with the module off).

In a second closed-loop run, with chilled coolant and the ring out of the bath, the ring went from 30 to 4.5 °C in 352 s and held 3.98 °C on 1.65 A. The firmware held the drive at 2.9 A, so the pull-down was slower than that of the SCEC Gen 3 ring ex vivo, which completed 90% of its fall in about 2 min.

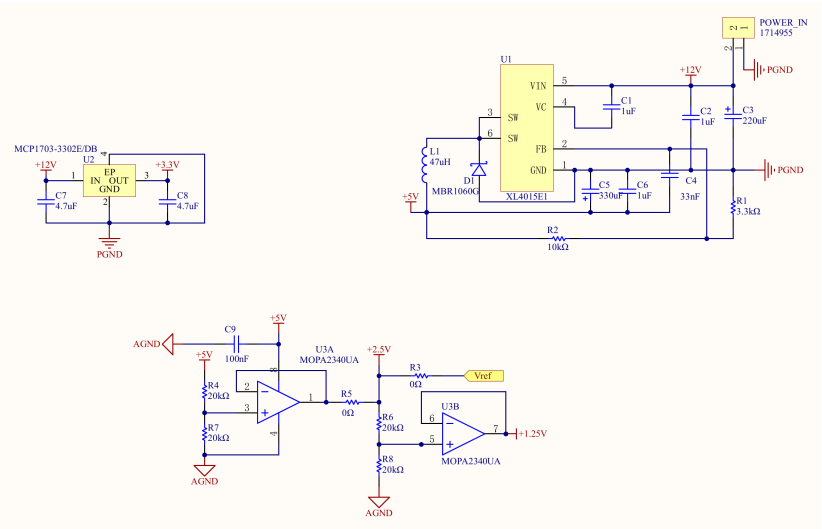
The other functions of the board worked, with limits. All six thermistor channels read, and in room air five agreed within 3 mV. Both flow sensors read about 0.1 mL min^{-1} with the pump off and 570 to 800 mL min^{-1} with it on, the upper values beyond their range. In later sessions they dropped off their bus during the reference fault of Section 3.3.8 and returned only when restarted, so that a flow reading accompanied 28% of the samples of one session. The board switched the pump and the fan, but neither could be graded: the pump gave no flow below 65% pulse-width modulation (PWM) duty, and the fan turned only at full duty. The ambient sensor read 25.3 to 25.9 °C over 12 min and was not compared with a reference. One of the two pressure inputs was connected to a transducer and worked.

F.3. Schematics and Drawings

Figures F.2 and F.3 reproduce the five schematic sheets of the board of Section 3.2.4 (design PCB_Project_LM_260407, revision 1.0) as designed and fabricated; the reworks are not drawn (CS4 moved from PA4 to PH1, PA4 wired to the 2.5 V reference, and the ring signal to IN1N cut). Figure F.4 gives the assembly and fabrication drawings. The board has four copper layers, a total thickness of 1.61 mm, and

741 plated holes. Each sheet is cropped to its drawing, without the frame and title block.

Sheet 1 of 5: Power



Sheet 2 of 5: ADN8831 controller and output stage

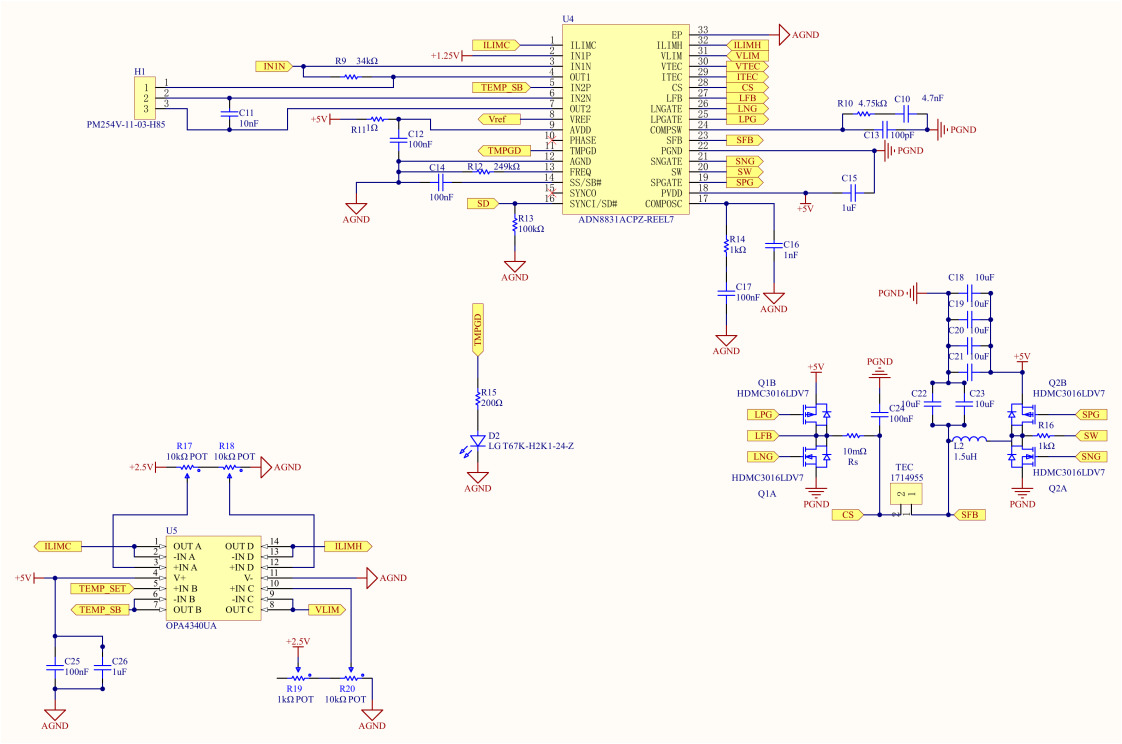
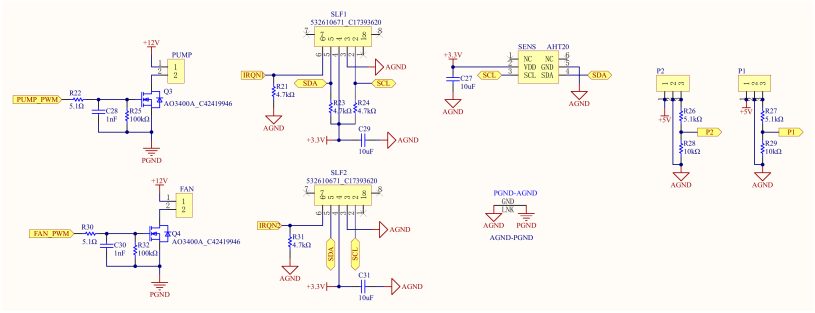
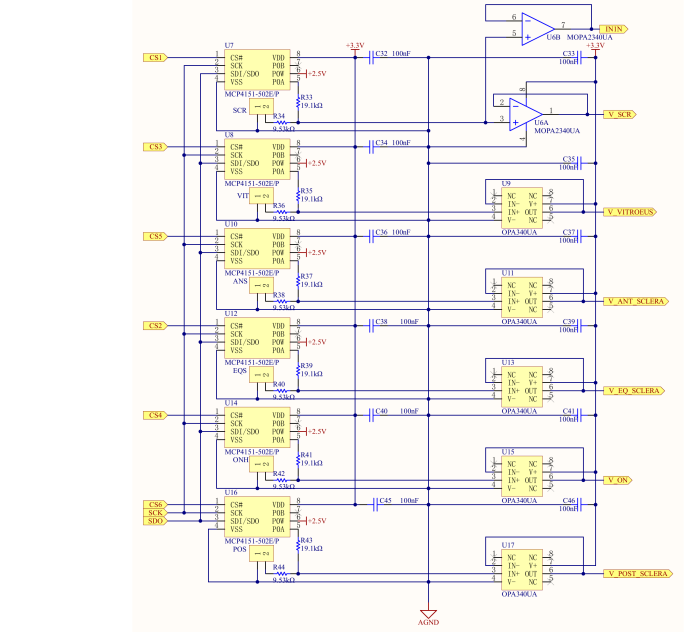


FIGURE F.2. Schematic of the integrated control board, sheets 1 and 2 of 5. Top, power: the 12 V input, the buck converter from 12 to 5 V (XL4015), the linear regulator from 12 to 3.3 V (MCP1703), and the buffered 2.5 V and 1.25 V references. Bottom, the thermoelectric-module controller (ADN8831), its output stage and H-bridge of four MOSFETs with the current-sense resistor and filter inductor, and the trimmers that set the current and voltage limits.

Sheet 3 of 5: Peripherals



Sheet 4 of 5: Thermistor channels



Sheet 5 of 5: Microcontroller

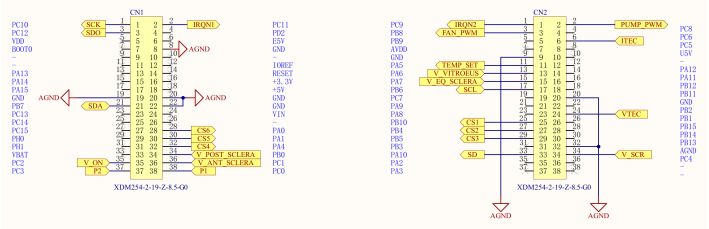


FIGURE F.3. Schematic of the integrated control board, sheets 3 to 5 of 5. Top, peripherals: low-side drivers of the pump and fan, the two coolant-flow sensors, the ambient temperature and humidity sensor, and the inputs of the two pressure transducers. Middle, thermistor channels: for each of the six thermistors, a digital potentiometer forming the divider from the 2.5 V reference and a buffer amplifier feeding the converter of the microcontroller. Bottom, microcontroller interface: the connections to the microcontroller development board through its expansion headers.

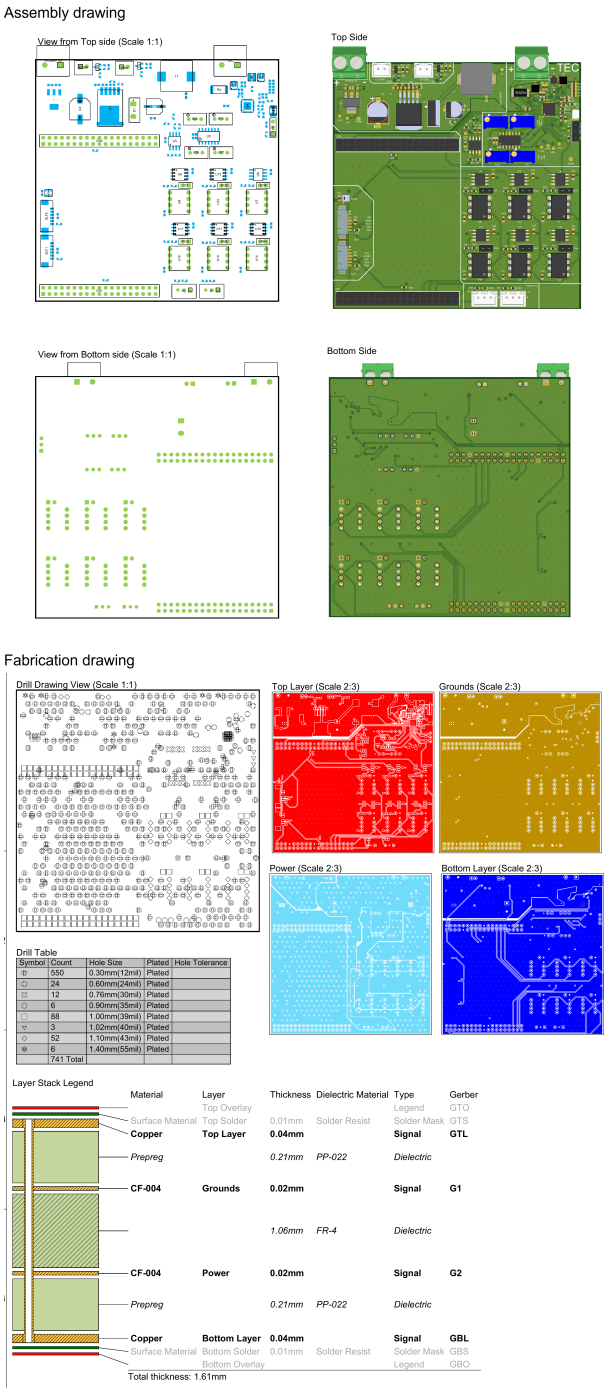


FIGURE F.4. Assembly (top) and fabrication (bottom) drawings of the integrated control board. Assembly: component placement on the top and bottom sides at full scale (left) and rendered views of the populated board (right). Fabrication: drill drawing and drill table, the four copper layers (top, ground, power, bottom), and the layer stack, 1.61 mm in total.

APPENDIX G

ACC Approval Documentation

The animal-research approval documentation for Aim 3 is reproduced in this appendix. The University of Illinois Chicago Animal Care Committee (ACC) approved protocol `IPROTO202600000055` on April 30, 2026; the approval letters are addressed to John Hetling. The original approval period is April 30, 2026, through April 29, 2029.

The original approval is followed by four amendment approvals. Amendment `IAMEND202600000339` was approved on June 15, 2026. Amendment `IAMEND202600000409` was approved on July 30, 2026, with its letter dated July 31, 2026. Amendment `IAMEND202600000587` was approved on October 12, 2026, and amendment `IAMEND202600000642` on November 6, 2026. The letters of amendments `IAMEND202600000409`, `IAMEND202600000587`, and `IAMEND202600000642` use the revised protocol title *Initial Safety Study: Evaluating Ocular Irritation and Retinal Function During Use an Ocular Cooling Device in Live Pigs*, quoted as issued.

Each letter is reproduced in full after its text transcription. All five letters carry the following institutional letterhead:

> Office of Animal Care and Institutional Biosafety (OACIB)
> Administrative Office Building (AOB)
> 1737 W. Polk St. | Room 206, MC 672 | Chicago, IL 60612
> P: (312)-996-1972 | F: (312) 996-9088 | E: oacib@uic.edu
> W: `https://research.uic.edu/compliance/`

G.1. Original Protocol Approval

April 30, 2026
John Hetling
Biomedical Engineering
Dear Dr. Hetling,
The protocol indicated below was approved in accordance with the Animal Care Policies of the University of Illinois Chicago on 4/30/2026. This protocol is approved for a period of 3 years: 4/30/2026 to 4/29/2029.
Title of Protocol: Ocular Cooling Device Safety in Live Pigs
ACC Protocol Number: IPROTO202600000055
Thank you for complying with the Animal Care Policies and Procedures of UIC.
Sincerely,
Leon Tai, PhD
Chair, Animal Care Committee

UNIVERSITY OF ILLINOIS CHICAGO
Office of the Vice Chancellor for Research

Office of Animal Care and Institutional Biosafety (OACIB)
Administrative Office Building (AOB)
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W: https://research.uic.edu/compliance/

April 30, 2026

John Hetling
Biomedical Engineering

Dear Dr. Hetling,

The protocol indicated below was **approved** in accordance with the Animal Care Policies of the University of Illinois Chicago on **4/30/2026.** **This protocol is approved for a period of 3 years: 4/30/2026 to 4/29/2029.**

Title of Protocol: **Ocular Cooling Device Safety in Live Pigs**

ACC Protocol Number: **IPROTO202600000055**

Thank you for complying with the Animal Care Policies and Procedures of UIC.

Sincerely,

Leon Tai, PhD
Chair, Animal Care Committee

G.2. Amendment IAMEND202600000339

June 15, 2026
John Hetling
Biomedical Engineering
Dear Dr. Hetling:
The modifications requested in the protocol below have been reviewed and approved in accordance with the Animal Care Policies of the University of Illinois Chicago on 6/15/2026.
Title of Protocol: Ocular Cooling Device Safety in Live Pigs
ACC Protocol Number: `IPROTO202600000055`
Amendment Number: `IAMEND202600000339`
Thank you for complying with the Animal Care Policies and Procedures of UIC.
Sincerely,
Leon Tai, PhD
Chair, Animal Care Committee

UNIVERSITY OF ILLINOIS CHICAGO
Office of the Vice Chancellor for Research

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W: https://research.uic.edu/compliance/

June 15, 2026

John Hetling
Biomedical Engineering

Dear Dr. Hetling:

The modifications requested in the protocol below have been reviewed and approved in accordance with the Animal Care Policies of the University of Illinois Chicago on **6/15/2026**.

Title of Protocol: Ocular Cooling Device Safety in Live Pigs

ACC Protocol Number: IPROTO202600000055

Amendment Number: IAMEND202600000339

Thank you for complying with the Animal Care Policies and Procedures of UIC.

Sincerely,

Leon Tai, PhD
Chair, Animal Care Committee

G.3. Amendment IAMEND202600000409

July 31, 2026
John Hetling
Engineering
Dr. Hetling:
The modifications requested in the protocol below have been reviewed and approved in accordance with the Animal Care Policies of the University of Illinois Chicago on 7/30/2026.
Title of Protocol: Initial Safety Study: Evaluating Ocular Irritation and Retinal Function During Use an Ocular Cooling Device in Live Pigs
ACC Protocol Number: `IPROTO202600000055`
Amendment Number: `IAMEND202600000409`
Thank you for complying with the Animal Care Policies and Procedures of UIC.
Sincerely,
Leon Tai, PhD
Chair, Animal Care Committee

UNIVERSITY OF ILLINOIS CHICAGO
Office of the Vice Chancellor for Research

Office of Animal Care and Institutional Biosafety (OACIB)
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W: https://research.uic.edu/compliance/

July 31, 2026

John Hetling
Engineering

Dr. Hetling:

The modifications requested in the protocol below have been reviewed and approved in accordance with the Animal Care Policies of the University of Illinois Chicago on **7/30/2026**.

Title of Protocol:	**Initial Safety Study: Evaluating Ocular Irritation and Retinal Function During Use an Ocular Cooling Device in Live Pigs**
ACC Protocol Number:	**IPROTO202600000055**
Amendment Number:	**IAMEND202600000409**

Thank you for complying with the Animal Care Policies and Procedures of UIC.

Sincerely,

Leon Tai, PhD
Chair, Animal Care Committee

G.4. Amendment IAMEND202600000587

October 12, 2026
John Hetling
Engineering
Dr. Hetling:
The modifications requested in the protocol below have been reviewed and approved in accordance with the Animal Care Policies of the University of Illinois Chicago on 10/12/2026.
Title of Protocol: Initial Safety Study: Evaluating Ocular Irritation and Retinal Function During Use an Ocular Cooling Device in Live Pigs
ACC Protocol Number: `IPROTO202600000055`
Amendment Number: `IAMEND202600000587`
Thank you for complying with the Animal Care Policies and Procedures of UIC.
Sincerely,
Leon Tai, PhD
Chair, Animal Care Committee

UNIVERSITY OF ILLINOIS CHICAGO
Office of the Vice Chancellor for Research

Office of Animal Care and Institutional Biosafety (OACIB)
Administrative Office Building (AOB)
1737 W. Polk St. | Room 206, MC 672 | Chicago, IL 60612
P: (312)-996-1972 | **F:** (312) 996-9088 | **E:** oacib@uic.edu
W: https://research.uic.edu/compliance/

October 12, 2026

John Hetling
Engineering

Dr. Hetling:

The modifications requested in the protocol below have been reviewed and approved in accordance with the Animal Care Policies of the University of Illinois Chicago on **10/12/2026**.

Title of Protocol:	**Initial Safety Study: Evaluating Ocular Irritation and Retinal Function During Use an Ocular Cooling Device in Live Pigs**
ACC Protocol Number:	**IPROTO202600000055**
Amendment Number:	**IAMEND202600000587**

Thank you for complying with the Animal Care Policies and Procedures of UIC.

Sincerely,

Leon Tai, PhD
Chair, Animal Care Committee

G.5. Amendment IAMEND202600000642

November 6, 2026
John Hetling
Biomedical Engineering
Dr. Hetling:
The modifications requested in the protocol below have been reviewed and approved in accordance with the Animal Care Policies of the University of Illinois Chicago on 11/06/2026.
Title of Protocol: Initial Safety Study: Evaluating Ocular Irritation and Retinal Function During Use an Ocular Cooling Device in Live Pigs
ACC Protocol Number: `IPROTO202600000055`
Amendment Number: `IAMEND202600000642`
Thank you for complying with the Animal Care Policies and Procedures of UIC.
Sincerely,
Leon Tai, PhD
Chair, Animal Care Committee

UNIVERSITY OF ILLINOIS CHICAGO
Office of the Vice Chancellor for Research

Office of Animal Care and Institutional Biosafety (OACIB)
Administrative Office Building (AOB)
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W: https://research.uic.edu/compliance/

November 6, 2026

John Hetling
Biomedical Engineering

Dr. Hetling:

The modifications requested in the protocol below have been reviewed and approved in accordance with the Animal Care Policies of the University of Illinois Chicago on **11/06/2026**.

Title of Protocol:	**Initial Safety Study: Evaluating Ocular Irritation and Retinal Function During Use an Ocular Cooling Device in Live Pigs**
ACC Protocol Number:	**IPROTO202600000055**
Amendment Number:	**IAMEND202600000642**

Thank you for complying with the Animal Care Policies and Procedures of UIC.

Sincerely,

Leon Tai, PhD
Chair, Animal Care Committee

APPENDIX H

Anesthesia Records and Necropsy of Animal 3

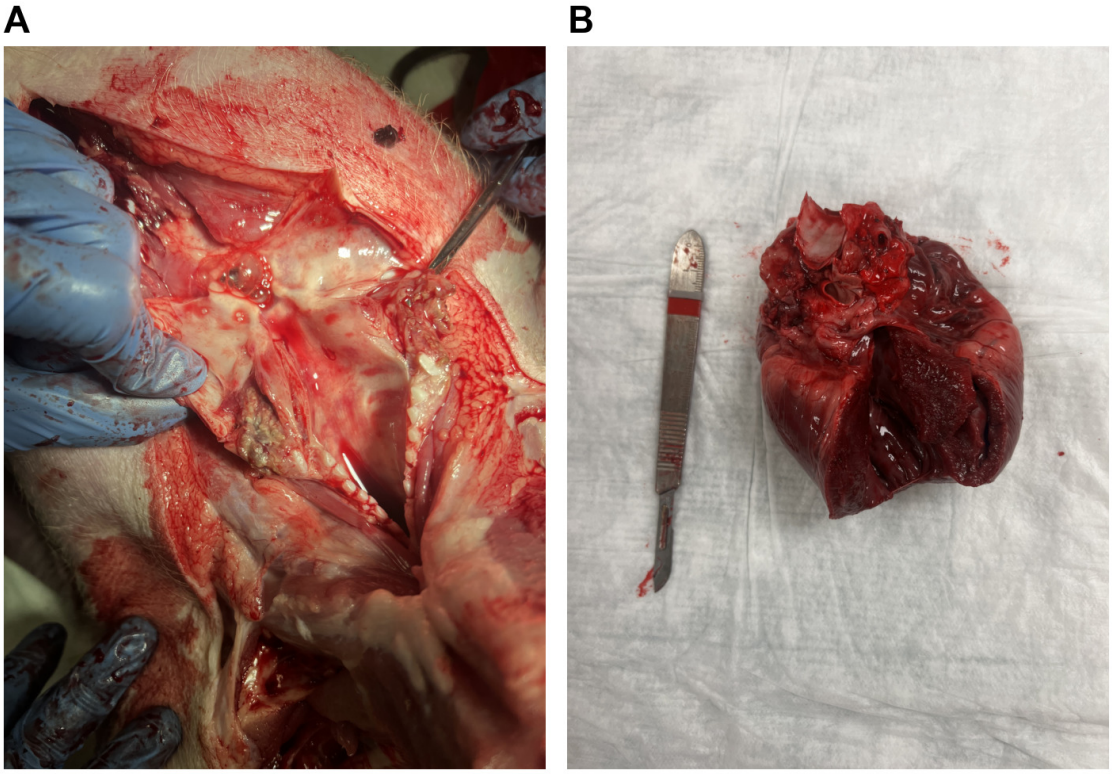

FIGURE H.1. Necropsy of animal 3, which died in recovery from session 3 (Section 4.3.6): (A) the opened larynx and trachea, showing the laryngeal necrosis, most likely from a traumatic intubation in session 2; (B) the opened heart, showing the cardiac hypertrophy, judged incidental; a scalpel gives the scale. Neither was attributed to the device.

Table H.1 gives the drugs and doses of the BRL anesthesia record of each session (Section 4.2.3). After rocuronium the animal was weaned from the ventilator until spontaneous breathing resumed, and normal respiration and oxygen saturation were confirmed before it was returned to its housing.

TABLE H.1. Drugs given in the sessions, from the BRL anesthesia records. A, animal; S, session; IM, intramuscular; IV, intravenous.

Drug	**Dose**	**Route**	**Given in**
Ketamine and medetomidine (100 and 20 mg/mL)	Protocol 10 and 0.08 mg/kg; 5 and 0.2 mg/kg on the BRL record from A2 onward, given as 4.5 to 11.6 and 0.18 to 0.32 mg/kg; 13.6 and 0.32 mg/kg in A1 S3	IM	All sessions except A1 S1 and S4 of A1, A5, and A6
Tiletamine–zolazepam, xylazine, butorphanol (100, 40, 6 mg/mL)	0.8 mL, 4.4 mg/kg of the combined drugs	IM	A1 S1
Tiletamine–zolazepam with xylazine (300 mg/mL combined)	1.4 mL in S4 of A1, A5, and A6 (14.7, 11.9, and 12.3 mg/kg), 0.7 mL in A4 S4 (5.8 mg/kg), and 0.5 mL in A6 S2 (4.6 mg/kg)	IM	S4 of A1, A4, A5, and A6; A6 S2
Isoflurane	1–2%	Inhaled	All
Rocuronium (10 mg/mL)	20 mg, then 10 mg about every 20 to 30 min (in A2 S3, 10 mg first and 15 mg last); every 20 min in A2 S4; gaps of up to 60 min in A2 S2 and 140 min in A3 S2	IV	From A2 onward
Sugammadex	2 mg/kg (0.7 mL in A2 S1, S2 and A4 S1); none recorded in the other sessions	–	Session end
Flunixin	Protocol 2.2 mg/kg; 3.7 mg/kg recorded in A2 S1	IM, IV	Sessions 1 to 3
Propofol	1.5 and about 1.8 mg/kg, when premedication did not allow intubation	IV	A1 S1, S3
Hydromorphone	Two doses in A1 S1; in A6 S1 ticked, amount not recorded	–	A1 S1, A6 S1
Lidocaine and ketamine infusion	Adjunct to isoflurane	IV	A2 S3, S4; all sessions of A3 to A6

APPENDIX I

Selected Ocular Surface Photographs and Histological Sections

Figures I.1 to I.23 give, for every session, the photographs selected for grading: redness (top) and fluorescein staining (bottom), of the test and control eyes, before and after each cooling session and once in the terminal session, with the grades given to each on the 0-to-4 scales of Section 4.2.8. Animal 3 died after session 3 and has no terminal session. All grades are the author's. Animal 1 is exploratory. The redness photographs are cropped to a centered square, and the staining photographs lose only the banner and the gray bar of the instrument.

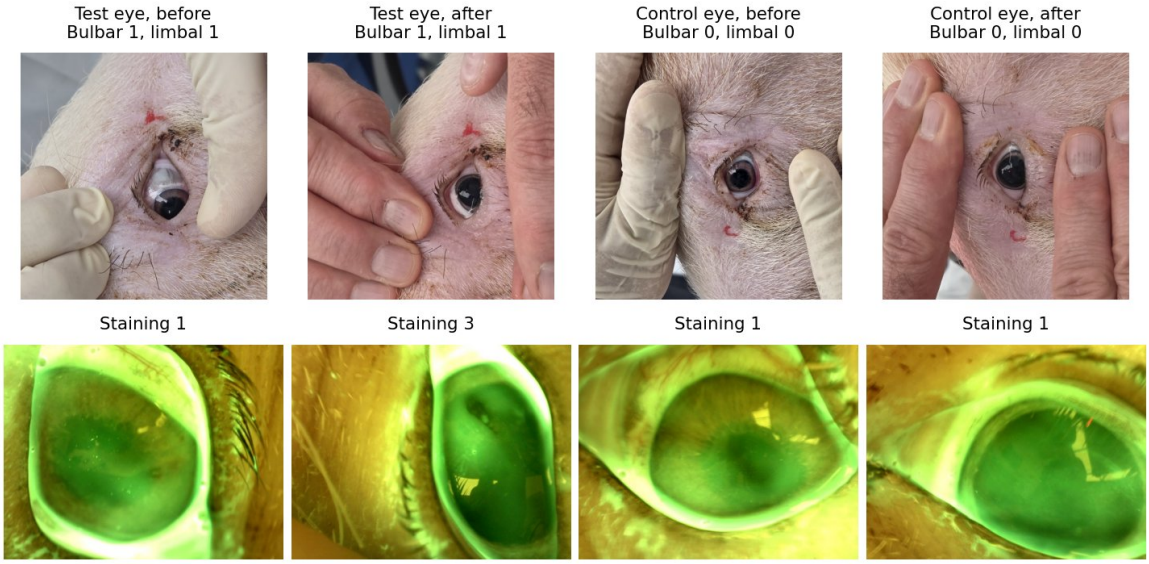

FIGURE I.1. Selected photographs of animal 1, session 1 (30-min session), with the grades of record.

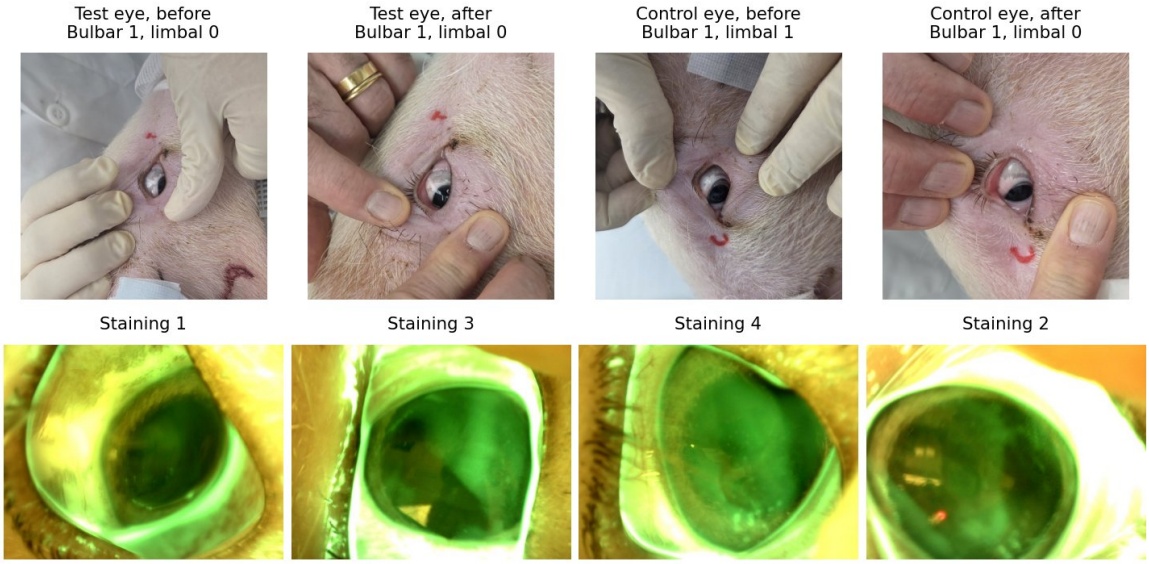

FIGURE I.2. Selected photographs of animal 1, session 2 (60-min session), with the grades of record.

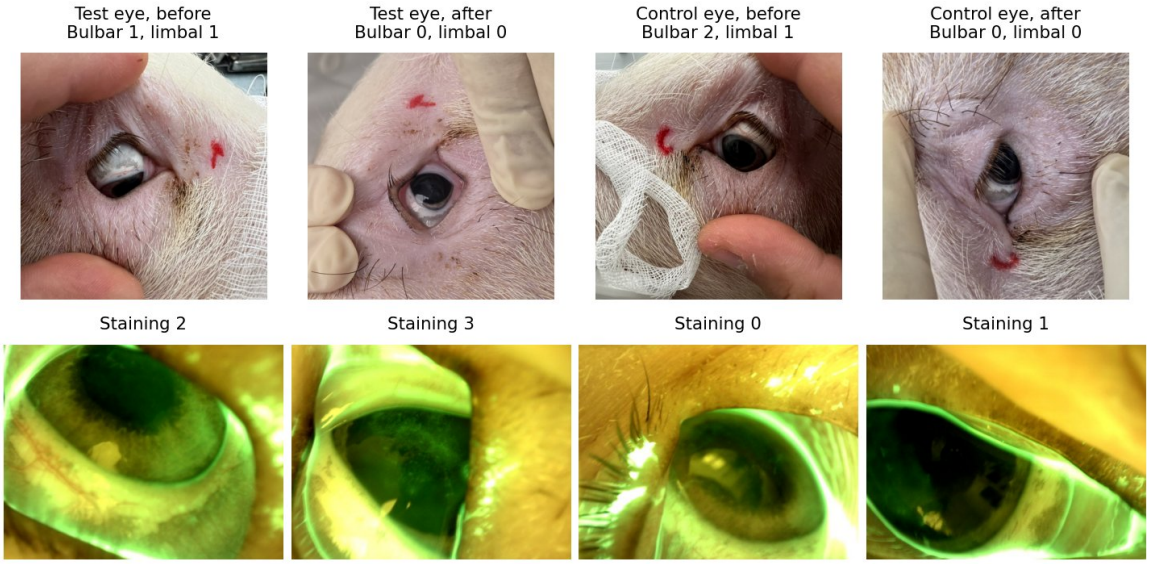

FIGURE I.3. Selected photographs of animal 1, session 3 (120-min session), with the grades of record.

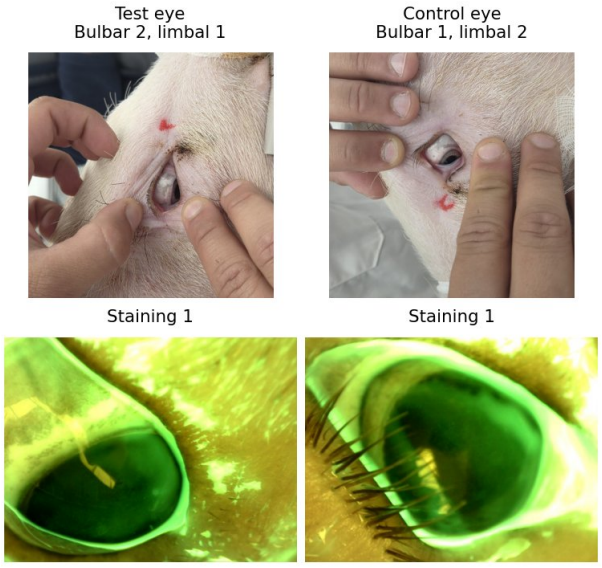

FIGURE I.4. Selected photographs of animal 1, session 4 (terminal session), with the grades of record.

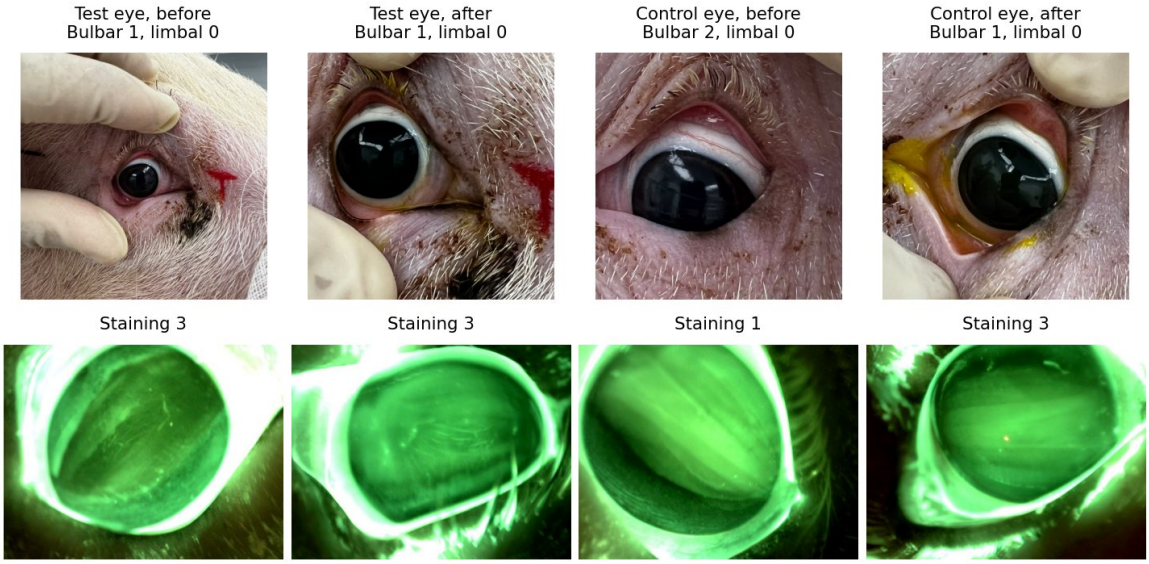

FIGURE I.5. Selected photographs of animal 2, session 1 (30-min session), with the grades of record.

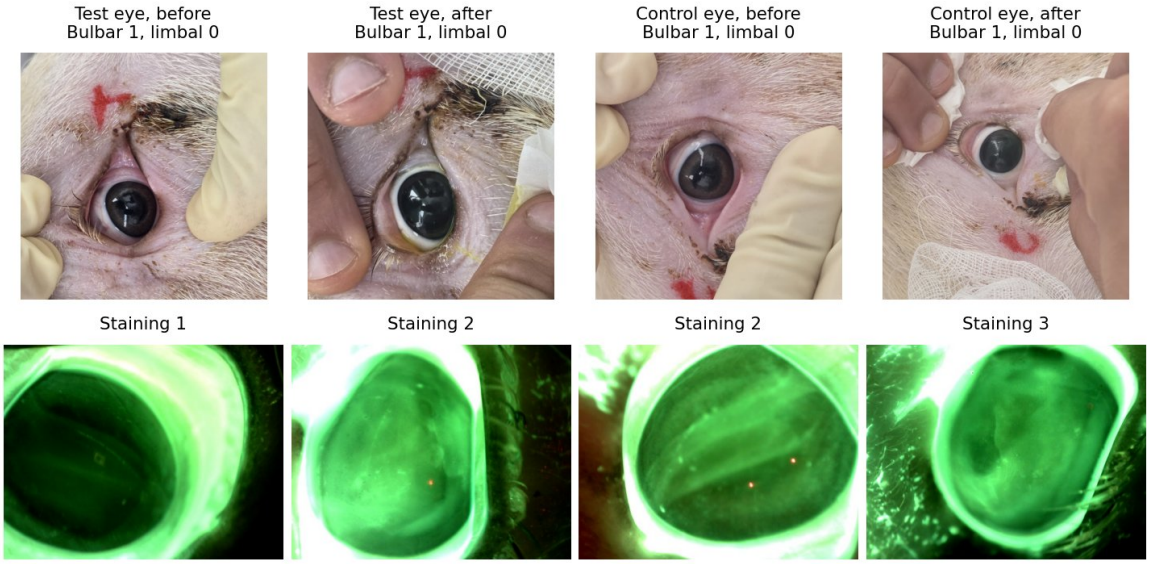

FIGURE I.6. Selected photographs of animal 2, session 2 (60-min session), with the grades of record.

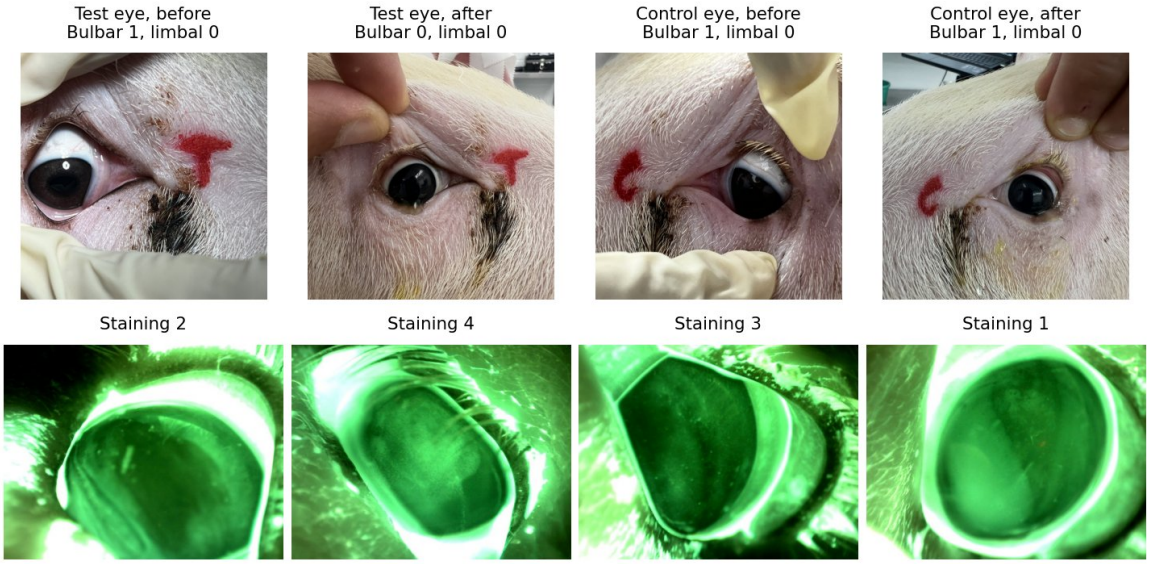

FIGURE I.7. Selected photographs of animal 2, session 3 (120-min session), with the grades of record.

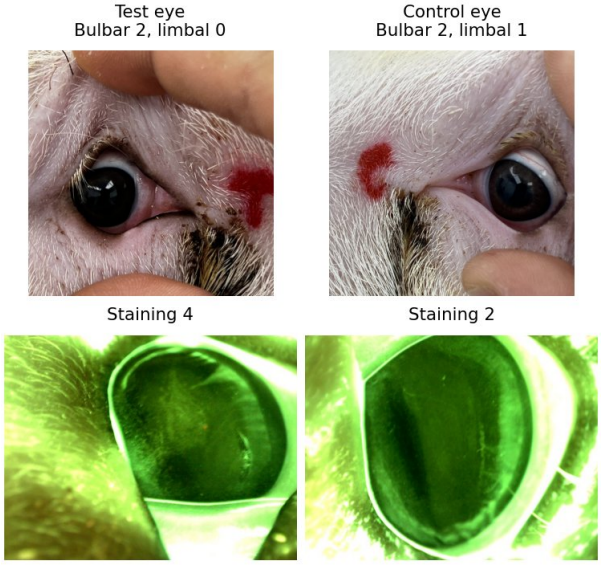

FIGURE I.8. Selected photographs of animal 2, session 4 (terminal session), with the grades of record.

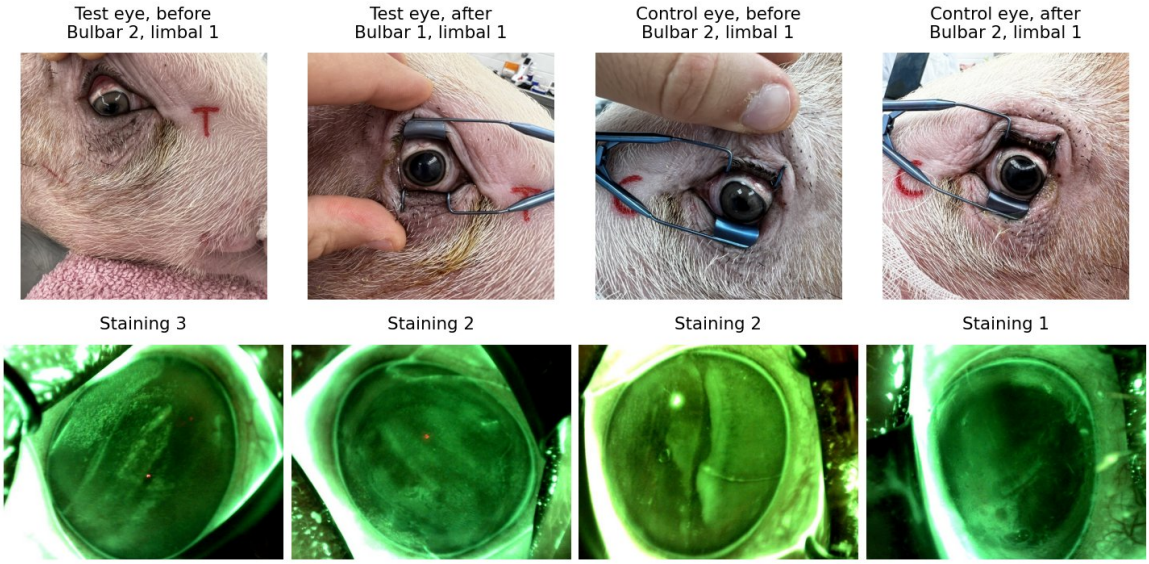

FIGURE I.9. Selected photographs of animal 3, session 1 (30-min session), with the grades of record.

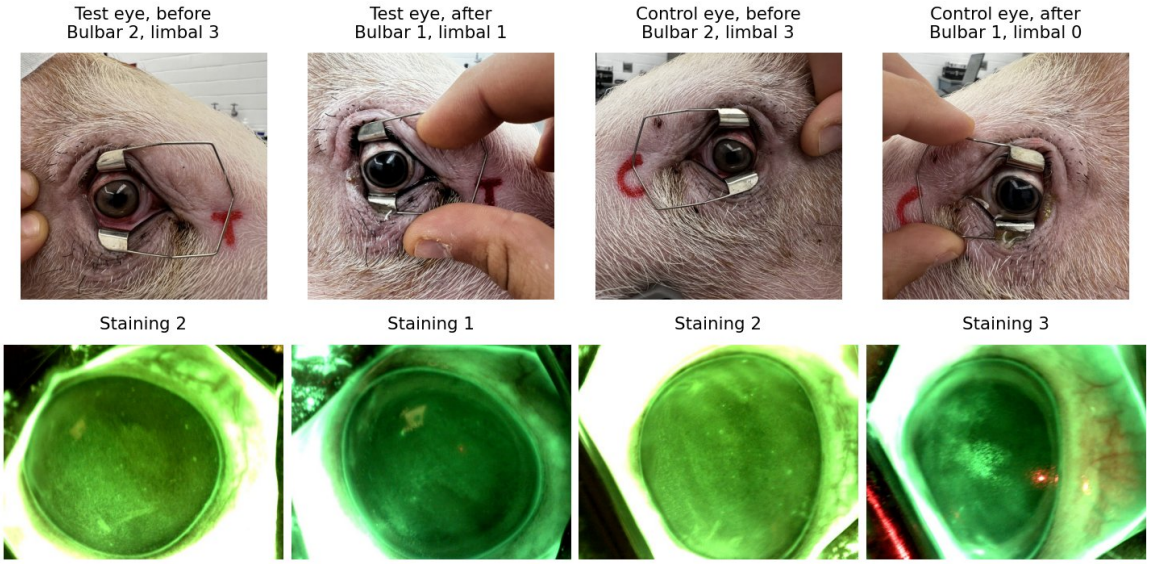

FIGURE I.10. Selected photographs of animal 3, session 2 (60-min session), with the grades of record.

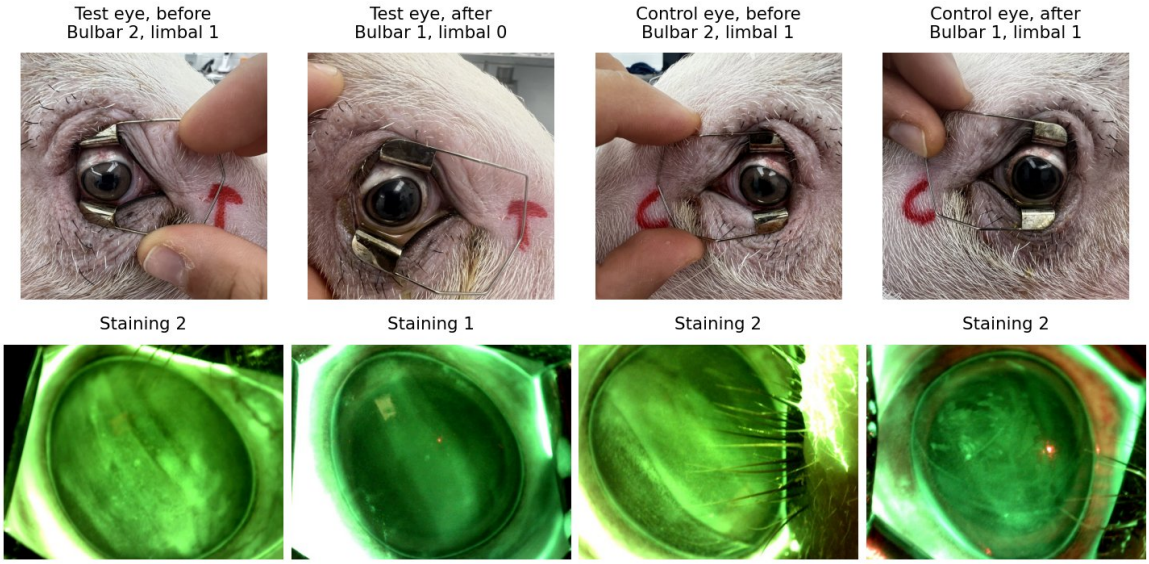

FIGURE I.11. Selected photographs of animal 3, session 3 (120-min session), with the grades of record.

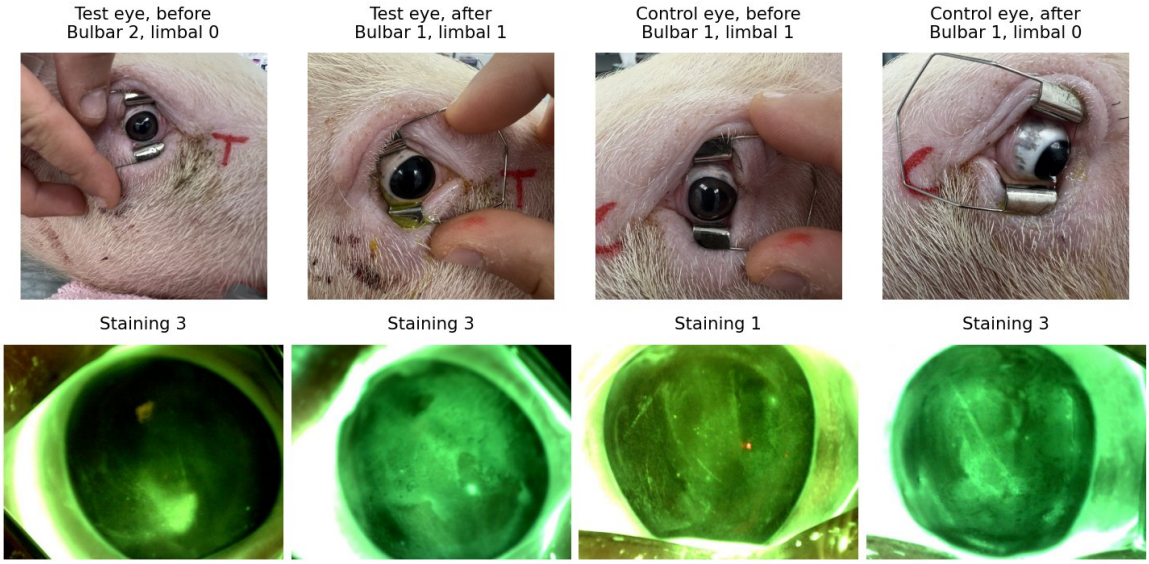

FIGURE I.12. Selected photographs of animal 4, session 1 (30-min session), with the grades of record.

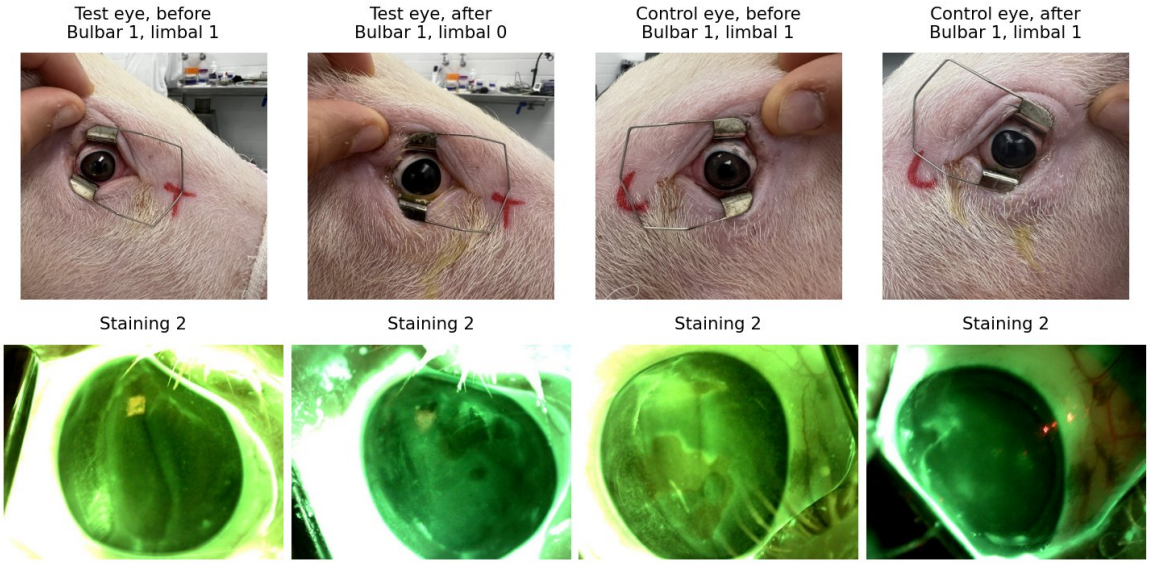

FIGURE I.13. Selected photographs of animal 4, session 2 (60-min session), with the grades of record.

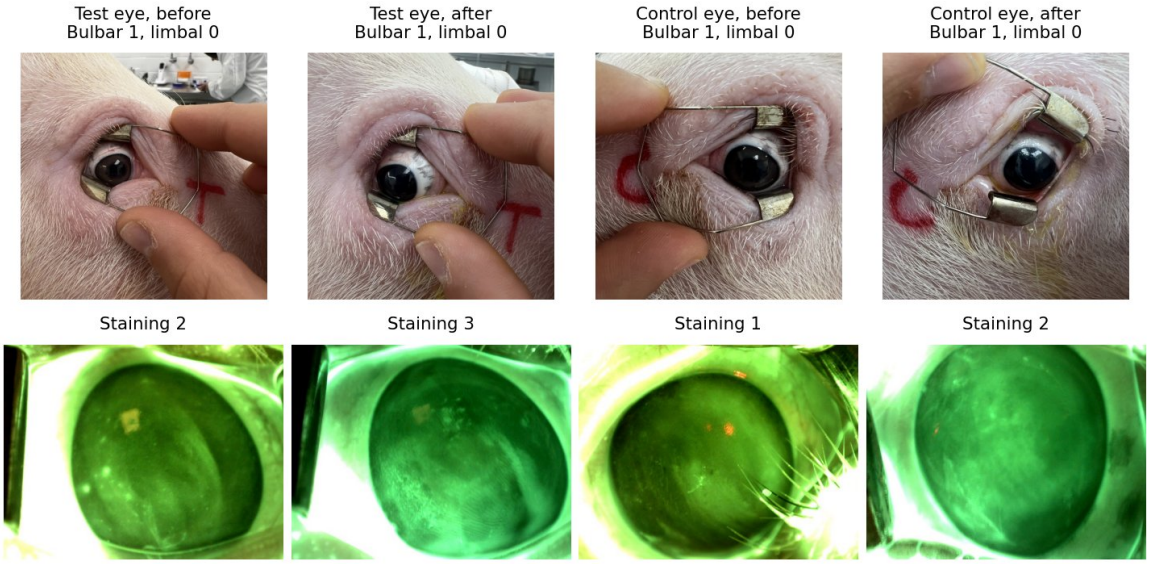

FIGURE I.14. Selected photographs of animal 4, session 3 (120-min session), with the grades of record.

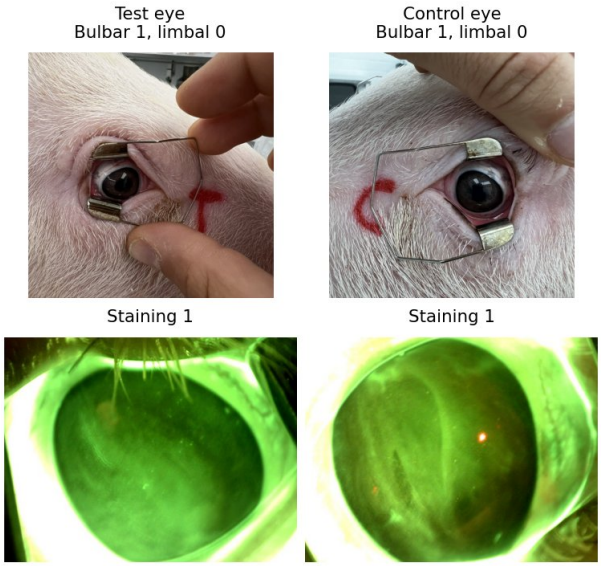

FIGURE I.15. Selected photographs of animal 4, session 4 (terminal session), with the grades of record.

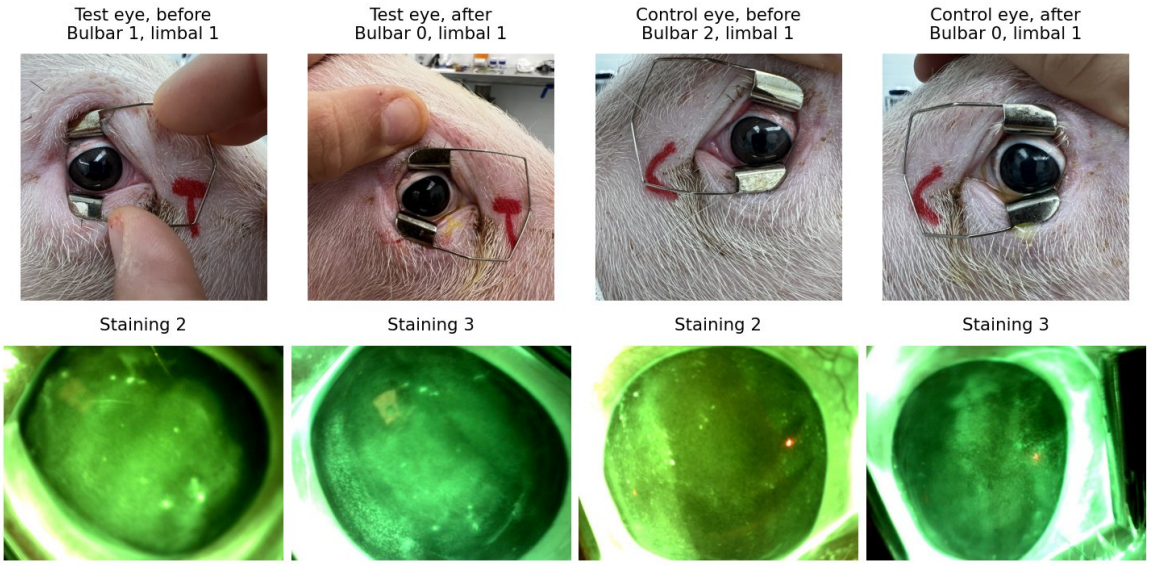

FIGURE I.16. Selected photographs of animal 5, session 1 (30-min session), with the grades of record.

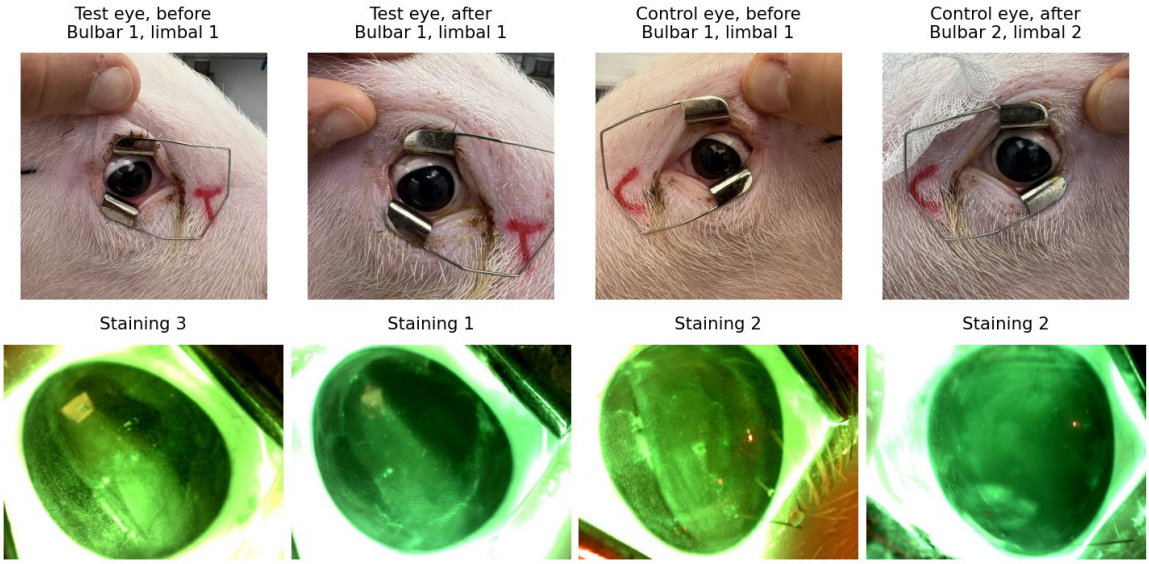

FIGURE I.17. Selected photographs of animal 5, session 2 (60-min session), with the grades of record.

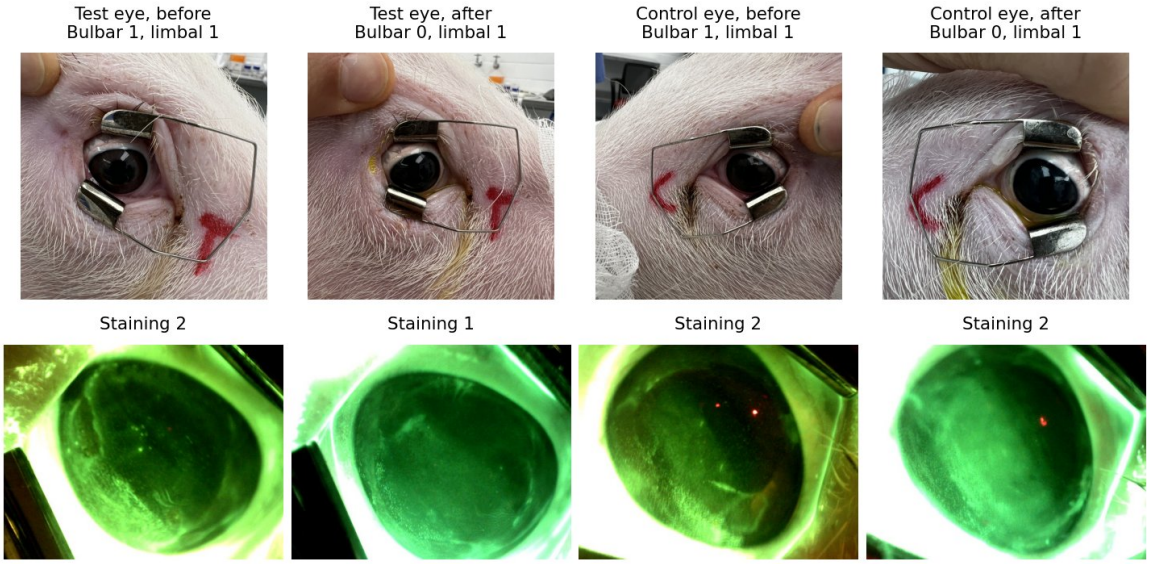

FIGURE I.18. Selected photographs of animal 5, session 3 (120-min session), with the grades of record.

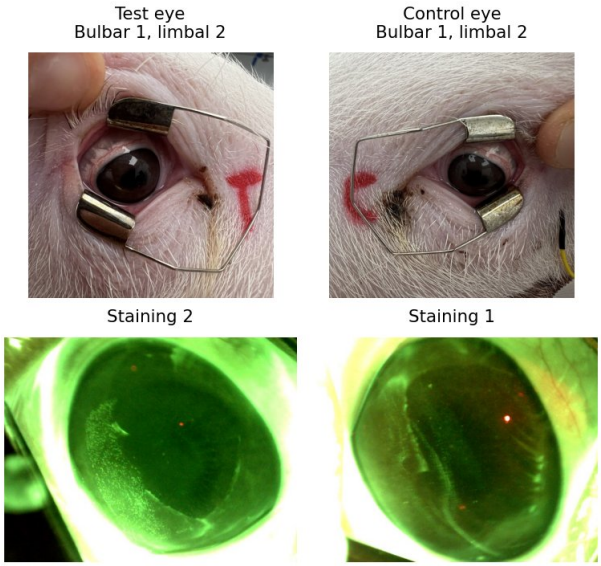

FIGURE I.19. Selected photographs of animal 5, session 4 (terminal session), with the grades of record.

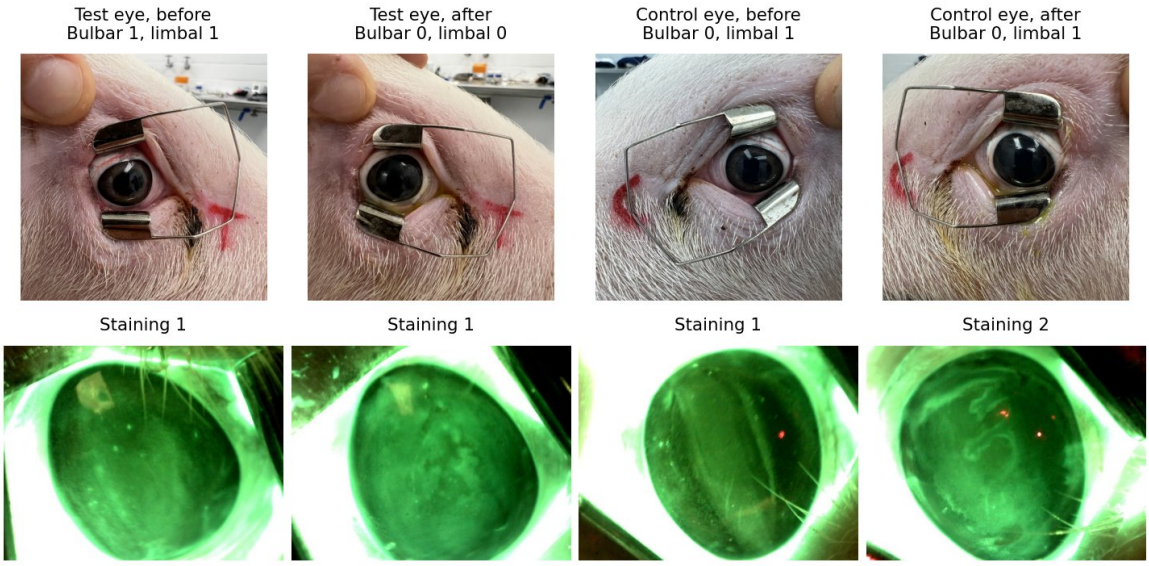

FIGURE I.20. Selected photographs of animal 6, session 1 (30-min session), with the grades of record.

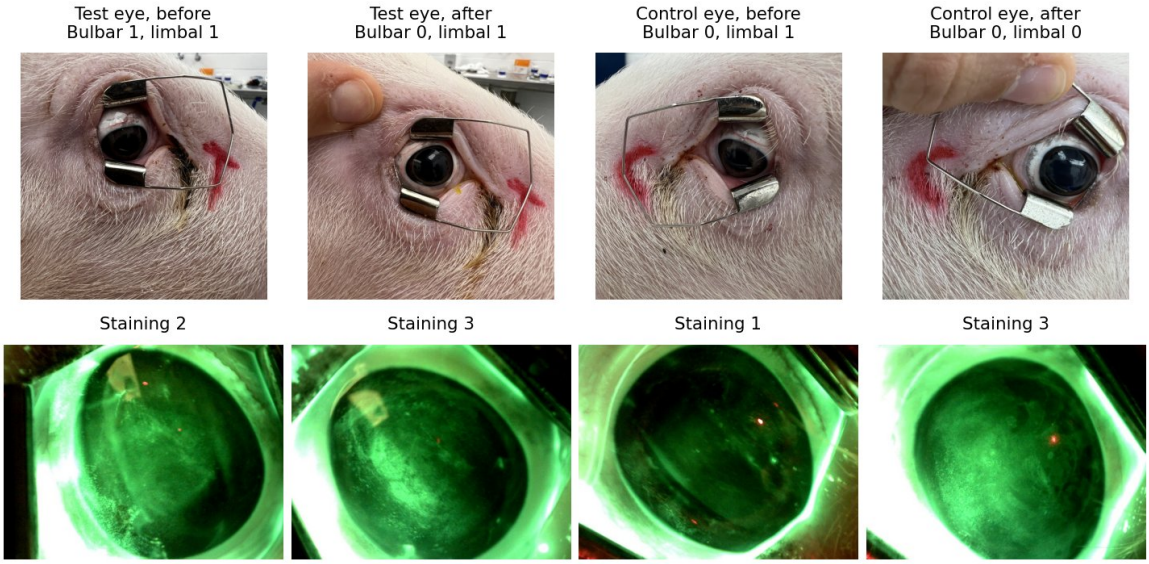

FIGURE I.21. Selected photographs of animal 6, session 2 (60-min session), with the grades of record.

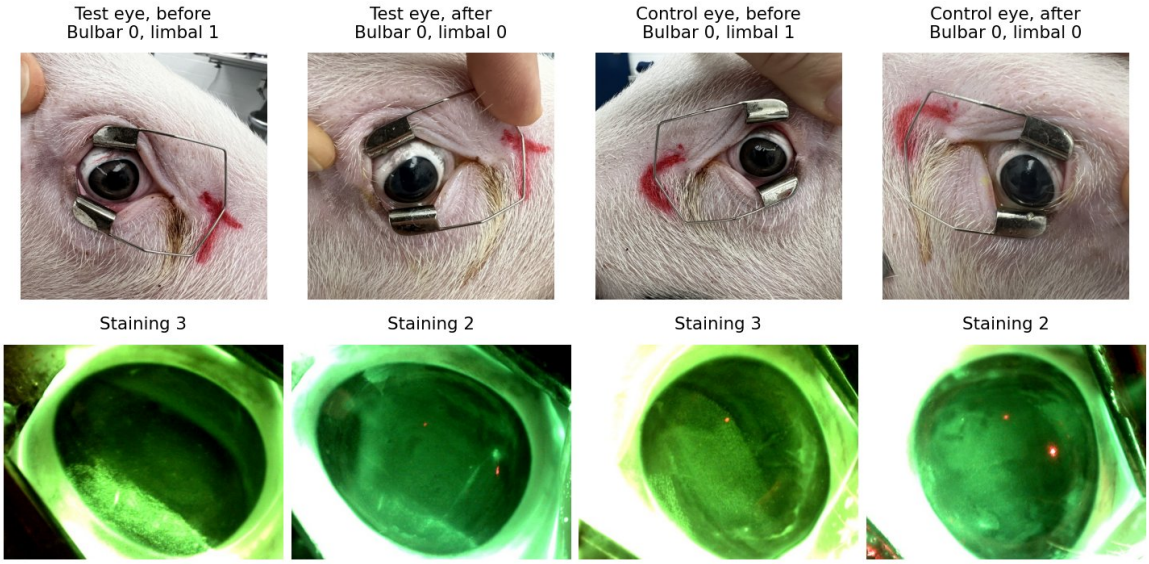

FIGURE I.22. Selected photographs of animal 6, session 3 (120-min session), with the grades of record.

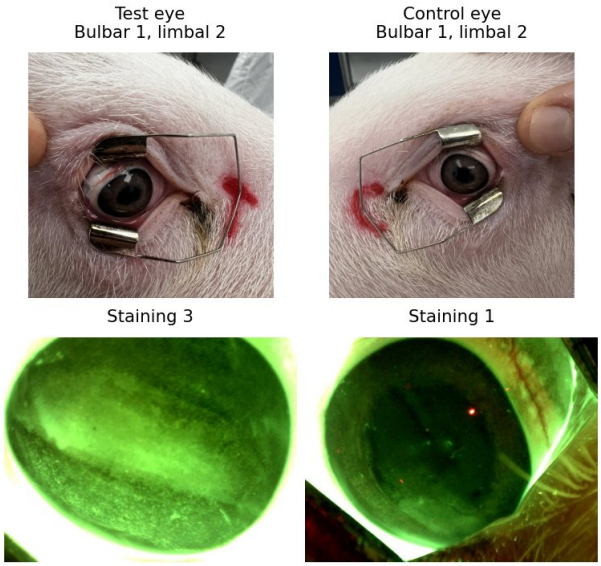

FIGURE I.23. Selected photographs of animal 6, session 4 (terminal session), with the grades of record.

I.1. Histological Sections

Slide 5 of the hematoxylin and eosin sections of the test and control eyes of each animal, as scanned (Section 4.2.8); for animal 1, which has no slide 5, slide 10, and for the test eye of animal 3, slide 10. Animal 3 died after its third session and its eyes were collected after death.

FIGURE I.24. Hematoxylin and eosin sections of animal 1 (exploratory), slide 10: test eye (left) and control eye (right).

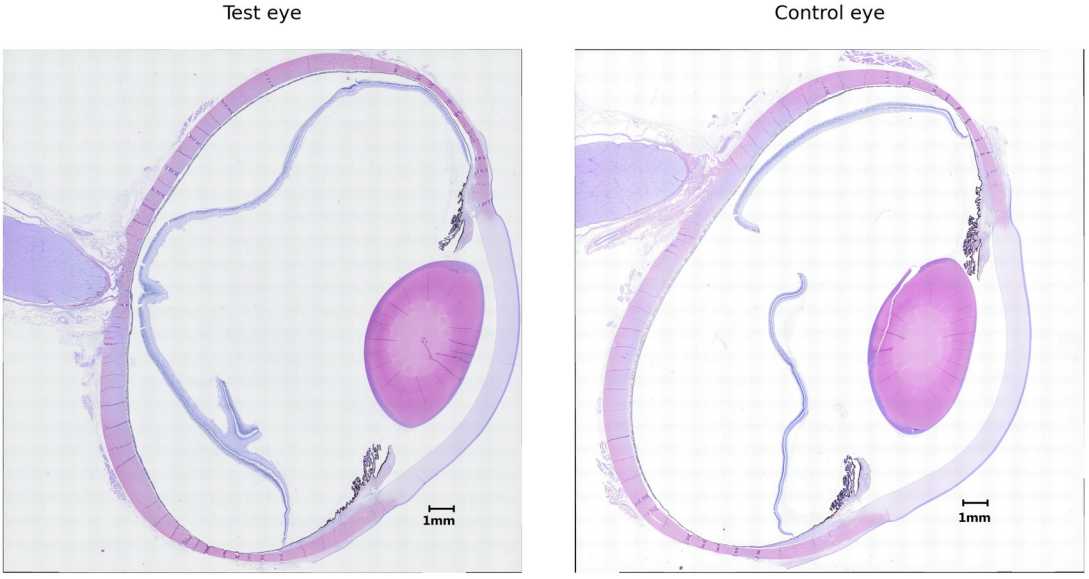

FIGURE I.25. Hematoxylin and eosin sections of animal 2, slide 5: test eye (left) and control eye (right).

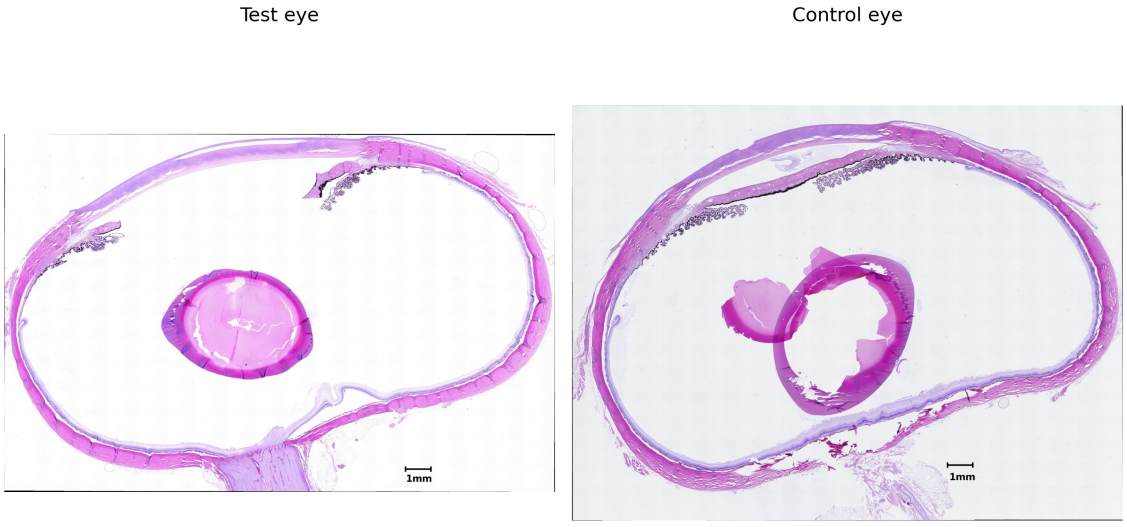

FIGURE I.26. Hematoxylin and eosin sections of animal 3: test eye, slide 10 (left), and control eye, slide 5 (right).

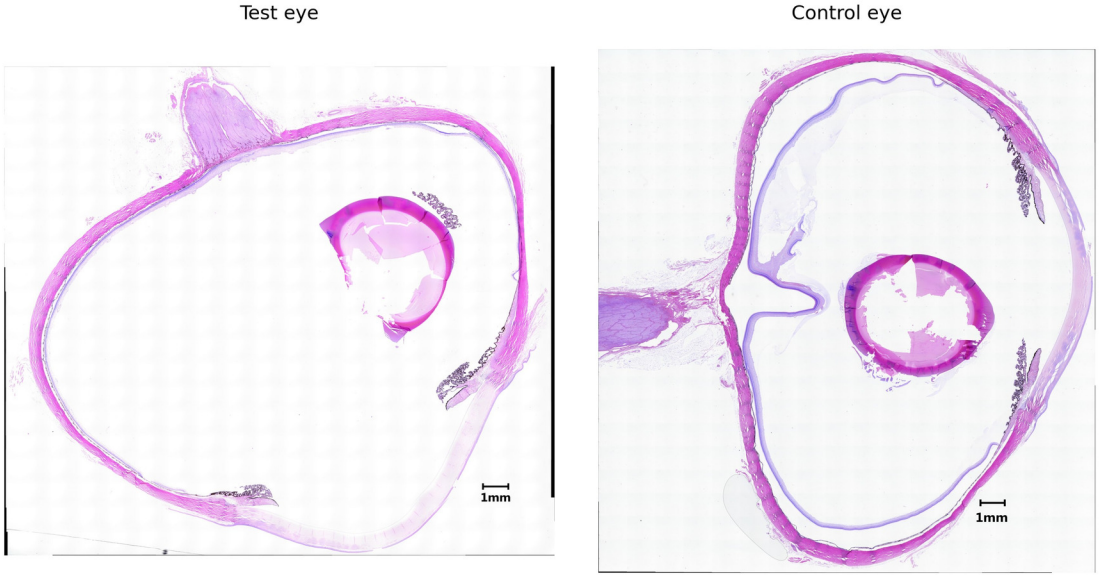

FIGURE I.27. Hematoxylin and eosin sections of animal 4, slide 5: test eye (left) and control eye (right).

APPENDIX J

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Figure 1.3	Sobotta and McMurrich 1914, *Atlas and Text-book of Human Anatomy*, vol. 3, Fig. 743	Public domain (published 1914)	Not required; printed caption cropped, attributed in the caption
Figure 1.5	Kur et al. 2012, *Prog. Retin. Eye Res.*, Fig. 1 [**25**]; drawings from Anand-Apte and Hollyfield, *Encyclopedia of the Eye*, 2010	Crown copyright; published by Elsevier	Licensed by Elsevier, RightsLink license no. 6357801012581 (Sep. 28, 2026); the original drawings (*Encyclopedia of the Eye*, Elsevier) licensed by Elsevier, RightsLink license no. 6364320254388 (Oct. 8, 2026)
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Figure 2.1C	Nakamura et al. 2026, *Ann. Biomed. Eng.*, Fig. 2A [**1**]	CC BY 4.0 (open access)	Not required; attributed in the caption
Figure 2.4	Nakamura et al. 2026, *Ann. Biomed. Eng.*, Fig. 1 [**1**]	CC BY 4.0 (open access)	Not required; attributed in the caption

APPENDIX K

Use of Generative Artificial Intelligence

A generative artificial intelligence (AI) tool (Claude, Anthropic, accessed via Claude Code) was used during the preparation of this dissertation. Its use, as described below, was approved by all five members of the dissertation committee by e-mail between October 23 and November 4, 2026.

Writing. The tool assisted with literature search and organization, verification of cited sources against their original publications, drafting and revision of prose, and formatting of the LaTeX source. All AI-assisted text was reviewed, checked against the primary literature, and edited by the author.

Code. Under the author's direction, the tool helped write, revise, test, and run the analysis code of Chapters 2 and 3 and the scripts that generate their figures and tables, and helped transfer the results into the text. It wrote the Python finite-element program of Chapter 3 and its test scripts, and it reviewed the code and results of that program, which was accepted only after the program had reproduced the solutions of the commercial software (Section 3.3.1). Every analytical decision was made by the author.

The tool was not used to design experiments or to collect data. The author takes full responsibility for the accuracy and content of this dissertation.

Cited Literature

[1] Y. Nakamura, L. Mecacci, and J. R. Hetling, "Therapeutic levels of hypothermia achieved in isolated porcine eyes using a scleral contact interface," *Annals of Biomedical Engineering*, vol. 54, no. 2, pp. 570–579, 2026 (published online Nov. 19, 2025). doi:10.1007/s10439-025-03905-w.

[2] B. Mac Grory, M. Schrag, V. Biousse, K. L. Furie, M. Gerhard-Herman, P. J. Lavin, L. Sobrin, S. I. Tjoumakaris, C. M. Weyand, and S. Yaghi, "Management of central retinal artery occlusion: A scientific statement from the American Heart Association," *Stroke*, vol. 52, no. 6, pp. e282–e294, 2021. doi:10.1161/STR.0000000000000366.

[3] J. A. Leavitt, T. A. Larson, D. O. Hodge, and R. E. Gullerud, "The incidence of central retinal artery occlusion in Olmsted County, Minnesota," *American Journal of Ophthalmology*, vol. 152, no. 5, pp. 820–823.e2, 2011. doi:10.1016/j.ajo.2011.05.005.

[4] J. Kim, S. J. Byun, S. J. Woo, K. H. Park, and S. J. Park, "Assessment of trends in the incidence rates of central retinal artery occlusion in Korea from 2002 to 2015," *JAMA Ophthalmology*, vol. 139, no. 4, pp. 399–405, 2021. doi:10.1001/jamaophthalmol.2020.6860.

[5] D. D. Hwang, K. E. Lee, Y. Kim, M. S. Kim, T. H. Rim, M. Kim, H. Kim, D. S. Kyoung, and J. I. Park, "Incidence of retinal artery occlusion and related mortality in Korea, 2005 to 2018," *JAMA Network Open*, vol. 6, no. 3, art. e233068, 2023. doi:10.1001/jamanetworkopen.2023.3068.

[6] S. S. Hayreh and M. B. Zimmerman, "Central retinal artery occlusion: Visual outcome," *American Journal of Ophthalmology*, vol. 140, no. 3, pp. 376–391, 2005. doi:10.1016/j.ajo.2005.03.038.

[7] M. Schrag, T. Youn, J. Schindler, H. Kirshner, and D. Greer, "Intravenous fibrinolytic therapy in central retinal artery occlusion: A patient-level meta-analysis," *JAMA Neurology*, vol. 72, no. 10, pp. 1148–1154, 2015. doi:10.1001/jamaneurol.2015.1578.

[8] S. Prabhakaran, N. R. Gonzalez, K. S. Zachrison, et al., "2026 guideline for the early management of patients with acute ischemic stroke: A guideline from the American Heart Association/American Stroke Association," *Stroke*, vol. 57, pp. e316–e436, 2026. doi:10.1161/STR.0000000000000513.

[9] M. Schumacher et al. (EAGLE Study Group), "Central retinal artery occlusion: Local intra-arterial fibrinolysis versus conservative treatment, a multicenter randomized trial," *Ophthalmology*, vol. 117, no. 7, pp. 1367–1375.e1, 2010. doi:10.1016/j.ophtha.2010.03.061.

[10] C. Préterre et al. (THEIA collaborators), "Intravenous alteplase versus oral aspirin for acute central retinal artery occlusion within 4.5 h of severe vision loss (THEIA): A multicentre, double-dummy, patient-blinded and assessor-blinded, randomised, controlled, phase 3 trial," *The Lancet Neurology*, vol. 24, no. 11, pp. 909–919, 2025. doi:10.1016/S1474-4422(25)00308-4.

[11] S. J. Ryan et al. (TenCRAOS Investigators), "A randomized trial of tenecteplase in acute central retinal artery occlusion," *New England Journal of Medicine*, vol. 394, no. 5, pp. 442–450, 2026. doi:10.1056/NEJMoa2508515.

[12] ClinicalTrials.gov, "Early reperfusion therapy with intravenous thrombolysis for recovery of VISION in acute central retinal artery occlusion (REVISION)," registry entry NCT04965038, status Terminated (stopped after prespecified interim analysis due to futility), last updated May 18, 2026, `https://clinicaltrials.gov/study/NCT04965038`, accessed Oct. 28, 2026.

[13] B. Mac Grory, C. Preterre, P. Lebranchu, S. J. Ryan, Ø. Kalsnes Jørstad, M. C. Moe, et al., "Intravenous thrombolysis for acute central retinal artery occlusion: Protocol for a systematic

review and individual participant data meta-analysis of randomized controlled trials," *PLoS ONE*, vol. 21, no. 3, art. e0342739, 2026. doi:10.1371/journal.pone.0342739.

[14] E. Ardila Jurado et al. (Swiss Stroke Registry Investigators), "Central retinal artery occlusion: Current practice, awareness and prehospital delays in Switzerland," *Frontiers in Neurology*, vol. 13, art. 888456, 2022. doi:10.3389/fneur.2022.888456.

[15] C. Stretz, J. E. Paddock, T. M. Burton, T. Bakaeva, M. Freeman, A. Choudhury, S. Yaghi, K. L. Furie, M. Schrag, and B. Mac Grory, "IV thrombolysis for central retinal artery occlusion: Real-world experience from a comprehensive stroke center," *Journal of Stroke and Cerebrovascular Diseases*, vol. 33, no. 4, art. 107610, 2024. doi:10.1016/j.jstrokecerebrovasdis.2024.107610.

[16] A. K. Rudkin, A. W. Lee, and C. S. Chen, "Central retinal artery occlusion: Timing and mode of presentation," *European Journal of Neurology*, vol. 16, no. 6, pp. 674–677, 2009. doi:10.1111/j.1468-1331.2009.02616.x.

[17] Y. Shen, M. Drum, and S. Roth, "The prevalence of perioperative visual loss in the United States: A 10-year study from 1996 to 2005 of spinal, orthopedic, cardiac, and general surgery," *Anesthesia & Analgesia*, vol. 109, no. 5, pp. 1534–1545, 2009. doi:10.1213/ane.0b013e3181b0500b.

[18] T. Calway, D. S. Rubin, H. E. Moss, C. E. Joslin, K. Beckmann, and S. Roth, "Perioperative retinal artery occlusion: Risk factors in cardiac surgery from the United States National Inpatient Sample 1998–2013," *Ophthalmology*, vol. 124, no. 2, pp. 189–196, 2017. doi:10.1016/j.ophtha.2016.10.025.

[19] L. A. Lee, S. Roth, K. L. Posner, F. W. Cheney, R. A. Caplan, N. J. Newman, and K. B. Domino, "The American Society of Anesthesiologists Postoperative Visual Loss Registry: Analysis of 93 spine surgery cases with postoperative visual loss," *Anesthesiology*, vol. 105, no. 4, pp. 652–659, 2006. doi:10.1097/00000542-200610000-00007.

[20] Postoperative Visual Loss Study Group, "Risk factors associated with ischemic optic neuropathy after spinal fusion surgery," *Anesthesiology*, vol. 116, no. 1, pp. 15–24, 2012. doi:10.1097/ALN.0b013e31823d012a.

[21] American Society of Anesthesiologists Task Force on Perioperative Visual Loss, North American Neuro-Ophthalmology Society, and Society for Neuroscience in Anesthesiology and Critical Care, "Practice advisory for perioperative visual loss associated with spine surgery 2019," *Anesthesiology*, vol. 130, no. 1, pp. 12–30, 2019. doi:10.1097/ALN.0000000000002503.

[22] I. Bekerman, P. Gottlieb, and M. Vaiman, "Variations in eyeball diameters of the healthy adults," *Journal of Ophthalmology*, vol. 2014, art. 503645, 2014. doi:10.1155/2014/503645.

[23] R. C. Augusteyn, D. Nankivil, A. Mohamed, B. Maceo, F. Pierre, and J.-M. Parel, "Human ocular biometry," *Experimental Eye Research*, vol. 102, pp. 70–75, 2012. doi:10.1016/j.exer.2012.06.009.

[24] L. A. Remington, *Clinical Anatomy and Physiology of the Visual System*, 3rd ed. St. Louis, MO: Elsevier Butterworth-Heinemann, 2012. ISBN 978-1-4377-1926-0.

[25] J. Kur, E. A. Newman, and T. Chan-Ling, "Cellular and physiological mechanisms underlying blood flow regulation in the retina and choroid in health and disease," *Progress in Retinal and Eye Research*, vol. 31, no. 5, pp. 377–406, 2012. doi:10.1016/j.preteyeres.2012.04.004.

[26] M. Ferrara, G. Lugano, M. T. Sandinha, V. R. Kearns, B. Geraghty, and D. H. W. Steel, "Biomechanical properties of retina and choroid: A comprehensive review of techniques and translational relevance," *Eye*, vol. 35, pp. 1818–1832, 2021. doi:10.1038/s41433-021-01437-w.

[27] P. G. Watson and R. D. Young, "Scleral structure, organisation and disease. A review," *Experimental Eye Research*, vol. 78, no. 3, pp. 609–623, 2004. doi:10.1016/S0014-4835(03)00212-4.

[28] R. F. Spaide, "Choroidal blood flow: Review and potential explanation for the choroidal venous anatomy including the vortex vein system," *Retina*, vol. 40, no. 10, pp. 1851–1864, 2020.

[29] S. S. Hayreh, "Blood flow in the optic nerve head and factors that may influence it," *Progress in Retinal and Eye Research*, vol. 20, no. 5, pp. 595–624, 2001. doi:10.1016/S1350-9462(01)00005-2.

[30] S. S. Hayreh, M. B. Zimmerman, A. Kimura, and A. Sanon, "Central retinal artery occlusion. Retinal survival time," *Experimental Eye Research*, vol. 78, no. 3, pp. 723–736, 2004. doi:10.1016/S0014-4835(03)00214-8.

[31] S. Tobalem, J. S. Schutz, and A. Chronopoulos, "Central retinal artery occlusion – Rethinking retinal survival time," *BMC Ophthalmology*, vol. 18, art. 101, 2018. doi:10.1186/s12886-018-0768-4.

[32] C. E. Riva, J. E. Grunwald, S. H. Sinclair, and B. L. Petrig, "Blood velocity and volumetric flow rate in human retinal vessels," *Investigative Ophthalmology & Visual Science*, vol. 26, no. 8, pp. 1124–1132, 1985.

[33] G. T. Feke, H. Tagawa, D. M. Deupree, D. G. Goger, J. Sebag, and J. J. Weiter, "Blood flow in the normal human retina," *Investigative Ophthalmology & Visual Science*, vol. 30, no. 1, pp. 58–65, 1989.

[34] J. E. Grunwald, C. E. Riva, J. Baine, and A. J. Brucker, "Total retinal volumetric blood flow rate in diabetic patients with poor glycemic control," *Investigative Ophthalmology & Visual Science*, vol. 33, no. 2, pp. 356–363, 1992.

[35] J. P. S. Garcia Jr., P. T. Garcia, and R. B. Rosen, "Retinal blood flow in the normal human eye using the Canon laser blood flowmeter," *Ophthalmic Research*, vol. 34, no. 5, pp. 295–299, 2002. doi:10.1159/000065600.

[36] A. Yoshida, G. T. Feke, H. Ogasawara, D. G. Goger, and J. W. McMeel, "Retinal hemodynamics in middle-aged normal subjects," *Ophthalmic Research*, vol. 28, no. 6, pp. 343–350, 1996. doi:10.1159/000267926.

[37] S. Szegedi, N. Hommer, M. Kallab, S. Puchner, D. Schmidl, R. M. Werkmeister, G. Garhöfer, and L. Schmetterer, "Repeatability and reproducibility of total retinal blood flow measurements using bi-directional Doppler OCT," *Translational Vision Science & Technology*, vol. 9, no. 7, art. 34, 2020. doi:10.1167/tvst.9.7.34.

[38] A. Bill and G. O. Sperber, "Control of retinal and choroidal blood flow," *Eye*, vol. 4, no. 2, pp. 319–325, 1990. doi:10.1038/eye.1990.43.

[39] L. M. Parver, "Temperature modulating action of choroidal blood flow," *Eye*, vol. 5, no. 2, pp. 181–185, 1991. doi:10.1038/eye.1991.32.

[40] L. M. Parver, C. R. Auker, and D. O. Carpenter, "The stabilizing effect of the choroidal circulation on the temperature environment of the macula," *Retina*, vol. 2, no. 2, pp. 117–120, 1982.

[41] A. Alm and A. Bill, "Ocular and optic nerve blood flow at normal and increased intraocular pressures in monkeys (*Macaca irus*): A study with radioactively labelled microspheres including flow determinations in brain and some other tissues," *Experimental Eye Research*, vol. 15, no. 1, pp. 15–29, 1973. doi:10.1016/0014-4835(73)90185-1.

[42] T. M. Nork, C. B. Y. Kim, D. Shanmuganayagam, M. S. Van Lysel, J. N. Ver Hoeve, and J. D. Folts, "Measurement of regional choroidal blood flow in rabbits and monkeys using fluorescent microspheres," *Archives of Ophthalmology*, vol. 124, no. 6, pp. 860–868, 2006. doi:10.1001/archopht.124.6.860.

[43] Y. Zhang, O. San Emeterio Nateras, Q. Peng, C. A. Rosende, and T. Q. Duong, "Blood flow MRI of the human retina/choroid during rest and isometric exercise," *Investigative Ophthalmology & Visual Science*, vol. 53, no. 7, pp. 4299–4305, 2012.

[44] C. J. Pournaras, E. Rungger-Brändle, C. E. Riva, S. H. Hardarson, and E. Stefansson, "Regulation of retinal blood flow in health and disease," *Progress in Retinal and Eye Research*, vol. 27, no. 3, pp. 284–330, 2008. doi:10.1016/j.preteyeres.2008.02.002.

[45] C. E. Riva, P. Titze, M. Hero, A. Movaffaghy, and B. L. Petrig, "Choroidal blood flow during isometric exercises," *Investigative Ophthalmology & Visual Science*, vol. 38, no. 11, pp. 2338–2343, 1997.

[46] K. Shinoda, K. Yagura, S. Matsumoto, G. Terauchi, A. Mizota, and Y. Miyake, "Intraocular temperature at different sites in eye measured at the beginning of vitreous surgery," *Journal of Clinical Medicine*, vol. 10, no. 15, art. 3412, 2021. doi:10.3390/jcm10153412.

[47] R. Kobylianskyi, K. Przystupa, V. Lysko, J. Majewski, L. Vikhor, V. Boichuk, O. Zadorozhnyy, O. Kochan, M. Umanets, and N. Pasyechnikova, "Thermoelectric measuring equipment for perioperative monitoring of temperature and heat flux density of the human eye in vitreoretinal surgery," *Sensors*, vol. 25, no. 4, art. 999, 2025. doi:10.3390/s25040999.

[48] G. J. Ríos-Nequis, J. E. Covarrubias-Avilés, A. Levine-Berebichez, A. C. Esquivel-Muciño, and J. A. Ramírez-Estudillo, "Intraocular thermodynamics during vitreoretinal surgery," *Journal of VitreoRetinal Diseases*, 2026 (published online Oct. 22, 2026, ahead of print). doi:10.1177/24741264261496265.

[49] O. S. Zadorozhnyy, A. R. Korol, V. O. Naumenko, N. V. Pasyechnikova, and L. L. Butenko, "Heat exchange in the human eye: A review," *Journal of Ophthalmology (Ukraine)*, no. 6, pp. 50–58, 2022. doi:10.31288/oftalmolzh202265058.

[50] E. H. Ooi, W. T. Ang, and E. Y. K. Ng, "Bioheat transfer in the human eye: A boundary element approach," *Engineering Analysis with Boundary Elements*, vol. 31, no. 6, pp. 494–500, 2007. doi:10.1016/j.enganabound.2006.09.011.

[51] J. A. Scott, "A finite element model of heat transport in the human eye," *Physics in Medicine and Biology*, vol. 33, no. 2, pp. 227–241, 1988. doi:10.1088/0031-9155/33/2/003.

[52] V. M. M. Flyckt, B. W. Raaymakers, and J. J. W. Lagendijk, "Modelling the impact of blood flow on the temperature distribution in the human eye and the orbit: Fixed heat transfer coefficients versus the Pennes bioheat model versus discrete blood vessels," *Physics in Medicine and Biology*, vol. 51, no. 19, pp. 5007–5021, 2006. doi:10.1088/0031-9155/51/19/018.

[53] D. Gongal, J. R. Hetling, and C. D. Foster, "Hypothermia treatment for ocular ischemia: A multiphase porous thermal model of human eye," *Journal of Thermal Biology*, vol. 131, art. 104139, 2025. doi:10.1016/j.jtherbio.2025.104139.

[54] M. H. Geiser, M. Bonvin, and O. Quibel, "Corneal and retinal temperatures under various ambient conditions: A model and experimental approach," *Klinische Monatsblätter für Augenheilkunde*, vol. 221, no. 5, pp. 311–314, 2004. doi:10.1055/s-2004-812878.

[55] D. Gongal et al., "Thermal finite element analysis of localized hypothermia treatment of the human eye," *Medical Engineering & Physics*, vol. 111, art. 103928, 2023. doi:10.1016/j.medengphy.2022.103928.

[56] K. H. Polderman, "Mechanisms of action, physiological effects, and complications of hypothermia," *Critical Care Medicine*, vol. 37, no. 7 (Suppl.), pp. S186–S202, 2009. doi:10.1097/CCM.0b013e3181aa5241.

[57] M. A. Yenari and H. S. Han, "Neuroprotective mechanisms of hypothermia in brain ischaemia," *Nature Reviews Neuroscience*, vol. 13, no. 4, pp. 267–278, 2012. doi:10.1038/nrn3174.

[58] Hypothermia after Cardiac Arrest Study Group, "Mild therapeutic hypothermia to improve the neurologic outcome after cardiac arrest," *New England Journal of Medicine*, vol. 346, no. 8, pp. 549–556, 2002. doi:10.1056/NEJMoa012689.

[59] S. A. Bernard, T. W. Gray, M. D. Buist, B. M. Jones, W. Silvester, G. Gutteridge, and K. Smith, "Treatment of comatose survivors of out-of-hospital cardiac arrest with induced hypothermia," *New England Journal of Medicine*, vol. 346, no. 8, pp. 557–563, 2002. doi:10.1056/NEJMoa003289.

[60] N. Nielsen, J. Wetterslev, T. Cronberg, et al., "Targeted temperature management at 33°C versus 36°C after cardiac arrest," *New England Journal of Medicine*, vol. 369, no. 23, pp. 2197–2206, 2013. doi:10.1056/NEJMoa1310519.

[61] J. Dankiewicz, T. Cronberg, G. Lilja, et al., "Hypothermia versus normothermia after out-of-hospital cardiac arrest," *New England Journal of Medicine*, vol. 384, no. 24, pp. 2283–2294, 2021. doi:10.1056/NEJMoa2100591.

[62] A. Lavinio, K. M. Busl, J. P. Coles, K. Donadello, R. Helbok, M. S. Sekhon, M. B. Skrifvars, F. S. Taccone, S. Wahlster, and C. Robba, "Temperature control in acute brain injury," *Intensive Care Medicine*, vol. 52, pp. 714–728, 2026. doi:10.1007/s00134-026-08367-9.

[63] J. P. Nolan, C. Sandroni, A. Cariou, T. Cronberg, S. D'Arrigo, K. Haywood, A. Hoedemaekers, G. Lilja, N. Nikolaou, T. M. Olasveengen, C. Robba, M. B. Skrifvars, P. Swindell, and J. Soar, "European Resuscitation Council and European Society of Intensive Care Medicine guidelines 2025: Post-resuscitation care," *Resuscitation*, vol. 215, suppl. 1, art. 110809, 2025. doi:10.1016/j.resuscitation.2025.110809.

[64] K. S. Alotaibi, N. Saleem, T. A. Chandos Snow, P. Arina, A. Dyson, and M. Singer, "Does time to achieve a targeted body temperature matter for survivors of cardiac arrest? A systematic review and meta-analysis," *Critical Care Medicine*, vol. 54, no. 2, pp. 312–323, 2026. doi:10.1097/CCM.0000000000006964.

[65] T. Kass-Hout, C. Horn, C.-H. J. Sun, A. Yoo, O. O. Zaidat, et al., "Adjuvant systemic hypothermia during mechanical thrombectomy (ReCCLAIM II): A multicenter, randomized, controlled trial," *Stroke*, vol. 57, 2026 (published online Oct. 9, 2026, ahead of print). doi:10.1161/STROKEAHA.126.056651.

[66] A. Quiñones-Hinojosa, J. Y. Malek, A. Ames III, C. S. Ogilvy, and K. I. Maynard, "Metabolic effects of hypothermia and its neuroprotective effects on the recovery of metabolic and electrophysiological function in the ischemic retina in vitro," *Neurosurgery*, vol. 52, no. 5, pp. 1178–1187, 2003. doi:10.1227/01.neu.0000057836.64972.a0.

[67] A. M. Mueller-Buehl et al., "Oxidative stress-induced retinal damage is prevented by mild hypothermia in an ex vivo model of cultivated porcine retinas," *Clinical & Experimental Ophthalmology*, vol. 48, no. 5, pp. 666–681, 2020. doi:10.1111/ceo.13731.

[68] P. Klemm, J. Hurst, M. Dias Blak, T. Herrmann, M. Melchinger, K. U. Bartz-Schmidt, G. Zeck, M. Schultheiss, M. S. Spitzer, and S. Schnichels, "Hypothermia protects retinal ganglion cells against hypoxia-induced cell death in a retina organ culture model," *Clinical & Experimental Ophthalmology*, vol. 47, no. 8, pp. 1043–1054, 2019. doi:10.1111/ceo.13565.

[69] M. Coassin, K. G. Duncan, K. R. Bailey, A. Singh, and D. M. Schwartz, "Hypothermia reduces secretion of vascular endothelial growth factor by cultured retinal pigment epithelial cells," *British Journal of Ophthalmology*, vol. 94, no. 12, pp. 1678–1683, 2010. doi:10.1136/bjo.2009.168864.

[70] E. M. Salido, D. Dorfman, M. Bordone, M. Chianelli, M. F. González Fleitas, and R. E. Rosenstein, "Global and ocular hypothermic preconditioning protect the rat retina from ischemic damage," *PLoS ONE*, vol. 8, no. 4, art. e61656, 2013. doi:10.1371/journal.pone.0061656.

[71] S. Becker, Z. L'Ecuyer Morison, J. Allen, S. Saeid, L. Sturgis, A. Adderley, A. Koskelainen, and F. Vinberg, "Healing of ischemic injury in the retina," *Science Advances*, vol. 12, art. eadx7204, 2026. doi:10.1126/sciadv.adx7204.

[72] L. R. Wolin and L. C. Massopust Jr., "Selective cooling of the eye: Effects on evoked potentials of the visual system," *Archives of Ophthalmology*, vol. 76, no. 5, pp. 723–728, 1966. doi:10.1001/archopht.1966.03850010725020.

[73] P. Lachapelle, J. Benoit, and P. Guité, "The effect of in vivo retinal cooling on the electroretinogram of the rabbit," *Vision Research*, vol. 36, no. 3, pp. 339–344, 1996. doi:10.1016/0042-6989(95)00126-3.

[74] M. Pitkänen, O. Kaikkonen, and A. Koskelainen, "A novel method for mouse retinal temperature determination based on ERG photoresponses," *Annals of Biomedical Engineering*, vol. 45, no. 10, pp. 2360–2372, 2017. doi:10.1007/s10439-017-1872-y.

[75] O. Kaikkonen, T. T. Turunen, A. Meller, J. Ahlgren, and A. Koskelainen, "Retinal temperature determination based on photopic porcine electroretinogram," *IEEE Transactions on Biomedical Engineering*, vol. 69, no. 2, pp. 991–1002, 2022. doi:10.1109/TBME.2021.3111533.

[76] M. Amirkavei, O. Kaikkonen, T. Turunen, A. Meller, J. Åhlgren, A. Kvanta, H. André, and A. Koskelainen, "Non-damaging laser treatment with electroretinography-based thermal dosimetry activates hormetic heat response in pig retinal pigment epithelium," *Nature Communications*, vol. 16, art. 9533, 2025. doi:10.1038/s41467-025-64095-6.

[77] D. R. May, R. J. Freedland, S. Charles, C. Wang, and J. Bakos, "Ocular hypothermia: Anterior chamber perfusion," *British Journal of Ophthalmology*, vol. 67, no. 12, pp. 808–813, 1983. doi:10.1136/bjo.67.12.808.

[78] K. Tamai, E. Toumoto, and A. Majima, "Local hypothermia protects the retina from ischaemic injury in vitrectomy," *British Journal of Ophthalmology*, vol. 81, no. 9, pp. 789–794, 1997. doi:10.1136/bjo.81.9.789.

[79] Y. Zhang, M. Li, B. Yu, S. Lu, L. Zhang, S. Zhu, Z. Yu, T. Xia, et al., "Cold protection allows local cryotherapy in a clinical-relevant model of traumatic optic neuropathy," *eLife*, vol. 11, art. e75070, 2022. doi:10.7554/eLife.75070.

[80] B. S. Kim, Y. J. Kim, S. W. Seo, et al., "Long-term results from cyclocryotherapy applied to the 3-o'clock and 9-o'clock positions in blind refractory glaucoma patients," *Korean Journal of Ophthalmology*, vol. 29, no. 1, pp. 47–52, 2015. doi:10.3341/kjo.2015.29.1.47.

[81] N. Faberowski, E. Stefansson, and R. C. Davidson, "Local hypothermia protects the retina from ischemia: A quantitative study in the rat," *Investigative Ophthalmology & Visual Science*, vol. 30, no. 11, pp. 2309–2313, 1989.

[82] T. Yahalomi, I. Hecht, O. Lagstein, A. Nemet, L. Pe'er, F. Hadad, et al., "Reduction of postintravitreal injection pain using ice: An open-label interventional randomized controlled trial," *Retina*, vol. 40, no. 7, pp. 1434–1438, 2020. doi:10.1097/IAE.0000000000002608.

[83] T. Kataoka, M. Zako, M. Takeyama, A. Ohno-Jinno, et al., "Cooling prevents induction of corneal damage by argon laser peripheral iridotomy," *Japanese Journal of Ophthalmology*, vol. 51, no. 5, pp. 317–324, 2007. doi:10.1007/s10384-007-0460-3.

[84] C. G. Besirli, S. J. Smith, D. N. Zacks, et al., "Randomized safety and feasibility trial of ultra-rapid cooling anesthesia for intravitreal injections," *Ophthalmology Retina*, vol. 4, no. 10, pp. 979–986, 2020. doi:10.1016/j.oret.2020.04.001.

[85] D. L. Chao, N. T. Rinella, A. M. Khanani, C. C. Wykoff, and G.-H. Kim, "Cooling anesthesia for intravitreal injection: Results of the prospective open-label, dose-ranging COOL-1 trial," *Clinical Ophthalmology*, vol. 15, pp. 4659–4666, 2021. doi:10.2147/OPTH.S336653.

[86] U.S. Food and Drug Administration, "De Novo classification request for OcuCool," De Novo request DEN230011, sponsor RecensMedical, Inc., received Feb. 16, 2023, granted Sept. 30, 2024. Established the generic device type "ophthalmic cooling anesthesia device" (21 CFR 886.4190, Class II, product code QZQ).

[87] L. I. Anatychuk et al., "Thermoelectric device for contact cooling of the human eye," *Physics and Chemistry of Solid State*, vol. 21, no. 1, pp. 140–145, 2020. doi:10.15330/pcss.21.1.140-145.

[88] R. R. Kobylianskyi, V. V. Lysko, O. S. Zadorozhnyy, and N. V. Pasechnikova, "Complex of thermoelectric equipment for diagnostics and treatment of ophthalmological diseases," *Journal of Thermoelectricity*, no. 3, pp. 101–116, 2025. doi:10.63527/1607-8829-2025-3-101-116.

[89] A. Galor et al., "A randomized double-masked study evaluating safety and efficacy of a novel topical cooling device for chronic ocular surface pain," *Ophthalmology and Therapy*, vol. 15, pp. 2033–2050, 2026. doi:10.1007/s40123-026-01392-3.

[90] A. Galor, B. Henderson, C. Ifantides, J. A. Stefater, and T. P. Stryjewski, "The safety and tolerability of a single scleral treatment with a cooling device in rabbits," *Translational Vision Science & Technology*, vol. 14, no. 10, art. 2, 2025. doi:10.1167/tvst.14.10.2.

[91] J. C. B. Chiang, M. Wu, J.-H. Lee, A. Curkpatrick, C. Ifantides, T. Nguyen, A. Galor, H. Wu, R. Lindstrom, V. Thompson, and L. E. Downie, "Impact of an ophthalmic cooling device on corneal immune and sensory nerve features in chronic ocular surface pain: A prospective, feasibility study," *Ophthalmology and Therapy*, vol. 15, pp. 2979–2996, 2026. doi:10.1007/s40123-026-01454-6.

[92] C. Ifantides et al., "Improved corneal sensitivity after treatment with a novel topical ophthalmic cooling device in patients with chronic ocular surface pain," *Investigative Ophthalmology & Visual Science*, vol. 67, p. 3458, 2026 (ARVO Annual Meeting abstract).

[93] F. J. DiSalvo, "Thermoelectric cooling and power generation," *Science*, vol. 285, no. 5428, pp. 703–706, 1999. doi:10.1126/science.285.5428.703.

[94] X.-L. Shi, J. Zou, and Z.-G. Chen, "Advanced thermoelectric design: From materials and structures to devices," *Chemical Reviews*, vol. 120, no. 15, pp. 7399–7515, 2020. doi:10.1021/acs.chemrev.0c00026.

[95] Tark Thermal Solutions, Inc., "Annular RH series RH14-14-10-L1-W4.5 thermoelectric cooler (MFG part 71062-514)," product datasheet, Rev. 00, dated June 1, 2022, print date Aug. 11, 2025.

[96] S. H. Zaferani, M. W. Sams, R. Ghomashchi, and Z.-G. Chen, "Thermoelectric coolers as thermal management systems for medical applications: Design, optimization, and advancement," *Nano Energy*, vol. 90, part A, art. 106572, 2021.

[97] B. Hu, X.-L. Shi, J. Zou, and Z.-G. Chen, "Thermoelectrics for medical applications: Progress, challenges, and perspectives," *Chemical Engineering Journal*, vol. 437, art. 135268, 2022.

[98] M. Zhang, Z. Li, Q. Wang, Y. Xu, P. Hu, and X. Zhang, "Performance investigation of a portable liquid cooling garment using thermoelectric cooling," *Applied Thermal Engineering*, vol. 214, art. 118830, 2022.

[99] M. A. McCall, "Pig models in retinal research and retinal disease," *Cold Spring Harbor Perspectives in Medicine*, vol. 14, no. 4, art. a041296, 2024. doi:10.1101/cshperspect.a041296.

[100] I. Sánchez, R. Martín, F. Ussa, and I. Fernández-Bueno, "The parameters of the porcine eyeball," *Graefe's Archive for Clinical and Experimental Ophthalmology*, vol. 249, no. 4, pp. 475–482, 2011.

[101] T. W. Olsen, S. Sanderson, X. Feng, and W. C. Hubbard, "Porcine sclera: Thickness and surface area," *Investigative Ophthalmology & Visual Science*, vol. 43, no. 8, pp. 2529–2532, 2002.

[102] S. Nicoli, G. Ferrari, M. Quarta, C. Macaluso, P. Govoni, D. Dallatana, and P. Santi, "Porcine sclera as a model of human sclera for in vitro transport experiments: Histology, SEM, and comparative permeability," *Molecular Vision*, vol. 15, pp. 259–266, 2009.

[103] F. Lorget, A. Parenteau, M. Carrier, et al., "Characterization of the pH and temperature in the rabbit, pig, and monkey eye: Key parameters for the development of long-acting delivery ocular strategies," *Molecular Pharmaceutics*, vol. 13, no. 9, pp. 2891–2896, 2016. doi:10.1021/acs.molpharmaceut.5b00731.

[104] J. Rootman, "Vascular system of the optic nerve head and retina in the pig," *British Journal of Ophthalmology*, vol. 55, no. 12, pp. 808–819, 1971. doi:10.1136/bjo.55.12.808.

[105] S. Fouquet, O. Vacca, F. Sennlaub, and M. Paques, "The 3D retinal capillary circulation in pigs reveals a predominant serial organization," *Investigative Ophthalmology & Visual Science*, vol. 58, no. 13, pp. 5754–5763, 2017. doi:10.1167/iovs.17-22097.

[106] S. Middleton, "Porcine ophthalmology," *Veterinary Clinics of North America: Food Animal Practice*, vol. 26, no. 3, pp. 557–572, 2010. doi:10.1016/j.cvfa.2010.09.002.

[107] M. L. Beauchemin, "The fine structure of the pig's retina," *Albrecht von Graefes Archiv für Klinische und Experimentelle Ophthalmologie*, vol. 190, no. 1, pp. 27–45, 1974. doi:10.1007/BF00414333.

[108] A. Hendrickson and D. Hicks, "Distribution and density of medium- and short-wavelength selective cones in the domestic pig retina," *Experimental Eye Research*, vol. 74, no. 4, pp. 435–444, 2002. doi:10.1006/exer.2002.1181.

[109] P. Törnquist, A. Alm, and A. Bill, "Studies on ocular blood flow and retinal capillary permeability to sodium in pigs," *Acta Physiologica Scandinavica*, vol. 106, pp. 343–350, 1979.

[110] Y. Zhang, J. M. Harrison, O. San Emeterio Nateras, S. Chalfin, and T. Q. Duong, "Decreased retinal-choroidal blood flow in retinitis pigmentosa as measured by MRI," *Documenta Ophthalmologica*, vol. 126, no. 3, pp. 187–197, 2013. doi:10.1007/s10633-013-9374-1.

[111] O. San Emeterio Nateras et al., "Choroidal blood flow decreases with age: An MRI study," *Current Eye Research*, vol. 39, no. 10, pp. 1059–1067, 2014. doi:10.3109/02713683.2014.892997.

[112] S. Vurgese, S. Panda-Jonas, and J. B. Jonas, "Scleral thickness in human eyes," *PLoS ONE*, vol. 7, no. 1, art. e29692, 2012. doi:10.1371/journal.pone.0029692.

[113] J. Ruiz-Ederra, M. García, M. Hernández, H. Urcola, E. Hernández-Barbáchano, J. Araiz, and E. Vecino, "The pig eye as a novel model of glaucoma," *Experimental Eye Research*, vol. 81, no. 5, pp. 561–569, 2005. doi:10.1016/j.exer.2005.03.014.

[114] H. Morén, P. Undrén, B. Gesslein, G. K. Olivecrona, S. Andreasson, and M. Malmsjö, "The porcine retinal vasculature accessed using an endovascular approach: A new experimental model for retinal ischemia," *Investigative Ophthalmology & Visual Science*, vol. 50, no. 11, pp. 5504–5510, 2009. doi:10.1167/iovs.09-3529.

[115] S. J. Dreyer, "Design, fabricate, and test an in vitro system to investigate therapeutic hypothermia in porcine eyes," M.S. thesis, University of Illinois at Chicago, Chicago, IL, 2017.

[116] L. Mecacci, "Development and validation of a novel device to induce localized therapeutic hypothermia," M.S. thesis, University of Illinois at Chicago, Chicago, IL, 2022.

[117] L. Mecacci, Y. Nakamura, and J. R. Hetling, "Ocular hypothermia delivery system: in vitro validation using a non-perfused pig eye model," ARVO Annual Meeting, 2023 (poster; abstract control ID 3879496).

[118] L. Mecacci, Y. Nakamura, and J. R. Hetling, "Ocular hypothermia, in-vitro validation of a novel therapeutic apparatus using non perfused porcine eye model," Summer Biomechanics, Bioengineering and Biotransport Conference (SB3C), Lake Geneva, WI, June 11–14, 2024.

[119] L. Mecacci and J. R. Hetling, "Ocular hypothermia delivery system: in vitro validation using non-perfused human donor eyes," ARVO Annual Meeting, 2026 (poster; abstract control ID 4485539).

[120] J. Kong and P. Gouras, "The effect of body temperature on the murine electroretinogram," *Documenta Ophthalmologica*, vol. 106, no. 3, pp. 239–242, 2003. doi:10.1023/A:1022988332578.

[121] S. G. Michel, G. M. LaMuraglia II, M. L. L. Madariaga, and L. M. Anderson, "Innovative cold storage of donor organs using the Paragonix Sherpa Pak devices," *Heart, Lung and Vessels*, vol. 7, no. 3, pp. 246–255, 2015.

[122] R. L. Lindstrom et al., "Optisol corneal storage medium," *American Journal of Ophthalmology*, vol. 114, no. 3, pp. 345–356, 1992. doi:10.1016/S0002-9394(14)71803-3.

[123] A. V. Shakurov, A. V. Pushkarev, V. A. Pushkarev, and D. I. Tsiganov, "Prerequisites for developing new generation cryosurgical devices (review)," *Sovremennye Tehnologii v Medicine*, vol. 9, no. 2, pp. 178–187, 2017. doi:10.17691/stm2017.9.2.23.

[124] TE Connectivity, "GA10K3MCD1 Micro BetaCHIP (MCD) thermistor probes," datasheet, Feb. 2018.

[125] Fluke Calibration, "9102S Handheld Dry-Well," product specifications, `https://www.fluke.com/en-gb/product/calibration-tools/temperature-calibrators/fluke-calibration-9102s`, accessed Oct. 20, 2026.

[126] S. Zhang, Z. You, B. Rosner, and G.-S. Ying, "Evaluation of statistical methods for clustered eye data with skewed distribution," *Ophthalmology Science*, vol. 6, no. 2, art. 101020, 2026. doi:10.1016/j.xops.2025.101020.

[127] L. I. Los, R. J. van der Worp, M. J. A. van Luyn, and J. M. M. Hooymans, "Age-related liquefaction of the human vitreous body: LM and TEM evaluation of the role of proteoglycans and collagen," *Investigative Ophthalmology & Visual Science*, vol. 44, no. 7, pp. 2828–2833, 2003. doi:10.1167/iovs.02-0588.

[128] International Electrotechnical Commission, *IEC 60601-1: Medical electrical equipment – Part 1: General requirements for basic safety and essential performance*, consolidated ed. 3.2 (IEC 60601-1:2005+AMD1:2012+AMD2:2020), Subclause 11.1.2.2 and Table 24 (allowable maximum temperatures of applied parts). Geneva: IEC, Aug. 20, 2020.

[129] A. Krell, P. Blank, H. Ma, T. Hutzler, M. P. B. van Bruggen, and R. Apetz, "Transparent sintered corundum with high hardness and strength," *Journal of the American Ceramic Society*, vol. 86, no. 1, pp. 12–18, 2003. doi:10.1111/j.1151-2916.2003.tb03270.x.

[130] R. Apetz and M. P. B. van Bruggen, "Transparent alumina: A light-scattering model," *Journal of the American Ceramic Society*, vol. 86, no. 3, pp. 480–486, 2003. doi:10.1111/j.1151-2916.2003.tb03325.x.

[131] F. Kotz, K. Plewa, W. Bauer, et al., "Liquid glass: A facile soft replication method for structuring glass," *Advanced Materials*, vol. 28, no. 23, pp. 4646–4650, 2016. doi:10.1002/adma.201506089.

[132] F. Kotz, K. Arnold, W. Bauer, et al., "Three-dimensional printing of transparent fused silica glass," *Nature*, vol. 544, no. 7650, pp. 337–339, 2017. doi:10.1038/nature22061.

[133] H. H. Pennes, "Analysis of tissue and arterial blood temperatures in the resting human forearm," *Journal of Applied Physiology*, vol. 1, no. 2, pp. 93–122, 1948. doi:10.1152/jappl.1948.1.2.93.

[134] C. Kiel and A. J. E. Foss, "Steady-state thermal homeostasis model of heat generated and dissipated in the human retina," *Physical Biology*, vol. 23, art. 046008, 2026. doi:10.1088/1478-3975/ae9665.

[135] Tark Thermal Solutions, Inc., "Annular RH series RH14-14-06-L1-W4.5 thermoelectric cooler (MFG part 71063-505)," product datasheet, web page, `https://tark-solutions.com/products/thermoelectric-cooler-modules/peltier-annular-series/RH14-14-06-L1-W4.5/pdf`, accessed Oct. 31, 2026.

[136] K. Heil, R. Thomas, G. Robertson, A. Porter, R. Milner, and A. Wood, "Freezing and non-freezing cold weather injuries: A systematic review," *British Medical Bulletin*, vol. 117, no. 1, pp. 79–93, 2016. doi:10.1093/bmb/ldw001.

[137] C. Yap, A. M. Wong, J. Naor, and D. S. Rootman, "Corneal temperature reversal after storage in Chen medium compared with Optisol GS," *Cornea*, vol. 20, no. 5, pp. 501–504, 2001. doi:10.1097/00003226-200107000-00012.

[138] A. G. Robson, L. J. Frishman, J. Grigg, R. Hamilton, B. G. Jeffrey, M. Kondo, S. Li, and D. L. McCulloch, "ISCEV Standard for full-field clinical electroretinography (2022 update)," *Documenta Ophthalmologica*, vol. 144, pp. 165–177, 2022. doi:10.1007/s10633-022-09872-0.

[139] Johnson & Johnson Vision, *Clinical Assessment Guide*, Johnson & Johnson Middle East Inc., 2025 (document 2025PP11584). Grading scales adapted from N. Efron, *Contact Lens Practice*, 2nd ed., Butterworth-Heinemann, 2010.

[140] N. Efron, *Efron Grading Scales for Contact Lens Complications*, Millennium ed. Oxford, U.K.: Butterworth-Heinemann, 2000.

[141] American Veterinary Medical Association, *AVMA Guidelines for the Euthanasia of Animals: 2020 Edition*. Schaumburg, IL, USA: American Veterinary Medical Association, 2020. ISBN 978-1-882691-09-8.

[142] Y. Rotenstreich, G. A. Fishman, R. J. Anderson, and D. G. Birch, "Interocular amplitude differences of the full field electroretinogram in normal subjects," *British Journal of Ophthalmology*, vol. 87, no. 10, pp. 1268–1271, 2003. doi:10.1136/bjo.87.10.1268.

[143] K.-E. Choi, V. T. Q. Anh, J.-H. Oh, C. Yun, and S.-W. Kim, "Normative data of axial length, retinal thickness measurements, visual evoked potentials, and full-field electroretinography in female, wild-type minipigs," *Translational Vision Science & Technology*, vol. 10, no. 12, art. 3, 2021. doi:10.1167/tvst.10.12.3.

[144] G. A. Fishman, A. V. Chappelow, R. J. Anderson, Y. Rotenstreich, and D. J. Derlacki, "Short-term intervisit variability of ERG amplitudes in normal subjects and patients with retinitis pigmentosa," *Retina*, vol. 25, no. 8, pp. 1014–1021, 2005. doi:10.1097/00006982-200512000-00010.

[145] C. Sommer, F. Turgut, A. Saad, M. D. Becker, M. Al-Sheikh, C. Q. Davis, U. Rahn, M. T. S. Barboni, D. C. DeBuc, and G. M. Somfai, "The reproducibility of handheld ERGs recorded with skin electrode and natural pupils," *Documenta Ophthalmologica*, 2025 (published online, ahead of print). doi:10.1007/s10633-025-10065-8.

[146] K. S. Freeman, K. L. Good, P. H. Kass, S. A. Park, B. Nestorowicz, and R. Ofri, "Effects of chemical restraint on electroretinograms recorded sequentially in awake, sedated, and anesthetized dogs," *American Journal of Veterinary Research*, vol. 74, no. 7, pp. 1036–1042, 2013. doi:10.2460/ajvr.74.7.1036.

[147] N. B. Sørensen, A. T. Christiansen, T. W. Kjær, K. Klemp, M. la Cour, and J. F. Kiilgaard, "Time-dependent decline in multifocal electroretinogram requires faster recording procedures in anesthetized pigs," *Translational Vision Science & Technology*, vol. 6, no. 2, art. 6, 2017. doi:10.1167/tvst.6.2.6.

[148] M. Gencer and M. Yalvaç, "Comparison of the effects of spinal versus general anesthesia on the perioperative intraocular pressure: A prospective, randomized clinical trial," *Saudi Medical Journal*, vol. 47, pp. 1549–1557, 2026. doi:10.15537/1658-3175.8815.

[149] M. Mohammadzadeh, R. Azargoun, and R. Mohammadi, "Sedative effects of dexmedetomidine and ketamine or dexmedetomidine and midazolam on tear production, ocular perfusion pressure and pupil diameter in rabbits," *Veterinary Medicine and Science*, vol. 12, art. e71017, 2026. doi:10.1002/vms3.71017.

[150] C. M. Nicou, A. Pillai, and C. L. Passaglia, "Effects of acute stress, general anesthetics, tonometry, and temperature on intraocular pressure in rats," *Experimental Eye Research*, vol. 210, art. 108727, 2021. doi:10.1016/j.exer.2021.108727.

[151] G. J. Ortiz, D. J. Cook, M. E. Yablonski, H. Masonson, and G. Harmon, "Effect of cold air on aqueous humor dynamics in humans," *Investigative Ophthalmology & Visual Science*, vol. 29, no. 1, pp. 138–140, 1988.

[152] T. C. B. de Oliveira, J. L. M. Ferreira, H. T. de Lima, C. O. A. Bezerra Filho, and J. C. Ribeiro, "Intraocular pressure modulation with thermal stimuli," *Arquivos Brasileiros de Oftalmologia*, vol. 88, no. 1, art. e2023-0083, 2025. doi:10.5935/0004-2749.2023-0083.

[153] L. Michaud, "The impact of scleral lenses on intraocular pressure," *Journal of Clinical Medicine*, vol. 15, no. 4, art. 1635, 2026. doi:10.3390/jcm15041635.

[154] S. Orgül, J. Flammer, D. Stümpfig, and P. Hendrickson, "Intraocular pressure decrease after local ocular cooling is underestimated by applanation tonometry," *International Ophthalmology*, vol. 19, no. 2, pp. 95–99, 1995. doi:10.1007/BF00133179.

[155] P. S. Bilkhu, J. S. Wolffsohn, S. A. Naroo, L. Robertson, and R. Kennedy, "Effectiveness of nonpharmacologic treatments for acute seasonal allergic conjunctivitis," *Ophthalmology*, vol. 121, no. 1, pp. 72–78, 2014. doi:10.1016/j.ophtha.2013.08.007.

[156] G. W. Aylward, K. A. McClellan, R. Thomas, and F. A. Billson, "Transient corneal changes associated with the use of gold foil electrodes," *British Journal of Ophthalmology*, vol. 73, no. 12, pp. 980–984, 1989. doi:10.1136/bjo.73.12.980.

[157] B. C. Ackermann, L. Skrzypczyk, L. Geisweid, N. Junker, S. Wohlfart, P. Uhl, G. U. Auffarth, V. A. Augustin, and M. Hammer, "Increasing dark adaption times and ERG response in landrace pigs – An intraindividual comparison in 20 animals," *Experimental Eye Research*, vol. 262, art. 110733, 2026. doi:10.1016/j.exer.2025.110733.

[158] P. Dhungel, M. K. Alanazi, P. Caroline, L. Yudcovitch, and M. Liu, "Short-term impact of scleral lens wear on intraocular pressure and retinal nerve fiber layer thickness," *Life*, vol. 16, no. 7, art. 1094, 2026. doi:10.3390/life16071094.

[159] T. Ban, "Development of the multi-electrode electroretinogram: Technology, application and commercialization," Ph.D. dissertation, University of Illinois at Chicago, Chicago, IL, 2011.

[160] Y. Krakova, "Development of a multi-electrode electroretinography system and characterization of meERG signals in rat," M.S. thesis, University of Illinois at Chicago, Chicago, IL, 2012.

[161] G. Balasubramanian, "Development of multielectrode electroretinography (meERG) for human clinical use," Ph.D. dissertation, University of Illinois at Chicago, Chicago, IL, 2025.

[162] S. Patangay, Z. Derafshi, T. S. Vajaranant, J. C. Park, E. Ghahari, J. J. McAnany, and J. R. Hetling, "Three dimensional stimulus source for pattern electroretinography in mid- and far-peripheral retina," *Translational Vision Science & Technology*, vol. 7, no. 1, art. 8, 2018. doi:10.1167/tvst.7.1.8.

[163] P. Roy, "Peripheral pattern electroretinography: System upgrade, novel protocol development, & clinical validation," Ph.D. dissertation, University of Illinois Chicago, Chicago, IL, 2025.

[164] L. Bentivoglio, "Design and fabrication of a synchronously updated 3-D source for retinal electrodiagnostic testing," M.S. thesis, University of Illinois at Chicago, Chicago, IL, 2021.

[165] E. Puddu, "Design, fabrication and testing of a novel stimulus source for visual electrophysiology," M.S. thesis, University of Illinois at Chicago, Chicago, IL, 2022.

[166] S. Salameh, "First working prototype of a configurable full-field stimulus source for electrodiagnostic vision testing," M.S. thesis, University of Illinois at Chicago, Chicago, IL, 2024.

[167] L. Jones et al., "TFOS DEWS II management and therapy report," *The Ocular Surface*, vol. 15, no. 3, pp. 575–628, 2017. doi:10.1016/j.jtos.2017.05.006.

[168] R. W. Carr, S. Pianova, J. Fernandez, J. B. Fallon, C. Belmonte, and J. A. Brock, "Effects of heating and cooling on nerve terminal impulses recorded from cold-sensitive receptors in the guinea-pig cornea," *The Journal of General Physiology*, vol. 121, no. 5, pp. 427–439, 2003. doi:10.1085/jgp.200308814.

[169] Joint Committee for Guides in Metrology, *Evaluation of Measurement Data — Guide to the Expression of Uncertainty in Measurement*, JCGM 100:2008 (GUM 1995 with minor corrections), 1st ed. Sèvres: BIPM, 2008.

[170] IT'IS Foundation, *Tissue Properties Database*, version 5.0 (released Aug. 21, 2025). Zurich: IT'IS Foundation. doi:10.13099/VIP21000-05-0; `https://itis.swiss/database`. Values compared: blood density 1049.75 $\mathrm{kg\,m^{-3}}$ and heat capacity 3617 $\mathrm{J\,kg^{-1}\,K^{-1}}$; water heat capacity 4178 $\mathrm{J\,kg^{-1}\,K^{-1}}$ and thermal conductivity 0.6045 $\mathrm{W\,m^{-1}\,K^{-1}}$.

[171] Z. Hashin and S. Shtrikman, "A variational approach to the theory of the effective magnetic permeability of multiphase materials," *Journal of Applied Physics*, vol. 33, no. 10, pp. 3125–3131, 1962. doi:10.1063/1.1728579.

[172] K. Trier, "The sclera," in *The Biology of the Eye*, ser. Advances in Organ Biology, vol. 10. Amsterdam: Elsevier, 2005, pp. 353–373. doi:10.1016/S1569-2590(05)10013-5.

[173] P. Asbell and D. Brocks, "Cornea overview," in *Encyclopedia of the Eye*, D. A. Dartt, Ed. Oxford: Academic Press, 2010, vol. 1, pp. 522–531. doi:10.1016/B978-0-12-374203-2.00058-0.

[174] S. Hayes, T. White, C. Boote, C. S. Kamma-Lorger, J. Bell, T. Sorenson, N. Terrill, O. Shebanova, and K. M. Meek, "The structural response of the cornea to changes in stromal hydration," *Journal of the Royal Society Interface*, vol. 14, no. 131, art. 20170062, 2017. doi:10.1098/rsif.2017.0062.

[175] Y. M. Michelacci, "Collagens and proteoglycans of the corneal extracellular matrix," *Brazilian Journal of Medical and Biological Research*, vol. 36, no. 8, pp. 1037–1046, 2003. doi:10.1590/S0100-879X2003000800009.

[176] R. F. Fisher and B. E. Pettet, "Presbyopia and the water content of the human crystalline lens," *The Journal of Physiology*, vol. 234, no. 2, pp. 443–447, 1973. doi:10.1113/jphysiol.1973.sp010353.

[177] N. K. Tram and K. E. Swindle-Reilly, "Rheological properties and age-related changes of the human vitreous humor," *Frontiers in Bioengineering and Biotechnology*, vol. 6, art. 199, 2018. doi:10.3389/fbioe.2018.00199.

[178] O. İnam, H. J. Kaplan, and T. H. Tezel, "Retinal hydration assessment with optical coherence tomography: Unraveling its significance in retinal fluid dynamics, macular edema and cell viability," *Translational Vision Science & Technology*, vol. 12, no. 8, art. 4, 2023. doi:10.1167/tvst.12.8.4.

[179] R. M. Forbes, A. R. Cooper, and H. H. Mitchell, "The composition of the adult human body as determined by chemical analysis," *Journal of Biological Chemistry*, vol. 203, no. 1, pp. 359–366, 1953. doi:10.1016/S0021-9258(19)52646-1.

[180] H. H. Mitchell, T. S. Hamilton, F. R. Steggerda, and H. W. Bean, "The chemical composition of the adult human body and its bearing on the biochemistry of growth," *Journal of Biological Chemistry*, vol. 158, no. 3, pp. 625–637, 1945. doi:10.1016/S0021-9258(19)51339-4.

[181] Y. Choi and M. R. Okos, "Effects of temperature and composition on the thermal properties of foods," in *Food Engineering and Process Applications*, vol. 1, M. Le Maguer and P. Jelen, Eds. London: Elsevier Applied Science, 1986, pp. 93–101.

[182] ASHRAE, "Thermal properties of foods," in *2002 ASHRAE Handbook—Refrigeration* (SI ed.). Atlanta, GA: American Society of Heating, Refrigerating and Air-Conditioning Engineers, 2002, ch. 8, pp. 8.1–8.30.

[183] E. E. García Rojas, J. S. R. Coimbra, and J. Telis-Romero, "Thermophysical properties of cotton, canola, sunflower and soybean oils as a function of temperature," *International Journal of Food Properties*, vol. 16, no. 7, pp. 1620–1629, 2013. doi:10.1080/10942912.2011.604889.

[184] H. Fischer, I. Polikarpov, and A. F. Craievich, "Average protein density is a molecular-weight-dependent function," *Protein Science*, vol. 13, no. 10, pp. 2825–2828, 2004. doi:10.1110/ps.04688204.

[185] M. E. Langham, R. A. Farrell, V. O'Brien, D. M. Silver, and P. Schilder, "Blood flow in the human eye," *Acta Ophthalmologica*, vol. 67, suppl. 191, pp. 9–13, 1989. doi:10.1111/j.1755-3768.1989.tb07080.x.

[186] G. T. Dorner, E. Polska, G. Garhöfer, C. Zawinka, B. Frank, and L. Schmetterer, "Calculation of the diameter of the central retinal artery from noninvasive measurements in humans," *Current Eye Research*, vol. 25, no. 6, pp. 341–345, 2002. doi:10.1076/ceyr.25.6.341.14231.

[187] J. W. Kiel, M. Hollingsworth, R. Rao, M. Chen, and H. A. Reitsamer, "Ciliary blood flow and aqueous humor production," *Progress in Retinal and Eye Research*, vol. 30, no. 1, pp. 1–17, 2011. doi:10.1016/j.preteyeres.2010.08.001.

[188] L. M. Wilcox Jr., E. M. Keough, R. J. Connolly, and C. E. Hotte, "Comparative extraocular muscle blood flow," *Journal of Experimental Zoology*, vol. 215, no. 1, pp. 87–90, 1981. doi:10.1002/jez.1402150110.

[189] O. A. Larsen, N. A. Lassen, and F. Quaade, "Blood flow through human adipose tissue determined with radioactive xenon," *Acta Physiologica Scandinavica*, vol. 66, no. 3, pp. 337–345, 1966. doi:10.1111/j.1748-1716.1966.tb03208.x.

[190] Z. Wang, Z. Ying, A. Bosy-Westphal, J. Zhang, B. Schautz, W. Later, S. B. Heymsfield, and M. J. Müller, "Specific metabolic rates of major organs and tissues across adulthood: Evaluation by mechanistic model of resting energy expenditure," *The American Journal of Clinical Nutrition*, vol. 92, no. 6, pp. 1369–1377, 2010. doi:10.3945/ajcn.2010.29885.

[191] R. M. Werkmeister, D. Schmidl, G. Aschinger, V. Doblhoff-Dier, S. Palkovits, M. Wirth, G. Garhöfer, R. A. Linsenmeier, R. A. Leitgeb, and L. Schmetterer, "Retinal oxygen extraction in humans," *Scientific Reports*, vol. 5, art. 15763, 2015. doi:10.1038/srep15763.

[192] U. Cicekli, "Computational model for heat transfer in the human eye using the finite element method," M.S. thesis, Louisiana State University, Baton Rouge, LA, 2003. LSU Master's Theses 2762.

[193] E. Y. K. Ng and E. H. Ooi, "FEM simulation of the eye structure with bioheat analysis," *Computer Methods and Programs in Biomedicine*, vol. 82, no. 3, pp. 268–276, 2006. doi:10.1016/j.cmpb.2006.04.001.

[194] A. F. Emery, P. Kramar, A. W. Guy, and J. C. Lin, "Microwave induced temperature rises in rabbit eyes in cataract research," *Journal of Heat Transfer*, vol. 97, no. 1, pp. 123–128, 1975. doi:10.1115/1.3450259.

[195] F. A. Duck, *Physical Properties of Tissue: A Comprehensive Reference Book*. London: Academic Press, 1990.

[196] T. Okuno, "Thermal effect of infra-red radiation on the eye: A study based on a model," *The Annals of Occupational Hygiene*, vol. 35, no. 1, pp. 1–12, 1991. doi:10.1093/annhyg/35.1.1.

[197] J. J. W. Lagendijk, "A mathematical model to calculate temperature distributions in human and rabbit eyes during hyperthermic treatment," *Physics in Medicine and Biology*, vol. 27, no. 11, pp. 1301–1311, 1982. doi:10.1088/0031-9155/27/11/001.

[198] S. C. DeMarco, G. Lazzi, W. Liu, J. D. Weiland, and M. S. Humayun, "Computed SAR and thermal elevation in a 0.25-mm 2-D model of the human eye and head in response to an implanted retinal stimulator—Part I: Models and methods," *IEEE Transactions on Antennas and Propagation*, vol. 51, no. 9, pp. 2274–2285, 2003. doi:10.1109/TAP.2003.816395.

[199] P. Bernardi, M. Cavagnaro, S. Pisa, and E. Piuzzi, "SAR distribution and temperature increase in an anatomical model of the human eye exposed to the field radiated by the user antenna in a wireless LAN," *IEEE Transactions on Microwave Theory and Techniques*, vol. 46, no. 12, pp. 2074–2082, 1998. doi:10.1109/22.739285.

[200] K. C. Gokul, D. B. Gurung, and P. R. Adhikary, "Thermal effects of eyelid in human eye temperature model," *Journal of Applied Mathematics & Informatics*, vol. 32, no. 5–6, pp. 649–663, 2014. doi:10.14317/jami.2014.649.

[201] A. A. Abusharha, A. Al Yami, K. Alsreea, R. Fagehi, A. Alsaqr, S. Alanazi, and A. Masmali, "Repeatability and reproducibility of tear film evaporation rate measurement using a new closed-chamber evaporimeter," *The Open Ophthalmology Journal*, vol. 15, pp. 117–121, 2021. doi:10.2174/1874364102115010117.

[202] R. Fagehi, G. A. El-Hiti, M. H. Alsubaie, A. Abusharha, M. A. Alanazi, A. M. Masmali, and T. Almubrad, "Measurements of tear evaporation rate in subjects with refractive errors using a portable evaporimeter," *Healthcare*, vol. 10, no. 2, art. 405, 2022. doi:10.3390/healthcare10020405.

[203] Coolants Plus, Inc., "Propylene Glycol, Industrial Grade (Starfire)," product data sheet, Form No. 117-01540-0604X-AAD. Typical properties at 25 °C: thermal conductivity 0.2061 $\mathrm{W\,m^{-1}\,K^{-1}}$, specific heat 2.51 $\mathrm{J\,g^{-1}\,K^{-1}}$, viscosity 48.6 mPa s.

[204] STMicroelectronics, "STM32L476xx: Ultra-low-power Arm Cortex-M4 32-bit MCU+FPU, 100DMIPS, up to 1MB flash, 128 KB SRAM, USB OTG FS, LCD, ext. SMPS," datasheet DS10198, Rev. 11, 2024.

[205] Analog Devices, "ADN8831: Thermoelectric cooler (TEC) controller," data sheet D04663, Rev. C, 2019.

[206] Sensirion, "SLF3S-4000B liquid flow sensor," data sheet, version 1.2, November 2022.

[207] Aosong Electronics, "AHT20 humidity and temperature sensor," data sheet, version 1.0, May 2021.

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- Engineering Intern, R&D, RetMap Inc., Chicago, Illinois, Summers 2023, 2024, and 2025.
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TEACHING

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PUBLICATIONS

- Y. Nakamura, L. Mecacci, and J. R. Hetling, "Therapeutic levels of hypothermia achieved in isolated porcine eyes using a scleral contact interface," *Annals of Biomedical Engineering*, vol. 54, no. 2, pp. 570–579, 2026. doi:10.1007/s10439-025-03905-w.
- L. Mecacci, Y. Nakamura, K. Fong, N. Shirdade, and J. R. Hetling, "An ex vivo system to study clinically feasible induction of therapeutic hypothermia in porcine eyes," *Journal of Neuroscience Methods* (manuscript pending submission).
- L. Mecacci and J. R. Hetling, "Therapeutic levels of hypothermia achieved in isolated human donor eyes using a scleral contact interface, and thermal concordance with the porcine eye," *Annals of Biomedical Engineering* (manuscript in preparation).

- L. Mecacci, Y. Nakamura, and J. R. Hetling, "Ocular hypothermia delivery system: in vitro validation using a non-perfused pig eye model," ARVO Annual Meeting, 2023 (poster).
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- L. Mecacci and J. R. Hetling, "Ocular hypothermia delivery system: in vitro validation using non-perfused human donor eyes," ARVO Annual Meeting, 2026 (poster).

PATENTS AND GRANTS

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